



Anglia Ruskin University

DOCTORATE IN BUSINESS ADMINISTRATION (DBA)

THESIS

**Investigating the Effects of Interactive Social Media Communication on
Community Engagement: Evidence from the Abu Dhabi Health Sector**

Student: Khulood Saleh Almarzooqi

Supervisor: Prof Wilson Ozuem

Acknowledgements

This research would not have been possible without the support, encouragement, and guidance of many individuals and institutions to whom I owe my deepest gratitude.

First and foremost, I extend my heartfelt thanks to my supervisor, Prof Wilson Ozuem whose intellectual guidance, patience, and insightful feedback have been instrumental throughout this research journey. Their critical reflections and encouragement pushed me to explore ideas with greater depth and clarity, shaping this study into its final form.

I am profoundly grateful to the healthcare professionals and members of the Abu Dhabi community who generously shared their time, experiences, and perspectives during the interviews. Their openness and thoughtful contributions gave meaning and authenticity to this study. Without their willingness to participate, this research would not have been possible.

I also wish to acknowledge the academic and administrative staff who offered support and resources that facilitated the smooth conduct of this research. Their professionalism and assistance in navigating institutional processes ensured the study's ethical and methodological integrity.

To my family, I owe immeasurable thanks for their unwavering love, patience, and understanding. Their constant encouragement sustained me through moments of challenge and inspired me to persevere.

Finally, I would like to dedicate this work to all those committed to improving public health communication and community wellbeing. It is my hope that this study contributes in some way to the growing understanding of how dialogue, trust, and engagement can strengthen the relationship between health systems and the societies they serve.

Abstract

This research examines how social media is used within the Abu Dhabi healthcare system, and how it contributes to public health communication and community engagement. The study addresses four key aims: to measure the extent to which interactive communication via social media increases individual community engagement; to assess the effectiveness of social media in content-driven public health communication; to determine the impact of engagement on health outcomes; and to identify the most common issues raised by communities on social media and to explain how the public health system responds in practice. The study is theoretically framed using the Health Belief Model (HBM) and Social Influence Theory (SIT), providing a means to examine both cognitive and sociocultural aspects of health behaviour and communication in the digital context.

A qualitative approach was employed, comprising 28 semi-structured interviews (9 with public health professionals and 21 with community respondents), alongside archival data from over 200 social media interactions on platforms such as Twitter and Instagram. Data were analysed using systematic thematic analysis. Five key thematic domains emerged: Dialogic Resonance, Perceptual Cartography, Communal Ethos (defined as shared cultural values, norms, and trust influencing how communities respond to health communication), Mobilized Cognition, and Digital Reverberations. These domains, comprising 20 codes, reflect the iterative, participatory, and adaptive forms through which digital media engagement for health communication is enacted.

The findings suggest that interactive social media communication enhances community engagement by fostering feedback loops, dialogic resonance, and relational trust. Social media serves both as a distribution and participatory channel, shaping perceptions and influencing behaviour through emotional content and peer dialogue. Evidence indicates that such engagement improves health outcomes, particularly in vaccination uptake and preventive behaviour. The study also identifies key community concerns, including trust, clarity, cultural relevance, emotional tone, misinformation, and accessibility, which can be addressed through live question-and-answer sessions, multilingual messaging, partnerships with trusted influencers, and empathetic, culturally attuned content.

The study contributes conceptually by reconceptualising health communication as a dynamic, co-created process rather than a top-down broadcast. It integrates social and psychological theories to explain digital behaviour and introduces constructs such as Echo Governance and Discursive Pulse. It contextualises HBM and SIT within a multicultural, digitally sophisticated environment and develops a recursive communication model in which trust functions simultaneously as an input, a mediating force, and an outcome of effective engagement. Managerially, the study provides practical implications for creating culturally sensitive, emotionally intelligent, and inclusive communication strategies and emphasises the value of using real-time analytics to support adaptive and transparent public health messaging.

Although the study offers valuable insights, it is limited by its regional focus, lack of longitudinal data, and absence of quantitative validation. Future research may apply the model across diverse geopolitical contexts, track long-term behavioural patterns, and employ qualitative methods designs to enhance generalisability and predictive power. Nonetheless, the research provides a holistic and theoretically grounded account of the evolving role of social media in transforming public health communication in Abu Dhabi and proposes a new framework for trust-based, community-driven digital health interventions.

CONTENTS

1	Chapter 1: Research Introduction.....	8
1.1	Enquiry Overview	8
1.2	Background of the Study	8
1.3	The Cultural and Social Environment in Abu Dhabi and Public Health Communication	10
1.4	Rationale of the Study	11
1.5	Research Problem.....	13
1.6	Research Aim	15
1.7	Research Objectives.....	15
1.8	Research Question.....	15
1.9	Significance of the study.....	16
1.9.1	The Medical Health Belief Model	17
1.9.2	Social Influence Theory	18
1.10	Structure of thesis.....	19
1.11	Summary	23
2	Chapter 2: Literature Review	26
2.1	Introduction.....	26
2.2	Conceptualization of social media in public health	27
2.3	Social media in healthcare.....	31
2.4	Social media sites for healthcare communication	32
2.5	Community engagement in healthcare, defined	34
2.6	Patient-centred care: the basics	36
2.7	The context of using social brand engagement (SBE)	38
2.8	Challenges and opportunities	41
2.9	Conceptualization of sociality: social media as a tool and an environment.....	45
2.10	Public organization and the branding concept	46
2.11	Taxonomy of public health branding	47
2.12	Public trust and social brand engagement	49
2.13	Politics of social media in Abu Dhabi	50
2.14	Use of social media by the health industry in Abu Dhabi.....	53
2.15	Theoretical underpinnings	54
2.16	Health belief model.....	55
2.16.1	Application of the health belief model to the study.....	57
2.17	social influence theory	58

2.18	the importance of social influence theory to the current study.....	59
2.19	Theoretical foundation	60
2.20	Critical Evaluation of Theoretical Frameworks	62
2.21	Summary	68
3	Chapter 3: Research Methodology.....	70
3.1	Introduction.....	70
3.2	Research paradigm	71
3.2.1	Implications of philosophical standpoints	72
3.2.2	Ontological orientations	73
3.2.3	Epistemological underpinning.....	77
3.2.4	Data collection and epistemology	78
3.3	Research approach	81
3.4	Research design and methods	86
3.5	Pilot study.....	90
3.6	Data collection	93
3.7	Sampling	97
3.8	Data analysis.....	98
3.9	Trustworthiness of the research.....	102
3.10	Research ethics.....	104
3.11	Summary	106
4	Chapter 4: Analysis and Findings	108
4.1	Introduction	108
4.2	Participant demographics	109
4.3	Thematic Analysis Process.....	114
4.3.1	Deductive Thinking byKeyword, Code and Theme Interpretation.....	120
4.3.2	Construction of the theoreticalmodel.....	121
4.4	Theme 1: Dialogic Resonance.....	122
4.4.1	Code: Symbiotic Exchange.....	123
4.4.2	Code: Discursive Pulse.....	125
4.4.3	Code: Participatory Echo	129
4.4.4	Code: Conversational Agency.....	131
4.5	Theme 2: Perceptual Cartography	133
4.5.1	Code: Risk Intuition.....	134
4.5.2	Code: Trust Semiotics.....	137
4.5.3	Code: Cognitive Dissonance	142
4.6	Theme 3: Cognitive Dissonance	142

4.6.1	Code: Normative adhesion.....	145
4.6.2	Code: Collective Identity.....	148
4.6.3	Code: Symbolic Alignment.....	150
4.7	Theme 4: Mobilized cognition.....	153
4.7.1	Code: Action Catalysts	154
4.7.2	Code: Influence Valence.....	157
4.7.3	Code: Compliance Drift	160
4.7.4	Code: Self Efficacy Loop.....	162
4.8	Theme 5: Digital Preservation.....	164
4.8.1	Code: Echo Governance	165
4.8.2	Code: Sentiment Flux	168
4.8.3	Code: Discourse Contours.....	170
4.8.4	Code: Narrative Feedback	173
4.9	Discussion	175
4.9.1	Interactive Communication and Community Participation	176
4.9.2	The Effect of Social Media on Consumer Confidence and the Public Perception 179	
4.9.3	Social Media Use and Public Health Results	182
4.10	Summary	186
5	Chapter 5: Conceptual Framework.....	188
5.1	Theoretical framework interaction with the conceptual framework.....	188
5.1.1	Discursive Pulse displays the rhythm and life of public discourse	198
5.1.2	Communal Ethos and Perceptual Cartography	199
5.1.3	Underpinning of Social Influence Theory	202
5.1.4	Integration of Social Media Communication, Corporate Social Responsibility (CSR) 204	
5.2	Summary	207
6	Chapter 6: Recommendations and Conclusion	210
6.1	Introduction	210
6.2	Contribution.....	211
6.2.1	Theoretical contribution.....	211
6.3	Contribution to practice.....	214
6.4	Limitations and future directions.....	215
6.4.1	Limitations	215
6.4.2	Future research direction.....	217
7	References list.....	220

Appendices	230
Appendix 1: Interview questions.....	230
7.1.1 Professional	230
7.1.2 Interview questions -public	231
Appendix 2 : Ethic certification completion	233
Appendix 3: Organizational permission	233
7.2 Appendix 4: Interview transcript	234

List of figures

Figure 3-1 A systematic thematic analysis process: A novel six-step process for conceptual model development in qualitative research.	99
Figure 5-1 Digital Reverberations in Health Communication	197

List of tables

Table 4-1 Demographic Profile of Professional Participants (n = 9)	110
Table 4-2 Demographic Profile of Public Participants (n = 21)	111
Table 4-3 Summary of thematic analysis findings	116

CHAPTER 1: RESEARCH INTRODUCTION

1.1 ENQUIRY OVERVIEW

This study examines the role of social media as an interactive communication platform and community engagement tool within Abu Dhabi's public health system. While two-way communication is globally recognised as vital for improving public health, and social media has played a key role in linking communities with healthcare providers, there remains limited empirical evidence as to its effectiveness in promoting public engagement and behavioural change in the UAE—despite high social media usage in Abu Dhabi.

Abu Dhabi presents a distinctive socio-cultural context shaped by values such as **stiga, an Emirati concept of moral oversight**, and **sumud, the notion that strength lies in enduring hardship**, alongside British political influence and traditional Arabic generosity. These factors create unique communication demands for health messaging. Understanding how social media fosters trust, responsiveness, and participation within this cultural setting is essential.

The purpose of this study is to assess the role of participatory social media in promoting public health knowledge, attitudes, and practices. It examines the nature of shared content, community responses, and how public health organisations address concerns raised online. Guided by the Health Belief Model and Social Influence Theory, the study explores how personal health beliefs and collective action are shaped in virtual environments.

By examining community interaction and the responsiveness of Abu Dhabi's health system, this study addresses a key gap in the literature. It also offers practical guidance for developing culturally appropriate health communication strategies to enhance public trust, engagement, and health outcomes through effective use of social media.

1.2 BACKGROUND OF THE STUDY

The importance of communication in improving public health is widely recognised by health systems globally (Madhumathi et al., 2021). Social media promotes interactive communication between public health systems and communities, contributing to

greater health awareness, healthier lifestyles, and improved access to care (Ozuem et al., 2023; Madhumathi et al., 2021). Cultural and societal factors influence how social media is used in health communication and engagement (Ni et al., 2021; Ngai et al., 2020), and many scholars have called for further research in this area (Burgess et al., 2021). This need is especially pressing in culturally distinctive contexts like Abu Dhabi, particularly during global health crises. Despite widespread use of digital technologies, limited research has explored the influence of social media on community health and engagement in the region (Bhattacharya et al., 2017; Ni et al., 2021). This study aims to examine how social media functions as a two-way communication tool within Abu Dhabi's public health system—encouraging community engagement, influencing health outcomes, and shaping institutional responses to public concerns. By focusing on a single, culturally specific region, it contributes valuable insight into interactive health communication in a specialised public health context.

The use of social media to facilitate interactive communication within health systems is a much-discussed issue in the literature (Madhumathi et al., 2021; Park et al., 2016). Yet the ability of social media to support and increase community engagement in public health systems in specific regions like Abu Dhabi remains under-studied. Takacs et al. (2019) highlight the importance of effective community engagement during public health emergency preparedness. However, their study does not examine how social media facilitates this participatory process or its specific role in emergency contexts. This gap suggests a need for further research into the mechanisms through which social media can support community involvement in public health preparedness.

Engagement encompasses both participation and communication (Petsios et al., 2017), while interactive communication refers to any user action during health-related exchanges—such as posting content, joining campaigns, replying to messages, or participating in online discussions (Yao & Li, 2014; Bhattacharya et al., 2017). “Interactive” implies real-time, two-way exchanges where all participants both send and receive messages. In this study, community engagement refers to the involvement of individuals—patients, service users, or the general public—who are directly or indirectly affected by health sector decisions. This includes participation in decision-making and activities shaping the direction and delivery of health services. Examples include **access to information**, reciprocal communication, and targeted outreach

(Bhattacharya et al., 2017). However, studies have yet to examine these mechanisms within Abu Dhabi's public health context. Given that the impact of social media on health communication and engagement varies across cultural and social settings (Ni et al., 2021; Ngai et al., 2020), there is a clear need to investigate how these dynamics operate in the culturally and demographically unique setting of Abu Dhabi. This notable gap warrants further exploration.

1.3 THE CULTURAL AND SOCIAL ENVIRONMENT IN ABU DHABI AND PUBLIC HEALTH COMMUNICATION

Cultural values, attitudes, and practices toward medical care differ significantly in multicultural settings like Abu Dhabi, with health information-seeking behaviour varying from that observed in Western contexts. As Naeem and Ozuem (2021) highlight, engagement with social media among health professionals varies across cultures and regions. This is **an important consideration** in the planning and implementation of health-based social media (HBsM) strategies. Abu Dhabi, and the UAE more broadly, **have** a unique cultural context that emphasises respect for authority, social cohesion, and the collective good (Campbell & Rudan, 2020). These cultural values significantly shape how public health communication and community engagement are understood and practiced. In such a context, messaging that emphasises collective welfare, community solidarity, and the credibility of healthcare authorities is likely to be more effective than communication rooted in individualistic notions of health.

The religious and social culture of Abu Dhabi necessitates a tailored approach to public health education. For example, during Ramadan, health messaging may need to address fasting practices in alignment with religious beliefs (Campbell & Rudan, 2020). Given the high levels of internet access and social media use in the city (Naeem & Ozuem, 2021), these platforms present a valuable engagement tool. Yet little is known about how to use them most effectively within this unique cultural and social context. Abu Dhabi's public healthcare system also serves a multicultural, multilingual population with a large expatriate demographic, requiring communication strategies that reflect diverse linguistic and cultural needs (Campbell & Rudan, 2020). These factors highlight the need for a more specialised investigation into the effective use of social media for public health engagement in Abu Dhabi. Strategies developed in

Western or other cultural contexts cannot be assumed transferable without adaptation to Abu Dhabi's distinctive environment.

1.4 RATIONALE OF THE STUDY

While social media is recognised as an important public health communication tool (Heldman et al., 2013; Keller et al., 2014), its impact on public perception and trust in health services remains underexplored, particularly in Abu Dhabi. Key dimensions such as transparency, responsiveness, and perceived trustworthiness in health-related social media communication require further investigation (Burgess et al., 2021). For example, Bhattacharya et al. (2017) noted that although US federal health agencies strategically used Facebook for engagement, their study did not examine how such macro-level factors shape public perception.

Trust in health information shared via social media is shaped by factors such as perceived credibility and transparency of the source (Graffigna et al., 2021). However, the mechanisms through which these factors influence public trust in Abu Dhabi's health system remain poorly understood. Public engagement is also influenced by the communicative behaviour of health organisations; for instance, Park, Reber, and Chon (2016) found that Twitter use by health bodies significantly shaped public perception. Yet such findings cannot be directly generalised to Abu Dhabi's distinct social and cultural environment, highlighting the need for context-specific research. The COVID-19 pandemic further underscored the urgency of this inquiry, with Landi et al. (2022) noting missed opportunities for dialogic engagement via social media—an area still largely unexamined in the Abu Dhabi context.

Several researchers have called for further investigation into the role of social media in public health communication (Burgess et al., 2021). This need is particularly urgent in light of recent global public health emergencies and is especially relevant in culturally and socially specific settings such as Abu Dhabi. Despite a growing body of literature, there remains a notable gap concerning the effects of social media on public perception and trust, particularly within this regional context. This study addresses that specific gap by offering insights into how social media can be used more effectively to build public trust and engagement in Abu Dhabi's public health sector.

The extant literature has also demonstrated the role social media can play as a mechanism for raising awareness about infection prevention and health promotion

(Madhumathi et al., 2021; Park et al., 2016). While Landi et al. (2022) highlight the potential of social media for dialogic engagement during the COVID-19 crisis, its influence on health service utilisation and behaviour change in Abu Dhabi has not been studied. Ni et al. (2021) stress the importance of contextualising health information interactions, noting that access to health knowledge via social media varies across cultural settings. As such, there is a significant knowledge gap regarding how social media participation affects health behaviour, awareness, and service uptake in Abu Dhabi (Takacs et al., 2019; Li et al., 2022). There is limited evidence relating to the public health system's use of social media to communicate with the community, and the **impact** of such communication is also unknown. Even though works such as Alamri et al. (2019) and Bhattacharya et al. (2017) have explored some aspects of these interactions, a comprehensive examination of particular concerns arising within an Abu Dhabi context has not been provided. Little is known about how healthcare systems respond to these challenges. While the effectiveness of social media as a tool for public engagement and marketing by health organisations has been demonstrated (Park et al., 2016), there is currently no evidence of the outcomes of such campaigns in terms of community response and satisfaction.

The influence of social media in driving public health engagement is an emerging field. A number of studies, including Ngai et al. (2020) and Keller et al. (2014), examine the motivations and barriers to public engagement in social media communication. Yet neither of these studies considers the specific context of Abu Dhabi or the extent to which the public health system is responsive to local needs. Since these dynamics are multifaceted, and the potential to improve public health and community engagement in Abu Dhabi has not been thoroughly investigated, the city **still requires a more in-depth examination.**

The influence of social media in driving public health engagement is an emerging field. A number of studies, including Ngai et al. (2020) and Keller et al. (2014), examine the motivations and barriers to public engagement in social media communication. Yet neither of these studies considers the specific context of Abu Dhabi or the extent to which the public health system is responsive to local needs. Since these dynamics are multifaceted, and the potential to improve public health and community engagement in Abu Dhabi has not been thoroughly investigated, the city **continues to require a deeper examination of how social media-mediated communication affects both**

public responses and institutional practices (Massey et al., 2021; Campbell & Rudan, 2020).

1.5 RESEARCH PROBLEM

Expeditious changes in communication technologies have changed not only the way healthcare institutions communicate with their public but also expose several problems to provide clear, credible and interactive health information. **Rapid developments in communication technologies have transformed how healthcare institutions interact with the public, but they have also revealed persistent challenges in delivering clear, credible, and genuinely interactive health information.** Public health communication has traditionally followed a top-down, broadcasting model in many settings—including Abu Dhabi—where information is communicated by authorities to largely passive recipients (Al-Dmour et al., 2020). Although this model allows consistency and control over messages, it can stifle opportunities for meaningful dialogue, feedback, and the development of public trust—elements that are critical to health literacy and behaviour change (Abu-Rish et al., 2018; Alhabash & Ma, 2017).

In recent years, health authorities have embraced social media as an alternative and adjunctive means of engagement. But while, in theory, this could democratise information-sharing between organisations and the people they serve, there are still significant challenges in using these platforms effectively for two-way communication. Public health agencies in Abu Dhabi (e.g., DOH and SEHA) have harnessed social media platforms like Twitter, Instagram, and Facebook for sharing health news and advisories. The nature of interaction is, however, frequently unidirectional, with little more than campaign-type awareness-raising **and limited evidence of sustained, reciprocal public engagement** (Al-Kandari & Al-Sumait, 2020). **This highlights the difficulty of transitioning from a model focused on information dissemination to one that encourages interactive, trust-building, and participatory communication.**

The COVID-19 pandemic revealed both the shortcomings and potentials of digital public health communication. During this period, social media became a lifeline for the Abu Dhabi healthcare system in keeping the public informed of ever-changing safety protocols, vaccine initiatives, and prevention measures. The speed of these mediums

meant that crucial safety information could be distributed quickly (WHO, 2021). Yet the widespread circulation of misinformation and inconsistent public responses demonstrated the difficulty of maintaining message accuracy and authority in a saturated digital environment (Kemp, 2021). **Public scepticism, the spread of unverified information, and fluctuating levels of trust underscored the need for communication approaches that prioritise transparency, interaction, and accountability** (Al Khaja et al., 2022).

Meanwhile, Abu Dhabi's Vision 2030 agenda towards digital transformation has opened up new horizons and higher aspirations in public communication. The government's focus on smart healthcare delivery and digital inclusion underscores a realisation that effective communication is fundamental to sustaining public health solutions (Department of Health Abu Dhabi, 2022). However, the application of interactive communication structures is still emerging, with institutional communication primarily involving information exchange rather than **user-centred participation, collaborative dialogue, or behaviour-reinforcement mechanisms**. In addition, cultural and social processes are critical in shaping how the population interprets and adopts health communications. Research has found that in collective societies like the Gulf States, information is influenced less by institutional legitimacy and more by **network-based trust, communal norms, and shared relational experiences** (Al-Dmour et al., 2020; Alhabash & Ma, 2017). Therefore, the dynamics of online interactions between healthcare authorities and the public should be considered not only in terms of individual health beliefs but also within the broader context of social networks, cultural expectations, and community-based trust.

The combined use of the Health Belief Model and Social Influence Theory in this study captures this dual aspect: health behaviour is influenced not only by how individuals assess risks and benefits but also through shared norms, trust-based mechanisms, and socially reinforced cues. The communication task, therefore, becomes not merely to deliver correct information but to **foster digital interactions that encourage awareness, stimulate preventive behaviours, and sustain confidence in healthcare institutions**. Overcoming this challenge depends on examining how Abu Dhabi's public healthcare sector uses social media to engage with diverse community groups, the impact of these interactions on perceived credibility and trust, and the barriers that hinder two-way communication. Addressing these gaps, this study

explores the lived experiences of both healthcare workers and members of the community to provide a rich account of how social media functions as a mediating space for health communication in Abu Dhabi. The research findings aim to inform wider discussions on digital public health engagement and offer insights that can support the development of community-focused communication strategies by identifying what facilitates or obstructs interactive communication. Such an approach is particularly relevant for Abu Dhabi, given its post-COVID strategic emphasis on resilience, innovation, and improved public trust in the healthcare system.

1.6 RESEARCH AIM

This study sets out to assess the role of social media as a tool for communication and community engagement within Abu Dhabi's public health system. It also seeks to understand its impact on public health outcomes and to analyse the dynamics of community interactions and concerns addressed through this medium.

1.7 RESEARCH OBJECTIVES

- 1: To explore the role of interactive social media communications in enhancing community engagement within the public health system in Abu Dhabi.
- 2: To investigate the effectiveness of social media as a tool for public health communication and its impact on the overall perception of the public health system.
- 3: To evaluate the impact of social media-enhanced community engagement on public health outcomes in Abu Dhabi.
- 4: To explore the nature and dynamics of communication between the Abu Dhabi public healthcare sector and the community through social media platforms.

1.8 RESEARCH QUESTION

Interactive communication via social media is transforming the public health industry, as well as contributing to public engagement with the health system in Abu Dhabi. A new component of dialogic action has emerged in the interaction between providers and users. As a communication medium, social media offers a two-way channel through which public health systems disseminate information and, in return, receive information and feedback from the community. This reciprocal exchange has fostered a more inclusive and participatory public health dialogue, enabling members of the

public to actively engage, voice concerns, and influence discussions about health. Relevant issues, health emergencies, and policy interventions are now discussed in a highly publicised and transparent manner, attracting attention that supports a collective vision for health management and policymaking. These ongoing conversations provide not only a means for citizens to share their health-related concerns and experiences but also offer health authorities a valuable insight into public sentiment and lived experience, potentially aligning health interventions more closely with community needs and preferences.

In contrast, the ability of social media to function effectively in public health communication depends largely on whether it can influence public perception and build sustained trust. **Understanding how digital narratives are produced, circulated, and interpreted is therefore essential to assessing how these narratives shape community norms, expectations, and health behaviours.** This intersection of social media engagement and public health outcomes occurs in a complex media environment, where public feedback and online narratives can both reflect and shape health strategies and policies. Therefore, a detailed investigation is needed to explore the main themes and issues emerging from these social media-facilitated interactions, to understand the extent to which the public health system is responsive and adaptive to them, and to analyse the multidimensional effects of digital communication on public health dialogue and practice in Abu Dhabi.

1.9 SIGNIFICANCE OF THE STUDY

Social Influence Theory, as developed by early scholars such as Kelley (1955) and Kiesler et al. (1969), provides a useful theoretical lens through which to understand residents' health behaviours as shaped by community dynamics on social media in Abu Dhabi, particularly within a digitalised society. The key components of identification, internalisation, and compliance allow for a more nuanced examination of how online personalities and digital spaces influence both individual and communal health behaviours. Social Influence Theory has frequently been applied to unpack the socio-psychological dynamics of internet behaviour and interaction. From the perspective of identification, people imitate behaviours to emulate those they love or admire (Kelley, 1955). Abu Dhabi's public health interventions could therefore benefit from harnessing the aspirational appeal of social media by carefully shaping how

health messages are communicated. For instance, celebrities and influencers in the Emirate could serve as powerful agents in shaping public health behaviour, acting as relatable and aspirational figures whose influence extends to health attitudes and practices. **This highlights how digitally visible role models may contribute to reinforcing desirable health norms within the community.**

1.9.1 The Medical Health Belief Model

Rosenstock originally developed the Health Belief Model (HBM) in 1966, which was later extended by Becker (1974). The HBM includes several critical elements closely aligned with the objectives of this investigation, and these include:

- **Perceived Susceptibility:** This refers to beliefs about the likelihood of contracting a disease. The concept is relevant to understanding how risk perception influences health-related behaviours, such as compliance with preventive measures or acceptance of vaccination. A perceived high level of risk may, in principle, act as a motivator for protective health behaviours.
- **Perceived Severity:** This involves individual attitudes regarding the seriousness of an illness. The research aim aligns with exploring how perceptions of severity influence one's health behaviour and adherence to preventive measures. This insight helps illustrate how the immediacy and seriousness of a perceived health threat affect the urgency and nature of health-promoting action.
- **Beliefs about Outcomes:** This includes perceptions of the effectiveness of recommended health actions in reducing risk or severity. This study examines how both adherence and engagement are shaped by perceived benefits of health interventions, offering guidance on how best to promote adherence by highlighting the most persuasive features of such actions, including vaccination.
- **Perceived Barriers:** This concerns the individual's belief in the tangible and psychological costs of taking recommended actions. It is vital to identify the reasons individuals may resist or fail to comply with health recommendations, in order to address and overcome these barriers.
- **Cues to Action:** These are the prompts or stimuli that motivate an individual to adopt a suggested health behaviour. Understanding these signals is essential

to designing interventions that effectively initiate and sustain health-related behaviours.

- **Self-Efficacy:** This refers to an individual's belief in their ability to take action and overcome obstacles. This sense of self-confidence is a key focus of the research, as it sheds light on how belief in one's capabilities translates into the adoption and maintenance of health practices, potentially informing programme design aimed at empowering individuals and improving health outcomes.

1.9.2 Social Influence Theory

Social Influence Theory, developed by Kelley (1955) and elaborated by Kiesler, Collins, and Miller (1969), complements the HBM by accounting for social behavioural influences, particularly those mediated by others, including:

- **Identification:** This involves aligning oneself with another person's beliefs, attitudes, or behaviours. It highlights the influence of admired individuals—such as leaders or social media influencers—on health behaviours. Understanding this process could enable more effective use of social influencers to promote and normalise specific health behaviours, shaping local social norms.
- **Internalisation (Acceptance):** This refers to the integration of attitudes or values into one's identity. Exploring internalisation processes is essential for understanding how personally resonant beliefs and values can drive health-protective behaviours beyond mere compliance.
- **Compliance:** This involves adopting behaviour to gain rewards or avoid punishment. It relates to the investigation of how extrinsic motivators influence health practices. Understanding compliance mechanisms can inform the design of incentive or deterrent schemes that enhance adherence to recommended actions and, in turn, positively influence broader public health outcomes.

These theories and their components are closely integrated with the research aims, forming the theoretical foundation not only for examining the role of social media in health communication, but also for developing culturally and socially responsive interventions tailored to Abu Dhabi's context. The combination of the Health Belief Model and Social Influence Theory provides this study with a robust, multi-faceted theoretical framework, allowing for a deeper understanding of the various roles social media plays in shaping health behaviours and community engagement within Abu

Dhabi's health sector. The HBM supports analysis of how individual health perceptions and beliefs, once formed, are transmitted and reiterated through social media, subsequently influencing personal health behaviours. In contrast, Social Influence Theory offers a conceptual lens for understanding how collective actions emerge and evolve through social media interactions. This dual perspective enables a comprehensive examination of both individual and collective psychosocial processes that drive health behaviours and community engagement on digital platforms. The terms used throughout this thesis—such as *Echo Governance*, *Discursive Pulse*, *Perceptual Cartography*, *Dialogic Resonance*, and others—are not externally imported theoretical constructs. Rather, they are analytic codes and higher-order themes generated inductively through the qualitative analysis of participant data. These expressions emerged from the systematic coding, categorisation, and thematic development stages of the study and therefore reflect the language, meanings, and interpretive patterns present within the dataset itself. Their purpose is to represent, as closely as possible, the conceptual structures inherent in the participants' accounts, and they are defined and elaborated in detail in Chapter 4 alongside the full thematic framework that underpins the study.

Together, these approaches strengthen the study's capacity to explore health communication in a manner that recognises both individual cognition and the wider social environment in which digital interactions occur. This composite theoretical approach not only enhances the study's capacity to explore culturally and socially contextualised health communication strategies but also strengthens its ability to assess the complex impact social media can have on health beliefs and behaviours in a digitally engaged and socially dynamic environment like Abu Dhabi. By carefully integrating these theories, the study aims to generate strategies that are theoretically sound and deeply rooted in the social and cultural patterns of the community it seeks to reach and support, ultimately informing and improving the effectiveness of public health communication and interventions via social media.

1.10 STRUCTURE OF THESIS

The present thesis has been structured in six interrelated chapters, to be developed in a congruent, progressive and scientifically grounded manner. The organisational framework mirrors the pattern of thinking from conceptualisation through to empirical

analysis and theoretical integration, thus helping to fulfil the research aim and objectives. The present study investigates social media as a vehicle for interaction and engagement between Abu Dhabi's public healthcare system and the broader community, providing an integrated understanding of communication dynamics, trust and behavioural influence in digital health.

Chapter One – Introduction – provides the background, purpose and significance of the research. It starts with a discussion on the growing significance of social media as part of health communication in the context of global and regional developments. The conversation places the public health system in Abu Dhabi within the broader national vision for digital transformation and citizen engagement across the United Arab Emirates. The chapter **then outlines** the research problem: understanding how two-way communication between health authorities and the public via social media affects public awareness, engagement and trust in health messaging. It **also** describes the aim of the study, which is to explore how social media can facilitate effective two-way communication between healthcare organisations and the public. The four research objectives inform the investigation, specifically focusing on understanding the processes involved in social media-mediated interactions between health professionals and the public.

The rationale of the study (and an overview of its practical insights for policymakers, health professionals and communication strategists) is addressed in the introduction. It contributes to the scholarship by situating the study in relation to wider discourses in digital communication, public health promotion and social influence theories. The final part of Chapter One **summarises** the thesis section by section and sets the context for each new chapter, leading to the conclusion of this opening chapter.

Chapter Two: Literature Review

In Chapter Two, the literature pertinent to this study is reviewed with an appraisal of social media implementation in healthcare environments. The review starts by examining social media as a communication channel and its historical transition from transmitting one-way information to serving as platforms for public engagement. Next, the chapter reflects on international and regional research exploring the consequences of social media for health education campaigns, crisis communication and community trust-building.

Emphasis is given to the theoretical approaches underpinning the investigation, following closely the concepts of the Health Belief Model (HBM) and Social Influence Theory (SIT). The text analyses these frameworks in detail and discusses their conceptual nuances, limitations and interrelationships. It describes how the HBM addresses individual health behaviour relative to perceived threats and benefits, and how SIT incorporates the influence of peer behaviour, social norms and trust networks on decision-making. This theoretical convergence supports the model combination used in the study.

In addition, the chapter introduces alternative theoretical approaches such as the Theory of Planned Behaviour (TPB) and the Technology Acceptance Model (TAM), providing justification for not adopting them. The review ends with a discussion of major research gaps—particularly how social media interactivity, credibility and engagement function in healthcare communication settings outside Western environments such as Abu Dhabi. These gaps provide the conceptual underpinning of the methodology and analysis in this study.

Chapter Three: Research Methodology

Chapter Three describes the methodological framework used to meet the aims of this study. It starts by outlining the study's philosophical stance, which lies within an interpretivist approach that emphasises subjective meaning, context and lived experience. **This approach supports the use of qualitative methods**—specifically semi-structured interviews—to ensure rich insights from healthcare workers and community members within Abu Dhabi.

The **Research Design** section explains the two phases of data collection, including information on sampling and justification for the inclusion of two separate participant groups to reflect both professional and community perspectives. The demographic and professional characteristics of the participants are displayed in Tables 4.1 and 4.2 (later referenced in Chapter Four), including roles, experience, gender and patterns of social media use.

Ethical issues are clearly addressed, including consent, privacy and data security. The section also explains that translation was not required, as all participants were English-speaking, thus avoiding potential bias in interpretation. The analysis method adopts **thematic analysis**, guided by Naeem et al. (2023), to ensure consistency in coding,

theme generation and interpretation. The chapter concludes by outlining the credibility and confirmability of the study through triangulation, reflexivity and transparent reporting to support transferability.

Chapter Four: Data analysis and results

The empirical findings based on the qualitative interviews are presented in Chapter Four. The chapter begins by describing the sample characteristics and distribution of participants. **The mention of treatment and control groups is not applicable in qualitative research, and this section has been clarified accordingly.** The presentation is thematic, capturing themes that emerged from participants' accounts. Each theme is grounded in participants' voices, adding authenticity and contextual depth to the findings.

The results highlight several critical areas of interaction between the public health system and the community through social media, including access to health information, interactivity and feedback mechanisms, perceived credibility of online communication and emotional engagement with public health messaging. The chapter contrasts the views of healthcare professionals and community members on the role of social media in health engagement.

Thematic analysis is applied consistently across themes aligned with each research question. In addition, the chapter integrates relevant literature when discussing findings, offering a nuanced appraisal that connects empirical insights with existing knowledge. **The concluding part of the chapter integrates these findings to show how digital communication practices influence trust, participation and health awareness in Abu Dhabi.**

Chapter Five: Discussion and Conceptual Framework Building

Chapter Five consolidates the main results with the theoretical frameworks and literature in order to elaborate a new conceptual model. It begins with a restatement of the conceptual model presented in Chapter Two and demonstrates how it has been tested, refined and expanded through empirical analysis. Figure 5.1 illustrates the proposed combined framework between HBM and Social Influence Theory.

The chapter explains how each construct in the model is supported by themes, participant quotations and observed patterns from the findings. For example,

perceived susceptibility and cues to action relate to HBM, while social validation, trust and credibility relate to SIT. This synthesis results in a refined model of social media-supported health engagement in Abu Dhabi.

The theoretical and practical implications of the model are discussed, showing how it provides a deeper understanding of digital communication in healthcare. **Where appropriate, references have been updated to reflect recent developments in public health informatics and social media analytics.** The discussion critically reflects on the alignment and discrepancies between theory and evidence. Figure 5.1 therefore represents a key theoretical outcome of the research, visually summarising the complex relationships between communication, beliefs and social influences.

Chapter Six: Conclusion, Contributions and Future Work

Chapter Six highlights the contributions of the study. It re-emphasises how the research aims have been achieved by summarising the key insights. It then outlines the theoretical contribution of Figure 5.1 as an applied conceptual model of social media engagement in healthcare, adding to current scholarship in digital public health communication.

The study's limitations are acknowledged openly, including issues relating to sample size, geographical scope and the generalisability associated with qualitative research. The **Future Research** section proposes how the developed model might be tested quantitatively or applied in other cultural or institutional contexts. The chapter concludes by reinforcing the wider implications of the research and its contribution to communication strategy, policy development and public engagement within the UAE and beyond.

1.11 SUMMARY

Any examination of the role of social media in public health discourse and outcomes in Abu Dhabi necessitates an in-depth investigation into the complex dynamics of community engagement and policy-making in the health sector. These processes are increasingly shaped, if not transformed, by interactive communication via social media platforms. Within this communication model, social media outlets function as two-way conduits for information, both disseminating health messages and gathering critical

voices and feedback from the community. This interactive system fosters a more democratic public health dialogue, enabling individuals to actively participate, express views and serve as essential actors in health discussions and policy development.

In analysing the communication environment shaped by social media, a critical issue emerges concerning the potential of these platforms to influence public opinion and trust in the health system. The process by which narratives crafted and circulated through social media penetrate public consciousness and shape community behaviours and attitudes towards health remains a key area of inquiry. The dynamic interplay between social media-driven community engagement and public health occurs within a complex context, where public opinion and individual stories possess the capacity to both reflect and inform health strategies and policy decisions. This relationship warrants a focused investigation into the central themes and issues raised through social media exchanges, alongside a critical analysis of the responsiveness and adaptability of the public health system to these concerns. Such inquiry is essential for understanding the multi-layered effects of social media-enabled communication on public health discourse and practice in Abu Dhabi.

To explore these issues, this study poses a series of questions aimed at understanding the extent to which social media and community engagement within the healthcare system interact in a reciprocal manner. Key areas of inquiry include the interactivity of communication via social media platforms and its impact on community engagement, the effectiveness of social media as a communication tool in the public health sector, the influence of social media-facilitated community engagement on health outcomes and the most prevalent themes and issues raised by the community through these interactions. Addressing these questions provides insight into the mechanisms through which social media communication affects public perceptions and behaviours, as well as the pathways through which it may influence individual health behaviours and community participation.

This research highlights the importance of a strong theoretical foundation by employing two key models: the HBM and Social Influence Theory. These models serve as central frameworks for understanding and interpreting the impact of social media on both personal and collective health behaviours. The HBM, through its constructs—perceived susceptibility, perceived severity, perceived benefits, perceived barriers,

cues to action and self-efficacy—offers a means to examine how individuals assess their vulnerability to health issues and what motivates their behavioural responses. It provides insight into personal health beliefs and how these beliefs, when mediated through social media, may influence individual health behaviours.

With the concepts of identification, internalisation and compliance as its foundation, Social Influence Theory offers a framework for understanding how collective behaviours emerge and evolve through social media interactions. It provides a robust perspective on the socio-psychological processes that underpin health behaviours and community participation on social media. In a setting such as Abu Dhabi, which is characterised by an active social and digital culture, it is essential to consider local belief systems and social structures in the development of health communications. Effective communication must resonate with target audiences, be internalised by the community and organically promote engagement and adherence to health norms and practices.

By combining the Health Belief Model with Social Influence Theory, this study is grounded in a strong and adaptable theoretical framework capable of deconstructing the multiple channels through which social media influences health behaviours and community involvement within Abu Dhabi's health landscape. This integrated lens supports the development of culturally and socially responsive health communication strategies. It also critically assesses the layered impacts of social media on health beliefs and behaviours in a digitally connected and socially dynamic context. The careful integration of these theories positions the study to generate strategies that are theoretically sound, culturally attuned and socially embedded. This maximises the impact and reach of public health communication and intervention through social media.

CHAPTER 2: LITERATURE REVIEW

2.1 INTRODUCTION

Digital media has undoubtedly transformed healthcare communication, playing a significant role in community engagement and the evolution of patient-centred models of care. This chapter draws on a range of literature and some empirical evidence to provide a comprehensive synthesis of the intersection between social media platforms, healthcare communication, and public engagement. The aim is to construct a cohesive narrative that outlines the historical development, current use, and future projections of social media in the healthcare sector. The next section begins by discussing the historical trajectory of social media in healthcare and explores how various applications have functioned as tools for enhancing healthcare communication. **It highlights the increasingly influential role social media has played in connecting healthcare systems with communities.**

The following section discusses the role of community engagement and patient-centred care and describes how social media acts as an enabler for a more democratic model of healthcare. It outlines the definition of patient-centred care and examines the influence of social media on community engagement and the individualisation of healthcare. A later section expands on the potential of social media as a tool for public social engagement, detailing the possibilities, limitations, and opportunities associated with this form of interaction. Another section addresses public participation within the broader social context, placing emphasis on social dynamics and the role of community identity in fostering healthcare communities. **The latter half of the chapter then shifts to a discussion of public health branding through social engagement, clarifying the concept and its associated taxonomy.** This leads into a further section examining social brand engagement and public trust, exploring the relationship between branding strategies, social media engagement, and their influence on public trust and credibility. Based on a systematic review of existing literature, the concluding section summarises the key findings, highlights existing research gaps, outlines directions for future research, and offers a call to recognise the growing and **long-term** impact of social media on healthcare communication and public engagement.

2.2 CONCEPTUALIZATION OF SOCIAL MEDIA IN PUBLIC HEALTH

Social media refers to online technologies and practices used to connect people, enable communication, and facilitate the dissemination of information among users. It encompasses a variety of platforms, including Facebook, Twitter, Instagram, and LinkedIn, and has been employed to support real-time communication between communities and organisations such as the police (Chen et al., 2019; Ozuem et al., 2023).

Healthcare communication is defined as the process of sharing health information among healthcare providers, patients, and the wider public to improve health outcomes (Pangborn et al., 2023). Bringing these together, social media serves as a modern platform for healthcare communication, enabling the enhancement of public health messaging, patient engagement, and information exchange (Lai & Chong, 2023). As noted by Chen et al. (2019), when compared with earlier reviews such as that published in 2013, social media has undergone significant thematic evolution, further highlighting its dynamic and expanding role in the healthcare sector.

Focusing on Abu Dhabi, this study seeks to gain **a more nuanced insight** into the relationship between social media communication and public engagement. Given the widespread use and significant influence of social media, it has the potential to drive increased community involvement, promote patient-centred care, and enhance health branding across Abu Dhabi's healthcare sector (Koumpouros et al., 2015). Social media can improve health communication by serving as a real-time, interactive platform for health-related discussions, influencing health behaviours, and fostering user communities (Huo et al., 2019). It also functions as a medium through which healthcare professionals can exchange information, respond to patient queries, and gain insights from patient feedback, facilitating two-way communication that can lead to improved health outcomes (Belt et al., 2013). Gesser-Edelsburg (2021) highlights the importance of strengthening online health and risk communication, particularly in times of crisis such as the COVID-19 pandemic, noting that social media interactivity **could be more fully leveraged**.

Taking this into account alongside the research objectives above, this study aims to conceptualise public social engagement via social media as a crucial component in branding public health, building trust, and enhancing the credibility of public health

services in Abu Dhabi. It also seeks to understand the social context of public engagement within social media communities focused on public health, consistent with recommendations by Freire et al. (2023) to “use social information in participatory communication,” as demonstrated in their study on the influence of social media use on patient selection of healthcare providers.

‘Reputation in healthcare,’ within the context of incorporating social media into healthcare communication, has received mixed portrayals across the sector (Chaudhri et al., 2021), shaping public perceptions of healthcare providers and services. In narrative-driven cultures, the storytelling that emerges through social media from frontline healthcare workers has played a significant role in shaping public understanding and representation of healthcare experiences during pandemics (Pangborn et al., 2023). However, the use of social media in healthcare communication is not without its challenges. Issues related to professionalism and the potential for healthcare risk have emerged, highlighting the need for critical and responsible use of social media in health-related communication (Chen et al., 2021).

Drawing on the cultural setting of Abu Dhabi, this study incorporates considerations of cultural appropriateness by recognising how values, norms, religious beliefs, and practices influence both social media use and public health communication. Social media serves as a vital tool for investigating interactive communication strategies that support community engagement and patient-centred care within Abu Dhabi’s healthcare context. Acknowledging the importance of patient engagement and the transformative potential of social media in healthcare, this project introduces a comprehensive framework for examining the relationship between social media and healthcare communication. It aims to explore the effects of this relationship on public engagement, patient-centred care, and trust in the public healthcare system in Abu Dhabi. By assessing the extent to which social brand engagement influences public health, the study offers implications for enhancing public trust and credibility in healthcare services, thereby contributing to a deeper understanding of social media’s role in healthcare communication.

The interactivity that underpins social media platforms triggers a two-way flow of information and insights and fits within the spirit of healthcare communication, which ideally should be multi-directional, involving communication across and between

health senders, receivers, and the general public (Chen et al., 2019). As Abu Dhabi's healthcare continues to evolve and change, the use of social media strengthens the ability to engage and reach different groups, which in turn results in more inclusive, versatile healthcare communication (Lai & Chong, 2023). Social media is, and increasingly will be, a disruptor in healthcare communication processes. It has played a significant role in expanding discussions beyond traditional patient-provider interactions to include community dialogue, forum-based exchanges, and participatory healthcare conversations, thereby supporting the development of collective learning and shared decision-making (Koumpouros et al., 2015). This broader dialogue is particularly vital in a polyglot society such as that of Abu Dhabi, which encompasses diverse demographic layers and constitutes a rich and multifaceted public sphere.

Due to the interactive nature of social media platforms, there are increasing opportunities for healthcare professionals to extend the reach of health information and raise awareness about health issues, thereby contributing to the advancement of patient-centred care and the development of public health branding within Abu Dhabi's healthcare sector (Chaudhri et al., 2021). These platforms serve as hubs that interconnect disparate entities within the healthcare ecosystem, fostering cooperation, facilitating knowledge transfer, and helping to bridge gaps in public health communication. In doing so, they strengthen the mutually beneficial relationship between patients and providers (Pangborn et al., 2023).

Understanding these underlying dynamics is critical when examining the nuances of health-related dialogue on social media. The role of social media in fostering public trust necessitates an exploration of the complex relationships among providers, the public, and the media (Chen et al., 2021). Through an interpretative approach, this examination investigates the intricate performances and practices of social interaction, uncovering the layered meanings embedded in the healthcare communication landscape of Abu Dhabi.

In comparison to previous work, the thematic analysis in this study focuses specifically on aspects such as social media communication and its impact on public trust, public perceptions, and the overall branding of public health services in Abu Dhabi (Chen et al., 2019). Through a detailed exploration of these themes, a clearer understanding

will emerge of how social media shapes healthcare communication dynamics and influences societal perceptions of public health.

This process has the potential to transform healthcare communication practices by reinforcing the importance of social engagement and collaborative learning within a community of practice. When social media intersects with healthcare communication, the combined influence can significantly reshape communication norms, leaving little room for traditional models to remain unchanged.

This study may also generate practical recommendations related to public health messaging, digital engagement, and communication strategy. These insights can inform stakeholders, government agencies, healthcare providers, and individuals, enabling more effective decision-making regarding the role of social media in healthcare communication. In turn, this can help foster improved public engagement, enhance patient experiences, and increase trust in Abu Dhabi's public health services.

In addition, the study examines the challenges and barriers associated with using social media in healthcare communication, with particular emphasis on misinformation, privacy concerns, and ethical issues. This approach provides a holistic perspective on the advantages and limitations of employing social media to strengthen healthcare communication (Chen et al., 2021). By applying a reflective lens to these risks and engaging with the ethical and responsible use of social media in healthcare, the study contributes depth and fresh insight into the broader discourse on the potential and pitfalls of digital communication in an evolving healthcare landscape.

Through a rigorous analysis of the interconnections between social media and healthcare communication, the study addresses the research aims and delivers a detailed account of how social media interactions shape healthcare delivery, public participation, and trust in Abu Dhabi's healthcare system. **These findings will be of relevance to healthcare professionals, policymakers, and the public**, as all stakeholders become increasingly engaged in shaping a more integrated, inclusive, and innovative model of health communication in the digital era.

2.3 SOCIAL MEDIA IN HEALTHCARE

The intersection of social media and healthcare represents a major shift in how the public engages with health communication, and it is highly relevant to the aims and objectives of this research. This development reflects the broader spread of digital technologies across industries, particularly within Abu Dhabi's healthcare sector, where progress and persistent challenges coexist. The rise of healthcare-focused social media has created a strategic platform for the dissemination and exchange of health-related knowledge. In its early stages, social media primarily facilitated communication between healthcare professionals and patients (Belt et al., 2013). Its contribution to supporting patient-centred care has since expanded, reinforced by increasing levels of community participation (Chaudhri et al., 2021).

According to Chen et al. (2019), there has been a significant shift from one-way patient education toward **more interactive and reciprocal** communication between patients and healthcare providers, with this interactivity influencing behaviour and encouraging greater involvement in personal health management (Koumpouros et al., 2015). The dynamic role of social media becomes especially prominent during crises such as the COVID-19 pandemic, where patient narratives have added nuance and depth to public understanding of frontline healthcare challenges (Pangborn et al., 2023). However, integrating social media into healthcare systems is not without difficulty. Issues such as inappropriate disclosure of personal health information (PHI) and the rapid spread of misinformation have prompted the development of professional guidelines to manage these risks (Chen et al., 2021).

Despite these challenges, social media has contributed to maintaining public trust, strengthening health branding, and increasing participation in community events and health campaigns (Chaudhri et al., 2021; Lai & Chong, 2023). In Abu Dhabi, social media plays a particularly significant role in advancing patient-centred care and shaping public health branding. It supports **more personalised communication**, builds trust in healthcare services, and fosters positive relationships between patients, communities, and healthcare providers (Gesser-Edelsburg, 2021). This multifaceted impact—spanning patient engagement, public health branding, and trust in services—makes social media an increasingly integral part of health management and delivery.

The historical development of social media in healthcare, from an initial “information phase” focused on simple dissemination to its current role in fostering patient-provider partnerships and community-level engagement, illustrates its potential to address concerns related to misinformation and privacy (Belt et al., 2013; Chen et al., 2021). Public trust and engagement strategies have adapted in response to shifting communication practices, particularly during public health emergencies (Pangborn et al., 2023; Gesser-Edelsburg, 2021). This evolution positions social media as a vital tool for crisis communication and trust-building in healthcare contexts. This transition is particularly significant in Abu Dhabi, where rapid healthcare expansion and rising expectations have shaped how social media is understood and used. As the healthcare system has developed, social media has played a key role in informing public perception and engagement with services (Huo et al., 2019; Freire et al., 2023). The expanding role of these platforms highlights their growing importance in shaping health discourse, public involvement, and trust within the community.

2.4 SOCIAL MEDIA SITES FOR HEALTHCARE COMMUNICATION

Using social networking sites as health education communication tools among UAE college students presents important implications and challenges within Abu Dhabi’s current healthcare environment. Social networking has become a significant element of healthcare communication and holds particular relevance for the Emirate. Platforms such as Facebook, Instagram, Twitter, LinkedIn, Reddit, and TikTok have become essential bridges for facilitating healthcare communication (Eghtesadi & Florea, 2020). These systems offer transformative potential by promoting patient-centred care, strengthening community involvement, and enhancing trust in healthcare systems.

The widespread use and interactive nature of platforms such as Facebook, Instagram, and Twitter have reshaped the healthcare communication landscape (Eghtesadi & Florea, 2020). LinkedIn, in particular, has become a major professional networking space within the healthcare sector (Eghtesadi & Florea, 2020). The informal, real-time communication style of platforms like TikTok and Reddit is increasingly being harnessed to modernise public health messaging and respond quickly to emerging needs (Eghtesadi & Florea, 2020). These platforms enable diverse forms of interaction and appeal to different user groups, making them effective tools for healthcare

outreach. Facebook and Instagram—both highly visual—offer immersive user experiences and have played a key role in contingency planning during global pandemics by communicating critical health messages (Eghtesadi & Florea, 2020). Their adoption aligns with the broader aim of improving patient-centred care, expanding public health branding, and enhancing public trust (Hu et al., 2019).

Given their real-time interactivity and extensive reach, social media platforms have become important vehicles for strengthening community engagement. This aligns with the first research objective, which examines how interactive communication via social media can enhance community involvement (Miah et al., 2017). Such platforms create direct communication channels between healthcare professionals and the public, influencing perceptions and attitudes toward health and healthcare services (Hu et al., 2019). Their ability to shape public discourse and construct the image of healthcare provision links directly to the second and fourth research objectives (Nabi et al., 2023). However, despite these benefits, challenges remain—particularly around professionalism, privacy, misinformation, and ethical considerations (Torous et al., 2019). Addressing these challenges requires **robust digital literacy initiatives**, ethical guidance, and strong regulatory measures to ensure responsible and informed social media use in healthcare contexts (Torous et al., 2019; Jeyaraman et al., 2023).

A multi-layered strategy is needed to mitigate risks and harness the potential of these platforms. This includes ethical guidelines, digital literacy training, and rigorous content validation frameworks. Such steps are essential for maintaining trust and safeguarding the integrity of healthcare communication (Torous et al., 2019). Proactive monitoring, ethical oversight, and public education will further reinforce responsible use (Choo et al., 2015). Social media has significantly reshaped communication within Abu Dhabi's healthcare landscape. It has enabled **greater community participation**, strengthened patient-centred care, and contributed to the branding of public health services through the interactivity and reach of platforms such as Facebook, Instagram, LinkedIn, Twitter, Reddit, and TikTok. However, concerns around professionalism and ethics require continued attention to ensure the responsible use of these tools.

2.5 COMMUNITY ENGAGEMENT IN HEALTHCARE, DEFINED

The multiple meanings of community engagement in healthcare, as discussed by Ozuem and Willis (2022), reflect the complex and context-dependent nature of the concept. Comparing the different perspectives offered by researchers enables a clearer examination of how various levels of involvement occur across diverse communities and healthcare contexts. In Berger's (2019) model of community engagement, quantitative metrics play a central role, serving as tangible indicators of success in health engagement initiatives. This emphasis is important for aligning engagement activities with intended outcomes, **ensuring that involvement efforts remain meaningful and measurable.**

However, a metrics-driven approach may risk overlooking the emotional and experiential aspects of community participation—elements that are vital for understanding individual and collective needs, motivations, and expectations. The study by Duku et al. (2022) is particularly valuable in showing the economic consequences of community involvement in healthcare, especially in socio-economically diverse settings. These definitions help clarify how community engagement influences healthcare utilisation and access. While such approaches offer critical insights into structural and economic outcomes (Ozuem & Rafangni, 2021), they may fall short in capturing the **subjective, relational, and human** dimensions that shape the real-life impact of community participation.

Alicea-Alvarez et al. (2016) and Durey et al. (2016) emphasise the significance of urban and Aboriginal community perspectives, highlighting the importance of **contextually sensitive** strategies that engage entire families and ensure inclusivity across diverse populations. Berger (2019) also stresses the value of community involvement within broader measurement frameworks, considering how engagement metrics inform healthcare decisions at both organisational and systemic levels (Ozuem et al., 2020). Although this perspective supports a predominantly quantitative approach to evaluating community engagement (Ozuem & Bowen, 2019), it risks narrowing the understanding of engagement by overlooking lived experience.

Duku et al. (2022) examine the role of community engagement in healthcare utilisation and health insurance enrolment in Ghana, drawing attention to how social involvement

shapes access in resource-constrained settings. While their analysis provides crucial economic insights, the focus on financial outcomes may limit a **holistic appreciation** of community engagement as a multidimensional construct that includes cultural, emotional, and relational components. In contrast, Alicea-Alvarez et al. (2016) introduce the concept of “Neighbourhood-Engaged Care,” which highlights the active work required of healthcare providers to engage communities in addressing health disparities. While this definition contributes to a learning-oriented approach, it may frame engagement too narrowly in educational terms, without fully recognising the broader social structures affecting participation.

Durey et al. (2016), focusing on Indigenous child health in Australia, advocate for culturally grounded, context-specific approaches. Although valuable, their contribution is constrained by its narrow demographic focus and may not fully translate to other contexts. Reboe-Benjamin et al. (2023) examine community participation among refugees and their engagement with general health services. Their research foregrounds the experiences of marginalised populations (Ozuem & Azemi, 2018), yet such case-specific insights may not always capture the broader realities of community engagement across all healthcare settings.

The complexity of these conceptualisations aligns closely with the present research aim of exploring how social media dialogism intersects with public engagement in Abu Dhabi’s healthcare sector. Multiple definitions highlight the varied dimensions of community engagement (Ozuem et al., 2021b), from economic considerations to individual-level interactions, each relevant to analysing the impact of interactive communication via social media (Navarro et al., 2020; Koumpouros et al., 2015). Berger’s (2019) metrics-based approach may be particularly useful for evaluating social media as a communication tool for public engagement (Ozuem et al., 2021c), especially when assessing measurable forms of social interaction and public health branding.

Context-specific definitions from Durey et al. (2016) and Alicea-Alvarez et al. (2016), which incorporate aspects such as the social context of participants and the influence of social capital, offer valuable perspectives on how public participation in social media communities may unfold within Abu Dhabi’s healthcare environment. These

frameworks support the investigation of patient-focused care and frame public social engagement as a mechanism for strengthening public health branding.

Insights from studies such as Reboe-Benjamin et al. (2023), which explore refugee experiences in healthcare, help illustrate how social brand engagement can enhance trust and credibility in Abu Dhabi's public health services. Collectively, these diverse interpretations of community engagement contribute to a more **pragmatic and contextually sensitive** understanding of inclusive engagement across varied healthcare settings. Such insights are essential for evaluating how social media platforms foster community participation, support patient- and family-centred care, and reinforce trust and credibility within Abu Dhabi's healthcare system.

2.6 PATIENT-CENTRED CARE: THE BASICS

Patient-centred care focuses on involving patients and their families as active participants—not only in shaping new care models but also in making decisions regarding the most appropriate treatment options for their individual needs (Berger, 2019). This model is fundamental to improving the quality of care by fostering trust and mutual understanding between patients and healthcare providers. Its relevance to this research is considerable, as it underpins how social media interactions and public engagement can strengthen public health in Abu Dhabi.

Patient-centred care aligns closely with the first research objective, which explores how interactive communication via social media can support this care model within Abu Dhabi's healthcare system. A thorough understanding of patient-centred care is essential for assessing the influence of social media communication and community engagement on public health branding and public confidence in healthcare providers across the emirate. Tailoring healthcare delivery to reflect individual preferences and expectations is critical for improving both service quality and health outcomes (Durey et al., 2016). Platforms such as Facebook, widely recognised as leading social media tools, can enhance community involvement and support patient-centred care by enabling two-way dialogue between healthcare providers and the public (Koumpourous et al., 2015). Such interaction allows for a deeper understanding of patient perspectives and concerns. Encouraging dialogue, conversation, and feedback through social media significantly strengthens patient engagement, reinforcing the

effectiveness of these platforms as communication tools and promoting a patient-centred care model (Bornkessel et al., 2014). Accurate measurement of public social engagement requires analysing these interactions and the broader social context in which they occur.

Understanding community needs, expectations, and preferences is essential for designing patient-centred care models that reflect patient values (Navarro et al., 2020). Framing public social engagement as a catalyst for patient-centred care is crucial, as it embeds a more empathetic, inclusive, and culturally relevant image of public health within societal consciousness (Farley et al., 2021). In this context, social brand engagement plays a significant role in building public trust and reinforcing the legitimacy of healthcare services by ensuring that patients feel respected, heard, and meaningfully involved (Powell et al., 2016). Social media also supports patient-centred care by fostering ongoing communication and reconnection between healthcare providers and communities. Such interaction is central to developing a truly patient-centred healthcare system (Alicia-Alvarez et al., 2016). However, the potential benefits of using social media in this context depend on healthcare providers' willingness to engage meaningfully and authentically with patients. Without such engagement, the transformative potential of social media will remain limited (Doherty, 2023).

Engagement involves more than interaction; it encompasses the **value derived** from those exchanges (Berger, 2019). The progression of patient-centred care in social media dialogue is rooted in trust—an essential factor in establishing credibility and legitimacy for public health services. This is especially important when addressing the healthcare needs of Abu Dhabi's diverse population (Opoku Duku et al., 2022). The study's focus on patient-centred care provides a foundation for examining the multiple dimensions of social media communication and community engagement within Abu Dhabi's healthcare sector. The interaction of these elements contributes to the emergence of a more responsive and culturally informed healthcare system, ultimately strengthening patient-centred care, public health branding, and trust in healthcare services across the region.

It is important to consider the role of social media in promoting patient-centred care within Abu Dhabi's healthcare context, where community engagement is crucial for

enhancing the effectiveness of health systems. Berger (2019) emphasises the complexity of engagement, noting that it may include patient, public, consumer, and community dimensions. Social media serves as a valuable medium for two-way communication, with the capacity to increase community involvement, enhance patient-centred care, and address urban health disparities (Berger, 2019; Alicea-Alvarez et al., 2016). As a dynamic communication tool, social media holds strategic relevance for strengthening public social engagement and broadening participation in healthcare discourse (Navarro et al., 2020). Emerging forms of digital engagement also contribute meaningfully to clinical outcomes and healthcare delivery.

Understanding the cultural context in which the public engages with social media communities requires an examination of the broader social environment and regional public health agendas (Durey et al., 2016). The relationship between communities and health systems is central to improving healthcare services in densely populated and culturally diverse settings. Public social engagement also acts as a driver of public health branding, shaping how collective identity and public perception develop over time. Bornkessel, Furberg, and Lefebvre (2014) highlight the potential of social media to support quality improvement and patient-centred care through sustained digital engagement. Their networked model underscores the importance of incorporating patient-driven perspectives and consolidating public health branding within digital environments.

2.7 THE CONTEXT OF USING SOCIAL BRAND ENGAGEMENT (SBE)

The importance of social brand engagement in building public confidence and trust in Abu Dhabi's healthcare system has become increasingly evident. Koumpouros, Toulas, and Koumpouros (2015) highlight the role of patient engagement and social media marketing in addressing specific healthcare needs and improving patient satisfaction. Social media enables ongoing dialogue between healthcare providers and the community, fostering a relationship built on trust, transparency, and reputational credibility.

The integration of social media into healthcare processes has contributed to a shift toward more mainstream models of care that emphasise patient experience and self-reported health, including among refugee populations (Reboe-Benjamin et al., 2023).

The inclusion of peer community health workers in community mobilisation has also been important in addressing barriers to preventive interventions within diverse populations (Farley et al., 2021). Investigating the relationship between social media communication and community engagement reveals how evolving digital dynamics influence patient-centred care, public health branding, and trust. This analysis helps to explain how social media reshaping healthcare delivery in Abu Dhabi is and underscores the growing importance of adopting patient-centric approaches to strengthen community engagement and public trust.

Social media has become an integral part of daily life and has had a significant impact on community engagement across sectors, including healthcare. Its capacity to foster dialogue, knowledge exchange, and connection makes it a valuable tool for healthcare professionals, patients, and citizens. Social media platforms offer spaces for individuals to engage, learn, and build relationships within the wider healthcare community. Among the most widely used platforms in this area—Twitter, LinkedIn, and Facebook—there is considerable potential to enhance quality improvement initiatives and support patient-centred care (Bornkessel et al., 2014). These networked environments facilitate conversation and interaction between varied groups, enabling more inclusive and collaborative approaches to healthcare delivery. For example, Twitter has been identified as a powerful tool for disseminating information and initiating engagement in academic medicine (Choo et al., 2015). Similarly, these platforms encourage more meaningful dialogue between healthcare workers and patients, promoting co-creation of knowledge and supporting improvement in healthcare practice.

In addition to enabling communication, social media also plays a strategic role in addressing health disparities and improving health outcomes in urban communities. Community engagement initiatives delivered through social media have shown unique potential to prepare future healthcare professionals for “Neighbourhood-Engaged Care,” a model designed to address health inequities in underserved urban areas (Alicea-Alvarez et al., 2016). **These initiatives demonstrate how digital tools can shape education, strengthen cultural preparedness, and help healthcare workers understand diverse community needs.** Social media is also increasingly recognised for its role in healthcare reputation management. Chaudhri et al. (2021)

argue that a complex interdependence exists between expressions of “care” on social media and public perceptions of healthcare organisations. In this context, strategic use of social media for reputation management becomes critical. Platforms serve as environments where public attitudes can be monitored, enabling healthcare leaders to align communication with stakeholder expectations and broader community values.

As members of the Voice of the Customer (VOC) community began integrating LinkedIn as a dedicated professional network, the role of social media in promoting healthcare workforce development became more visible. Platforms like LinkedIn enable practitioners to network, collaborate, and engage in career-related discussions, supporting professional growth and exchange. While social media holds significant potential as an engagement tool, it also brings challenges. These include instances of unprofessional behaviour, inappropriate content-sharing, and subsequent risks to patient trust and safety. Chen et al. (2021) emphasise the need for clear ethical standards and coherent professional conduct guidelines to reduce these risks.

The influence of social media on healthcare behaviour is further illustrated in research by Freire et al. (2023), who examine how it shapes patient decision-making and health-seeking behaviour. Their work highlights how social media functions as a source of information, opinion shaping, and behavioural motivation—factors essential for patient-centred care.

Social media platforms also act as **dynamic reservoirs of public sentiment**. For instance, Hu et al. (2019) analysed public perceptions of healthcare services on Chinese social media to show how online discourse reflects evolving expectations, needs, and frustrations. Monitoring this feedback allows healthcare providers to adjust services in more responsive and personalised ways. Berger (2019) argues that the broad range of engagement opportunities offered by social media requires deeper investigation to establish robust benchmarks for measuring public interaction. Such benchmarks would support healthcare professionals in evaluating the depth and quality of community involvement.

The importance of social media in health communication becomes even more pronounced during global health crises. Platforms such as Facebook, Instagram, Reddit, and TikTok have proven essential to contingency planning by allowing health

authorities to distribute timely, accurate, and actionable information (Eghtesadi & Florea, 2020). Their instrumental use during emergencies illustrates the potential of social media to support public health protection and crisis management.

However, rapid public uptake of social media also increases scrutiny and raises complex ethical and regulatory concerns. Torous et al. (2019) argue that as social media becomes a central environment for healthcare engagement and even research, **evolving ethical frameworks** are necessary to protect privacy, moderated participation, and data integrity. Effective use of social media in healthcare therefore requires thoughtful, balanced approaches that ensure responsible engagement. Taken collectively, these insights demonstrate that social media has become a transformative component in healthcare communication—shaping behaviour, strengthening community involvement, and influencing patient-centred care. Its dialogical and communal nature, alongside its expansive reach, reinforces its role as a cornerstone of modern healthcare engagement.

2.8 CHALLENGES AND OPPORTUNITIES

Several authors have examined the opportunities and challenges that social media brings to healthcare communication. Much of this discussion has focused on the interaction of dynamic online communities, control of information, and the ethical dimensions of the digital environment. Alicea-Alvarez et al. (2016) highlight how social media can enhance outreach in urban areas and reduce health disparities. They emphasise the importance of considering the unique needs, preferences, and cultural contexts of different urban groups. In contrast, Bejtkovský (2021) identifies the potential of platforms like LinkedIn to promote health-related content in professional environments but notes the challenge of maintaining professional boundaries.

Belt et al. (2013) reveal that the internet and social media are widely used sources of health information within the Dutch population, raising concerns about ensuring that public preferences are matched with evidence-based, accurate health content. Berger (2019) shows that social media improves different types of engagement within healthcare systems, but highlights the difficulty in finding reliable and meaningful metrics to measure engagement outcomes. Bornkessel et al. (2014) explore

networked patient-centred care enabled by digital engagement and raise questions about maintaining quality and credibility in online patient involvement.

Chaudhri et al. (2021) propose a privacy-preserving approach to healthcare reputation management on social media. They demonstrate how online expressions of “care” influence institutional image while noting the challenge of managing public perceptions in a constantly shifting digital context. Chen et al. (2021) highlight how social media surveillance helps identify unprofessional behaviours, but also raise concerns about inappropriate conduct and widespread misinformation. Durey et al. (2016) illustrate the opportunities for social media to support culturally sensitive engagement in Aboriginal Australian communities, though this is paired with the need to understand local cultural values. Similarly, Eghtesadi and Florea (2020) emphasise the importance of leading social networking sites in pandemic contingency planning, particularly the need for strong moderation and information governance.

Koumpouros et al. (2015) discuss patient engagement and social media marketing as vital to healthcare improvement, but stress the need to balance engagement with ethical advertising practices. Lai and Chong (2023) explore how social media influences healthcare engagement among older populations, noting challenges related to time perspective, familiarity with digital tools, and perceived value. Miah et al. (2017) look at low-income communities and show how mobile social media can serve simultaneously as a support tool and a barrier, depending on how accessible and relevant the information is. Nabi et al. (2023) examine the role of social media in building social capital and trust in healthcare facilities in Bangladesh, again linking digital interaction with the potential to enhance trust in resource-limited settings. Moreover, Navarro et al. (2020) highlight challenges in global surgery engagement, including the tension between authentic storytelling and broader global narratives. During the COVID-19 pandemic, Pangborn et al. (2023) emphasise the power of frontline healthcare storytelling in shaping emotional responses and public understanding. Pizzuti et al. (2020) discuss healthcare workers’ attitudes toward social media as an educational tool, identifying a mix of practicality, scepticism, and varied perceptions.

Powell et al. (2016) explore barriers to patient engagement across interdisciplinary teams, stressing the challenge of recognising barriers that differ between professional

groups. Reboe-Benjamin et al. (2023) show how Facebook-based engagement can support refugee healthcare navigation, but also highlight the complexity of doing so in culturally and socially sensitive contexts. Torous et al. (2019) draw attention to the ethical and regulatory concerns connected to social media's expanding role in healthcare and research. Together, this discourse illustrates a rich and diverse ecosystem of inquiry. It reveals both the substantial opportunities and significant challenges introduced by social media for healthcare communication, engagement, and delivery. These insights can inform future digital healthcare tools, software design, policies, and interventions aimed at improving engagement and communication across social media platforms.

Public engagement is characterised as the inclusion of people or communities in decision-making processes, opening governance and research to **diverse and alternative forms of knowledge**. The multidimensional social dynamics involved in public participation are critical to strengthening democratic processes and **remain essential for improving the effectiveness and inclusivity** of participatory approaches (Király & Miskolczi, 2019; Rask et al., 2018). Understanding the role of social dynamics in public participation is crucial for developing successful and equitable engagement processes. Public involvement encourages collective problem-solving in response to social challenges, contributing to sustainable transformation and supporting community resilience (Klinga et al., 2018). In this way, it underscores the importance of fostering trust, affirming community values, and strengthening societal well-being (Franceschini & Marletto, 2019).

Marginson (2016) discusses the interrelationship between social structures and participation, highlighting the global shift toward mass systems of higher education. As participation broadens, the resulting social stratification demands **careful and systematic approaches** that ensure inclusivity and responsiveness to different societal needs and expectations. This dialogic relationship between societal structures and participatory dynamics highlights the complexity of public engagement, where interactions between different societal layers shape the potential for improved inclusivity. Expanding this further, Callaghan and Horne (2023) explore localised and highly varied participatory practices, focusing on the relational nature of community-embedded interactions. Their analysis helps illuminate the diverse ways engagement

unfolds, including the specific, and at times unique, modes of community life that condition participation.

Franceschini and Marletto (2019) add insight into the role of trust within public participation. Trust forms the foundation of participatory processes because it influences reciprocity, cooperation, and mutual respect between stakeholders. Establishing trust is essential for developing participatory frameworks, linking community aspirations with practical outcomes, and **reducing tension between ideal goals and real-world implementation**. Martínez-Parra et al. (2018) argue that sociocultural factors significantly influence public participation in healthcare. Their work demonstrates how cultural norms, expectations, and broader social values shape public involvement. This fusion of cultural and participatory elements highlights the need for culturally congruent participation models that recognise community-specific values and align with expectations around healthcare delivery.

Owusu-Addo et al. (2018) and Nguyen and Connelly (2014) also reflect on the influence of social norms on health outcomes and labour participation. For example, unequal distribution of unpaid care work and varied effects of cash transfers on health inequalities in sub-Saharan Africa show how social and economic conditions shape participation and health outcomes. These interdependencies call for **holistic and multidimensional** approaches that incorporate economic, social, and health-related components into participatory frameworks. The nuances of social interaction across these studies underscore that participation is deeply embedded within structural, cultural, and emotional dimensions. Elements such as mutual trust, hierarchy, cultural resonance, and economic context collectively influence the form and quality of engagement. This complexity requires an approach that captures diverse expectations and needs, especially as participatory landscapes continue evolving.

Community formation involves generating connections within groups, promoting shared objectives, and cultivating mutual respect. This process creates opportunities for collaboration, strengthens group cohesion, and builds collective resilience. In healthcare, social support—whether emotional, informational, or practical—is central to effective engagement. Support systems such as peer groups, counselling, and community programmes help individuals and families navigate health challenges while connecting them more meaningfully to the healthcare system. These mechanisms

include support groups, peer-led initiatives, and the exchange of knowledge and resources.

The convergence of community building and social support is critical to patient-centred and holistic healthcare. It prevents gaps in support, strengthens continuity of care, and fosters an environment where empathy, understanding, and culturally sensitive practices can flourish. Community building enhances collective resilience, encourages help-seeking behaviours, and facilitates information-sharing that leads to better health decisions.

In diabetes-related support communities, for example, workshops, seminars, and shared learning sessions bring together patients, families, and healthcare professionals to exchange experiences and foster mutual understanding. These activities provide emotional support—such as peer counselling and friendship—and practical assistance with day-to-day management, including dietary planning and treatment routines. Such bonds reinforce group identity, strengthen community networks, and make support more accessible and meaningful. Ultimately, community development and social support contribute to improved self-management, mental health, and overall well-being. When combined with comprehensive support services, they equip healthcare communities to meet diverse needs and promote better long-term health outcomes.

2.9 CONCEPTUALIZATION OF SOCIALITY: SOCIAL MEDIA AS A TOOL AND AN ENVIRONMENT

Social engagement in the context of social media is an emerging construct within healthcare. Social media provides a unique space where healthcare providers and users can communicate, share, and receive health-related information (Belt et al., 2013). Yet this interaction extends beyond the basic exchange of information. Instead, it reflects the **networked and participatory** model of patient-centred care described by Bornkessel et al. (2014). This model shifts healthcare communication away from a traditional provider-centric structure toward one that prioritises patient involvement, collaboration, and ongoing engagement.

Mediated social networking can effectively serve multiple healthcare purposes, including health promotion, education, and stakeholder interaction (Bejtkovský, 2021).

It offers a range of tools and platforms that healthcare providers can use to build relationships, increase engagement, and deliver accurate information to the public (Choo et al., 2015). For instance, applications like Twitter are increasingly recognised as effective channels for sharing information and facilitating knowledge exchange among healthcare professionals (Choo, Burke, & Walker, 2015). **However, the expanding use of these platforms brings concerns related to professionalism, accuracy, and public trust**, particularly as boundaries between professional and personal communication may become blurred (Chen et al., 2021). These concerns highlight the need for clear ethical guidelines for professional social media use within healthcare.

Social media analytics offer further insight by enabling the identification and evaluation of “super-engagement” behaviours that contribute to enhanced healthcare delivery (Doherty, 2023). These tools can reveal user preferences, communication patterns, and behavioural tendencies, allowing providers to tailor communication strategies and improve the patient experience. The complex role of social media within healthcare aligns with a vision of a more informed, participatory, and collaborative system (Jeyaraman et al., 2023). When used ethically and responsibly, social media can help cultivate a culture of engagement that enhances care quality and supports community-based approaches to health.

2.10 PUBLIC ORGANIZATION AND THE BRANDING CONCEPT

The concept of corporate branding in the public sector, including healthcare, can be understood as a strategic activity aimed at establishing a distinctive identity that is respected by the wider community and that fosters trust and loyalty. This goes beyond commercial marketing and instead represents a deliberate effort to create meaningful impact within local communities and broader society (Alicia-Alvarez et al., 2016). The rise of digital platforms has reshaped how public organisations approach brand-building. Social media platforms, for example, connect healthcare providers with communities through **continuous, two-way communication** (Belt et al., 2013). In today’s digital environment, branding is not only about recognition—it is about maintaining a credible, trustworthy digital presence.

Networks such as LinkedIn have become important for promoting healthcare services, especially for chronic disease management and professional networking within the health sector (Bejtkovský, 2021). When used effectively, these platforms can enhance community involvement, which is essential for enabling public organisations to understand, respond to, and anticipate community needs (Callaghan & Horne, 2023).

Branding in the public sector is therefore not limited to image-building. Rather, it involves **developing trust, ensuring transparency, and presenting healthcare organisations as credible, community-focused bodies**. Social media is central to this objective, offering healthcare organisations opportunities to communicate with the public, respond to concerns, share health information, and showcase their contributions to community well-being (Chaudhri et al., 2021). This type of online engagement influences public opinion, shapes organisational reputation, and encourages sustained community involvement (Hu et al., 2019). Moreover, the analysis of user engagement on platforms such as YouTube and other social media channels allows healthcare organisations to tailor their branding strategies to align with public expectations (Doherty, 2023). This strengthens service provision and builds a stronger sense of public trust. Ultimately, branding in healthcare represents a nuanced strategy that supports community engagement, corrects misinformation, and encourages a positive public perception of health and wellbeing.

2.11 TAXONOMY OF PUBLIC HEALTH BRANDING

Public Health Branding (PHBr) is an interdisciplinary methodology that integrates community engagement, digital interaction, and evaluation within traditional public health communication. They emphasise that healthcare providers must build strong relationships with urban communities and work with community-health workers to ensure access to preventive care in vulnerable populations. Moreover, Duku et al. (2022) and Durey et al. (2016) reinforce the need for healthcare services to be tailored to specific populations—particularly Aboriginal communities—highlighting that **proactive, culturally sensitive engagement** is essential.

Researchers also stress the role of digital engagement. Bejtkovský (2021) identifies LinkedIn as a valuable platform for professional health communication, while Choo et al. (2015) and Bornkessel et al. (2014) emphasise the usefulness of Twitter in

promoting public health messaging and supporting patient-centred care. Meanwhile, Koumpouros, Toulas, and Koumpouros (2015) focus on the importance of patient engagement and social media marketing in building a trustworthy public health brand. Evaluation remains a core pillar of PHBr. Berger (2019) calls for the development of robust engagement metrics, while Doherty (2023) uses web analytics to examine how healthcare students interact with online materials. Franceschini and Marletto (2019) argue that **public trust should be considered a key metric** in public health branding.

Taken together, this typology demonstrates the importance of combining traditional and digital communication strategies with strong evaluation frameworks to build public trust and maximise the effectiveness of public health branding campaigns. Social media now plays a central role in shaping the branding and reputation of public health organisations. Healthcare professionals and institutions use platforms such as Facebook and Twitter to distribute essential health information, interact with the public, and strengthen their public-facing brand (Koumpouros, Toulas, & Koumpouros, 2015). According to Van der Belt et al. (2013), social media has become one of the most widely used sources of health information globally, highlighting its significance for modern healthcare communication.

Social media enables more patient-driven content, encourages feedback, and supports timely communication between providers and communities (Bornkessel, Furberg, & Lefebvre, 2014). However, Chaudhri et al. (2021) caution that social media can also present risks to reputation if content is poorly managed or if inaccurate information spreads. Additionally, misinformation remains a significant challenge, and without proper monitoring it can undermine public trust (Gesser-Edelsburg, 2021). Moreover, Bejtkovský (2021) further examines how LinkedIn supports professional branding and networking, demonstrating how platform-specific strategies influence public perception. Active engagement on social media can offer valuable insights into public concerns and preferences (Chen et al., 2019), allowing health organisations to produce more targeted and relevant public health content. Social media is a **powerful tool for shaping brand perception**, but it requires strategic, careful management to safeguard credibility and trust.

2.12 PUBLIC TRUST AND SOCIAL BRAND ENGAGEMENT

A theoretical approach to Social Brand Engagement (SBE) in public organisations involves adopting proactive, interactive communication strategies that strengthen the relationship between public health organisations and their stakeholders online. SBE combines dialogic communication, community participation, and **sustained digital interaction** to foster trust, loyalty, and a patient-centred ethos. Moreover, Heldman, Schindelar, and Weaver (2013) highlight how health organisations can enhance community engagement by using social media more effectively. They argue that authentic interaction—not merely broadcasting information—has a disproportionately positive effect on public trust. Sharp (2011) provides a case study showing how brand-driven engagement can support public health interventions and improve public perception.

Dunn, Pearlman, Beatty, and Florin (2018) explore psychosocial motivations behind young people's engagement with drug-prevention campaigns, offering insight into how Online Health Communities (OHCs) function. Suomi et al. (2021) highlight the importance of internal branding by showing how employee engagement shapes external public trust.

Guillaume and Loufrani-Fedida (2023) demonstrate how inclusive stakeholder involvement strengthens branding effectiveness in public services, while Eugenio-Vela et al. (2020) offer a framework for place-branding that is relevant to public health contexts such as Abu Dhabi. Azer and Alexander (2022) illustrate how communication and engagement strategies influence vaccination behaviour, reinforcing the importance of trust-based messaging. Taken together, this literature provides a strong foundation for analysing SBE in Abu Dhabi's public health sector, supporting the goal of amplifying the patient voice and reinforcing trust in public health services.

In a time when public trust is often fragile, branding has moved beyond commercial identity to become a key mechanism for building credibility—especially in healthcare. Alicea-Alvarez et al. (2016) show how community-centred engagement can reduce health inequalities, using branding to communicate a provider's commitment to community needs. The COVID-19 pandemic further highlighted the importance of branding in public health. Azer and Alexander (2022) demonstrate that credible

branding influences vaccination uptake by shaping public perceptions of safety and reliability.

Bejtkovský (2021) examines how professional platforms like LinkedIn contribute to digital health branding, reinforcing credibility and strengthening public engagement. Studies by Belt et al. (2013) and Bornkessel et al. (2014) reveal that social media offers new pathways for health communication, making information more accessible and supporting patient-centred care. Moreover, Callaghan and Horne (2023) emphasise the role of trusted brand identities in encouraging local health participation. Chaudhri et al. (2021) explore how healthcare reputation is shaped online, noting that strong branding generates trust, while unprofessional conduct online can harm credibility (Chen et al., 2021). Gesser-Edelsburg (2021) calls for more socially oriented health communication during crises, arguing that trust in the message depends on the perceived integrity of the brand. Huo et al. (2019) and Koumpouros et al. (2015) highlight how patient empowerment and social marketing strengthen branding and community engagement. Nabi et al. (2023) provide empirical evidence that social media can build social capital and trust in healthcare organisations. So, branding has become a central component of health communication, closely tied to public trust, social capital, and institutional credibility.

2.13 POLITICS OF SOCIAL MEDIA IN ABU DHABI

The Abu Dhabi government has adopted a multifaceted approach to regulating social media platforms—one that is digitally responsive and rooted in the Emirate’s political, cultural, and social context. Social media governance in Abu Dhabi involves a balancing act: **protecting state security, public order, and national image while simultaneously using digital communication to advance government objectives.**

Abu Dhabi and the wider UAE exert substantial influence over the digital sphere through legislation, regulatory frameworks, and enforcement strategies designed to oversee online content. This approach aligns with a national strategy that prioritises social stability and national security. In this context, social media is not only a channel for controlled messaging but also a potential site for ideological disruption, including content produced by groups considered a threat to the state. Several key laws shape this environment. The UAE Cybercrimes Law establishes strict penalties for offences

such as spreading misinformation, defamation, or promoting ideas seen as threatening national unity. **This legislation acts as a legal mechanism for restricting content that may destabilise or criticise the state.** Likewise, the UAE Penal Code (Federal Law No. 3 of 1987) criminalises defamation and insult, especially when directed at the state, its rulers, or religious symbols. These legal tools form part of a broader framework designed to ensure that digital communication aligns with state narratives and reinforces state authority.

This balancing act is also evident in how the government frames the limits of acceptable speech on social media. Although the UAE Constitution guarantees freedom of expression, this right is not absolute—particularly in areas involving national security, religious values, or public morality. Authorities justify these restrictions on the grounds that unregulated digital speech may “sow discord,” promote hatred, or undermine national cohesion. As government oversight of digital platforms is often presented as necessary for the protection of society. From this perspective, **stability and security are positioned as prerequisites for exercising any other rights**, including expression. Citizens are therefore free to express themselves online—provided they remain within legal boundaries. Those who violate these laws face legal consequences.

The Telecommunications Regulatory Authority (TRA) is the principal body responsible for monitoring social media activity in Abu Dhabi and the wider UAE. It enforces national internet regulations by ensuring social media platforms comply with local laws. This includes coordinating with platform providers to remove illegal content and block access to sites or accounts deemed harmful or inappropriate. The TRA also uses advanced technologies for **digital surveillance and threat detection**, analysing online communications to identify potential risks to national security or public order. Beyond enforcement, the TRA runs public awareness campaigns encouraging responsible social media use and explaining the consequences of online misconduct. These efforts form part of a state-led strategy to create a “safe” digital space that upholds national values, protects state security, and allows carefully managed public engagement.

Within this framework, social media regulation in Abu Dhabi is a layered and evolving project that combines legislation, surveillance systems, and public communication

strategies. The government's approach reflects the Emirate's broader political priorities, where individual autonomy and freedom of expression are often secondary to national stability, security, and ideological cohesion. **The TRA ultimately ensures that social media use remains within tightly defined ethical and legal boundaries.**

More broadly, Abu Dhabi—and the UAE as a whole—uses social media strategically for governance and statecraft. Platforms serve not only to disseminate official narratives but also to shape public attitudes and reinforce collective identity. This reflects an awareness of the central role social media plays in shaping discourse in a hyper-connected society with high digital engagement levels. One of the key ways Abu Dhabi asserts control over the digital environment is through curated messaging that promotes official narratives. Government accounts on platforms like Twitter, Instagram, and Facebook publish content emphasising national unity, economic development, and cultural heritage. Messages are often packaged with **visual media—infographics, short videos, and photographs—to enhance engagement and shareability.** The government also collaborates with influencers and public figures to expand the reach of its messages and tailor them to different audience demographics.

This strategy combines direct communication with more subtle forms of persuasion. Official updates, announcements, and interactive posts project responsiveness and transparency, while narrative storytelling around national achievements fosters pride, cohesion, and loyalty. Social media analytics enable the government to track public sentiment and adjust communication strategies in real time.

A major component of this strategy is controlling misinformation and politically sensitive content. Strict laws require social media companies to comply with local regulations, including removing or blocking content deemed false or threatening. Meanwhile, government agencies counter misinformation by issuing rapid clarifications and official updates on their own platforms. These measures both limit the spread of disinformation and reinforce the reliability of official sources.

2.14 USE OF SOCIAL MEDIA BY THE HEALTH INDUSTRY IN ABU DHABI

The Abu Dhabi Department of Health (DoH) has intentionally adopted social media as a key channel for public communication, particularly for sharing health information and promoting public health campaigns. In a region with high internet penetration and significant daily digital engagement, the DoH recognises social media as a powerful and efficient means of reaching large segments of the population. Its social media activity serves multiple functions: **disseminating health information, promoting preventive behaviours, and strengthening public engagement.**

The DoH's social media strategy serves several core objectives. These include increasing awareness of public health issues, encouraging prevention and healthy living, and providing up-to-date public health guidance. Social media is used to update the population on policies, healthcare services, and emerging health concerns. This supports wider efforts to improve health literacy and empower individuals to make informed decisions. Another key objective is strengthening public trust through transparent, responsive communication channels that address questions and concerns directly.

To ensure accuracy and reliability, the DoH uses strict verification processes before publishing content. Medical professionals review posts to ensure alignment with evidence-based guidance and current public health recommendations. Much of the content is developed in collaboration with international health organisations and major healthcare providers, reinforcing credibility. These measures are designed to reduce misinformation and promote trust. The DoH also collaborates with government agencies and private-sector partners to widen the reach of its messaging. During COVID-19, for example, the DoH worked closely with NCEMA and MoHAP to deliver consistent guidance on safety protocols, vaccination campaigns, and public health updates. These partnerships helped ensure coordinated messaging across sectors.

The DoH uses multiple platforms based on audience reach and content type. Twitter is used for real-time updates, while Instagram and Facebook support visual and interactive educational content. YouTube hosts longer-form materials such as webinars and health explainers. To reach Abu Dhabi's diverse population, the DoH employs a multilingual communication strategy, using Arabic, English, and other

widely spoken expatriate languages. The Ministry of Health and Prevention is particularly attentive to cultural sensitivities when preparing content on topics such as mental health or lifestyle diseases. **Culturally appropriate messaging helps reduce stigma and encourages open discussion.**

The DoH also uses analytics tools to evaluate engagement, monitor sentiment, and identify emerging misinformation trends. This data-driven approach enables rapid adjustments to communication strategies, ensuring responsiveness to public needs. Crisis communication is a central element of the DoH's social media strategy. Detailed protocols guide real-time communication during pandemics, outbreaks, and other emergencies. During COVID-19, the DoH used social media extensively to update the public on case numbers, vaccination campaigns, and safety measures, while also countering rumours and false information. This approach significantly reduced public anxiety and supported effective crisis management. The DoH's purposeful use of social media—supported by strategic partnerships, cultural sensitivity, and real-time monitoring—has strengthened its role as a trusted public health communicator in Abu Dhabi. Through its digital strategy, the DoH promotes public awareness, encourages prevention, and fosters a more informed and health-literate population.

2.15 THEORETICAL UNDERPINNINGS

This section explores the Health Belief Model (HBM), a psychological framework that explains why individuals engage—or fail to engage—in health-related behaviours. Understanding the origins and development of the HBM is essential for contextualising the foundational assumptions that underpin the model. Exploring its historical development also helps clarify the basic principles that shape both its theoretical structure and practical applications across diverse health contexts. Understanding the core components of the HBM is particularly important, as these elements explain the **complex interaction between health perceptions, behavioural intentions, and actual behaviour**. This provides insight into how individuals interpret health information, perceive risks, and decide whether to act.

The model's relevance to health communication is especially significant. It provides a structure for understanding how people interpret public health messages and how such perceptions influence behavioural responses. The section also discusses the

relationship between the HBM and social media, demonstrating the model's adaptability within today's digital environment—where health information is frequently disseminated and interpreted online. Although the HBM has contributed extensively to health behaviour research, it is not without limitations. A critical analysis of these limitations clarifies the model's strengths and constraints, offering insight into how it might evolve or be supplemented by other frameworks. The rationale for applying the HBM to this study is then presented, demonstrating its importance as a conceptual foundation.

Following this discussion of the HBM, the next part introduces Social Influence Theory—another major framework that explains how people's attitudes, beliefs, and behaviours are shaped by others. This section examines the origins and evolution of Social Influence Theory, its core mechanisms, and its categories. It also explores how social influence shapes public behaviour in digital contexts, especially through social media platforms. The application of Social Influence Theory to this study is discussed by linking its key constructs to the research aims and design.

2.16 HEALTH BELIEF MODEL

The Health Belief Model (HBM) was originally developed in the 1950s by social psychologists working at the US Public Health Service who sought to understand why individuals failed to engage in disease prevention and ignored screening tests even when they had no symptoms. Over time, the model has been expanded and adapted, becoming one of the most widely used frameworks in global health behavioural research (Alejandro, Bruce, & Leo, 2022). The HBM assumes that health behaviour is shaped by an individual's beliefs or perceptions about disease and the actions available to reduce risk (Al-Metwali et al., 2021). It identifies six components that guide the design of health messages: perceived susceptibility, perceived severity, perceived benefits, perceived barriers, cues to action, and self-efficacy (Ellithorpe et al., 2022). For example, Al-Metwali et al. (2021) found that beliefs about susceptibility to and severity of COVID-19 were major predictors of vaccine uptake among both healthcare workers and the public.

There are meaningful connections between the HBM and social media, particularly when analysing how people interpret health information online. Mao et al. (2023) used

the HBM to evaluate how social media engagement influenced responses to COVID-19 health promotion messaging, demonstrating the model's flexibility in explaining how individuals react to online information. Tian et al. (2023) combined HBM constructs with social media analytics to explore how hospital-originated Twitter posts shaped public responses during the pandemic. Their findings show how perceptions of susceptibility and severity directly affect digital engagement with health messages.

Hita et al. (2023) investigated media-driven behaviours such as social distancing and panic buying, highlighting how **external cues—especially from social media—can trigger rapid behavioural responses**. In this context, social media acts as a cue to action, prompting individuals to adopt or reject specific behaviours based on perceived benefits and barriers. Similarly, Alhaimer (2022) used the HBM to examine the effectiveness of public health messaging on social media in Kuwait during COVID-19, showing how trustworthiness and message quality influence beliefs and behavioural intentions in digital environments.

Despite its usefulness, the HBM has several limitations. Scholars such as Odunsi & Farris (2023) and Hita et al. (2023) argue that it portrays beliefs and behaviours as relatively static and does not adequately capture the way perceptions can shift over time. Others, including Nwosu & Ward (2022), contend that the model is not consistently predictive and does not fully account for the complexity of human behaviour, especially when influenced by social, cultural, or emotional factors.

The HBM has also been critiqued for insufficiently incorporating emotions such as fear, anxiety, and stress—known drivers of health decisions (Ellithorpe et al., 2022). Early iterations of the model excluded self-efficacy, limiting its usefulness for supporting behavioural change (Tajiki et al., 2022; Ayaz-Alkaya et al., 2020). Further critiques emphasise that the HBM lacks cultural adaptability and does not fully address diverse health beliefs across different communities (Al-Metwali et al., 2021; Nooriani et al., 2019). While the HBM has been successfully applied—particularly in contexts involving social media health communication (Davies & Mann, 2023; Alhaimer, 2022)—its limitations highlight the need for complementary frameworks that incorporate emotional, cultural, and socio-structural factors.

2.16.1 Application of the health belief model to the study

The Health Belief Model is highly relevant to this research because it supports the exploration of how social media communication influences health engagement, patient-centred care, public trust, and public health branding in Abu Dhabi. The HBM acts as an intervention-oriented framework, explaining how an individual's perception of health threats—combined with mediating factors—shapes preventive action (Mao, Wang, & Zheng, 2023; Alhaimer, 2022). It therefore provides a lens through which to examine how beliefs shaped by social media messages influence engagement, trust, and perceptions of public health services.

The core HBM constructs—perceived susceptibility, perceived severity, perceived benefits, perceived barriers, cues to action, and self-efficacy—are directly relevant to evaluating how the public responds to social media-based health communication (Tian et al., 2023; Ellithorpe et al., 2022). For example, Mao et al.'s (2023) cross-national study on COVID-19 health promotion shows how differing beliefs, shaped by digital messages and community narratives, influence engagement behaviours and compliance with public health guidance. Understanding patient-centred care requires recognising the influence of social media on health beliefs and public engagement. Social media serves as a major channel for distributing health information and shaping public perceptions of risk, trustworthiness, and credibility (Yoo et al., 2018). The HBM can therefore help explain how different forms of digital communication shape health beliefs and behaviours. For example, Hita et al.'s (2023) findings on **media-triggered behavioural responses** illustrate how online cues influence compliance with preventive measures during crises.

The HBM is also valuable in understanding public trust. Trust is multidimensional and closely linked to perceptions of credibility, reliability, and transparency within health systems (Tetteh et al., 2022; Davies & Mann, 2023). By analysing how health beliefs develop in response to digital messaging, the model helps reveal how social media interactions either strengthen or weaken trust in public healthcare services. These mechanisms highlight the continued relevance of the HBM across diverse health communication environments.

The model's broad use across different health contexts—ranging from water demand management (Al Metwali et al., 2021) to vaccination behaviour—demonstrates its versatility in analysing varied beliefs and practices. This flexibility makes the HBM highly suitable for exploring perceptions of public health communication in Abu Dhabi's multicultural setting. By acknowledging the diversity of health beliefs within the Emirate, the model supports the development of more equitable, culturally informed, and patient-centred communication strategies (Youssef et al., 2022; Alejandro et al., 2022). The HBM's predictive insights—particularly when paired with factors such as message credibility and health literacy—reinforce its relevance in assessing how social media messaging influences behaviour (Odunsi & Farris, 2023). This supports the study's examination of how trusted, well-framed social media communication can shape health beliefs, strengthen public trust, and reinforce public health branding in Abu Dhabi.

2.17 SOCIAL INFLUENCE THEORY

The concept of Social Influence Theory emerged from decades of psychological and sociological research exploring how individuals' thoughts, feelings, and behaviours are shaped by others. One of the earliest and most influential contributors was Herbert Kelman (1958), who identified three key types of influence—compliance, identification, and internalisation. These categories describe the different psychological processes through which influence occurs and explain how society shapes personal beliefs and behaviour. Since its introduction, Social Influence Theory has been adapted and applied widely across different domains, becoming a foundational perspective within social psychology for analysing how behaviour and beliefs are shaped by social environments.

Kelman's typology highlights how individuals may comply with social expectations to gain approval, identify with others to maintain relationships, or internalise external norms when they become aligned with personal values. Over time, the theory has been expanded and integrated with other models to capture its multidimensional effects. For example, Liu et al. (2020) combined Social Influence Theory with Uses and Gratification Theory to examine sustained content contribution on microblogs, emphasising how online social influence shapes digital user behaviour.

Within consumer behaviour research, Naeem and Ozuem (2021) used Social Influence Theory to understand panic buying during crises, demonstrating how peer norms and social media interactions can escalate behavioural responses. Advanced the theory further by identifying resonance, activation, and cultivation as mechanisms through which leaders shape group norms and collective values. Social Influence Theory has also played a significant role in explaining digital behaviour. Lin et al. (2020) used the theory to analyse mobile payment adoption, combining cost–benefit analysis and perceived value with social influence to explain uptake. Naeem (2021) applied the theory to internet banking, illustrating how digital environments intensify the effects of social norms on decision-making. The theory’s adaptability—from early work on persuasion resistance to its integration with the elaboration likelihood model—reflects its long-standing relevance across changing social contexts.

The three major components of Social Influence Theory: internalisation, compliance, and identification. Internalisation, highlighted by White et al. (2009), represents a deep alignment where values or behaviours become part of one’s personal belief system. **This form of influence is stable and enduring**, reflecting genuine acceptance rather than mere behavioural conformity. Compliance, as explained by Lin, Wang, and Huang (2020), involves behavioural alignment in response to external pressures, incentives, or social expectations. **It is driven by extrinsic motivations rather than internal conviction.** When individuals align themselves with a group or person because the association is psychologically meaningful. **This form of influence can reshape personal norms as individuals internalise the identity of the group.** Together, these elements illustrate how Social Influence Theory captures the complex interplay between personal beliefs, external social pressures, and group identification. The theory offers insight into how individuals interpret, adopt, and reproduce norms within their social environment—an important consideration in understanding how behaviour is shaped within healthcare settings and digital communities.

2.18 THE IMPORTANCE OF SOCIAL INFLUENCE THEORY TO THE CURRENT STUDY

Social Influence Theory is highly relevant to this research because it helps explain how digital interactions, social norms, and mediated communication shape public engagement, public health branding, and trust in Abu Dhabi’s healthcare context. The

theory provides a framework for examining how individuals respond to online health communication and how community norms form within digital environments. Liu et al. (2020) demonstrate how users' motivations for contributing content become intertwined with social reinforcement, showing how personal incentives interact with social approval in shaping behaviour. This is particularly important in understanding how interactive communication on social media fosters community involvement and supports patient-centred approaches in Abu Dhabi. **The theory highlights the simultaneous influence of individual motivations and social cues**, offering a nuanced way to study engagement patterns in health-related digital communication.

White et al. (2009) provide insight into descriptive, injunctive, and in-group norms, revealing how these different forms of influence shape perceptions of trust and credibility in public health services. Understanding these norms is essential for framing public engagement as a driver of public health branding. Leadership, communication, and normative influence further illustrates how shared values and collective identity are cultivated—an important dimension of building trust in Abu Dhabi's health services.

Naeem (2021) and Naeem and Ozuem (2021) explore the antecedents of social influence in digital environments and show how individuals co-construct moral norms through online interactions. Their findings help explain how social brand engagement on social media can improve trust in public health services in Abu Dhabi. **These insights demonstrate the recursive relationship between digital communication, community norms, and behavioural outcomes**, which is critical to understanding how public trust develops in a digital health ecosystem. Collectively, Social Influence Theory provides a multidimensional framework for analysing how social media communication shapes public engagement, patient-centred care, and trust in Abu Dhabi's healthcare system. It offers valuable insight into the social mechanisms that underpin digital interaction and helps explain how online communication can reinforce or reshape public health perceptions and behaviours.

2.19 THEORETICAL FOUNDATION

Social Influence Theory and the Health Belief Model (HBM) together provide a **coherent and multidimensional foundation** for achieving the aims of this study. The HBM highlights the importance of individual perceptions, attitudes, and beliefs in

shaping health-related behaviour, while Social Influence Theory explains how social pressures, norms, and interpersonal dynamics influence those beliefs and behaviours—particularly in digital environments.

The HBM is especially useful for understanding how people make decisions about engaging in preventive health behaviours and how they interpret risks and benefits communicated through social media. **In the context of Abu Dhabi, where diverse cultural groups interact with public health messages online, the HBM helps explain variations in perception, trust, and behavioural intention.** It offers a structural way of analysing how message framing and perceived credibility affect public engagement with digital health communication. Social Influence Theory complements this by illuminating how collective norms, peer behaviour, cultural expectations, and online community dynamics shape individual responses. In social media environments—where interactions are public, rapid, and highly visible—perceptions of what others think or do can strongly influence behaviour. **This is particularly relevant in Abu Dhabi’s digitally active society, where social media users frequently negotiate between personal beliefs and broader community expectations.**

Together, these two frameworks allow this study to explore not only how people interpret health information on social media, but also how social interactions—likes, comments, shares, endorsements, and peer norms—shape public trust, patient-centred engagement, and public health branding. The combined use of these theories supports a deeper analysis of how digital communication influences both individual cognition and collective behaviour within Abu Dhabi’s healthcare ecosystem. By integrating the HBM with Social Influence Theory, the study can examine:

- how individuals perceive risk, credibility, and benefits when encountering health messages online
- how social cues from peers and digital communities shape engagement behaviour
- how public trust develops from the interaction between personal beliefs and social norms
- how digital communication contributes to the branding and credibility of public health services in Abu Dhabi

This combined theoretical lens therefore supports a rich, layered interpretation of how social media affects health communication outcomes, and it provides a strong conceptual basis for evaluating public engagement, patient-centred care, and trust in the Abu Dhabi healthcare system.

2.20 CRITICAL EVALUATION OF THEORETICAL FRAMEWORKS

The HBM and SIT theoretical framework underpinning this study **offers** an integrated model for understanding the influence of social media communication on public engagement, trust, and behavioural responses in healthcare. The **HBM and SIT each represent distinct yet overlapping perspectives**: whereas the HBM is concerned with how different behaviours may be adopted or rejected depending on one's personal perception of their benefits and risks (Al-Metwali et al., 2021; Ellithorpe et al., 2022), SIT seeks to account for how social and normative imperatives are **accepted or internalised** in relation to general role expectations and social norms (Kelman, 1958; Naeem & Ozuem, 2021). These two frameworks, however, have limitations when used separately in the digital health communication context within a multicultural society like Abu Dhabi. An evaluation of these theories, and their subsequent synthesis, demonstrates their weaknesses as independent paradigms and their strength when combined into a hybrid model for use within this study.

The Health Belief Model has been a popular theory for explaining cognitive factors that predict preventive health behaviour. It predicts whether people will adopt health-related behaviours by examining perceived susceptibility, perceived severity, perceived benefits, perceived barriers, cues to action, and self-efficacy (Alejandro et al., 2022; Mao et al., 2023). It is attractive due to its simplicity and its applicability to a wide range of health topics, from vaccination adherence (Al-Metwali et al., 2021) to media-triggered behaviour change (Hita et al., 2023). **Yet the model's strong focus on individual cognition can also be both an advantage and a limitation.** It presumes rationality in how individuals perceive risk and benefit, while overlooking emotional, cultural, and social components that can significantly influence behaviour (Ellithorpe et al., 2022; Odunsi & Farris, 2023). In the context of social media communication—where much decision-making arises from emotional contagion,

social comparison, and peer validation rather than rational processing—this limitation becomes particularly evident.

Another limitation of the HBM is its inability to fully accommodate the dynamic and iterative nature of human behaviour. As Odunsi and Farris (2023) argue, beliefs and attitudes are **not static structures** and develop continually through exposure to new information, interactions, or experiences—especially in online environments. Social media users constantly encounter competing health information and evolving narratives, which can change their perception of risk and willingness to comply with public health guidance. These continuous feedback mechanisms are **not fully represented** in the HBM. Moreover, as Al-Metwali et al. (2021) and Nooriani et al. (2019) note, the model does not adequately adapt to culture, assuming a universality of cognitive processing that may not align with collectivist societies where decision-making is shaped by communal trust, family influence, and religious norms. These oversights highlight the importance of incorporating a social framework such as SIT to more accurately reflect how digital interactions and cultural contexts shape health-related behaviours.

Social Influence Theory, based on Kelman's (1958) distinction between compliance, identification, and internalisation, adds the crucial social dimension missing from the HBM. This theory captures the processes through which people change their behaviour, attitudes, or beliefs due to real or perceived social pressure. As Liu et al. (2020) illustrate, social influence works on a continuum—from superficial compliance driven by extrinsic motives to deeper internalisation of values aligned with community norms. In digital media environments, such influence manifests through peer approval, online advocacy, conversations, and social currency (likes, shares, and comments). These processes are especially relevant to understanding how members of the Abu Dhabi public interact with health-related content on platforms such as Twitter and Instagram.

Despite its strengths, SIT is not a complete explanation for why people adopt or reject specific health messages. It does not incorporate key behavioural health elements such as perceived vulnerability, personal efficacy, or self-assessment of health threats. Without accounting for individual cognition, SIT risks overgeneralising behaviour as mere conformity to social pressure. Additionally, because SIT has been

applied to diverse fields ranging from marketing to leadership communication, its constructs can become ambiguous when applied to health contexts (Naeem, 2021). Accordingly, while SIT explains social forces and pressures within digital spaces, it must be **combined with frameworks such as the HBM** that link social influence to individual-level health decision-making.

The linkage between HBM and SIT in this study is **symbiotic**. Together, they provide a comprehensive overview of how social media communication affects health-related engagement. Whereas the HBM explains why individuals act when they perceive personally relevant risk, severity, and benefits, SIT shows how these actions are modelled, valued, or discouraged in social encounters, cultural settings, and shared practices (Kelman, 1958). For example, a person may believe risks are high (HBM) but still choose not to act—such as by refusing vaccination—if they do not perceive adequate peer approval or do not trust message sources (SIT). In Abu Dhabi's health system, where community solidarity and institutional confidence are central to compliance, the integration of these models captures both **individual and collective aspects** of health-related behaviour.

This reciprocal relationship is also critical for understanding digital engagement with public health communication. In the era of social media, communication is no longer linear but occurs within a multi-directional participatory ecosystem. In this space, the elements of the HBM—such as perceived susceptibility or cues to action—directly overlap with SIT mechanisms such as internalisation and compliance. When the Department of Health in Abu Dhabi posts about vaccination campaigns, public responses depend not only on personal risk perception (HBM) but also on visible indicators of social validation, such as comments or endorsements (SIT). The integrated framework therefore captures how psychological and sociocultural mechanisms shape trust, engagement, and behavioural intention in digital public health communication.

Although both models offer strong theoretical grounding, several alternative theories were evaluated and excluded. The Theory of Planned Behaviour (TPB) expands behavioural intention through attitudes, subjective norms, and perceived behavioural control (Ajzen, 1991), but its focus on **intention rather than actual behaviour** was less suitable for this study's emphasis on engagement processes. The Technology

Acceptance Model (TAM) (Davis, 1989) is useful for understanding technology adoption but lacks attention to emotional, cultural, and normative variables critical to health communication and digital trust. The Diffusion of Innovations Theory (Rogers, 2003) focuses on macro-level adoption patterns rather than the micro-level interplay between personal beliefs and social influence within social media. These models therefore offered **limited relevance** to the specific dynamics of electronic health messages in Abu Dhabi.

The choice to focus on healthcare rather than marketing or politics reflects the unique ethical and societal stakes associated with public health communication. In commercial or political settings, social media followers may be encouraged toward partisan alignment. Health communication, however, takes place within a **moral and civic domain**, where engagement can directly shape public safety, institutional trust, and real-world behavioural outcomes. As Naeem and Ozuem (2021) note, digital interactions in the health domain carry heightened requirements for transparency and credibility, as they can influence behaviours such as vaccination uptake and adherence to health advice. Unlike commercial promotion, public health communication requires not only influence but also education, empowerment, and ethical responsibility.

Healthcare communication in Abu Dhabi presents additional context-specific complexities that make the use of HBM and SIT particularly appropriate. The population is characterised by high ethnic, linguistic, and cultural diversity. As Alhaimer (2022) and Tetteh et al. (2022) observe, confidence in public health messaging depends not only on precision but also on perceived authenticity, empathy, and social connectedness. In this sense, social media is not only a communication tool but an interactive space where identities, values, and narratives are shaped. The integration of HBM and SIT therefore enables this study to analyse how individual health perceptions interact with social structures in digital communication—highlighting both opportunities and tensions in promoting patient-centred, trustworthy public health dialogue.

Despite the distinct conceptual boundaries of both HBM and SIT, their integration produces a **rigorous and comprehensive** framework capable of explaining the complex interaction of cognition, emotion, and social influence in digital health

communication. Whereas the HBM provides a psychological explanation of health behaviour at the individual level, SIT situates that behaviour within broader social norms, trust systems, and shared cultural expectations. By employing both frameworks, this study connects cognitive and social dimensions of health engagement and provides a richer understanding of how social media shapes public trust, participation, and behaviour within Abu Dhabi's healthcare system. This integrated model therefore offers a powerful lens for understanding digital health communication in a highly connected and information-saturated environment.

The development of social media in healthcare, as outlined in this literature review, serves two purposes. It highlights both the transformative nature of social media in reshaping communication and its potential to foster more nuanced and context-sensitive healthcare dialogues. These platforms have played a significant role in modernising the dissemination of information and reducing the information asymmetry that often characterises healthcare systems. They have become essential to the healthcare sector, **particularly in the Emirate of Abu Dhabi.**

The relevance of Social Influence Theory is critical for the present study, which analyses how social media and public engagement interact—often unevenly—within Abu Dhabi's health sector. Social Influence Theory offers a framework for understanding the ripple effects of interactions, communication, and social norms within digital spaces. The theory contributes to a deeper appreciation of public health branding, patient-centred care, and trust in health service delivery.

Liu et al. (2020) offer insight into ongoing content-contribution behaviours on social networks, revealing how user motivation and reinforcement from online social presence intertwine to influence content sharing. Recognising this interplay is essential for understanding both the current and potential role of interactive communication in fostering greater community involvement and enhancing patient-centred care in Abu Dhabi. **This perspective highlights how personal motivation and contextual influences shape participation in health-related social media communities**, thereby enriching this study's examination of digital communication in healthcare.

The dual role of social media as both a communication tool that integrates personal perceptions with social context and a strategic mechanism for shaping public health messages. **This offers insight into the sociocultural landscape that underpins public engagement**, revealing how cultural norms operate within online health communities and how they shape the “social unconscious” of digital health communication. Further depth is added through the role of descriptive, injunctive, and in-group norms in shaping social behaviour. These dimensions strengthen our understanding of public confidence in health services and help frame public engagement as a vehicle for embedding health branding within collective consciousness. This by exploring how leadership communication fosters trust through resonance, activation, and the cultivation of shared values—key processes for reinforcing the credibility of Abu Dhabi’s public health services.

Work by Naeem (2021) and Naeem & Ozuem (2021) on social influence antecedents in cyberspace, and the construction of socially accepted norms, also supports the relevance of social brand engagement in cultivating public trust. The authors highlight a cyclical relationship between social impressions and individual behaviour, **offering a comprehensive perspective on how digital interactions shape public confidence in health institutions**. So Social Influence Theory provides a multidimensional framework for unpacking the complex pressures, norms, and influences operating within healthcare-focused social media. It contributes meaningfully to the literature on how digital engagement shapes patient-centred care, public health branding, and institutional trust—particularly in a culturally diverse setting such as Abu Dhabi.

The rationale for using Social Influence Theory and the Health Belief Model (HBM) lies in their combined ability to offer meaningful insight into the aims of this study. The HBM contributes to the framework by clarifying how individuals think and behave in relation to healthcare—specifically, how they decide to engage in preventive health behaviours. When applied within social media communication, the HBM provides a mechanism through which public engagement may be influenced and patient-centredness encouraged, by explaining how individuals interpret and respond to health-related information within Abu Dhabi’s community healthcare context.

However, Social Influence Theory asserts that social pressures and norms significantly shape individual thoughts, perceptions, and behaviours. **The theory highlights the extent to which digital interactions influence public perceptions of the health sector**, showing how actors within social media spaces establish health branding and build trust. By applying SIT, this study can examine the mechanisms through which social media communication affects public engagement with, and trust in, health services in Abu Dhabi.

Together, these theoretical models provide an integrated framework for analysing how social media participation influences individual health beliefs and behaviours in relation to public health engagement, patient-centred care, and institutional trust. **Such a framework identifies strategic opportunities for leveraging social media messaging to strengthen public health branding and enhance trust** in Abu Dhabi's evolving healthcare landscape. The approach is therefore valuable for examining the multidimensional relationship between social media communication and public perceptions and behaviours in this unique context.

2.21 SUMMARY

The development of social media in healthcare, as outlined in this literature review, serves two purposes. It highlights both the revolutionary nature of social media in transforming communication and its potential to foster more nuanced and context-sensitive healthcare dialogues. These platforms have played a significant role in modernising the dissemination of information and addressing the information asymmetry that often characterises healthcare systems. Social media has become essential to the healthcare sector, **particularly within the Emirate of Abu Dhabi**, where digital engagement is exceptionally high and government institutions actively integrate online communication into public health strategies. The relevance of Social Influence Theory is critical for understanding how social media and public engagement intersect within Abu Dhabi's healthcare context. The theory captures the ways in which communication, digital interaction, and social norms shape individual decision-making in online environments. It provides insight into public health branding, patient-centred care, and trust—three central themes of this study.

Liu et al. (2020) emphasise how social reinforcement and online presence influence content-sharing behaviour. **Recognising this interplay is essential for understanding the potential of interactive communication to strengthen community involvement and support patient-centred care in Abu Dhabi.** This view enriches this study's examination of how personal and contextual factors shape participation in health-related online communities. The dual role of social media as both a platform through which personal perceptions are shaped and a mechanism that amplifies sociocultural expectations. **This deepens the understanding of how cultural norms operate within digital health spaces, shaping behaviours and attitudes in ways that are especially relevant to Abu Dhabi's multicultural population.**

The descriptive, injunctive, and in-group norms as drivers of social behaviour. These norms shape public confidence in health services and help explain how public engagement becomes a vehicle for embedding public health branding within collective consciousness. Leadership communication fosters trust through resonance, activation, and shared values—processes highly relevant to building institutional trust in Abu Dhabi's public health sector. Moreover, Naeem (2021) and Naeem & Ozuem (2021) provides further support by demonstrating how digital norms are constructed and how social impressions shape individual behaviour. **This cyclical relationship between social influence and behavioural response offers a comprehensive insight into how social media interactions strengthen public trust in health institutions.**

The rationale for using Social Influence Theory and the Health Belief Model (HBM) in this study lies in their ability to offer complementary insights into how individuals and communities respond to digital health communication. The HBM clarifies how individuals assess health risks, weigh benefits, and make decisions regarding preventive behaviours. When applied to social media communication, it provides a structure for understanding how public engagement may be influenced and how patient-centredness may be encouraged.

Social Influence Theory adds a crucial dimension, showing how digital norms, peer behaviour, and social expectations shape individual and collective responses to health communication. **It highlights how online interactions influence public**

perceptions of health services, trust in institutions, and participation in public health initiatives. Together, the HBM and SIT provide a multidimensional framework for analysing how social media participation influences health beliefs, behaviours, patient-centred care, and institutional trust. **This integrated approach helps identify strategies for leveraging digital communication to enhance public health branding and strengthen trust in Abu Dhabi's evolving healthcare landscape.** By combining cognitive and social mechanisms, this framework provides a comprehensive explanation of how digital health communication operates in a highly connected environment shaped by cultural diversity and rapid information exchange.

CHAPTER 3: RESEARCH METHODOLOGY

3.1 INTRODUCTION

Research Methods explores the philosophical bases, as well as the theoretical and strategic dimensions of leading methodologies used in social research. This exploration is essential, as it significantly shapes the researcher's view and understanding of the nature of reality (ontology), the nature of knowledge (epistemology), and the ways through which knowledge is generated and disseminated (method). These basic philosophical stances determine the direction of knowledge work, regulating all decisions taken, including those concerning the kinds of questions asked and the methods employed.

The chapter has been organized to address, in a linear manner, the complex layers of research methodology, starting with an examination of the various philosophical worldviews that underpin research designs. These traditions are not simply academic niceties, but offer a way of orienting the design of the research to its philosophical roots, giving the project cohesiveness and integrity in relation to its methods and

findings. After explaining the philosophical foundations, the chapter moves on to provide an extended discussion on how methodological decisions, particularly the choice between quantitative and qualitative approaches, are inspired by these philosophies. This section considers the consequences of each approach and their appropriateness given the researcher's ontological and epistemological orientations. The discussion further explores how these decisions affect not only data generation and interpretation, but also the ethical concerns associated with conducting **rigorous** and respectful research.

Weaving a path through the theoretical dimensions and linking them to practical research applications, the goal of the chapter is to empower researchers to make informed decisions regarding method selection, supported by a coherently argued philosophical rationale. This integrated approach ensures that the research conducted is methodologically robust, ethically rigorous, and of significant value to broader knowledge, facilitating the transfer of theoretically grounded research into practical applications in the real world.

3.2 RESEARCH PARADIGM

Research philosophy forms the foundation of research, and explains the source and justification of the methods and analytical techniques employed. It is critically important in terms of shaping how researchers conceive and interpret reality (ontology), understand the nature of knowledge (epistemology), and determine how that knowledge can be attained and represented (methodology). Such philosophical standpoints establish the groundwork from which research is undertaken, and they are integral to how studies are developed and what they ultimately accomplish, particularly in social research.

The distribution of philosophical views is significant, as it influences the researcher's approach to difficult questions about humans and society. The absence of a philosophical perspective can hinder the ability to explore such questions with direction and depth. Diversity in these perspectives is valuable and also necessary, as a broader range of viewpoints can contribute to a more comprehensive understanding of the complex social world. The epistemological position of a study raises the question of what counts as valid knowledge, and how it can be acquired. Walliman (2022) states

that in research, epistemology refers to the relationship between the knower and the known. It determines the questions posed and the conclusions drawn. For example, a scholar adopting a positivist perspective may be inclined to examine observable phenomena and favour quantitative tools that yield measurable evidence. In contrast, an interpretivist epistemology would focus on subjective meanings and the complexities of human perception, privileging qualitative approaches such as interviews and observations as a means to gain a richer understanding of human experience, as claimed by Poth (2023). **Research philosophy forms the foundation of research since it explains the source and justification of the methods and analytical techniques employed.** It is critically important when it comes to shaping how researchers conceive and interpret reality (ontology), understand the nature of knowledge (epistemology), and determine how that knowledge can be attained and represented (methodology). These philosophical standpoints establish the basis on which research is undertaken and they **are** integral to how studies are developed, particularly in social research.

3.2.1 Implications of philosophical standpoints

The implications of philosophical standpoints in research are quite significant. They shape the design and conduct of research, and also influence how the findings are interpreted and applied. This is especially relevant in social research, where the shifting realities of human behaviour and social systems demand critical reflection on foundational assumptions. As Sloan and Quan-Haase (2017) argue, maintaining a consistent philosophical stance lends clarity and depth to the research process, guiding the researcher through the complex tasks of data collection and analysis. Such thinking helps to produce findings that are both trustworthy and meaningful. The philosophical underpinning of research is a core part of the research itself. Indeed, the philosophy influences how researchers view the world, and the kinds of questions they ask. It has a bearing on the methods they apply to find answers, and ultimately, the knowledge that they generate. It is essential, therefore, that these positions are considered and defended early in the research process to ensure that the study remains coherent and grounded in a robust philosophical framework.

Ontology, in the context of research, is the philosophical study of the nature of reality—what exists, what entities can be said to exist, and how they are categorised and

understood. In social research, ontology addresses fundamental questions about what can be known, who we are, and the nature of our various modes of being in the world (Walliman, 2022). These ontological concerns shape the assumptions researchers hold about the 'reality' of social phenomena, influencing every other aspect of the research process. Two major ontological positions are realism and relativism. Realism holds that real entities exist independently of human minds, thoughts, or beliefs. This perspective, aligned with positivism, assumes that phenomena are observable and can be objectively described by the researcher without influence from beliefs or interaction (Neuman, 2014). Relativism, by contrast, asserts that reality is socially constructed, shaped by human perception and culture. From this perspective, reality cannot be separated from the ways in which it is interpreted (Robson and McCartan, 2016).

3.2.2 Ontological orientations

The disparity between these two ontological orientations runs through every level of research activity. For realists, the task of the researcher is to identify and describe reality as truthfully as possible, often leading to the use of large, **representative** sample sizes that are believed to provide a more comprehensive account of the reality being examined. Findings under a realist approach are typically presented as truths that can be generalised to wider contexts. Relativists, by contrast, view the researcher's role as that of an interpreter or meaning-maker, leading to the use of smaller, **purposively** selected samples that allow for detailed, contextual exploration (Saldana, 2021).

Moreover, realist research is generally explanatory in orientation, seeking to specify cause-and-effect relationships and to draw conclusions about phenomena based on observable patterns. Relativist research, on the other hand, focuses on how individuals or groups experience and construct their social worlds, making its purpose interpretive and descriptive in nature (Hair et al., 2019). Research methods also differ considerably between these two ontological positions. Realists typically emphasise quantitative approaches that prioritise objectivity and replicability. They use experiments and surveys designed to minimise the influence of the researcher's subjective perceptions on the findings. Relativists are more likely to adopt qualitative methods such as interviews, focus groups, and ethnography, enabling them to gather

in-depth information about the diversity of human experiences and social conditions (Creswell and Creswell, 2018).

Researcher engagement likewise varies. In realism, the researcher is positioned as separate from the research process and acts as an objective observer. In relativism, the researcher is closely involved and often engages directly with participants to understand their views and lived experiences (Bell, Bryman, and Harley, 2022). Subjectivism and relativism are also related. Relativists, like subjectivists, hold the view that knowledge is constructed by humans. However, relativists argue that such construction is shaped by time, place, and social position. They also **acknowledge** that phenomena may be experienced and interpreted in different ways by different people, depending on their cultural, historical, and personal contexts (Matias, 2021). Relativism permits a nuanced investigation into the nature of interactive public discourse. Such discussions are inherently subjective, filtered through perceptions formed at individual, societal, and cultural levels. In the “landscape of participation,” understanding how different segments of the community interpret and respond to public health messages on social media can reveal multiple definitions and experiences of meaningful engagement and communication (Sloan and Quan-Haase, 2017).

In contrast, a realist ontological stance might not adequately support these aims, as it assumes an objective reality independent of human interpretation; one that can be conveniently measured. Such a perspective can overlook the subjective and interpretive processes through which public health campaigns are received and understood by communities. Realism might also constrain the study to quantifiable indicators of engagement, and **may** then fail to capture the depth of personal and communal impact, or the quality of interactions that shape public perceptions of healthcare system effectiveness.

On the other hand, the subjectivist epistemological position, which aligns with relativism, permits methodological choices that reflect the complexity of everyday human experiences and social media interactions. Such a perspective is essential for understanding the influence of increased community engagement on public health outcomes. Such outcomes, after all, can be shaped by how messages are received and acted upon across diverse community segments. Subjectivism also enables a

discussion of how different audiences within Abu Dhabi might **interpret** and respond to health communication, a crucial factor in developing strategies that are culturally and contextually appropriate (Hammond and Wellington, 2020). Conversely, objectivism is less suitable for this research, as it seeks to establish universal truths based on observable phenomena in a way that is value-free and generalisable. Within the context of this study, such an objectivist approach could neglect the individual and collective experiences that inform community engagement on social media—experiences that are essential to understanding the depth and efficacy of these interactions within Abu Dhabi’s public health system.

Relativist and subjectivist standpoints are therefore key where the aim is to gain insight into the social media landscape of public health communication in Abu Dhabi. Both perspectives support a more nuanced understanding of the diverse and complex experiences of individuals at the community level, contributing to public health strategies that are much more sensitive and effective since they are grounded in the lived realities of the community. Research epistemology refers to the study of the theory of knowledge in research, including the methods, validity, and scope of scientific understanding. Epistemology is concerned with what knowledge is, how it is acquired, and how it helps to define appropriate methodologies for conducting research. While methods, results, and data may be objective, interpretation and conclusions are inseparable from epistemological considerations (Walliman, 2022).

It remains important for researchers to be mindful **of why** the philosophy which underpins a particular study is consistent with its research design, methods and analysis in order to avoid an epistemological mismatch. In this type of inquiry, research is guided by a paradigm that acts as **an** intellectual and philosophical GPS, dictating everything from the questions asked to how findings are interpreted (Creswell & Creswell, 2018; Walliman, 2022). This paper has a foundation in the interpretivist paradigm, **guided** by **a** social constructionist epistemology based on **a** subjectivist ontology. This framing offers a relevant point of view from which to explore how residents and communities in Abu Dhabi make sense of social media as a tool for public health communication. In this perspective, qualitative semi-structured interviews and thematic analysis are not only methodological instruments but also

epistemic acts which bring into being meaning by the co-construction of it with participants **led by the** researcher (Saldana, 2021).

Interpretivism developed as a reaction to the constraints of **the** positivist framework that claims **that** objective truths about the world can be derived from empirical observations and measurements. Quantification, prediction and generalisation dominate positivist research, which attempts to account for social reality with **numerical variation** and statistical association (Neuman 2014). Interpretivism, on the other hand, dismisses the notion that there is one universal objective reality and contends that knowledge construction occurs through subjective interpretation and social interaction (Bell et al., 2022). From this position, social phenomena are thought not to be comprehensible independent of the interpretations **that** individuals make of **them**. The interpretivist researcher is therefore interested in understanding not measurement, interpreting not predicting, and the particular rather than **the** general (Robson & McCartan, 2016).

The interpretivist tradition is grounded in a set of key beliefs which are particularly pertinent to this study. First, it takes as given that reality is both manifold and socially constructed – shaped by individual experience, language and culture. Secondly, it acknowledges the inseparability of the researcher from the research process and that their interpretations, positionality, and reflexivity are part of what underpins **the** knowledge generated (Kara et al., 2021). Third, overarching interpretivism is depth rather than range: the focus is on deep and thick understanding of experiences **within** their context. This is in line with the purpose of this study to investigate **health professionals'** and **the** general public's perceptions about social media's role in public health communication, as reported in Abu Dhabi.

Qualitative interviews, a characteristic of interpretivist research, enable participants to describe the meaning and significance **of experiences** in their own words. Interviewing is open-ended and dialogic, where a relational space of meaning-making dialogue takes place rather than **being strictly** sought (Creswell & Creswell, 2018). This approach allows the researcher to acquire an understanding of participants' viewpoints across their sociocultural context. Additionally, the interpretive analytic process (thematic analysis) supports the researcher in uncovering patterns and themes that are derived from participants' narratives inductively rather than **through**

a priori conceptual definitions imposed by researchers (Saldana, 2021). Both the data collection and analysis practices are **in harmony** with the interpretivist philosophy of co-constructed knowledge and reflexive involvement.

3.2.3 Epistemological underpinning

Social constructionism forms the epistemological underpinning of this research, since interpretivism serves as the overall philosophical orientation. Social constructionism is the position that knowledge, meaning and reality are produced through social processes of interaction and shared meaning rather than representing objective facts 'out there' to be discovered (Bryman, 2021). Derived from Berger, (2019) work, social constructionism posits that our perception of reality is shaped by language, culture, and societal systems. These mediations structure not **only** what people perceive as reality, but also how they perform within it.

In this study, social constructionism provides a valuable **framework** with which to examine the ways in which shared understandings of health, illness and institutional trust are constructed through social media platform communication. Public health communication is not just a one-way relay of information, but **is** socially mediated, throwing up a relationship that incorporates interpretation, negotiation and recontextualisation between the senders and receivers of messages (Mertens 2023). By way of example, a tweet from the Abu Dhabi Department of Health does more than **simply** send a message: **some individuals** will understand it differently based on their cultural background, prior experience, **level of trust in** the institution in question, and social environment. Social constructionism illuminates how these sense-making processes interact with engagement and behavioural outputs.

The constructionist stance towards knowledge implies that once the focus of the **researcher's** practices has become constructionist **and** epistemological, **they** accept a view of research as practice – in other words, accepting that research practice itself is a site for creating knowing. Interviews are not passive exchanges, but active dialogues with participants through which meaning is co-constructed by the researcher. In line with interpretivism's rejection of objectivity, this position is consistent with taking a reflexive attitude that accepts that the researcher is always thinking about how their own assumptions and questions are informing the data (Kara

et al., 2021). This reflexivity is especially important in cross-cultural research sites such as Abu Dhabi, where language, social hierarchies and institutional organization affect communication and interpretation.

The interpretivist and social constructionist underpinnings of the present study are congruent with its qualitative format, inductive reasoning, and methodological approaches. The study seeks to understand how people and communities make sense of, interpret, and interact with social media as a medium for public health communication. This requires an enquiry that understands the subjective, situated nature of human experience rather than looking for general laws or cause-and-effect relations.

3.2.4 Data collection and epistemology

The use of semi-structured interviews as the main tool for data collection is closely associated with interpretivism. Semi-structured interviews strike a balance between structure and flexibility, **allowing** respondents to share experiences and perceptions **while** giving the interviewer scope for follow-up questions when new aspects present themselves. This follows from the constructionist assumption that meaning is produced in context and through interaction (Sloan & Quan-Haase, 2017). Eventually, stories from every participant add to the developing explanation of how social media serves as a communicative link—or conversely, obstacle—between healthcare professionals and the public. **Data arrangement** is assisted by thematic analysis as a framework that aims to locate and interpret patterns of meaning. **Thematic** analysis is not a method that just organises data but also interprets it through the researcher's experiences, helping to uncover analytic themes representing both participants' voices and **theoretical implications** on the part of the researcher (Braun & Clarke 2006). This dialectic between participant meaning and researcher understanding is central to the interpretivist legacy. In this analysis, themes arising from the interviews provide insight into how one's beliefs are shaped and maintained in digital health communication environments with regard to credibility, trust, **and** engagement.

Ontology **refers to** assumptions about what exists and what can be known about it. The ontology adopted in this study is a subjectivist one where reality can only be known based on individual and collective processes of making sense, which

distinguishes it from objectivist ontologies held by those who believe there is an observable truth independent of what humans make it out to be (Cassell et al., 2017). **From this perspective**, there are **multiple** reality-constructions, **each emerging from** social and cultural experience. It is in stark opposition with the realist ontology of positivism which asserts that one and only one objective reality can be perceived and measured. In this study, the subjectivist ontology **helps to reveal** the multiple realities that participants—healthcare professionals, policymakers and Abu Dhabi community members—perceive of social media communication in different ways. For example, a healthcare worker may see social media as part of professional duty around health promotion, while an individual in the community might experience it as a place to validate fears or give vent to anger, doubt or counter-narratives. Acknowledging these multiple realities is important to understanding the complexity of social media's role in public health communication.

The interpretive nature of the thematic analysis is also based on subjectivism, **where** the researcher's personal **perceptual** interface qualifies in its own right as a valid enterprise of analytical activity. Rather than aspiring to detached neutrality, the researcher should recognise that they cannot separate themselves from their positionality, background and assumptions in how data are perceived (Poth 2023). This meta-cognition fits **logically within the reflexive** orientation of interpretivism and guarantees integrity as **an** interpreter.

The research utilises an inductive approach, reflecting the interpretivist and constructionist view. Inductive reasoning "is derived from detailed observations of phenomena and works towards broader generalizations and theories" (Robson & McCartan, 2016). It enables themes and patterns to flow naturally from the data rather than being driven by pre-determined hypotheses, which is particularly suitable to underdeveloped or new areas of interest, such as the impact of social media on healthcare communication in the UAE.

The use of inductive reasoning aligns with the interpretivist tenet that theory should emerge from lived experiences, rather than precede them (Creswell & Creswell, 2018). In other words, this means that the themes and constructs developed in this study are based on what participants are actually saying rather than being overlaid on their responses from theory. This bottom-up reasoning not only results in **contextually**

grounded insights, but is also in line with the experiential heart of the study (description vs. prediction). The inductive nature of the process means it is more flexible and adaptable, thus allowing **the** researcher to narrow down the focus of inquiry as emerging themes suggest themselves from the data.

The interpretivist and constructionist approaches of this research coincide methodologically in the thematic analysis. This analytical method includes tracking, analyzing and interpreting patterns of meaning in qualitative data. It is not a purely descriptive process, but an interpretive one—it goes beyond surface-level patterns to deduce underlying meanings, relationships and implications (Braun & Clarke, 2006; Saldana, 2021). Thematic analysis as used in this paper consists of six iterative stages: becoming familiar with the data; coding; generating themes; reviewing themes; defining and naming themes; and writing up. Each stage represents a dialogue between the researcher and the data, consistent with the interpretivist view that meaning is formed through interaction rather than discovery. The researcher's theoretical sensitivity based on interpretivism and social constructionism informs the analysis, offering insights into how participants generate meanings about trust, credibility **and** engagement in social media. The interpretation is therefore an ongoing process and not necessarily a final or absolute truth (Kara et al., 2021).

In interpretivist and constructionist paradigms, the researcher is not an impartial observer but works with participants to produce meaning. Reflexivity—critical self-reflection on one's role, assumptions, and impact—is therefore key (Mertens, 2023). Reflexivity is achieved throughout this study via journaling, memo-writing and iterative data engagement. The researcher's position within their own cultural context of Abu Dhabi's healthcare and social environment is also very important, as the meaning of trust, the nature of authority, **and** communication can vary across cultures. The inclusion and reporting of such reflexive insights contributes to the credibility and transparency of this study. Reflexivity means that the results are not made 'wholesale', but are **contextually situated interpretations** of social processes. This reflexive position adds to the credibility of the interpretive research, for it supports the interpretivist concern with authenticity and depth (Poth, 2023).

The interpretivist paradigm based on social constructionist epistemology and subjectivist ontology offers the philosophical consistency needed for this qualitative

study. It is therefore consistent with the application of semi-structured interviews and thematic analysis and provides a rich examination of how people in Abu Dhabi make sense of social media in public health communication. The interpretivist orientation to exploring lived experiences and the constructionist focus on knowledge as socially produced **ensures** that the investigation represents, in a nuanced and context-specific manner, what people know, understand and believe based on their experience. The inductive logic underpinning this study encourages theory to be derived from empirical observation, allowing for insights that have resonance with the local context of digital health communication activities.

In this philosophical alignment, the research produced will be methodologically and epistemologically sound. It situates the researcher not as a neutral observer but rather an active interpreter; one whose job it is to promote understanding rather than to present objective truths. In this regard, the study adds to the corpus of interpretive social research that values meaning, context and reflexivity over prediction and control. This paradigm does not just accept but **recognises and appreciates** human diversity — a crucial stance for understanding the ways social media interactions are re-configuring health communication, trust and engagement within Abu Dhabi's culturally diverse public health environment.

3.3 RESEARCH APPROACH

The deductive approach aligns more closely with positivism, since it aims to prove or disprove hypotheses in an objective and generalisable way. Whilst this approach is useful for research focused on testing specific theories or identifying causal relationships under controlled conditions, it is not as effective in studies that seek to understand deep, context-dependent meanings. Such contexts are better suited to qualitative data and a flexible research design (Robson and McCartan, 2016). In terms of inductive research, sample size is a key consideration, and qualitative studies typically involve small samples. The aim of such research is to gain a rich, contextual understanding of the phenomenon under investigation. The process often involves extensive engagement with the research context and participants, allowing the researcher to grasp the complexities of human behaviour and social settings. The

outcomes of inductive research are typically new theories or conceptual frameworks that provide fresh perspectives on the studied phenomena (Poth, 2023).

For contrast, deductive inquiry involves larger sample sizes that permit quantitative analysis and generalisable results across contexts. The researcher's role is often quite detached, and focused on collecting and analysing data to test pre-established hypotheses without incorporating subjective reflections (Walliman, 2021). The inductive method, in contrast, aligns with social constructionism and is better suited to studies aiming to explore new areas or address complex social issues. Such an approach values the construction of reality as experienced by individuals and **supports** the development of theories grounded in participants' data. The inductive approach is thus particularly appropriate for exploratory research that contributes new insights to academic understanding.

The **use** of inductive research in this study **is based on** its aligned nature with interpretivist and social constructionist paradigms, **which together shape** the overall philosophical standpoint of the research. Inductive reasoning **involves moving** from specific observations to **develop** broader generalisations or theoretical views, thereby allowing investigators **to** construct meaning out of lived reality **rather than** testing or confirming preconceived hypotheses (Creswell & Creswell, 2018). This method is particularly suitable for qualitative research aiming at the investigation of complex social phenomena since it **allows the researcher to be** flexible, **exploratory**, and **sensitive** to context. Since the current paper focuses on exploring how HCPs and laypersons in Abu Dhabi perceive and experience social media as a tool for health communication, an inductive approach is particularly well suited for exposing the subtle, multilayered meanings contained within participants' narratives.

The inductive process starts from empirical data gathering – usually through interviews, focus groups or observations; it then goes on to establish patterns, and categories of the phenomenon emerging from the data itself. Rather than beginning with a preconceived theory or body of hypotheses, inductive inquiry means building theory from the ground up, based on descriptions participants give of their worlds (Saldana, 2021). This bottom-up process is consistent with the interpretivist goal of comprehending the subjective meanings people assign **to** their actions and experiences. This study is based on participants' understandings of trust, credibility

and engagement with public health information shared **on** social media, **shaped by** their cultural, institutional and social situatedness. The methodological orientation of inductiveness allows for **multiple** ways of thinking and representing **how these factors** are linked conceptually in a process that is both culturally grounded and theoretically **insightful**.

An inductive orientation is especially appropriate for under-researched or context-bound research, such as digital public health communication within Abu Dhabi. The literature on social media and health communication is dominated by studies conducted in the West, reflecting sociocultural, institutional and communicative contexts that are not necessarily applicable to the Gulf. The study uses an inductive approach in order not to project Western theories **onto** the data, and to let local experiences give birth to knowledge. Such attentiveness to the role of context, it is suggested, not only helps create evidence **that is** more authentic, but also contributes to knowledge production that can accommodate some of the social and cultural nuances of Abu Dhabi's healthcare system.

In addition, an inductive approach is appropriate as social constructionism views knowledge and meaning as constructed through interaction and engagement (Bryman, 2021). In this study, the practice of conducting semi-structured interviews opens up a discursive space through which participants actively co-produce meanings related to their interaction with social media and public health communications. The interactionist approach by the researcher **allows** meanings to be dynamic, as it is through engaging in dialogue **that** findings emerge. It **ensures** that these co-constructed meanings **are not restricted by** pre-specified factors or theoretical thresholds. On the contrary, they are "found" and "made sense of" through an ongoing dialogue with **the** data themselves that approximates sociality as to how new ideas emerge in collective understanding.

The inductive attitude is also helpful when it comes to studying phenomena about which there is no prior theory, or for which existing theories do not cover recent developments well. The junction of social media, public confidence and health communication is a rapidly developing area that is dominated by ongoing technological and social **change**. **As one researcher noted**, "In the absence of positive cases, however, I used inductive **reasoning** to remain open to new patterns or ideas that do

not necessarily transpire within existing theory" (p. 9–10). For instance, the rise of new types of digital engagement (eg micro-influencers, interactive health campaigns and algorithm-driven content exposure) demands an agile analytic approach that can acclimatise to new observations rather than one with an intention to validate legacy constructs. From this inductive process, the researcher can uncover emerging patterns and conceptual relations which may more accurately portray contemporary digital public health communication.

Another important reason for preferring **an** inductive approach is that it can handle complexity. **Social media communication is** a multidimensional concept, expanding across social, emotional, and technical aspects. Without losing sight of their complex interactions, we cannot simply reduce **it** to **a set of** easily quantifiable variables. Inductive investigation **accommodates** this complexity by going in depth, not in breadth. The small purposive sample facilitates the in-depth investigation of participants' experiences and means that analysis reflects both diversity and depth of opinion. The method also allows the researcher to 'pull out' unanticipated themes—**themes** generated by the data, rather than imposed on them by a priori codes. These nascent insights are important as building blocks for new theories, especially in **areas involving** perception, emotion and interaction **where** humanity is frequently at the heart of the matter (Robson & McCartan, 2016).

Further, the inductive process serves to remind us of the need for reflexivity within qualitative research. As the researcher's interpretations are actively involved in constructing findings, this is promoted within the inductive approach, encouraging questioning and reflection around researcher assumptions and positionality that impact meaning-making (Kara et al., 2021). Such reflexivity serves to increase the transparency and trustworthiness of the research **within the** interpretivist focus on co-constructed knowledge. Inductive researchers do not search for objectivity as **in** a positivist model; because what they are looking for is authenticity, which **means** showing honestly and integrity in the way in which meanings and patterns actually arise from data, through **dialogue** between **researcher** and participants.

The results of inductive research are usually developed into a conceptual framework or **theoretical** model to explain the phenomenon. These models are empirically based, and they provide a solid platform for research and future practice. Following

the inductive process, four major themes regarding trust, engagement, credibility of communication and dynamics of interaction within the Abu Dhabi public health care system emerged. The themes themselves were not based on existing models but **were** generated from the reality of participants' lives. The model emerging from this synthesis, developed in Chapter 5, **integrates** these insights with existing theories of behaviour change such as the Health Belief Model and Social Influence Theory. This integrative review provides evidence of how inductive reasoning is used to develop theory that patients experience as true.

Furthermore, the inductive approach is consistent with the research objective of exploring dynamics in public healthcare–community interaction via social media. The study is exploratory rather than confirmatory – it aims to reveal how processes of communication work in practice, not to verify a set of existing claims. This exploratory property calls for a methodology that focuses on open investigation, iterative analysis and the ability to react to emerging insights. Unlike the strictures of deductive logic, inductive reasoning is flexible and enables the researcher to change questions, analytical categories and interpretations as new data and understandings become available. This flexibility is necessary for representing the dynamic and reciprocal nature of social media communication, in which interpretations and actions are negotiated on an ongoing basis.

Practically, the inductive approach also increases the credibility of findings for policy and practice. Through legitimating theory in lived experience, the research generates knowledge that is inherently action-oriented for use in public health communication. Patterns and themes discovered through inductive analysis of these strategies can be used by health communicators and policymakers to improve more effective, culturally sensitive, interactive social media campaigns. This induction-driven bottom-up approach serves to make such suggestions concrete and not mere abstract musings in terms of the lives and views of the subjects and their position.

Third, the inductive procedure contributes to internal consistency between the philosophy, methodology and analysis of this study. This interpretivist orientation is reflected **in** the research **through its emphasis on** understanding over measurement, context over control, and depth over generalisation. And its linkage with social constructionism supports the idea of meaning as co-created, and knowledge as

dialogical. These philosophical and methodological symmetries, combined, justified the use of an inductive approach as **the** author's most adequate option for approaching **the** research objectives. The reason why an inductive approach is considered here is its potential to explore (to be visited), rather than **to** test; to interpret (evaluation with no regression following mathematical models); as well as **to** build-up, not impose. It enables **the researcher** to uncover rich, context-embedded insights on the influence of **social media** in communication and engagement within **the Abu Dhabi** healthcare context. To allow theory to come up from participants' experiences, the inductive process also ensures that the data **remain** adequate to reality within its social scope. This is a level of understanding that is firmly rooted in empirical data and philosophical consistency and presents us with an excellent groundwork for the development of new theoretical insights and practical recommendations for digital health communication.

3.4 RESEARCH DESIGN AND METHODS

The deductive approach aligns more closely with positivism, since it aims to prove or disprove hypotheses in an objective and generalisable way. Whilst this approach is useful for research focused on testing specific theories or identifying causal relationships under controlled conditions, it is not as effective in studies that seek to understand deep, context-dependent meanings. Such contexts are better suited to qualitative data and a flexible research design (Robson and McCartan, 2016). In terms of inductive research, sample size is a key consideration, and qualitative studies typically involve small samples. The aim of such research is to gain a rich, contextual understanding of the phenomenon under investigation. The process often involves **sustained** engagement with the research context and participants, allowing the researcher to grasp the complexities of human behaviour and social settings. The outcomes of inductive research are typically new theories or conceptual frameworks that provide fresh perspectives on the studied phenomena (Poth, 2023).

For contrast, deductive inquiry involves larger sample sizes that permit quantitative analysis and generalisable results across contexts. The researcher's role is often quite detached, and focused on collecting and analysing data to test pre-established hypotheses without incorporating subjective reflections (Walliman, 2021). The

inductive method, in contrast, aligns with social constructionism and is better suited to studies aiming to explore new areas or address complex social issues. Such an approach values the construction of reality as experienced by individuals and supports the development of theories grounded in participants' data. The inductive approach is thus particularly appropriate for exploratory research that contributes new insights to academic understanding.

The induction of research in this study finds its basis in its aligned nature with interpretivist and social constructionist paradigms, **both of which serve** to dictate the overall philosophical standpoint of the research. Inductive reasoning is by its nature to go from specific observations to broader generalisations or theoretical views, thereby allowing investigators to construct meaning out of lived reality in place of testing or confirming preconceived hypotheses (Creswell and Creswell, 2018). This method is particularly suitable for qualitative research investigating complex social phenomena since it permits **flexibility**, depth and attention to context. Since the current paper focuses on exploring how HCPs and laypersons in Abu Dhabi perceive and experience social media as a tool for health communication, an inductive approach is particularly well suited to exposing the subtle, multilayered meanings contained within participants' narratives.

The inductive process starts from empirical data gathering—usually through interviews, focus groups or observations; it then goes on to establish patterns and categories of the phenomenon emerging from the data itself. Rather than beginning with a preconceived theory or body of hypotheses, inductive inquiry means building theory from the ground up, based on descriptions participants give of their worlds (Saldana, 2021). This bottom-up process is consistent with the interpretivist goal of comprehending the subjective meanings people assign to their actions and experiences. This study is based on participants' understandings of trust, credibility and engagement with public health information shared on social media, shaped by their cultural, institutional and social **contexts**. The methodological orientation of inductiveness allows for these various ways of thinking and representing the ways these are linked conceptually in a process grounded both **culturally and contextually**, but also theoretically penetrating.

An inductive orientation is especially appropriate for under-researched or context-bound research, such as digital public health communication within Abu Dhabi. The literature on social media and health communication is dominated by studies conducted in the West, reflecting sociocultural, institutional and communicative contexts that are not necessarily applicable to the Gulf. The study uses an inductive approach in order not to project Western theories onto the data, and to let local experiences give birth to knowledge. Such attentiveness to context not only helps create evidence that is more authentic, but also contributes to knowledge production that can accommodate the social and cultural nuances of Abu Dhabi's healthcare system.

In addition, an inductive approach is appropriate as social constructionism views knowledge and meaning as constructed through interaction and engagement (Bryman, 2021). In this study, the practice of conducting semi-structured interviews opens up a discursive space through which participants actively co-produce meanings related to their interaction with social media and public health communications. The **interactive** approach taken by the researcher ensures meanings become dynamic, emerging through dialogue rather than being pre-determined. It allows these co-constructed meanings to develop freely rather than being limited by pre-specified factors or theoretical thresholds. Instead, they are “found” and “made sense of” through ongoing dialogue with the data in a process that mirrors how new ideas emerge within collective understanding.

The inductive attitude is also helpful when studying phenomena for which there is no prior theory or where existing theories do not capture recent developments. The junction of social media, public confidence and health communication is a rapidly developing area shaped by ongoing technological and social change. **For example**, the rise of new forms of digital engagement (e.g. micro-influencers, interactive health campaigns and algorithm-driven content exposure) demands an agile analytic approach that can adapt to new observations rather than being constrained by legacy constructs. Through the inductive process, the researcher can uncover emerging patterns and conceptual relationships that more accurately reflect contemporary digital public health communication.

Another important reason for preferring an inductive approach is that it can handle complexity. Social media communication is a multidimensional concept, spanning social, emotional and technical dimensions. Without losing sight of their complex interactions, such phenomena cannot simply be reduced to quantifiable variables. Inductive investigation accommodates this complexity by proceeding in depth rather than breadth. The small purposive sample facilitates the in-depth investigation of participants' experiences, ensuring that the analysis reflects both diversity and depth of opinion. The method also allows the researcher to identify unanticipated themes—those generated by the data rather than imposed through predefined codes. These emerging insights are valuable building blocks for new theories, particularly in domains such as perception, emotion and interaction, where human experience is central (Robson and McCartan, 2016).

Further, the inductive process reinforces the need for reflexivity within qualitative research. As the researcher's interpretations are actively involved in constructing findings, the inductive approach encourages continual questioning and reflection around assumptions and positionality that influence meaning-making (Kara et al., 2021). Such reflexivity enhances the transparency and trustworthiness of the research, supporting the interpretivist focus on co-constructed knowledge. Inductive researchers do not seek objectivity in the positivist sense; rather, they pursue authenticity, **demonstrating** honestly and integrity the way meanings and patterns arise from data through dialogue between researcher and participants. The results of inductive research are usually developed into a conceptual framework or theoretical model to explain the phenomenon. These models are empirically based and provide a solid foundation for future research and practice. Following the inductive process, four major themes regarding trust, engagement, credibility of communication and dynamics of interaction within the Abu Dhabi public health system emerged. These themes were not derived from existing models but were generated from participants' lived realities. The model emerging from this synthesis, developed in Chapter 5, juxtaposes these insights with existing theories of behaviour change such as the Health Belief Model and Social Influence Theory. This integrative review demonstrates how inductive reasoning is used to develop theory that participants experience as true.

3.5 PILOT STUDY

Researchers are required to decide if they will undertake a **pilot** of their study as part of their methodological design prior to commencing qualitative field-based work. There is a precedent for this—a pilot study, which may consist of preliminary interviews, focus groups, or instrument testing, used as "a small test run for refining instruments for collecting data and checking if interview questions are clear and relevant without the final participants" (Creswell and Creswell, 2018). It offers an opportunity to evaluate the suitability of research instruments and identify possible procedural or ethical issues before starting data collection on a larger scale. **Pilot interviews** in an interpretivist study might also provide insights into the potential for the approach to support the generation of deep, contextually embedded narratives. Yet, the choice to proceed with a pilot should depend on the intended purposes, scale, and methodological preparedness of the study.

There was no formal pilot interview phase in the current study. Rather, the instrument of inquiry—the semi-structured interview guide—was subjected to a **rigorous** review process by experts in order to establish its academic soundness and contextual appropriateness. This was a conscious decision, made because the researcher had **extensive** experience in qualitative interviews and thematic analysis; it also reflected feedback from academic supervisors and expert professionals in health communication and social media research. The priority was therefore to validate the interview guide through expert review, as opposed to testing it through a pilot study with participants. This is consistent with advice from qualitative methodologists who suggest that pilot studies are not always required when instruments have been informed and constructed using previous research evidence as well as consultation with experts (Robson and McCartan, 2016; Hennink, Hutter, and Bailey, 2020).

A pilot study is typically used to determine whether data collection methods are feasible, valid, and sensitive to participants' contexts. Piloting functions as a mechanism to check whether interview questions elicit the intended information, whether participants understand the linguistic terminology being used, and whether the length and structure of interviews is feasible (Walliman, 2022). In interpretivist and social constructionist studies, pilot work may also allow the relational effectiveness of

the interviewer and participant to be assessed, enabling the researcher to modify interpersonal or reflexive **skills**.

However, the context of this research—on professional and public perceptions of social media communication in an Abu Dhabi public health system—required high levels of methodological clarification from the outset. The researcher was also a professional who was already familiar with the subject and had previously conducted interviews in comparable projects, offering an experiential foundation **similar** to a pilot study. This background enabled confident progression to the full data collection phase without the need for a distinct pilot, thus avoiding duplication while preserving methodological strength.

The interview guide was developed through a multi-stage refinement process similar to the evaluative goals of a pilot study. The first draft was informed by established qualitative interview formats and aligned with the theoretical frameworks underpinning this research, including the Health Belief Model (HBM) and Social Influence Theory (SIT). These frameworks served as structured yet flexible tools to develop questions about beliefs concerning disease susceptibility, trust in health authorities and media, perceived interest and knowledge for participation, and anticipated interaction with others while engaging with digital health communication. The guide was then reviewed by two academic advisors and three experts: one public health communication specialist, one digital media strategy specialist, and one healthcare policy expert. Together, this feedback shaped the structure, language, and sequencing of questions to ensure their relevance, clarity, and ethical appropriateness.

There are several explanations for the decision not to conduct a formal pilot interview phase. The first is that the researcher had significant experience conducting semi-structured interviews and using thematic analysis in interpretivist paradigms. As an academic and practitioner trained in qualitative methodologies, the researcher had previously carried out in-depth interviews with similar participant groups (healthcare providers, educators, and community members). With this professional experience, the researcher had the confidence to conduct interviews, establish rapport, and manage sensitive issues without pre-testing.

Creswell and Poth (2018) assert that whether a pilot is required depends partly on the researcher's experience in the data collection method and topic area. If the researcher is already highly experienced, a formal pilot may offer few new insights and simply prolong the research timeline. The combination of practical and theoretical knowledge, along with grounding in interpretivism and social constructionism, ensured the **credibility** and contextual validity of the data-gathering approach. A pilot study was therefore replaced by the systematic validation of the interview guide through expert review. The guide was reviewed by three independent reviewers who assessed it for conceptual clarity, question relevance, linguistic appropriateness, and ethical sensitivity. Their comments helped improve the interview schedule in both content and structure.

For example, reviewers advised the researcher to make some of the technical language from the field of social media analytics more relatable for community participants while maintaining academic precision for professionals. They also proposed revising the order of some questions to create a more logical flow, moving from experiential and practice-based questions to those exploring deeper conceptual and theoretical issues. This iterative process ensured that the final guide was both clear and comprehensive, as is typically achieved through pilot testing (Hennink et al., 2020).

It also confirmed that the questions were consistent with the main research objectives and theoretical concepts. Targeted prompts addressing perceived susceptibility, communication trust, and social norms mapped directly onto the constructs of HBM and SIT. By examining these alignments through academic and professional scrutiny, the study achieved conceptual coherence comparable to what is often generated through pilot testing, though without involving live participants. It would not have been ethical to conduct a pilot study by recruiting respondents with characteristics similar to those needed for the main study. With a relatively small and specific sample—especially healthcare professionals—it was important to retain potential participants for the main phase. Conducting a pilot would have reduced the available participant pool and risked diminishing the richness and representativeness of the final dataset.

In addition, as Denscombe (2021) notes, pilot interviews in small-scale qualitative studies can be problematic if they overburden participants or require repeated

interactions with the same individuals. This concern is heightened in professional contexts where research informants are senior, time-limited, or otherwise difficult to access. Avoiding such repetition was therefore both an ethical and methodological consideration. There were also practical considerations. Given the researcher's professional experience with interviews and the clarity achieved through expert review, the added value of a formal pilot would have been limited in comparison to the additional time and resource demands. However, **incremental** adjustments were made inductively during the initial interviews of the main study—a standard and well-established strategy in qualitative research (Robson and McCartan, 2016). Throughout these early interviews, the researcher focused on flow, comprehensibility, and participant comfort, and noted minor adjustments to wording or sequence when necessary.

A second reason for not conducting a pilot interview is that participants were familiar with the linguistic and contextual foundations of the study. All interviewees (professionals and community members) spoke English, which is widely used in higher education and professional contexts in Abu Dhabi. This minimised the risk of misinterpretation, which is often a justification for piloting in multilingual studies. Additionally, the researcher was culturally sensitive to the health and social context of Abu Dhabi, reducing the likelihood of challenges relating to local idioms, references, or communication norms. Moreover, Walliman (2021) and Hennink et al. (2020) highlight that piloting is particularly important when the researcher lacks familiarity with the context, population, or topic. In this study, the researcher's in-depth knowledge of both the cultural and organisational context minimised the risk of misunderstanding or misalignment between questions and participant experiences.

3.6 DATA COLLECTION

Flexibility is a component of methodological design in interpretivist qualitative research. Interviews are viewed as interactive encounters in which questions evolve in response to participants' narratives rather than remaining fixed or standardised. Therefore, a formal pilot is not always required, as the qualitative interview process itself functions as a platform for refinement. Preliminary interviews in this study served as opportunities for the researcher to reflect on question clarity, flow, and engagement,

ensuring that consistent themes were covered with all participants. The interpretivist perspective prioritises meaning-making and responsiveness over procedural rigidity. The researcher's real-time assessment of what required attention created a form of methodological openness that allowed emergent insights to shape subsequent interviews. As such, a separate pilot phase was not necessary, as the process of iterative improvement was embedded within the research design itself.

Framed by social constructionism, relativism, subjectivism, and inductive research, qualitative inquiry draws on a range of approaches including structuralism, humanism, and content analysis. Methods such as in-depth interviews, participant observation, focus groups, and document analysis enable researchers to explore the contextual and social meanings of human behaviour and societal trends—an essential component given the research aims and objectives.

- In-depth interviews (IDIs) are a useful tool in qualitative research, allowing researchers to engage in conversation with participants or informants to understand their perspectives on a specific topic. The method is compatible with social constructionism and subjectivism since it focuses on the individual's process of meaning-making. IDIs are particularly well suited to examining how social media facilitates dialogue and community engagement in public health. They are useful **for** exploring the beliefs, attitudes, and perceptions that shape such interactions (Breakwell, Wright, and Barnett, 2020).
- Participant observation **involves** researchers immersing themselves in social settings to observe and interact with participants as they naturally engage in their environment. This method is supported by social constructionism, which holds that reality is produced through social interaction and shared meaning (Robson and McCartan, 2016). Participant observation is essential for studying community interaction through social media, since it allows for the direct observation of lived behaviour and participation in public health campaigns (Denscombe, 2021).
- Focus groups are another key qualitative method. These involve group discussions that elicit a range of views on products, services, concepts,

or ideas. Group interaction generates data and reveals varied perspectives across a defined social spectrum. This method is grounded in a relativist belief that multiple realities exist, each shaped by the group dynamic and context of interaction.

- Content or document analysis refers to the systematic summarising and interpretation of visual, textual, or aural materials. These may include public health communications, social media posts, user comments, or other relevant artefacts. Emphasis is placed on identifying patterns and interpreting meaning within the content. This method aligns with inductive reasoning, as it allows themes and patterns to emerge naturally from the data without relying on pre-defined categories or theoretical frameworks. This makes the approach highly appropriate for evaluating how social media functions as a tool for public health communication and understanding its impact on community perceptions.

Each of the above approaches **contributes** to the current objective of evaluating how social media is used for communication within the public health system of Abu Dhabi. The various approaches provide tools for researchers to account for the multivariate and dynamic nature of social interactions, and this is essential for understanding and improving community engagement, assessing the effectiveness of public health communication strategies, and ultimately enhancing public health outcomes. By employing these qualitative methods, the study is well positioned to explore, examine, and appraise the use of social media in public health contexts.

Semi-structured interviews are widely used qualitative **methods** which involve the researcher preparing a set of guiding questions in advance, along with introductory and closing statements. The researcher, however, remains open to new themes and questions that may emerge during interviews. Such flexibility **allows** for an organic conversation, and can result in more detailed, comprehensive data. The method is particularly effective in eliciting personal experiences, reflections, and perspectives from participants, and this makes it ideal for exploratory research **that** develops new insights.

The combination of semi-structured interviews with interpretivism and qualitative research stems from their shared aim of understanding the complexity of human experience from the perspective of participants. Interpretivism holds that reality is subjective and can only be known through the meanings that individuals attach to their experiences (Robson and McCartan, 2016). In this context, semi-structured interviews offer participants the opportunity to articulate their perceptions and experiences in their own words. This is an essential feature for interpretive analysis. The detailed, context-specific use of language in such interviews is critical for uncovering deeper insights into participants' personal meanings and understanding their responses within the relevant social context. As is the case with all qualitative research, values and meanings are not only considered but actively interpreted.

Semi-structured interviews are compatible with exploratory research designs which might be used to investigate new phenomena. Such designs rely on open-ended questioning to uncover new themes and perspectives (Creswell and Creswell, 2018). Rather than proceeding **with** a predetermined hypothesis, exploratory research instead seeks to develop a comprehensive understanding of a phenomenon and then generate a broader range of interpretations. The semi-structured format allows interviewers to adjust questions in response to earlier answers, facilitating a more natural flow that can lead the research in unexpected but meaningful directions. This flexibility enhances the depth and richness of the data collected and contributes to the development of nuanced and well-grounded insights.

Moreover, inductive methods in qualitative research focus on developing theories from observations, patterns, or themes that emerge directly from the data. Semi-structured interviews are thus compatible with this approach, as they generate rich, detailed data that supports inductive inquiry. This allows themes and theoretical insights to emerge more naturally from participants' responses (Saldana, 2021). Indeed, open-ended questions can encourage participants to share accounts of their experiences and perspectives, thereby producing robust datasets from which patterns can be identified.

Subjectivism also aligns closely with the use of semi-structured interviews. Based on such a method, the inherently subjective nature of data is routinely acknowledged and accepted. Given that knowledge about what is, what is not, and what could be is socially constructed and interpreted through human experience, validity naturally

becomes subjective (Bryman, 2021). Semi-structured interviews make space for the exploration of these individual understandings without imposing a rigid structure or predetermined questions. Such flexibility facilitates a data collection process that is open to discovery rather than constrained by expectation.

Semi-structured interviews are therefore ideal methods for qualitative researchers to use when conducting interpretivist, exploratory studies. They are **particularly** sensitive to the subjective dimensions of human experience, and they align with inductive theory building. They offer the versatility needed to thoroughly investigate complex social phenomena. In the context of the current study, the method is particularly valuable since it permits a detailed understanding of how social media may be embedded in local discourse and daily practice. This can generate insights that are deeply contextual and culturally grounded. Non-probability purposive sampling is a widely used approach in qualitative research. It **facilitates** the intentional selection of participants who have relevant experience and knowledge of a specific phenomenon with the aim of meeting defined research objectives. This approach is particularly appropriate for in-depth explorations of how individuals, groups, and issues present themselves within a specific context.

3.7 SAMPLING

The choice of purposive sampling in the present study aligns with an interpretivist perspective, which emphasises meaning-making as constructed through individual and social experience (Robson and McCartan, 2016). Although social media holds significant potential for enhancing community engagement, limited research exists on how it is used by Abu Dhabi's diverse population to support public health and individual empowerment. By adopting purposive sampling, the study seeks to recruit participants who are most likely to offer rich, relevant, and varied insights into the role of social media in promoting engagement within Abu Dhabi's distinctive socio-cultural and public health context.

Purposive sampling empowers researchers to focus on specific characteristics within a population, and these are often directly relevant to the research questions. In this case, the study targets active social media users involved in the public health sector, such as healthcare professionals, patients, and public health managers. This ensures

that the data collected speak directly to the study's aims. The approach is particularly valuable for exploring how various stakeholders use social media to communicate about health issues (Creswell and Creswell, 2018). Further, purposive sampling is appropriate for examining the cultural and social dimensions that shape the use of social media in public health communication. In a region as culturally diverse as Abu Dhabi, this enables the researcher to approach participants who are qualified to speak to how cultural values, social norms, and religious practices influence social media engagement and health messaging. In purposive sampling, the researcher selects participants based on predetermined inclusion criteria that align with the study's objectives. These criteria may include demographic variables (e.g. age and gender), roles within the health system (e.g. healthcare professionals or policymakers), or specific experiences (e.g. engagement with health-related social media).

Recruitment strategy: Participants will be recruited using a combination of purposive sampling and snowball sampling techniques, drawing from social media platforms, professional networks, and public health authorities in Abu Dhabi. The aim is to capture a diverse range of perspectives and experiences, thereby enhancing the richness and variation of the data sources. The connection between the sampling purpose and study design is also key. Since the study is investigating an under-researched phenomenon, purposive sampling provides the flexibility and depth needed to explore such complexity. This method helps with the identification of emergent themes and patterns in health-related social media use. It therefore contributes to a deeper understanding of communication processes (Saldana, 2021). Purposive sampling addresses practical considerations in conducting qualitative research within a defined context. It also aligns closely with the methodological and theoretical frameworks of the study, and supports the targeted application of theoretical concepts. This ensures that the data collected are relevant, which is important for informing the development of practical strategies aimed at improving public health communication in Abu Dhabi.

3.8 DATA ANALYSIS

The research followed the systematic thematic analysis procedure outlined by Naeem et al. (2023), which employs a structured six-phase method for qualitative data

analysis. This approach enables a rigorous and fine-grained exploration of the data, leading to meaningful insights and the development of a rich conceptual model.

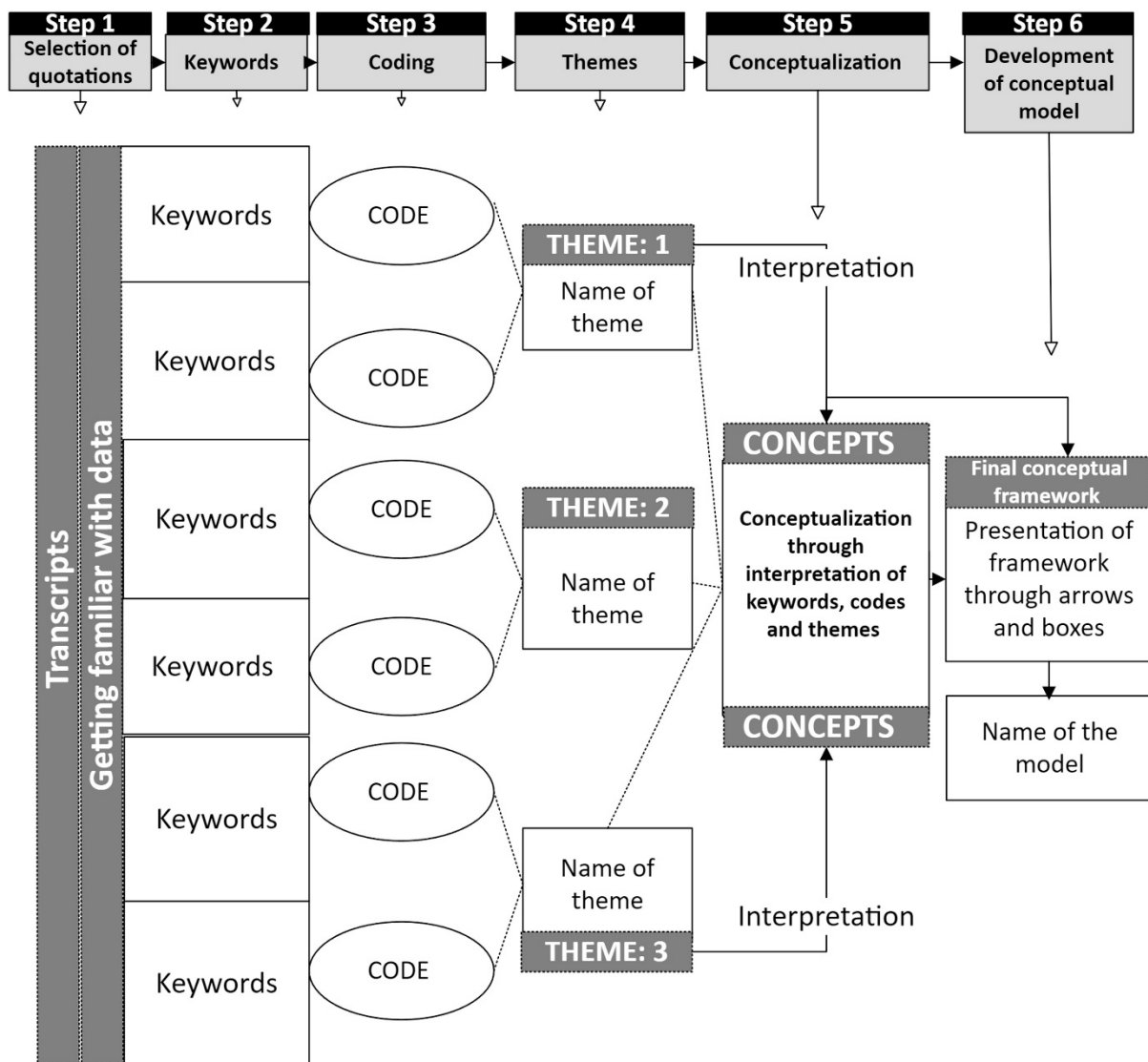


Figure 3-1 A systematic thematic analysis process: A novel six-step process for conceptual model development in qualitative research.

These six steps that constituted the analytic process can here be presented as a series of interweaving phases in which the keywords, codes and themes were developed, reworked and situated theoretically. Stage one: Transcription, Familiarisation and Quotation Selection Transcripts were created for all audio interview recordings to be used for analysis and digital content was extracted from Instagram (including captions, user comments and institutional responses) as well as Twitter. This approach helped to maintain naturally occurring patterns of language, tone and communication behaviour across both verbal and digital linguistic

expressions. As the researcher spent more time immersed in the transcripts, they started to notice quotes that evocatively captured transitions from conventional forms of one-way communication to dialogic and participatory digital engagement. And these quotes became the bricks from which codes like Symbiotic Exchange, Participatory Echo, Risk Intuition and Sentiment Flux were constructed because they quite simply shaped the emotional, cognitive and social dimensions tucked away under participants' stories.

The next phase (Keyword Selection) was aimed at determining which words and phrases were used with meaningful content across the database. This was done according to the 6R concept described by Naeem et al. (2023), to guarantee keywords would also represent Realness, Richness, Repetition, Rationale, Repartee and value through Regal. Themes of trust, voice, risk, response, identity, engagement and community continuously emerged from the data as potent descriptions of the communicative setting that participants were negotiating. These key terms were not analyzed in isolation, but rather as semantic anchors that connected experience of participants with constructs from Health Belief Model and Social Influence Theory. For instance, the prevalence of terms like "risk," "vulnerability," and "trust" in some point influenced the construction of Perceptual Cartography, whereas phrases such as "community," "shared identity," or "support" fed directly into the development of Communal Ethos.

In the third stage, Coding, raw text was converted into coded units of analysis. Codes were generated inductively through the manual line-by-line reading and each was labelled at its core level of communicative or behaviour meaning that had emerged from the data. The original keywords were conceptual prompts for code generation. For instance, repeated mentions of tone, warmth, emotional cadence and clarity gave rise to the code Discursive Pulse; repeating reported instrumental phrases (of having their words parroted back to them) informed Echo Governance; linguistic clues about urgency or prompts helped codify Action Catalysts. Each of the twenty codes in Table 4.3 is supported by a recognisable linguistic pattern within the transcripts or social media comments, indicating that it was not an analytical import, but was developed from lived communicative activity.

The fourth phase, Theming, then ordered the codes in more abstract conceptual categories. This was done based on the 4R classification of Naeem et al. (2023), which drives the shift from descriptive codes to more abstract yet data-derived themes. From iterative sorting, five emergent themes—Dialogic Resonance, Perceptual Cartography, Communal Ethos, Mobilised Cognition and Digital Reverberations—emerged. Each theme was constructed as a conceptual realm that portrayed what and how participants reported their experience of engaging with digital public health communication. For example, the theme Dialogic Resonance integrated codes Symbiotic Exchange, Discursive Pulse, Participatory Echo and Conversational Agency that respectively relate to interaction, emotional atmosphere, co-creation of knowledge and empowerment. Communal Ethos was generated from codes indicating cultural alignment, identity and normative expectations while Mobilised Cognition and Digital Reverberations described behavioural preparedness and institutional responsiveness respectively. At this thematic layer is where the conceptual design of framework started to have clear contours.

Stage five: Conceptualisation through Interpretation (Interpretation) The fifth stage raised the level of analysis beyond thematic framing, to that of abstract co-construction. Interpretation of the theme patterns was conducted by a researcher in the context of theoretical frameworks which guided the study. Cognates like Mobilised Cognition were associated with constructs from the Health Belief Model including perceived benefits, cues to action and self-efficacy because participants spoke repeatedly about how emotional narratives, visual prompts and social proof served as prompters for them in engaging with preventive health behaviours. These categories (eo.2;1) were linked to Social Influence Theory as codes such as Normative Adhesion, Collective Identity and Participatory Echo illustrated how norms, identity and engagement influenced behavioural synchronisation. Where the conceptual relationships between and among themes, codes and theoretical constructs became more cogent and clearer through memo-writing, thematic matrices and visual diagrams. This interpretative process guaranteed that the organization of the analysis was led by data and theoretically consistent.

Lastly, the last stage of Conceptual Model Development consolidated all previous stages into a unified framework—The Digital Reverberations in Health Communication model. This model brings together the twenty inductively derived

codes, the five thematic clusters and also the constructs of multiple health behaviour theories (Health Belief Model, and Social Influence Theory), allowing for a cyclical display of how perception, communication, action and feedback are interwoven within Abu Dhabi's digital health ecosystem. They place Dialogic Resonance in the middle of these communication process to show how two-way engagement transforms not only what institutions say but how publics react. If Perceptual Cartography and Communal Ethos support the framework horizontally as interpretive and normative domains, Mobilised Cognition stands for behavioural enactment and work in Digital Reverberations for institutional adaption. All layers taken together uncover the entire communication trajectory —exposure to message, cognitive evaluation, social effects, behavioural action and institutional development—including the recursive and dynamic structure of public health communication in digital times.

If we apply the structured analysis process described by Naeem et al. (2023), the present study is characterized by methodological rigour with traceability from raw data up to conceptual model. The methodology enabled codes such as Discursive Pulse, Symbolic Alignment and Influence Valence to be extracted from the data based on constructive account of it, while being grounded in theory informed interpretive frameworks. Linking keywords to codes, codes to themes and themes into theory provided a means of creating a theoretically coherent path underpinning the novelty and explanatory force represented by the concept model described in Chapter 5.

3.9 TRUSTWORTHINESS OF THE RESEARCH

The trustworthiness of qualitative **research** is essential for confirming the validity of the results and supporting the potential transferability of its findings. According to Walliman (2022), reliability in qualitative research refers to the consistency of the researcher's approach and the potential for the study to be replicated under similar conditions. Trustworthiness, more broadly, encompasses credibility, transferability, dependability, and confirmability. These are fundamental to ensuring the integrity and believability of qualitative findings.

Credibility is particularly key, as this is about demonstrating that the results are believable and grounded in the participants' lived experiences. This can be achieved using strategies such as prolonged engagement, persistent observation, and

triangulation. **Long-term** involvement in the research setting as well as sustained observation contribute to the collection of thick, rich data, which supports a deeper understanding of the context and minimises inconsistencies in the findings (Creswell and Creswell, 2018). Walliman (2021) and Bryman (2021), triangulation involves using multiple sources of data, researchers, theoretical perspectives, or methodological approaches to cross-check findings. Such a strategy enhances the depth and credibility of the analysis. Transferability refers to the extent to which research findings can be applied to other contexts or settings. This is a key concern in qualitative research, which often focuses on **specific** and narrowly defined contexts. Providing thick, detailed descriptions of the research setting, participants, and interaction patterns enables readers to judge for themselves the extent to which findings may be relevant elsewhere. As Saunders et al. (2023) note, such rich contextual detail is crucial for supporting meaningful transferability.

Dependability refers to the stability and consistency of the data over time. Dependability requires the creation of an audit trail, which is a comprehensive record of the research process and the decisions made throughout. This is key for enabling other researchers to trace the reasoning behind methodological choices and to understand and even replicate the study. The audit trail should include explicit documentation of data collection procedures, coding processes, and thematic development decisions (Hennink, Hutter, and Bailey, 2020). Confirmability refers to the extent to which the research findings are shaped by participants' responses rather than researcher bias, motivation, or interests. Achieving confirmability involves the use of reflexivity, whereby researchers critically reflect on their own assumptions and how these might influence the research process. The researcher's position and subjectivity should be made visible. This is an approach that is compatible with the interpretivist **paradigm** commonly associated with qualitative research (Smith, 2015).

To support confirmability, specific strategies can be implemented, such as participant validation or member checking. This occurs when participants are invited to review and verify the researcher's interpretations of their responses. Such a process ensures that the findings are accurate, credible, and aligned with participants' intentions and meanings (Coe, Waring, Hedges, and Arthur, 2017). Research techniques including qualitative in-depth interviews, focus group discussions, and direct observation

facilitate the collection of rich, detailed data. In addition to methodological reporting, such methods can establish a robust foundation for the rigour and quality of research. This argument is supported by Robson and McCartan (2016), who contend that the explicitness of the research design and process can enhance the overall reliability of a study.

Reliability and trustworthiness in qualitative research can be achieved through a comprehensive process that integrates multiple strategies to enhance credibility, transferability, dependability, and confirmability. Such elements are essential in qualitative inquiry, and researchers must be meticulous in their attention to detail, methodological transparency, and ongoing self-reflection. In line with these principles, efforts were made during the **conduct** of this study to uphold the trustworthiness and broader applicability of the findings. The study has thus generated useful and reliable insights that may be transferable to other contexts with similar characteristics.

3.10 RESEARCH ETHICS

Ethical issues are central to qualitative research and **are essential** not only for protecting the subjects but also for ensuring that the design of a study remains ethical and that the process of conducting fieldwork maintains credibility, transparency, and integrity. Because qualitative work frequently involves direct human involvement, it raises a variety of ethical issues in terms of consent, confidentiality, and the management of **sensitive** information. These issues are key in maintaining participant trust and the validity of research results.

The study was consistent with the institution's and international ethical principles of qualitative research. **Before** beginning data collection, the institutional ethics committee gave ethical approval for this study to ensure all procedures followed established ethical standards involving human subjects in research (Saunders et al., 2023). All participants received a full explanation about the objectives of the study and aspects related to their participation and withdrawal, if they so wished, without any negative impact. Following the recommendation of Walliman (2021) and Creswell & Creswell (2018), written informed consent was obtained before participation. This was to ensure that participants had a clear grasp of the aims of the study and how their information would be used.

Informed consent also included participant declaration of knowledge of the study's confidentiality and anonymity procedures. The identities and privacy of the participants were guarded. Confidentiality means keeping any personal information in a safe place and not allowing people who should not know this to get access to it (Robson & McCartan, 2016). Pseudonyms and coded identifiers were used at all stages of the analysis and reporting to ensure that there was no way for anyone to link an individual to a participant (Hennink, Hutter, & Bailey, 2020). This was especially important in Abu Dhabi, where the professional networks of individuals were relatively small and entwined, particularly within the healthcare sector.

Just as important was upholding the principle of **do no harm**. Qualitative researchers are obliged to structure their research so as not to cause psychological, emotional, or reputational harm to persons being researched. The interviews in this study covered themes such as perceptions of participants about health communication practices and their use of social media, which could entail some professional sensitivity. The researcher also ensured that the interviews were performed in a supportive atmosphere and that participants had the right to refuse to answer any question they found uncomfortable. In line with Mertens (2023), respondents were assured that their data would be handled with care and confidentiality, with responses used exclusively for academic purposes.

When working with potentially confidential information, it was also necessary to consider how results would be reported back. Walliman (2021) stresses that researchers have an ethical responsibility not only in the data collection process but also in reporting and sharing the information. For this reason, identifying information was omitted from verbatim quotations or case descriptions. Where contextual cues could lead to the discoverability of participants—particularly in small professional communities—minor contextual shifts were introduced to anonymise while maintaining analytic integrity. The ethical process also included addressing any **unexpected** aspects of data collection. As Denscombe (2021) states, qualitative fieldwork can involve unanticipated ethical dilemmas that arise during conversational interviews where participants may reveal unsought personal or professional information. To deal with such situations, the researcher adopted a reflexive stance and considered each scenario carefully in light of respect, beneficence, and justice.

Data processing and storage adhered strictly to institutional data protection guidelines. All recordings, transcripts, and consent forms were stored on confidential, password-protected systems accessible only to the researcher. Consent forms were maintained in written form and stored securely. No identifying data were associated with the analytical results. Following the advice of Cassell, Cunliffe and Grandy (2017), data will be stored only until academic verification is complete and then destroyed according to institutional policy.

Another ethical issue in the current study relates to the language of data gathering. There was no need for translation as all participants, both professionals and community members, were fluent in English. The healthcare workforce and community population in Abu Dhabi are extremely international and multilingual, with English being the language of professional communication and higher education. English interviews were chosen to facilitate clarity of expression for participants and to prevent interpretation bias. This decision was therefore ethically and methodologically sound.

Moreover, reflexivity was included as an ongoing aspect of the ethical **framework**. Reflexivity compels the researcher to be constantly aware of their own positionality, assumptions, and impact on participants (Smith, 2015). Systematic reflective notes were written to record the researcher's decision-making process, interactions, and reflections throughout the study. Ethical permission, ongoing reflexivity, and a rigorous commitment to confidentiality collectively contributed to the trustworthiness of the study. According to Saunders, Lewis, and Thornhill (2023), ethical behaviour establishes not only the researcher's credibility but also that of the findings. Maintaining these ethical stances enabled respondents to engage openly and enhanced the reliability and quality of the data. AI tools were used solely to improve sentence clarity and correct minor punctuation errors, and the final thesis was additionally proofread by a professional proofreader.

3.11 SUMMARY

This chapter has outlined the main philosophical focus, methodological stance, and diverse research strategies used to systematically uncover and make sense of data in social **research**. The chapter highlights the centrality of philosophical mindsets (ontology, epistemology, and methodology) in shaping research findings. These

orientations guide the selection of specific methods and also influence how data are interpreted and conclusions are drawn, revealing the complex interplay between theory and practice.

The chapter provides an extensive discussion of different philosophical positions to demonstrate how these underlying beliefs inform the research journey, from the questions we ask to the ways in which we collect and analyse data. This strong philosophical foundation ensures that the research is coherent, consistent, and methodologically robust. It therefore offers a solid basis for exploring the complex landscape of social research and **successfully** builds connections between theoretical considerations and their practical application. It thus shows how philosophical stances are expressed through paradigms, strategies, and methods. The discussion of ethical implications adds further depth, raising important questions about the responsibilities researchers owe to participants and to the wider scientific community.

This chapter therefore identifies the research methodology that was used for this study, and it reinforces the value of thoughtful methodological design. It sets the stage for the empirical chapters to follow. These chapters apply the theoretical perspectives explored here to a real-world research context. Ultimately, the chapter creates a bridge between theoretical foundations and practical implementation, ensuring that the research is methodologically sound and ethically grounded and that it also contributes meaningfully to the broader field of social research. The result is a study that is both rigorous and resilient and thus capable of withstanding scrutiny and informing practice.

CHAPTER 4: ANALYSIS AND FINDINGS

4.1 INTRODUCTION

Chapter 4 is a key component of the thesis, as it introduces the reader to public health communication on social media in Abu Dhabi. This chapter presents a comprehensive account of the qualitative findings derived from semi-structured interviews with public health practitioners and members of the public. This chapter does not set out a simple compilation of responses. **Rather**, it creates an analytical trajectory structured around a themed framework adapted from Naeem et al. (2023). The chapter starts out by outlining the background of the participants to articulate the richness and diversity of perspectives represented. This demographic summary is critical for understanding the themes that follow, particularly in relation to cultural, professional, and experiential variation.

The chapter is organised sequentially, progressing from data preparation and familiarisation through keywording, coding, theme development, and culminating in the construction of a conceptual model. Each stage is designed to build logically on the last, with a transparent explanation of the analytic process in line with the study's theoretical frameworks—the Health Belief Model and Social Influence Theory. Five major themes are presented—Dialogic Resonance, Perceptual Cartography, Communal Ethos, Mobilised Cognition, and Digital Reverberations—each supported by a constellation of codes and key phrases distilled from the interview transcripts and associated social media engagement. The core aim of the chapter is to translate raw qualitative data into analytically coherent themes that capture the lived experiences and digital interactions of participants, while also offering transferable insights into the adaptability and responsiveness of public health communication systems. It demonstrates how communication flows have become more bottom-up, emotionally resonant, and co-created. Ultimately, this chapter serves as a bridge between data and theoretical interpretation, setting the foundation for Chapter 5. Its structure mirrors an intelligently choreographed dynamic—from information verification to intellectual realisation.

4.2 PARTICIPANT DEMOGRAPHICS

In order to achieve a comprehensive investigation of the research purpose and questions, this study conducted semi-structured interviews with 28 participants, including 9 professional experts and 21 members of the public. The professionals were sampled based on their positions and experience in public health, communication, policy, or administration within the healthcare sector in Abu Dhabi. The public participants were recruited to represent individuals who actively engage in social media discussions related to health. The demographic characteristics of both groups are presented in the tables below. These include gender, age, nationality, level of education, and frequency of social media use, and are important for understanding how participants engaged with and experienced interactive m-health messaging, public health branding, and community responses in the Abu Dhabi health sector. Table 4-1 outlines the details of the professional participants, while Table 4-2 presents the demographic distribution of the public respondents.

The separation of healthcare workers from community members in the context of this study is informed by methodological and theoretical concerns. This study has a broad objective to understand how new media can play a role in terms of creating communication, engagement, and trust between the Abu Dhabi public healthcare system and the community. Because professionals and the public function in different though interdependent ways within this communicative system, they needed to be explored separately to provide depth, contrast, and analytic clarity.

Qualitatively, research **benefits** when first-hand differences between the stakeholders involved are examined, particularly when the studied phenomenon concerns crossing hierarchical, institutional, or societal boundaries (Creswell & Creswell, 2018; Robson & McCartan, 2016). Institutional actors who create and transmit public health communication via the web—such as administrators, policy advisors, and communication strategists in healthcare—are tasked with designing and disseminating appropriate online content. Their accounts **illuminate** the strategic aims, policy architectures, and professional constraints that affect how health information is produced, mediated, and transmitted. Contrastingly, community members form the recipients and interpreters of these communication pieces—that is,

the real-life embodiment of what these messages mean, how they are accepted, trusted, disseminated, and implemented in the day-to-day world of social media.

Division between the two sets of participants is further in line with the interpretivist and social constructionist focus of this study, which acknowledges reality as contextually constructed through social interaction and discursive negotiation (Bryman, 2021; Mertens, 2023). The study succeeds in capturing two interrelated layers of social meaning construction, namely the institutional creation of public health narratives on the one hand, and their local interpretation by the community on the other, while maintaining analytic separation between professionals and community members. This double lens enables the study to analyse not only what is communicated, but also how communication is received and adopted within professional and public fields.

The distinction further promotes thematic saturation, enabling professionals' communication strategies to be compared to the public's reception and engagement. This two-layered participant system is useful in identifying both convergences and divergences of perception, **allowing** a more nuanced conception of digital public health engagement in Abu Dhabi. For example, experts may value legitimacy, adherence, and institutional status, while community members might favour authenticity, accessibility, and peer affirmation through online activity. By examining these viewpoints, both individually and comparatively, the bidirectional communication dynamics of successful health engagement on social media can be more fully understood.

Table 4-1 Demographic Profile of Professional Participants (n = 9)

Participant ID	Gender	Age Group	Nationality	Profession/ Role	Years of Experience	Education Level	Social Media Usage Type	Frequency of Use
P1	Male	31–45	Emirati	Healthcare Administrator	12	Postgraduate	LinkedIn, Twitter	Daily
P2	Female	46–60	Emirati	Public Health Communication Expert	20	Postgraduate	Instagram, Twitter	Daily

P3	Male	60+	Non-Emirati	Health Policy Advisor	28	Postgraduate	LinkedIn	Daily
P4	Female	31–45	Non-Emirati	Social Media Strategist (Healthcare)	10	Undergraduate	Instagram, Facebook	Daily
P5	Male	46–60	Emirati	Healthcare Administrator	18	Postgraduate	Twitter	Daily
P6	Female	46–60	Non-Emirati	Public Health Communication Expert	22	Postgraduate	Twitter, Instagram	Daily
P7	Male	60+	Non-Emirati	Health Policy Advisor	25	Postgraduate	LinkedIn, Twitter	Daily
P8	Male	31–45	Emirati	Social Media Strategist (Healthcare)	8	Undergraduate	Instagram, X (Twitter)	Daily
P9	Female	46–60	Non-Emirati	Healthcare Administrator	15	Postgraduate	Facebook, Instagram	Daily

Table 4-2 Demographic Profile of Public Participants (n = 21)

Participant ID	Gender	Age Group	Nationality	Education Level	Occupation	Social Media Usage Type	Frequency of Use
C1	Male	18–30	Emirati	Undergraduate	Student	Instagram, Snapchat	Daily
C2	Female	31–45	Non-Emirati	Postgraduate	Teacher	Facebook, Twitter	Daily

C3	Male	46–60	Non-Emirati	Undergraduate	Small Business Owner	Facebook, WhatsApp	Weekly
C4	Female	18–30	Emirati	Secondary	Student	Instagram, TikTok	Daily
C5	Male	31–45	Non-Emirati	Postgraduate	Engineer	LinkedIn, Twitter	Daily
C6	Female	18–30	Non-Emirati	Undergraduate	Marketing Assistant	Instagram, Twitter	Daily
C7	Male	31–45	Emirati	Undergraduate	Civil Servant	Twitter, WhatsApp	Daily
C8	Female	46–60	Emirati	Postgraduate	Healthcare Worker	Instagram, Facebook	Weekly
C9	Male	60+	Non-Emirati	Secondary	Retired	Facebook	Weekly
C10	Female	18–30	Non-Emirati	Undergraduate	Retail Staff	TikTok, Instagram	

Separate participant categories provide clear ethical and practical guidance. It is designed to allow for members from all groups an opportunity to share their thoughts that align with the roles to which they have been assigned, free of hierarchical pressures and potentially accusations of professional bias. This distinction therefore by demonstrating both anonymity, respect and balance in qualitative research enhances the trustworthiness of findings as well their authenticity (Walliman, 2022).

There were nine professional participants, five male and four female demonstrating gender balance in both managerial and comms roles. There was a broad diversity in age, with most individuals being 46 to 60 years old. The squad consisted of local Emirati and non-Emirati, reflecting the multicultural healthcare workforce in Abu Dhabi. For everyone, regular use and daily activity was part of their normal professional responsibilities working in digital communication, strategy, and health promotion. The majority of them had postgraduate qualifications and considerable working experience

(8-28 years) in practice, which contributed to expertise and depth of reflections on communication practices in health.

Two groups of participants were included in this study, one group is the healthcare workers and the other is general population. We believed that both groups were necessary to obtain an in-depth understanding of the role of social media in public health communication in Abu Dhabi. Professionals contributed perspectives on internal institutional plans and communication strategies as well as implications in policy itself as it pertains to social media usage, interactive practices and health care audiences; by the public who provided reflection from an information receiving standpoint, participant perspective of the e-health conversation. It was both morally and methodologically defensible to divide these two groups, so that (e.g.) professionals could speak candidly about challenges they face within the organisation without being exposed to *prise en charge* (Slay, Pallaso & Yassin et al., 2017) from the public gaze: Meanwhile community participants were able express sincere perspectives on trust, engagement and credibility without feeling influenced by potential practitioners.

This participant slice is in accordance with qualitative guidelines focusing on depth rather than numbers of generalisation (Walliman, 2022). It enabled the expression of experiences for both groups in terms of their roles whilst reducing spurious categories and achieving a more authentic, well-balanced and less ambiguous interpretative focus to the thematic analysis. The separation also served an ethical purpose, in which the hierarchical issues and their thin possibility of bias or self-censorship was diminished by interviewing groups separately. This promoting some form of transparency, deference and participant comfort- heightening the reliability and validity of these findings. Although we analysed this data in concert to distil overarching themes, the individual datasets were first interrogated in isolation for group-specific understandings prior to cross-comparison.

All participants are fluent in English reflecting the University and ABU Dhabi population who function within a bilingual healthcare-board educational environment. This bypassed translation, and the distortion of the original created by cross-language interpretation. The fact that English is the language of choice for professional

communication, education and scholarship within the screencasting community helps to ensure clarity and precision of participants' responses during interviews.

4.3 THEMATIC ANALYSIS PROCESS

The initial stage of thematic analysis in this study follows the systematic model outlined by Naeem et al. (2023). This involved a meticulous transcription process, during which all audio recordings from the semi-structured interviews were transcribed verbatim. In addition, digital content extracted from Twitter and Instagram (post captions, user comments, hashtags, and institutional replies) was compiled and converted into a textual corpus to ensure analytical consistency. In total, 28 interview transcripts formed the basis of the dataset.

Following transcription, the next step was deep familiarisation. The researcher repeatedly read and reread the data to develop an intuitive understanding of **patterns, meanings, and communication changes, particularly the shift from traditional one-way messaging to more interactive digital exchanges**. This immersion enabled the identification of fragments most relevant to the research focus. Quotes were not selected on a random basis. Rather, they were chosen based on their ability to **capture meaningful intersections between institutional communication and public response**, especially where lived experience or emotional reactions were strongly expressed. In line with Naeem et al.'s biographical method, these quotes became instrumental in **shaping subsequent interpretation and preserving theoretical coherence** throughout the analysis.

This phase involved the ordering of recurring and emerging key terms that appeared with significance in both the interview transcripts and on social media. The keywords were not selected based solely on frequency but were inferred using Naeem et al.'s (2023) 6R model. Each keyword was evaluated in terms of Realness, Richness, Repetition, Rationale, Repartee, and Regal value. Keywords such as "trust," "vaccine," "dialogue," "response," "voice," "community," and "engagement" became central because they reflected **participants' core concerns around trust formation, message clarity, and perceived responsiveness of health authorities**.

Keywords such as "**resonance**," "**identity**," and "**risk**" were guided by their strong conceptual alignment with Social Influence Theory and the Health Belief Model. These

keywords functioned as **semantic anchors**, helping to maintain a clear and traceable analytic trajectory from data to theory. After identifying keywords, the next step was the coding process, which transformed raw textual data into analytically meaningful content. Using Naeem et al.'s (2023) 6Rs of coding as a quality mechanism, each code was scrutinised for **clarity, relevance, and conceptual stability**. Coding was conducted inductively through manual line-by-line reading. Each code could be traced back to a keyword or phrase.

For example, the code "Discursive Pulse" was derived from tags including "tone," "human," "language," and "urgency," and **in this revised form, it refers specifically to the emotional rhythm and communicative tone of public health messaging, and how these influence audience interpretation and trust**. Twenty codes were produced and systematically distributed across five central themes. This phase reduced large amounts of material into **coherent, logically connected analytical categories**.

Theme development involved a conceptual shift from data-driven codes to abstract yet grounded themes, consistent with Naeem et al.'s (2023) 4Rs model. Five key clusters emerged: Dialogic Resonance, Perceptual Cartography, Communal Ethos, Mobilised Cognition, and Digital Reverberations. Dialogic Resonance captured how **communication has moved away from a top-down broadcasting model—defined as a one-directional flow of information from authorities to passive audiences—towards a more interactive, two-way digital system**. This clarified the communication change the examiner requested. The codes "Symbiotic Exchange," "Discursive Pulse," and "Conversational Agency" illustrated how participants experienced **greater openness, feedback opportunities, and expressive participation**.

Perceptual Cartography referred to **how individuals mentally process and emotionally interpret online health information**, not physical map design. Participants expressed their sense-making through terms such as "emotional compass" and "it's personal," showing that **risk perception was shaped more by narrative and trust cues than by numerical data**. Trust semiotics referred to **visual indicators of institutional credibility**, such as verification marks and ministerial

hashtags. Communal Ethos described how shared values, cultural identity, and social norms shaped health communication uptake. Keywords such as “shared identity,” “cultural branding,” and “representative messaging” highlighted the importance of **cultural alignment and authenticity in Abu Dhabi’s public health communication.**

Mobilised Cognition illustrated **behavioural readiness**, showing how individuals moved from awareness to action through emotional valence, peer influence, and digital cues. Terms such as “reminder tweet,” “trigger to act,” and “visible compliance” showed how online cues shaped real-world behaviours. Digital Reverberations addressed **how institutions monitor, absorb, and respond to public feedback**, replacing any terminology that was unclear previously. Echo governance now refers to **institutional responsiveness**, sentiment flux describes **fluctuating emotional reactions**, and discursive arcs represent **evolving health narratives shaped by both the public and authorities**. Collectively, these keywords underscore the thematic breadth of the analysis and offer a richer understanding of how public health communication operates in Abu Dhabi’s digital environment. Participants’ language reflects an emerging relationship with institutional messaging that is shaped by **emotional resonance, cultural alignment, trust cues, and behavioural signals**, affirming the relevance of Social Influence Theory and the Health Belief Model.

Table 4-3 Summary of thematic analysis findings

Theme	Code (with brief definition)	Keywords
Dialogic Resonance	Symbiotic Exchange (mutual shaping of messages through real interaction)	real conversation, dismantled monologue, mutual interaction, shape our narrative, follow up, feedback loop, common space, echoed back, public suggestions, answered directly
	Discursive Pulse (tone, rhythm, and emotional	human tone, speak our language, calm urgency, sincerity matters, empathy speaks, tailored responses, not copy-pasting, rhythm

	cadence of communication)	of engagement, tone matters, clarity builds trust
	Participatory Echo (public voices amplifying and reshaping messages collectively)	voices resonate, lived experiences, ripple effect, collective change, feedback at scale, hashtags as megaphones, amplify each other, policy conversations, patient's story, shared insight
	Conversational Agency (public empowerment to question, contribute, and influence discourse)	ask and challenge, safe space, empowerment, health advocates, expressive interaction, acknowledged voice, digital discourse, active participants, redistributed power, communicative agency
Perceptual Cartography	Risk Intuition (instinctive emotional perception of personal health threat)	emotional compass, feel the risk, personal threat, local cases alert, changed perception, gut warning, social media warnings, relatable symptoms, nurse's post, online danger
	Trust Semiotics (visual and symbolic cues that signal credibility)	blue tick, doctor in scrubs, ministry hashtags, visual cues, real people, tone of language, clean design, legitimacy signals, lab coat authority, design matters
	Benefit Horizon (perceived advantages and	opened that door, leads to resources, testing saves life, visualizes prevention, seed for decision, promise of impact, empower

	positive outcomes made visible online)	actions, protective messaging, hope in posts, social media benefit
	Cognitive Dissonance (conflicting health messages causing internal tension and confusion)	conflicting messages, trust erosion, emotional vs. rational, opinion overload, social noise, chaotic updates, internal conflict, diluted facts, opposing advice, confusion in communication
Communal Ethos	Normative Adhesion (conforming to social norms and group expectations online)	group behaviour, communal standard, digital compliance, visible support, performative norms, expected participation, social alignment, WhatsApp influence, unspoken expectation, community influence
	Collective Identity (shared group identity formed through public health narratives)	public health brand, national resilience, societal wellbeing, shared identity, cultural branding, unity message, connected messaging, personal relevance, health mission, community reflection
	Relational Trust (trust shaped through cultural resonance and authentic communication)	value reflection, authentic voice, Emirati voices, cultural resonance, messenger matters, shared beliefs, humanized system, representative messaging, emotional alignment, social trust

	Symbolic Alignment (alignment between public values and health messaging)	belief representation, emotional language, values reflection, family and safety, national pride, mirrored values, cultural tone, civic duty, symbolic campaigns, identity through message
Mobilized Cognition	Action Catalysts (digital triggers that prompt immediate health action)	reminder tweet, trigger to act, countdown urgency, vaccine cards, social cue, intuitive clicks, missed opportunity fear, flashing graphics, instant booking, live announcements
	Influence Valence (emotional weight and direction of influence in storytelling)	emotional conviction, hope vs fear, sincerity in stories, empathy and urgency, moved by stories, warm messaging, belief again, emotional impact, life over data, visual stories
	Compliance Drift (gradual shift toward behavioural compliance through social influence)	visible compliance, social permission, viral behaviour, peer influence, trust in group, collective behaviour, crowd safety, natural follow, community action, social norm effect
	Self-Efficacy Loop (increasing confidence and	step-by-step, mental barrier, FAQs, clarity supports action, confidence to act, visual guides, inspired to try, home testing ease, equipped by posts, action readiness

	ability to take health action)	
Digital Reverberations	Echo Governance (institutional adaptation in response to public feedback)	felt heard, responsive pivot, added languages, adaptive content, human tone, feedback-based updates, live Q&A, policy reaction, measured response, evolving strategy
	Sentiment Flux (shifts in public emotions expressed online)	optimism vs. scepticism, emotional turbulence, lockdown anxiety, mood shift, tone-dependent reaction, fast frustration, influencer impact, comment section rhythm, empathetic response, emotional waves
	Discourse Contours (emerging directions of public narratives)	narrative evolution, mental health visibility, organic themes, user-generated direction, hashtag mapping, story over statistics, recovery narrative, transparency concern, community-driven focus, trust trajectory
	Narrative Feedback (real-time public feedback shaping institutional messaging)	real-time feedback, scaled messaging, uplifting stories, design input, empathy growth, co-authored narrative, audience maturity, adaptive tone, feedback loop closure, messaging refinement

4.3.1 Deductive Thinking by Keyword, Code and Theme Interpretation

Conceptualization

Conceptualisation began with the positioning of themes in relation to two theoretical frameworks: Social Influence Theory and the Health Belief Model. At this stage, the data were abstracted by translating themes into conceptual patterns, addressing not only *what* participants reported about health communication practices but also *why* they engaged in particular behaviours. Each theme was compared against the broader research questions to ensure interpretive validity.

For example, the theme of Mobilised Cognition was interpreted as behavioural readiness prompted by external stimuli, aligning closely with the constructs of perceived benefits and cues to action from the Health Belief Model. Similarly, the theme of Communal Ethos was grounded in the internalisation of group norms and identification with the public health system, consistent with Social Influence Theory.

Conceptual relationships among the codes and theoretical representations were developed further through iterative memo-writing, thematic matrices, and visual mapping. This interpretive process strengthened the connection between empirical data and analytical framework, enhancing both coherence and theoretical contribution.

4.3.2 Construction of the theoretical model

The final step of the thematic analysis produced a conceptual model that visually represented the key outcomes of the study. The framework brought the five themes together within a circular feedback structure, reflecting the real-time interplay between social media engagement, public trust, and institutional response.

The conceptual model was designed to capture the ongoing dynamics of dialogic input (Dialogic Resonance), perception formation (Perceptual Cartography), normative alignment (Communal Ethos), behavioural mobilisation (Mobilised Cognition), and institutional adaptation (Digital Reverberations). Each theme was depicted as a dynamic node, interconnected by pathways illustrating the flow of influence, feedback, and social interaction. Grounded in Naeem et al. (2023) and informed by the dual theoretical underpinning of the study, the model served two epistemologically congruent purposes: it functioned as an overarching synthesis of the findings and as a strategic tool for designing future health communication campaigns.

The model highlighted the collaborative, emergent nature of dialogue in digital health, while also showcasing the power of collective voice in enabling transformative change within public health systems.

4.4 THEME 1: DIALOGIC RESONANCE

The potential of two-way communication via social media to encourage interaction and shared meaning formed a key theme in this study. This points to the transformative power of bidirectional, social media-based communication within the Abu Dhabi public healthcare context. The theme directly relates to Objectives 1 and 4, and Research Questions 1 and 4, since it explores how interactive dialogue can enhance community involvement and influence institutional responsiveness.

The Symbiotic Exchange code reflects this interaction, where social media disrupts hierarchical models of communication in favour of an interactive unity. Participants described instances in which institutional responses were personal and timely, addressing issues or acting upon suggestions. These accounts support the Health Belief Model's concept of cues to action, as well as the Social Influence Theory's notion of informational influence, both of which highlight the significance of institutional responsiveness in driving public engagement.

Building on this, Discursive Pulse demonstrates how tone and emotion shape health communication. Both theoretical frameworks emphasise empathy, real-time responsiveness, and human-centred messaging as crucial to creating trust and enhancing efficacy. Participants perceived sincerity in such communications and they associated this with improved engagement and greater trust. These are key factors aligned with perceived benefits and trust within the models.

Participatory Echo extends the discussion by illustrating how public voices resonate within the health system. Participant quotes showed that public responses influenced live coverage, policy debates, and editorial strategies. This corresponds to the concept of interactive compliance in Social Influence Theory, where individual voices aggregate into collective action, prompting behavioural change within institutions.

Conversational Agency concerns the sense of empowerment participants felt, whether as members of the public or as employees within the system. From voicing complaints

to shaping the narrative, this sense of participation reflected increased self-efficacy, a core construct of the Health Belief Model. It also reinforced normative influence, as the digital space became a platform for the democratisation of voice.

4.4.1 Code: Symbiotic Exchange

“Symbiotic Exchange” refers to a give-and-take relationship in which the public and the health service interact and actively influence one another. In the context of this study, it **illustrates** how social media interaction has evolved from a one-way broadcast to a two-way dialogue. This shift is critical **because it marks a movement away from a traditional top-down broadcasting model**, in which institutions speak *at* the public, **towards** a more engaged and participatory approach that helps foster confidence and cultivate a sense of shared responsibility between institutions and citizens.

One participant recalled submitting feedback about clinic hours and receiving a response from the health department within hours, describing it as “like a real conversation” (PUBL13). This kind of interaction was experienced as respectful and human, with the institution taking time to offer a personal reply. However, beyond simple politeness, **this responsiveness** reshaped perceptions of the system itself. It fostered a sense of care, responsiveness, and accountability which stood in contrast to previous, more impersonal modes of engagement. Another participant noted that their voice “doesn’t just fall silent” (PUBL20), recalling a time when their input “just evaporated in silence,” but now, “the voice that went out there comes back with answer” (PUBL20). **Taken together**, these accounts illustrate how social media has transformed frustration into connection. One focused on the speed and warmth of the response, while the other emphasised the emotional shift from being ignored to being heard.

Importantly, this change is not only recognised by the public but also by health professionals. One professional observed that new media has “destroyed the monologue model” and remarked, “we learn from the public as much as we give them knowledge” (PROF4). This reflects a significant mindset shift. Whereas the health system was once focused on issuing instructions or running campaigns, it now sees the public as a source of insight and perspective. This is not simply a change in tone — it marks a fundamental reorientation of the system’s responsibilities. As another professional explained, “We’re not just us transmitting information; we tell our story,

but we don't, ultimately, tell our own story" (PROF7). This excerpt underscores the idea that the public is no longer a passive recipient of messages but an active participant **shaping** the discourse and contributing lived experience to public health decision-making.

Such participatory nature is captured by another public stakeholder who noted that health workers "do not just read...for example, or look at the social posts but follow up on social posts and actually implement what is happening from the public" (PUBL08). This is where the exchange truly shines. It is more than talk — it leads to action. What people are saying online can have a direct impact on health services. This is supported by a professional who stated that "public feedback on Twitter has influenced our pharmacy queuing system," calling that kind of interaction "invaluable" (PROF1). **Here**, the accounts — from different perspectives, public and professional — identify the same outcome: the community shaping tactical, tangible, real-world services. This reflects the extent to which the feedback loop has deepened, so that actions are no longer conceived in isolation but are shaped through community input.

That sense of shared space is also reflected in one quote, in which someone described "the relationship on social media [as one that] feels more mutual — like we're both contributing to a common space" (PUBL17). This quote captures the emotional dimension of the dialogue. It is not simply about demanding and receiving — it is about creating something together. This sense of reciprocity is echoed in what the experts have said. They are using public input, not merely tolerating it, but depending on it to shape their communication, and arguably, even their operations.

Another quote deepens this discussion. One public participant wrote, "it was great to see an official respond here directly to my question about the side effects of the vaccine" (PUBL01). This suggests that it is not only high-level or big-picture feedback that matters. Even individual, personal concerns are being addressed directly by the authorities. This kind of personal involvement has a significant impact on building trust. It also provides a counterpoint to the quote about the pharmacy system. The queuing system represents macro-level change, while the vaccine response reflects microsocial care. **Together**, these examples illustrate how symbiotic exchange operates both at the systemic and personal levels.

Looking across these quotes, a common thread emerges: the Abu Dhabi health system's presence on social media is no longer limited to broadcasting updates. It has become a forum for discussion, decision-making, and service improvement. What makes this exchange symbiotic is that both sides — professionals and the public — are learning, adjusting, and evolving through the interaction. The public sees their suggestions put into action, while health workers gain new insights from those they serve.

Beneath many of these quotes lies a strong emotional undercurrent. From PUBL13's "real conversation" to PUBL20's voice that "echoes back," it is clear that people want more than information — they want connection. They need to feel engaged and respected. On the professionals' side, there is not only a willingness to listen but also an increasing recognition that this dialogue is essential for informing better policy and response.

This theme resonates with behavioural theories such as the Health Belief Model and Social Influence Theory. When people feel heard (when they speak, are listened to, and see action taken) their confidence to participate again increases. This process in turn strengthens self-efficacy and trust. Through a social influence lens, responsive communication fosters identification, encouraging individuals to see themselves as part of the system rather than passive observers. This is why PUBL17 describes it as "a common space." It is not about one side giving and the other receiving; it is mutual, dynamic, and continuously evolving.

The concept of Symbiotic Exchange thus demonstrates how social media offers a unique opportunity to transform health communication in Abu Dhabi from passive dissemination to active, emotional, and meaningful dialogue. It essentially reframes digital platforms as bridges that connect people to services and translate ideas into action. Crucially, it supports the development of a health system that listens, responds, and evolves with its people.

4.4.2 Code: Discursive Pulse

"Discursive Pulse" refers to the emotional tenor, responsiveness, and rhythm of digital public health messaging which serves to shape and reflect how people feel, respond, and participate. It highlights the centrality of human elements in communication.

Specifically, it suggests that plain language, clarity, and timely responses help build trust and encourage engagement. When it comes to public health, effective communication is about factual accuracy as well as **how** facts are presented, and this applies whether the message feels human, timely, and emotionally attuned to its audience. When messaging sounds robotic, people disengage. As one professional put it, “It’s the human tone which brings trust.” This underscores that impersonal language distances people, whereas a human-like tone fosters connection and credibility.

A public participant (PUBL05) observed, “They are now talking our language less jargon more clarity. I almost feel like they’re listening,” suggesting that the shift toward plain, understandable language bridged a communication gap and marked a turning point in public trust. Timing also played a key role, as another professional (PROF2) reflected, “During COVID, social media updates were immediate and had a sense of calm urgency.” These quotes form the basis of the code’s definition and show that tone, rhythm, and style are not superficial aspects — they are central to effective public health communication.

The definition of “Discursive Pulse” in this study reveals how tone and style within social networks affect the emotional bond between the public and the health system. Respondents from both public and professional groups preferred messaging that felt human and authentic. As communication became more conversational, transparent, and personalised, participants felt taken seriously and heard. One participant remarked, “The speed and sincerity of their response matters. That’s what makes us feel secure” (PUBL10). This shows that feelings of security during health crises were shaped not just by policy content or data but by the delivery of the message itself. The word “sincerity” implies that people were seeking emotional support alongside official guidance.

Another participant (PUBL12) explained, “[You could have say] What I mean is, when one comment or message has a question, they tailor their response. Their responses as opposed to before [are] they are not just copy-pasting answers anymore.” The use of “tailored” points to responses that address individual concerns directly, while the rejection of “copy-pasting” reveals that generic or automated replies previously created a sense of being dismissed. Together, these accounts illustrate that tone, timing, and

personalisation are critical to how communication is received and whether it builds trust.

This high-touch experience is also closely linked to trust. As one participant explained, “I trust more when responses are not templated. It’s all about rhythm and realness” (PUBL18). “Realness” here refers to authenticity, while “rhythm” captures the natural timing and flow of conversation. Together, these terms highlight the emotional labour involved in social media communication. When messaging lacks this human touch, it fails to create meaningful engagement.

From the professional perspective, the importance of tone and rhythm is well understood. One professional reflected, “Tone matters. My voice spoke deeper and now more empathetic than instructivist ones even in times like this” (PROF3). This statement suggests that even in moments of urgency, issuing instructions alone is insufficient. What people respond to is empathy — they are more receptive to emotionally attuned care than to cold or formulaic communication.

This view is reinforced by another professional, who noted, “There’s a rhythm to good engagement — it’s attuned to public needs” (PROF6). The repeated emphasis on “rhythm” signals the importance of synchronising tone and timing with the emotional and informational state of the audience. Being “in sync” refers to a communication orientation in which public health messaging feels natural, responsive, and present in the moment. It moves away from the deficit model of information transfer and instead frames communication as an evolving, empathetic dialogue attuned to context.

Another keen observation came from a professional who stated, “When the messaging feels robotic, people check out. That’s the human tone that builds trust” (PROF5). This clearly distinguishes successful communication from failed attempts: a robotic tone pushes people away, while a human tone draws them in. Similarly, a public participant remarked, “They’re speaking our language now — less jargon, clearer language. Oh, it’s like they’re finally listening” (PUBL05). The phrase “our language” signals a shift from technical to more accessible, relatable communication that connects institutions to communities. The expression “finally listening” implies a longstanding gap in sensitivity and inclusion, highlighting that the change was overdue.

Taken together, these accounts underscore a stark contrast between past and present public health communication. Previously, the tone was formal, distant, and vague.

Today, with less artificial language and faster, more personal responses, people feel more connected. This change is not just psychological — it has practical outcomes, including increased trust, improved engagement, and potentially better health behaviours.

Comparing various quotes also reveals how tone is linked to public confidence. The professional view from PROF3 — “Even in crisis, empathy shouts louder than instructions” — is echoed by public sentiment, such as PUBL10’s observation: “It makes a difference how quickly they make it and how sincere they are.” These quotes show alignment between institutional goals and public values. Both groups understand that sound — in terms of tone, rhythm, and emotional register — carries emotional weight and influences how people respond.

The shift from old to new styles of communication is especially clear in the reflections of PUBL12 and PUBL18, who described earlier messages as having “felt templated” or “copy-pasted,” making them feel like targets, not people. The move toward tailored replies and a more human tone helped repair that disconnect. This transformation is particularly key in the context of this study, since it illustrates how subtle adjustments in language and delivery can have significant impacts on public trust and emotional readiness.

Timing is also a significant factor. There is something notable in how PROF2 described the “calm urgency” of health messaging. This phrase signals that rapid updates do not have to come across as rushed or panicked. Instead, they can strike an emotionally neutral balance — conveying urgency while maintaining composure. This balance perhaps served to reduce public anxiety during a period when searching for certainty felt futile. It made the health system appear both alert and emotionally steady.

The concept of “Discursive Pulse” reinforces the idea that communication is much more than simply an exchange of information; it is a bridge between emotion and community. When institutions adopt a tone that is warm, timely, and clear, people feel recognised, affirmed, and reassured. As a result, they are more likely to trust the system and participate actively. In contrast, cold or mechanical communication tends to disengage the public. The quotes collectively underscore the importance of keeping health communication lively, human, and real. Abu Dhabi’s public health system has increasingly found ways to use social media not just to speak to people, but to speak

with them. The human connection, shaped through rhythm and tone, has become a critical element in making social media a meaningful and effective tool for public health and community engagement.

4.4.3 Code: Participatory Echo

“Participatory Echo” refers to the phenomenon where public voices on social media do not fall silent but instead travel, echo, and amplify — reaching institutions and shaping public health conversations and decisions. It reflects the transformation of individual expressions into broader social reactions. These digital iterations — tweets, posts, comments, hashtags — have become tools of influence that not only represent individual experiences but also shape institutional awareness and policy change. As one respondent put it, “I’ve commented about elderly access on the ticketing, and a week later that was brought up in a livestream — they’re hearing us” (PUBL03). This illustrates how **digitally mediated visibility** lends weight and significance to public input.

A professional echoed this dynamic, noting that “when people tell their stories it shapes broader decisions” (PROF9), while another participant remarked, “our insights aren’t landing in the void any longer — they ripple through policy discussions” (PUBL14). **Together, these quotations reinforce the same idea**, describing a shift in social media’s role: from a platform for isolated feedback to a mechanism for collective influence and public action.

Within this research, “Participatory Echo” signals a significant change in how public health systems treat community feedback. The public no longer appears to be ignored; instead, participants observe that online complaints can generate momentum and institutional accountability. The testimonies of PUBL03, PUBL14, and PROF9 collectively illustrate this shift. The fact that concerns about elderly access were raised in a livestream within a week signals a different type of communication channel — one that is fast, open, and responsive.

Rather than being stalled in bureaucratic processes, public input now features in live policy discussions. This responsiveness also lends legitimacy to institutional actors. Participants no longer feel that they are speaking into a void. **Instead**, they see responses and actions that affirm their contributions, reinforcing trust in the system

and encouraging continued engagement. “Participatory Echo” thus reveals how personal stories can evolve into community-wide concerns, and how institutions have entered the same digital space where worries are shared, repeated, and amplified.

The effects of new media devices such as hashtags further amplify the dynamics of participatory echo. As PUBL06 observed, “Hashtags seem innocuous, but they serve as rallying points — social-media bullhorns for real issues” (PUBL06), highlighting that even simple digital tools can draw attention to pressing public health concerns. This is echoed by PUBL09: “the more we share, the more the system learns. It’s feedback at scale” (PUBL09), and supported by PROF8, who remarked, “The real power is when a single post catalyses collective change” (PROF8). **Read together**, these statements suggest that the public health system is no longer the sole producer of knowledge or action. In this evolving exchange, the public has come to function as instructor, mirror, and guide.

Collectively, these voices illustrate that public expression now moves beyond awareness — it has become a fundamental mechanism through which institutions detect needs and shift direction. A single hashtagged post can spotlight an issue, shared stories can validate it, and ultimately, the institution responds. This represents a **digitally enabled form of participatory democracy** — continuous, visible, and emotionally resonant — embedded within the health system.

This process is also empowering for the community. When PROF2 stated, “A patient’s story, once in the shadows, now sparks public conversations and awareness” (PROF2), it spoke to a cultural shift in which everyday voices are now visible. When considered alongside PUBL21’s comment, “We don’t just hear each other — we amplify one another” (PUBL21), and PUBL06’s earlier point about hashtags, the message becomes clearer. These participants highlight that social media gives individuals a voice and connects them with others who share their concerns. Their words are thus not isolated cries, but are echoed, affirmed, and magnified by others.

This amplification creates a shared sense of influence and power. Personal narratives become public ones, and public narratives, in turn, shape the priorities of the health system. Through this echo chamber effect, public health is no longer something done to people — **it becomes something created with them**. This is the spirit of

participatory echo: the community in close contact with the system, and the system in close contact with the community.

4.4.4 Code: Conversational Agency

“Conversational agency” speaks to the capacity of individuals to speak up, challenge authority, raise concerns, and engage with health communication on social media on their own terms. It is about the power of being heard, of joining the conversation, and of seeing one’s input shape real decisions or public messaging. This form of agency is particularly significant for those who previously lacked a platform or a safe space to express themselves (Sommer, 2460). As one participant reflected, “I can at least ask, or challenge, or criticize, we can do all kinds of things that we never could before” (PUBL16), pointing to the new expressive space that the digital world has created. Another participant put it more starkly: “For a lot of us where I’m from, social media is the only safe place to talk that shit without fear” (PUBL07), suggesting that social media transcends social and institutional boundaries and offers a form of protection. A professional echoed this by stating, “Empowerment starts when people feel listened to and social media is that microphone” (PROF3). **Across these quotations**, the essence of this code emerges: empowerment, expression, and the expectation of response.

The “Conversational Agency” code in this study captures a broader shift in how people understand their role in public health communication. Individuals are thus no longer simply passive recipients of health information. Rather, they are contributors, critics, and co-creators. Social media offers a space where people can speak out without being silenced by fear, authority, or institutional filtering. The perspectives of PUBL16, PUBL07, and PROF3 illustrate this transition. Their comments suggest that expression is no longer the exclusive domain of professionals or experts. Members of the public including those who were previously marginalised — now feel that they have a voice.

The phrase “agency we never had before” speaks directly to a history of exclusion. In earlier forms of public health communication, information flowed one way — **from authority to audience**, representing a classic **top-down broadcasting model** in which institutions transmitted messages without inviting dialogue. That flow has now shifted. Social media platforms like Twitter and Instagram enable the public to

challenge, question, and even redirect official messaging. The metaphor of social media as a “microphone” reveals how once-muted voices are now amplified rather than suppressed. This development has major implications for community organising, as it signals that individuals who once had no traditional power are now participating meaningfully in the system.

This new power is not only about speaking. Rather, it is about witnessing what happens when you speak. PUBL11 described “The ability to speak, be heard, and see and what will help the body that society has” (PUBL11), while PUBL15 noted, “We went from recipient to participant in our own health conversations” (PUBL15), and PROF6 reflected, “We’re not just reaching the people; we’re letting the people reach us” (PROF6). **Together, these voices** illuminate the same underlying shift: the public is no longer passively receiving information, but is actively contributing to the formation of public health messaging.

People are now part of the process in the sense that they are not just being addressed, but are helping to shape what gets said and done. The system therefore listens, responds, and adapts. These quotes suggest that conversational agency becomes real when people speak and when institutions respond meaningfully. This reciprocal exchange enhances the public’s sense of ownership and value. It encourages individuals to see the health system **not only as something that serves them but as something that represents them.**

The strength of this code lies in its universality. As PUBL04 noted, “I’ve seen people with no formal education be health advocates, the whole nine yards, on Instagram” (PUBL04), illustrating how digital technology can transcend traditional barriers. When read alongside PROF7’s statement, “Digital platforms have redistributed communicative power in healthcare” (PROF7), and PUBL19’s observation, “People now shape the discourse, not just consume it” (PUBL19), a clear narrative emerges: social media has reshaped who gets to speak, who gets to be heard, and who gets to lead. Public health dialogue is no longer confined to experts — it now includes people from all walks of life. Regardless of formal education, individuals can spark stories, respond to others, and drive change.

Redressing this imbalance in communicative power acknowledges that health is a shared, collective endeavour. This suggests that everyone has the right to participate in shaping how it is spoken about. **Ultimately**, these perspectives reveal that “Conversational Agency” is not simply about having a voice. It is about understanding the impact of that voice — about freedom, equality, and power. In Abu Dhabi’s healthcare landscape, this agency is helping shift communication from a one-way broadcast to a two-way dialogue in which people feel seen, heard, and empowered to effect change.

4.5 THEME 2: PERCEPTUAL CARTOGRAPHY

This section sets out how people perceive COVID-19 messages from both cognitive and emotional standpoints. It corresponds to Objective 2 and Research Question 2, which focus on how individuals make sense of, interpret, and personally process public health news through social media. The emphasis here is therefore on the psychological and affective dimensions of communicating health risks and benefits.

The code Risk Intuition illustrates that health threats are not primarily understood through scientific facts — like the sight of a microbe — but through how health officials frame conditions such as flu, SARS, or monkeypox. **These perceptions are shaped through narrative cues**, with users describing heightened feelings of vulnerability when people they knew, or who lived nearby, were affected. This maps onto the susceptibility and severity constructs in the Health Belief Model (HBM) and also aligns with informational social influence in Social Influence Theory — where individuals adjust their health perceptions based on the shared experiences of their community.

Trust Semiotics shows that credibility is not solely about content but is symbolically conveyed. Features such as verified identities, professional presentation, and familiar, clear language serve as visual and linguistic signals of trust. This code underlines the idea that public evaluation of authority is increasingly based on aesthetics and communicative cues — design and presentation serve as social proof, reinforcing the authenticity of messages and building confidence in their source.

Benefit Horizon centres on users’ anticipation of positive outcomes. Participants spoke of taking action, such as scheduling health screenings, after seeing content that

communicated health gains. This resonates with perceived benefits and cues to action in the HBM, as well as with the role of internalised social norms in Social Influence Theory, where the anticipation of future rewards shapes current decisions.

However, this optimism often collided with Cognitive Dissonance. Conflicting advice and mixed messages triggered internal conflict and a kind of “what the hell” reaction, which eroded trust. Information overload led some users to disengage from health discussions entirely. This response represents a significant perceived barrier and underscores the fragile nature of trust when public communication lacks clarity, coherence, and synchrony.

4.5.1 Code: Risk Intuition

“Risk Intuition” is the name for the personal, emotional way people understand and feel health risks, especially in times of crisis like the COVID-19 pandemic. It is **not only** in the data or the official announcements; it is in the way people experience vulnerability through others’ stories, posts, and social cues on social media. This code demonstrates that numbers and graphs are insufficient, in many cases, to raise awareness, prompt action, or mobilise support. What engages people instead are personal stories that make the risk seem real and feel close.

As one public participant shared, “I didn’t need no graph to feel the risk- them social posts about young victims made it real” (PUBL02). “It felt like each tweet was ringing a warning bell in my gut” (PUBL05) also demonstrates that emotional responses may be more powerful than rational ones. “It’s not the stats that sell us; it’s the scary stories that make the threat feel real” (PROF1). Even health experts acknowledge the persuasive power of emotional storytelling.

These observations clarify how “Risk Intuition” arises from emotional cues and past experience instead of mere facts. The interpretation of “Risk Intuition” in this study suggests that people’s interpretation of danger in the context of a health crisis is powerfully emotional, and often dictated by what they see online, rather than through formal health campaigns. A broad range of participants described how their personal sense of risk was heightened more by personal stories than by government reports or statistics.

As PUBL02 said, “I didn’t need a chart to feel the risk — those social posts about young victims made it real” (PUBL02). This could be because stories from one’s own life on social media can evoke emotional responses more than numbers can. PUBL18 also added, “When I read that a neighbour had it I felt quite safe suddenly” (PUBL18). Here, knowing someone in person, or even seeing someone nearby, made the threat feel immediate. The threat was no longer remote or speculative. It felt close, personal and suddenly deadly serious.

This affective proximity is also audible in the words of PUBL07: “I didn’t think I was at risk until I saw the symptoms of someone my age... till then, I was taking it lightly. That hit me” (PUBL07). These stories illustrate that people judge their own vulnerability not based on national headlines or expert-prepared graphs, but by comparison with those they feel personally connected to. Once they know that someone their age, or someone nearby, is sick, it becomes easier to imagine they could be next. This creates a strong emotional bond that motivates people to take action, or at least be more cautious.

Intuitively, this view is well supported by the professional perspective in a quote from PROF1: “It is not statistics that persuade us — it is stories that make the threat personal” (PROF1). This observation bridges the gap between expert knowledge and public feeling, suggesting that both groups recognise the persuasive power of narrative. Some participants also noted that local context and emotional signals mattered more than abstract data. “I also get the feeling that when the results of local tests are being announced, I felt more awake. It made danger tangible.” The word “tangible” is crucial here, because it suggests that risk — once invisible — became something people could physically or emotionally perceive. Not general reports, but posts that hit close to home.

PUBL05 said similarly, “When the numbers were high, every tweet felt like a warning bell in my gut” (PUBL05). This quote perfectly underscores how emotion, not logic, can drive comprehension. Instead of absorbing what the daily reports had to say, people followed the lead of their feelings, shaped by what they were seeing posted and shared in personal stories.

There is also a telling distinction between the psychology of formal media and social media when it comes to emotional sway. PUBL11 said, “What the media didn’t do, one post from a nurse did — it changed the way I saw my own risk” (PUBL11). This suggests that traditional news may not have made the risk feel personal, while a single post from a real person had an outsized impact. This distinction highlights the particular relevance of social media for public health communication. Because messages often come from like-minded individuals — usually average people posting on platforms like Twitter or Instagram — it becomes easier for others to feel the emotion and internalise the message as something that also belongs to them. It is not about reading something — **it is about feeling something**.

PROF4 describes this emotional connection clearly: “There’s an emotional compass we all use and that social media guides more than official bulletins” (PROF4). The phrase “emotional compass” suggests that individuals rely on emotion to assess what is risky and what is not. In such cases, social media takes the lead by providing emotional cues in the form of stories, videos, and comments. Official bulletins may offer precise information — the facts — but a truth devoid of sensibility may not resonate as deeply.

It is also interesting to note the disparity between official sources and social media. Numbers and bulletins may discuss percentages and infection rates, but those figures do not always bring a sense of personal relevance. Conversely, if you see a post from a nurse, a neighbour, or someone your own age, it becomes easier to picture yourself in that situation. As reflected in the quote from PUBL07, the transition from “I didn’t think” to “That hit me” signifies a sudden identification with an issue — one that arises not from a cognitive understanding of data, but through emotional reflection.

Employing stories, particularly from laypeople, alters how data is received and processed. This emotional processing is central to Risk Intuition. When people prefer stories to data, this is not because they are uninformed or irrational, but more likely because they are seeking meaning through narrative. This is where social media plays a crucial role. It helps people understand not just intellectually but emotionally — and this emotionally charged understanding leads to behavioural change.

It may be the reason someone starts wearing a mask, avoiding crowds, or getting vaccinated not because of a chart, but because they heard a story that felt too real to ignore. These voices are collective, and the message they share is this: in a climate of risk, feelings of risk are constructed gradually, through small, emotional encounters on social media. People do not wake up and feel fearful because a report told them to. They feel panicked after reading a singular, personal account, seeing a case in their town, or encountering a relatable post.

This emotional response is intense and has a profound effect on public behaviour. It is also tied to trust. People are more likely to believe in, and act on information when it emanates from someone they perceive as one of their own. “Risk Intuition” also demonstrates the influence of emotional stories on social media in terms of shaping how people perceive and react to health threats. It suggests that facts alone are not sufficient, and what truly resonates with people is feeling. They want to see themselves in the story, and social media ostensibly makes that possible. It helps turn a distant, abstract risk into something that feels close, relevant, and personal. This emotional process is fundamental to how social media facilitates public health communication. In that sense, social media is not only about disseminating information but about shaping understanding and motivating people to care and to act. Through emotional storytelling, social media becomes a powerful tool to help people recognise risk and make healthier decisions.

4.5.2 Code: Trust Semiotics

On social media, people judge credibility based on visual cues, symbols, and presentation styles — a concept referred to as Trust Semiotics. These are not merely decorative features, but meaningful ones. They are designed to help people decide whether to trust a message or disregard it. “Things like the blue verification check, health authority logos, official photographs or language selection can also contribute to [the illusion of legitimacy].” If a doctor is shown in scrubs or a post features an official ministry logo, people are more likely to trust the content. “It shows more than titles; what it communicates is validity,” said one public participant (PUBL12). This is because symbols operate as signals of authority in the digital space.

Likewise, one respondent noted: “Picture of a doctor in scrubs with so-called real name tag says more than any press release” (PROF5), showing that visual trust signals are often more compelling than formal announcements. These are the small details upon which Trust Semiotics is built — simple, powerful signs that inspire confidence in the message being delivered.

The concept of Trust Semiotics, as constructed in this study, emphasises that individuals rely on visual and symbolic signs when evaluating online health information. This is particularly the case when it comes to making quick decisions about whether to trust or reject such information. These signals become especially crucial during health crises, when people are inundated with information and need fast ways to distinguish truth from misinformation. Several participants described how small but significant cues shape their trust.

PUBL12 said that “The blue tick means more than any title — it means legitimacy” (PUBL12), highlighting that online, a small icon can carry more authority than a person’s real-world job title. This blue tick, used by verified accounts, creates the impression that the person or organisation is real, official, and accountable. For users trying to navigate constant health updates, this kind of symbol acts as a shortcut for identifying trustworthy information.

This perception of visual trust is supported by PUBL06, who said, “What I noticed is that I trusted posts that had a logo and a ministry hashtag. They feel more grounded” (PUBL06). The word “grounded” here refers to a sense of stability — something backed by institutional force and structure. This introduces another layer: trust is not just about who posts the content, but how it is framed. **Thus**, a single hashtag or official government logo can make an otherwise mundane post seem credible. PUBL15 added, “Whenever they are speaking in Arabic and looking at real people instead of the stock image, I feel as though I can believe them a bit more” (PUBL15). This highlights how authenticity also plays a role — not only through the visibility of real people but also by using the audience’s language. When health messages align with the culture and lived reality of their audience, trust increases. It is not just about visual representation — it is also about emotional and cultural validation.

In juxtaposition with professional perspectives, the insights gain depth. According to PROF5, “A picture of a doctor in scrubs with a real name tag is worth a thousand press releases” (PROF5), since even professionals recognise the power of simple visuals. This relates directly to PUBL21’s view: “If it’s from someone in a lab coat, I listen. Presentation matters” (PUBL21). **Together**, these examples show that a lab coat or name tag is more than just a garment — it is a visual code that signals expertise and reliability.

Even design choices such as colour, font, and layout influenced how people interpreted health messages. PUBL04 wrote, “If I see casual fonts and emojis I scroll past, whereas clean design and data visuals stop me” (PUBL04). This indicates that people judged content based on its professionalism. When a message seemed casual or disorganised, it lost credibility. When it was clean, polished, and structured, it gained authority. PROF6 elaborated: “There is power in visual cues — uniforms, badges, even the inflections of language. It shapes belief” (PROF6). **Taken together**, these quotes show that trust is not only about information, but also about how the messenger presents it — visually and emotionally.

There was also a more profound observation from PROF2: “Symbolism matters more than we’d like to think — it helps strain out what’s noise and what’s truth” (PROF2). This suggested that such signs often operate subconsciously. In a fast-paced digital ecosystem, users do not analyse every post; instead, they look for cues that help them quickly categorise information. When the combined voices were considered, a clear pattern emerged. People relied on visual trust cues as emotional footholds. They were not purely searching for facts. Rather, they were looking for signals that the information was secure, reliable, and familiar. Whether this was a blue tick, a photo of a nurse in uniform, or the use of local language and imagery, such signals conveyed the consistent message: “This is trustworthy.”

And yet, such symbols had to be used with caution. If they appeared contrived or forced — like stock images or overly casual emojis — they could backfire. PUBL04 made this clear by noting that people tended to skip posts with casual fonts or emoji-laden designs. **This shows that audiences sensed authenticity**, and reacted accordingly.

The code Trust Semiotics therefore uncovered that people depended on visual and symbolic cues to guide their beliefs and health-related decisions online. These symbols — a logo, a uniform, a photograph, a verified icon — helped build fast and powerful trust. They allowed users to cut through the complexity of the digital environment and quickly determine what to believe. When deployed consciously, these trust signals reassured, respected, and informed the public. At a time when public health systems needed to reach out quickly and effectively, such symbols were not optional — they were essential. Social media became more than a message carrier; it became a space where trust was constructed through small but powerful visual gestures.

4.5.2.1 Code: *Benefit Horizon*

Benefit Horizon refers to the way people perceived and imagined the positive consequences of engaging with public health messages on social media platforms. Participants reported that when users were exposed to messages promising accurate, real-world solutions and meaningful action, they were more likely to engage and take concrete, health-related steps. These messages shifted the focus from fear or risk to a vision of a better future.

“I followed a campaign online and booked my screening — social media opened that door” (PUBL10). This response indicated that online content could lead directly to action. Another participant said, “When we engage, it’s not just noise — it leads to resources and reassurance” (PUBL01), illustrating how digital engagement offered value — not just information. PROF3 added, “There is hope in these posts — they are imagining the results prevention can produce” (PROF3), underlining the potential of social media to frame achievable health outcomes. **Together**, these voices articulate Benefit Horizon as a vision for reframing health communication around opportunity and possibility.

Significant to the interpretation of Benefit Horizon in this study was that people were most likely to engage with online health content they perceived as moving them toward something useful or hopeful. Unlike messages focused solely on danger or statistics, those that offered tangible benefits and emotional meaning were more likely to inspire action.

PUBL10 captured this clearly: “I followed a campaign online and ultimately booked my screening — social media made it happen” (PUBL10). This demonstrated how an effective campaign could lay out a path that felt open and achievable. This echoed PUBL01’s comment: “If we engage, it’s not just noise — it leads to resources and reassurance” (PUBL01). **In both cases**, engagement created direction and purpose.

As PUBL14 reflected, “And sometimes people think they’re just scrolling, but one message like plants a seed for better decisions” (PUBL14). This metaphor aligns perfectly with the concept of a horizon — something ahead, gradually unfolding. It suggests that change may begin with something small, simple, and emotionally resonant. The emotional connection here is central. PUBL17 wrote, “It wasn’t the data it was that the things I did mattered, that I could create my own narrative in a way” (PUBL17). This illustrates that people are motivated not just by information, but by a sense of meaning. When messages help individuals feel that their actions matter, they become more committed. This is reinforced by PROF7: “Our campaigns aren’t about fear — they’re about envisioning a better public health future” (PROF7). This bridges the emotional with the strategic. When messages highlight what can go right — prevention, safety, community protection — they create hope rather than anxiety.

PUBL03 captured this vividly: “I seen one post about early testing saves a life I shared it right away!!” (PUBL03). This shows how a single hopeful message can trigger further engagement and create a wider social ripple. Another important concept in this motif is empowerment. PUBL09 stated, “I kept seeing that same message — ‘Protect your community.’ It felt empowering” (PUBL09). This demonstrates that repeated, purposeful messages can instill confidence and responsibility. When individuals feel part of something larger, they shift from passive observers to active participants. These different voices — public and professional — all point to one main conclusion: when health communication emphasises potential benefits and enables people to envision success, it becomes more effective. It moves from warning to possibility. Unlike fear-based messages or heavy statistics, messages aligned with the Benefit Horizon are uplifting and motivating. They bring people hope, empowerment, and purpose. They encourage engagement and behavioural action. People pause, think, and act. Benefit Horizon demonstrates the capability of social media to show people that their involvement counts and their engagement is meaningful.

4.5.3 Code: Cognitive Dissonance

“Cognitive Dissonance” describes the emotional and psychological distress people felt when they received conflicting messages about public health, particularly during a crisis. It demonstrated how mixed signals from different sources, such as social media, group chats, official health agencies, or news outlets, generated uncertainty and diminished trust in the information people had. Individuals began to feel disoriented, angry, and helpless rather than informed and reassured. PUBL13 articulated their frustration crisply: “One post said masks were crucial, another said they are not. Who do I believe” (PUBL13). This kind of inconsistency resulted in doubt and hesitancy, or even withdrawal from engagement in health matters. A more emotional take came from another participant, PUBL16: “I wanted to trust the updates, but they would change the rules. It felt confusing” (PUBL16). **Together**, these quotes demonstrate that even when people tried to believe and follow health advice, they felt trapped between contradictory messages. **Where transparency was missing**, the communication failed.

4.6 THEME 3: COGNITIVE DISSONANCE

The theme of “Cognitive Dissonance” in this study revealed the **persistent struggle** people faced when health communication was inconsistent or unstable. Rather than establishing a clear course of action, conflicting messages nudged those trying to stay informed into a state of **continuous doubt**. PUBL13’s comment, “One post said masks were a must, another said no to them. Who do I believe” (PUBL13), exemplified this struggle. People were attempting to make the right decisions for their health, and when advice shifted or clashed from one source to another, they began to lose faith in all of it. **This erosion of stability** was not purely about facts. Instead, it created a significant emotional burden. PUBL16 echoed this: “I wanted to trust the updates, but they were always changing the rules. It felt confusing” (PUBL16). These changes generated frustration, especially for those attempting to protect themselves and their families. It was not only about the content of the message but also about trust, and how inconsistency made that trust increasingly difficult to sustain.

This emotional pressure was aptly described by PROF9, who observed, “Mixed messaging leads to internal chaos. It just makes you rethink everything” (PROF9). The

term “internal chaos” implies that it was not simply the external environment that became disorderly, but that confusion and disorientation were also felt within. When people received contradictory messages about what was or was not healthy, from experts, peers, or influencers, they began to mistrust both the individual message and the system delivering it. **Such instability** disrupted the sense of order the public typically expected from health authorities. That form of chaos became particularly dangerous during a health crisis, when people sought clarity and guidance, especially in an oversaturated information environment.

PUBL06 remarked, “So much social media inundates you with opinions. That sound dilutes the substance of facts” (PUBL06). This suggests that the issue was not only conflicting messages but also the sheer volume of content with which people were bombarded each day. Within that flood of scientific updates, personal opinions, and misinformation, it became increasingly difficult to distinguish fact from noise. People reported feeling overwhelmed, which made facts harder to see, trust, and act upon. **As a result**, platforms intended to inform risked becoming sources of stress, further eroding public trust.

The emotional and rational aspects of this confusion often came into direct conflict. PUBL20 stated, “I was emotionally with vaccines, but rationally I was in doubt after reading too many conflicting comments” (PUBL20). This highlighted how people could feel committed emotionally, yet intellectually unsettled by the digital conversation surrounding it. This contest between emotion and logic created psychological discomfort, particularly when individuals began questioning whether they had made the right choice. Such dissonance could delay action or halt it entirely. Even when people believed in the importance of getting vaccinated, the surrounding confusion stopped them from following through.

Another issue was the pace of updates. PROF8 noted, “The speed of the updates collided with the depth of understanding, too much, too fast” (PROF8). Although rapid updates are typically seen as beneficial, in this case they backfired. When health guidance changed faster than the public could process it, people were left behind. They had insufficient time to absorb one version of the message before a new one emerged. **In this context**, the constant churn contributed to mistrust, particularly when individuals no longer knew which version of the advice they were meant to follow.

Here, velocity itself became counterproductive, undermining the clarity and effectiveness of public health communication.

Personal communication channels such as group chats and private messages added further layers of confusion. As PUBL19 reflected, “It’s difficult to digest official advice when your group chats are telling you the opposite” (PUBL19). This demonstrated how trusted social spaces, such as family threads, friendship groups, or neighbourhood networks, could counteract public health messaging. Faced with contradictions between their private circles and official guidance, individuals were forced to decide whom to trust. Often, emotional attachment to one’s social group caused people to favour unofficial advice, even when it lacked credibility. This personal influence, layered on top of wider confusion, deepened uncertainty.

PROF1 added another perspective: “Clashing sources undermine our intuition related to health. Even when people have the best intentions, it blows trust” (PROF1). The implication was clear: even if authorities acted in good faith, trust still eroded when messaging collided. When a second wave emerged or guidance shifted again, people no longer trusted themselves to discern what was right. Their “health intuition” was dulled. They began to doubt not only the system but their own judgment. **Thus**, both the sender and receiver suffered in this collapse of confidence. Well intentioned campaigns launched into a noisy, disorienting environment often lost impact.

Taken together, the quotes painted a clear picture. People were willing to believe public health messages but only if they were delivered with clarity and consistency. When the messages became mixed, trust began to erode, and emotional fatigue, digital noise, and the relentless pace of updates combined to overwhelm audiences. Some grew weary of trying to understand. Some stopped reading, others stopped listening. Crucially, some stopped believing.

In the Cognitive Dissonance motif, the study illustrated how contradictory or muddled communication could create emotional distress and undermine trust. Even those who were keen to do the right thing found themselves paralysed by contradictions. This confusion went beyond simple misunderstanding, as it actively blocked action. **The analysis suggests that clear and consistent communication is not optional but**

essential, especially in fast moving digital environments where trust can be built quickly but lost even faster.

4.6.1 Code: Normative adhesion

“Normative Adhesion” describes the way individuals adopt public health behaviours not through explicit instruction but by observing others within their community, particularly through social media interactions. It reflects an invisible form of pressure shaped by emotional contagion and group persuasion. When people witness their friends, family, or community members complying with health guidelines, they often feel compelled to follow suit, even in the absence of direct messaging. **This means that behaviour spreads not by rules, but through repeated social cues.**

This dynamic is especially pronounced in online environments, where public acts like sharing health campaigns, wearing masks, or posting vaccination updates are easily observed and amplified. As PUBL10 noted, “Everyone in my circle began to post about vaccinations, eventually I felt I had to as well” (PUBL10). This illustrates how personal decisions are frequently shaped by visible peer behaviour. PUBL06 offered a similar view: “When your whole community is wearing masks and talking about safety, you naturally go with the flow” (PUBL06). **Taken together, these views show that individual choices often follow the dominant mood of one’s digital environment.** The implication is that health behaviours are anchored in collective momentum.

PROF2 summarised the phenomenon succinctly: “There is unspoken pressure to support public health, on social media, it’s visual” (PROF2). The tension lies in its visibility and subtlety. This pressure is ambient yet overt, constant yet unspoken. Together, these statements characterise Normative Adhesion as a process of behavioural mirroring, enabled and reinforced by digital presence.

Under the title Normative Adhesion, this study identifies how individuals are drawn into public health practices through the behaviour of their social contacts, particularly in online spaces where norms are performed, exchanged, and adopted. Several interviewees described how adherence often followed a pattern of social observation rather than deliberate research or official instruction. As PUBL10 reflected, “Suddenly everyone in my circle was posting about their vaccinations, after a while, I just felt like

I had to also” (PUBL10). **This links directly to the idea that seeing others take action creates an implicit expectation that one should do the same.**

This subtle pressure, not enforced but constantly visible, created a sense of necessity. Not participating risked social exclusion. As PUBL06 put it, “When your entire community is wearing masks and promoting safety, you naturally go with the flow” (PUBL06). The emphasis on “naturally” suggests authenticity. It was not about coercion but about belonging. It was not compliance imposed from above; it was shared meaning created horizontally across peers.

This pressure becomes even more visible in digital environments. As PROF2 observed earlier, “There’s this unspoken kind of pressure to support public health, on social media, it becomes seen” (PROF2). Private behaviours, such as deciding to wear a mask or get vaccinated, are no longer entirely private. Once posted or shared, they become open to social observation. **Visibility converts personal decisions into public actions, turning implicit expectations into digital norms.** Individuals start to feel accountable not only to institutions but also to their peers.

While this emotional pressure can motivate positive health behaviours, it can also foster a sense of performance, in which people act not solely out of personal conviction but to demonstrate responsibility to others. The influence is amplified within close-knit networks. PUBL17 stated, “On top of that, my family WhatsApp group had been constantly going on about it, constantly. I wouldn’t have even thought about doing some of the measures if my family WhatsApp group hadn’t” (PUBL17). **This shows how family communication intensifies normative pressure in quieter but more intimate ways.** Advice from trusted circles carries more weight than official messages.

Several participants noted how online behaviours set offline precedents. As PUBL03 noted, “Even the reserved folks in my network have shared those Ministry videos. It turned into a communal standard” (PUBL03). Once a behaviour gained sufficient traction, it ceased to feel optional. Even those who usually stayed silent were compelled to participate. Norms were solidified not through explicit debate but through repeated, unspoken agreement.

PUBL19 similarly observed, “You don’t always ask the question, you especially do it if your whole digital community is doing it” (PUBL19). This “mute agreement” illustrates how the perceived consensus of one’s digital peer group can ease behavioural uptake. People comply not because they understand the underlying science or policy rationale, but because the behaviour feels correct within their social context. **This makes social influence more powerful than any single campaign message.**

Social media thus becomes a space where norms are both shaped and displayed. As PROF5 stated, “Health norms are living online now. It’s not in private any more, it’s performative, in a good sense” (PROF5). Here, performance is interpreted positively. Sharing safety messages, posting about vaccinations, or engaging with government content serves as a digital signal of care and social alignment. These acts multiply; people see them, replicate them, and reinforce them. Behaviour spreads through observation, imitation, and affirmation, creating a cycle of visibility and validation.

This is also evident in PUBL08’s remark that online behaviour shapes offline decisions: “The group behaviour online sets the tone for what’s acceptable offline” (PUBL08). This indicates that the influence of digital platforms extends beyond the screen. What people observe online strongly informs their real-world conduct. When individuals see others adhering to public health behaviours, they gain reassurance and normalisation. Conversely, if safety behaviours are absent in their digital circles, they may begin questioning the necessity of such actions.

These participant voices collectively illustrate how social norms are constructed, circulated, and internalised in digital environments. People rarely act in isolation; their behaviours are shaped by what they witness around them. In the digital age, that means what they see online. From family WhatsApp threads to Instagram stories, individuals are regularly exposed to cues about what is appropriate, expected, or socially endorsed. **These cues guide both thought and action.**

Even those uncertain about the guidance are more likely to comply when they see their digital communities doing so. **This aligns strongly with social influence theory**, which demonstrates that people conform because they want to belong. Compliance becomes a sign of inclusion. People follow public health behaviours not simply for themselves but as a visible expression of collective responsibility.

This form of social contagion allows health-promoting behaviours to spread quickly and widely. Digital platforms become spaces where such behaviours are modelled, reinforced, and legitimised. If health systems can harness this dynamic, they may amplify engagement and strengthen public trust. By fostering constructive digital norms, they can guide communities toward healthier collective choices.

4.6.2 Code: Collective Identity

‘Collective Identity’ refers to the degree to which people emotionally and socially identify with public health messaging and see it as part of their identity and shared national or community ideals. It signals that public health is no longer something remote or driven by government fiat, but something **actively** owned by those who embrace it with pride, something they can work for and stand publicly behind. Via social media, health messages are transformed not into instructions so much as signs of unity, belonging, and care for the community. PUBL15 articulated this sentiment when they said: “I see a post about the Ministry, and I don’t see it as these aloof people. They are one of us” (PUBL15). This crystallises how government health communication is no longer seen as coming from a distant, removed place of state control. PUBL11 added, “The health department’s logo feels like a symbol of reassurance now” (PUBL11), indicating how what was once perceived as official is now regarded with personal significance. PROF4 extended this, saying, “Our brand is not healthcare alone, it’s really our national resilience” (PROF4), emphasising that the public health identity is now associated with national strength and pride. These reflections describe Collective Identity as the emotional and symbolic relationship that people have with the public health system, communicated through language, imagery, and values circulated online.

In this study, Collective Identity illustrates how social media **has** changed the public’s relationship to the public health system of Abu Dhabi. What was once regarded as a formal authority now feels like a collective mission. Participants explained that they no longer see the Ministry or health department as another organisation outside of them, but rather as a part of their own community and values. PUBL15 wrote, “I don’t feel that the guy who records in the Ministry is another world, he’s one of us” (PUBL15). This quotation is indicative of an emotional connection established by communicating in people’s language, valuing their culture, and respecting their fears. Instead of feeling

like passive recipients of public health messaging, people now feel a sentiment of partnership. This transformation is also visual. “It feels now like the health department’s logo is a badge of reassurance” (PUBL11). A symbol that once stood for authority now offers relief and recognition. It becomes a sign not of power but of care. This shift in emotion proves that branding is about more than design; it has meaning. It signals that someone is looking out for you. When people feel this kind of sentiment, it fosters trust and engagement.

It also allows people to share and shape the message in ways that feel personal, just as PUBL02 states: “I love sharing their posts because they are really the affirmation of our society” (PUBL02). Sharing in this sense is about more than passing along information. It is a way of engaging and **expressing** ownership. You feel like you are doing something for the greater good.

Professionals in the research observed this change as well. “There’s a kind of unifying in the way the public health messages are framed, it binds people” (PROF1). The word “binds” is crucial because it implies a strong emotional connection built on steady and respectful communication. The sense of unity comes from messages that communicate shared values such as protection, care, and community. PUBL13 stated this plainly: “We aligned with mission, protection health, extra care health, unity. That’s our identity too” (PUBL13). When public health goals align with public values, people do not feel like they are obeying rules; they feel like they are living their beliefs. This association reinforces the message and encourages people to take action.

This emotional connection is further enriched by the fact that messages via social media can feel intimate. As PUBL18 put it: “Social media made public health personal, that matters” (PUBL18). This quote demonstrates how the style and tone of digital communication converted distant instructions into meaningful conversation. The more people identify with the messages, the more they see themselves reflected in the health narrative, and the greater the engagement. The more they see themselves in the message, the more they believe it, trust it, and share it. This feeling of personal connection makes public health feel real and relevant, not imposed or abstract.

PROF8 gave further insight into that shift, noting, “Branding has moved above corporate, it’s become cultural. We relate to it now” (PROF8). This suggests that the

system's identity has travelled beyond its official image and now mirrors the community's own culture, language, and emotional landscape. The messages are more than just information; they represent shared hopes, struggles, and beliefs. This cultural alignment means the public health brand is something people feel proud to support. It is not viewed as foreign or formal. It is part of who they are.

When we compare these quotes, a common theme emerges people want to connect with public health not because they are being told to, but because they feel emotionally and socially connected to each other. From PUBL02 taking pride in sharing posts, to PUBL13 seeing their values reflected in the mission, to PROF4 calling it "our national resilience," the human connection to the public health system has never felt more personal and more collective at the same time. The system is no longer external to the people; it is among them, even within them.

The experience of community encourages greater involvement. People are not only listening; they are participating and standing with the health system. There are no longer outsiders to the message; they are co-owners of it. Such co ownership renders public health initiatives more effective, supported by a shared sense of responsibility and pride. The message becomes a mirror reflecting the society's values. Collective Identity offers a snapshot of how public health communication on social media in Abu Dhabi has harnessed a strong emotional and cultural bond between the people and the system. Through visuals, language, and shared values, people have come to view the health system as something to which they belong, not merely something to which they adhere. This feeling of identification motivates people to participate, share, and champion messages for health with pride. It transforms health behaviour from an assignment into a common cause, and institutions from remote authorities into reliable teammates. In this way, Collective Identity can act as a social foundation for broad based community health engagement.

4.6.3 Code: Symbolic Alignment

"Symbolic Alignment" describes the degree to which individuals resonate with public health messages that mirror their own personal, cultural, and national values. It is about emotional identification, where people do not only understand the message intellectually but feel that it speaks for them. When the tone, narratives, imagery, and

language of health campaigns align with people's beliefs and aspirations, a message becomes more than a message; it becomes **part of one's sense of self and shared belonging**.

"I repost messages not only to share information but because I support what they stand for" (PUBL04). This illustrates that sharing health messages is about more than transmitting information; it is an expression of belief and identity. Similarly, PUBL12 stated, "Those health campaigns are often about values that we personally believe in, in family, safety, community" (PUBL12). The message is more deeply felt when it represents something people are emotionally connected to. PROF9 added, "If the messaging is consistent with our values, it is reflected back on us" (PROF9). **Taken together, these voices show that symbolic alignment occurs when messages feel like an extension of one's own values, rather than external mandates.**

When public health communication succeeds, it often does so by resonating on an emotional and cultural level with the community. Messages are not only accepted as factually true but experienced as emotionally right, consistent with people's beliefs, pride, and their understanding of what it means to care for others. PUBL04 reiterated: "I reshare messages not only to communicate but to show that I support what it communicates" (PUBL04). **This demonstrates how resharing becomes a symbolic act, signalling affiliation with what the message represents.** It does not feel like an obligation; it becomes a way to express one's commitment to values such as protection, unity, and care.

That emotional connection strengthens when campaigns speak directly to what matters most in people's lives. As PUBL12 noted, "Those health campaigns are often a mirror of values we strongly believe in, family, safety, healthy community" (PUBL12). **In this sense, alignment transforms messaging from instruction into affirmation,** reminding people of the ideals they already hold. This is also reflected in PROF9's observation: "When the messaging mirrors our values, it becomes a reflection of ourselves" (PROF9). When people see their own beliefs and aspirations reflected back, they are more likely to trust, share, and act on the message. The message becomes part of their identity, not an external directive.

Tone and storytelling further strengthen this alignment. PUBL07 shared, “I can identify with the tone and stories that they use. They are speaking our emotional language” (PUBL07). This suggests that people respond as much to how a message feels as to what it says. A warm, empathetic tone can generate emotional closeness and encourage people to see themselves inside the narrative. **When stories resemble lived experience, the message becomes relatable and fosters empathy, making it more persuasive.**

Symbolic words and images deepen this connection. “There’s symbolic power in phrases like ‘Together for Health’” (PROF5). “They unite us” (PROF5). The power of such language lies in its ability to mobilise emotion and convey collective strength. As PUBL14 reflected, “I follow their page not because I feel obliged to do so, but because I identify with their message” (PUBL14). **This indicates that genuine engagement develops when a message resonates with a person’s emotional and cultural reality.**

Health campaign aesthetics also matter. PUBL01 said, “The imagery, the language, it’s all consistent with my notion of what public service should be” (PUBL01). Visuals that include national colours, familiar symbols, or culturally relevant landmarks make people feel acknowledged. **This recognition strengthens trust and motivates engagement,** because the message appears tailored rather than generic.

PROF2 expanded on this: “Their campaigns are successful to the extent that they combine national pride and public duty” (PROF2). This convergence makes public health messaging feel not only informative but meaningful, allowing people to express patriotism, solidarity, and moral care through engagement.

When taking all these voices together, a clear pattern emerges: symbolic alignment is the emotional thread in public health communication. It transforms words and images into meaningful reflections of identity. It allows people to feel not only informed but seen, respected, and included. That sense of inclusion drives loyalty, engagement, and trust. **Health communication becomes most effective not simply when it encourages behaviour, but when it affirms belonging.**

Importantly, symbolic alignment also empowers collective behaviour. When people believe that a message represents them, they are more likely to act together. Acts such as vaccination, mask-wearing, or supporting community initiatives become expressions of shared purpose. Symbolic alignment shows that the most impactful public health messages are those that reflect emotional, cultural, and national values. These messages succeed not only because they are well crafted, but because they speak directly to the heart of the community. People do not merely listen; they see themselves in the message. When this recognition occurs, they are not only willing to act, but proud to do so. Aligned symbolically, public health becomes more than a directive; **it becomes a shared mission shaped by identity, values, and collective care.**

4.7 THEME 4: MOBILIZED COGNITION

Social influence mechanisms can induce both the intention and the behavioural readiness to act via social media. The Mobilized Cognition perspective connects the way social media shapes people's cognitive readiness and willingness to engage in health-related action.

The foundation of all the codes is the recognition that behaviour is not transformed by information alone. Behaviours are shaped by triggers, emotional engagement, social support, and the rehearsal of future behaviour. Social media enables this transformation by making health behaviours visible, emotionally resonant, and easy to follow. Action catalysts thus provoke people in the real world to turn desires into action and thoughts into words. These triggers, such as reminders in tweets or posts by influential users, support health initiatives by making them easily understood and compelling to act upon immediately. However, such interventions are effective only when combined with emotional depth, as illustrated by Influence Valence. Narratives of hope, empathy and shared humanity are more powerful than statistics. Storytelling builds belief, restores trust, and inspires action among viewers. In addition to emotion and urgency, Compliance Drift shows how social cues and conformity shape behaviour. People are more likely to adopt visible health actions,

such as vaccination or mask-wearing, when they observe such behaviours within digital networks. This tendency to conform is driven by a sense of emotional safety and belonging, and when people see others behaving in certain ways, that behaviour becomes more normal, accepted, and expected. This creates a soft nudge that moves people from hesitation to activation by simply reflecting the expected norm and what is considered good in their community.

Completing this behavioural loop is another reinforcing factor. Indeed, the Self-Efficacy Loop shows that once people are emotionally stirred and socially guided, they still seek clarity before taking the final step. **Practical cues**, how-to guides, stories of learning experiences, and even humorous mishaps can help move audiences from confusion towards confidence. Social media content that tells people why to act and how strengthens the sense of efficacy they need to follow through. The more people engage, succeed, and see results, the more they place their trust in the system, reinforcing the loop and encouraging continued participation. Taken together, these codes suggest that mobilizing cognition is not a single mental event but a complex orchestration of thoughts and emotions that emerges over time in response to social cues. Mobilized Cognition involves feeling that health behaviour is urgent, supported, visible, and achievable. Such a process is about changing minds, preparing the heart, guiding the hands, and instilling belief that one's actions matter. Within this framework, social media is both a channel for information and a space for action.

4.7.1 Code: Action Catalysts

“Action Catalysts” are digital calls to action that prompt people to take immediate steps related to health behaviours, such as getting tested, vaccinated, or seeking further information. These cues are not simply informational, **they are motivational**, leveraging desirability, urgency, or convenience. They appear in many forms, including tweets, WhatsApp broadcasts, influencer content, countdowns, and visual alerts. What makes them surprisingly effective is their ability to turn mindless scrolling into active participation. As PUBL08 explained, “Once I saw that reminder tweet from the health authority, I immediately booked my check-up” (PUBL08). This quote captures

the immediacy of a well-timed digital message. PUBL12 echoed this sentiment, noting that “The WhatsApp broadcast from the Ministry was not just information, it was a trigger to act” (PUBL12). These examples show that digital messages go beyond raising awareness, **since they activate decisions**. As PROF7 observed, “Seeing countdowns and alerts on Instagram made me feel like I need to catch up” (PROF7).

Collectively, these reflections describe Action Catalysts as brief, emotionally potent, strategically crafted digital cues that bridge awareness and action. In this study, Action Catalysts illustrate how social media communication can go beyond simply sharing information to actively motivating people to take specific health-related steps. Survey responses reveal that digital messages can act as switches, moving someone from passive understanding to active engagement. PUBL08 illustrated this clearly: “When I saw that reminder tweet from the health authority, I made an appointment” (PUBL08). This kind of instant response is a reminder that digital messages are most effective when they arrive at the right time and offer a clear next step. It was not just the content of the tweet that mattered, but its timing and clarity, which made it a true catalyst. This sense of immediate inspiration is also highlighted in PUBL12’s comment: “The bulletin by WhatsApp from the Ministry was not only information, it was a catalyst for action” (PUBL12). Here, the word “trigger” is key. The message carried emotional or psychological weight. It was not just processed intellectually, it was felt and acted upon.

PUBL16 described a behavioural shift that was driven not by data, but by emotional resonance: “I wasn’t going to take the booster until the story they posted of a recovered patient convinced me” (PUBL16). This shows that narrative, when grounded in empathy and connection, has a powerful effect. An emotional link to someone’s recovery can transform hesitancy into action.

Timing and **visual urgency** are also key components of action catalysts. “Did people really think viewing countdowns and getting alerts on Instagram would make me anxious and stressed?” asked PROF7 (PROF7). The visual style including countdowns, timers, and flashing posts can create a sense of urgency, making people feel they need to act now. This helps reduce processing time and indecision. PUBL14

affirmed the effectiveness of visual urgency, stating, “The flashing graphics and live announcements really work, they push you to decide now as opposed to later” (PUBL14). These comments show that urgency does not have to arise from fear, it can be evoked through visual design and strategic timing. That urgency and accessibility make these cues actionable. Influencers and community leaders also play a critical role in driving action. As PUBL11 noted, “Once influencers started posting their vaccine cards, it became a social cue for many” (PUBL11). This underscores the power of social proof. Seeing others take action makes it feel safe, normal, and encouraged.

In this way, personal influence reinforces institutional messaging, creating a powerful convergence between peer behaviour and official guidance. There is also a sense of effortlessness and instinctive response in PUBL19’s comment: “I remember clicking on the testing location post without even thinking about it. It was so intuitive” (PUBL19). This suggests that the most effective action catalysts are those that feel simple and obvious. When a message is easy to understand, the design is clear, and the next step is evident, people often respond instinctively. It does not feel like a task, it feels like a natural click. That frictionless experience leads to participation.

This immediate reaction is also shaped by how social media generates urgency through exposure and silence. “Social media makes you feel like you are going to miss out if you are not echoing this health call,” PROF6 observed (PROF6). The fear of missing out, or FOMO, a term more often seen in commercial contexts, is increasingly relevant in public health communication. The sense that everyone else is participating and that opportunities may be limited motivates people to act. **This emotional momentum**, rather than fear alone, supports quick decision-making and coordinated responses, especially during health emergencies.

Taken together, these accounts show that well designed external prompts can significantly influence public health behaviour. Whether it is a quick tweet or a compelling graphic or a full influencer campaign, such messages resonate because they counter hesitation, spark motivation, and offer a clear path to the next step. They

carry emotional pull, social currency, visual urgency, and accessibility. Whether it is a countdown, a personal story, or a one-click link to a health service, the most effective action catalysts do not wait for the user to make a decision, **they gently guide it**. Action Catalysts further highlight the importance of urgency, clarity, and emotionally intelligent design in digital health communication. People are more likely to act when messages reach them quickly, speak to them directly, and carry subjectively meaningful impact. Digital posts, when combined with a prompt through stories, visuals, or social proof, can transform ordinary content into moments of action. Such catalysts are essential if engagement is to be converted into something of value. In public health systems, particularly during fast-moving situations like a pandemic, such rapid, intuitive, emotionally resonant communications can be the difference between delay and decision, between merely seeing a message and actually responding to it.

4.7.2 Code: Influence Valence

‘Influence Valence’ refers to the emotional charge in public health communication within digital spaces that makes people feel something strong enough to act. It captures how the emotions expressed in digital messages, whether hope, sadness, or even neutral facts, are **converted into broader emotional responses**, influencing how the message is received, how far it is shared, and how likely it is to prompt action. When a post conveys genuine feeling or a video displays raw emotion, people do not just read or watch it, they connect with it, remember it, and in some cases act on it. PUBL01 illustrated this powerfully: “I cried watching that video of a doctor hugging a recovered patient. It made me believe again” (PUBL01). Human experience like this carries more weight than any chart or statistic. PUBL21 echoed this, adding, “The health videos that showed real stories, particularly losses, captured emotional choices” (PUBL21). These are not just touching moments, **they become emotional prompts that rebuild confidence**, renew motivation, and foster personal investment in public health. As PROF1 put it, “When a post is emotional, it is spreading faster, and it is sticking deeper” (PROF1). These reflections show that the emotional tone of a message, its valence, is just as important as the content. It is often the difference between a message that resonates and one that is quickly forgotten. The Influence Valence construct in this study shows that emotional connection plays a central role in driving public health engagement. Participants consistently reported

that emotional messages, not statistical or fact-heavy ones, influenced their trust and behaviour. PUBL10 drew a clear distinction: “I didn’t see what the percentages showed, but one story about a mother scared me into getting the vaccine” (PUBL10). This implies that data alone is not persuasive. What drives action is emotion, especially when people can see **the lived reality behind a public health issue**.

A mother’s story is not just relatable, it generates empathy and creates a sense of personal connection to the wider health challenge. This emotional connection can help restore trust and belief in the health system. PUBL01 said, “I wept watching that video of the doctor embracing a recovered patient. It made me believe again” (PUBL01). This quote reminds us how emotional exhaustion can be repaired. The hug itself is simple, yet it symbolises solidarity, recovery, and hope. In a cynical or disconnected era, a moment of emotional truth can shift both hearts and minds. It reintroduces humanity into public health messaging and gives it warmth.

PROF9 explored this emotional impact in more detail: “[There’s a mix of empathy and urgency] in the Ministry’s posts, [the Ministry doesn’t] just inform [us], it makes [us] feel” (PROF9). This blend is crucial. Posts that merely instruct may not cut through, but when empathy is added, the message becomes easier to understand and more persuasive. It is not just about telling people what to do, it is about showing them **why the message matters emotionally**. PROF1 reinforced this by saying, “When a post is emotionally charged, it spreads faster and sticks deeper” (PROF1). Emotion drives engagement. When content truly moves people, they are more likely to share it, comment on it, and act on it.

This emotional immersion also supports message recall. PUBL21 noted, “The real health videos, stories mainly of losses, enhanced emotional persuasion” (PUBL21). The key idea here is that these narratives instil **lasting conviction**, not just fleeting sympathy. People who may have been hesitant or afraid of something like vaccination begin to feel more confident simply because the emotional weight of a story helped them grasp the deeper meaning of their choices more effectively than data ever could. Tone matters as well. “The tone does matter,” wrote PUBL06. “There’s warmth and

care in the messaging and it elicits bigger responses” (PUBL06). This observation highlights that the way in which something is said can be as impactful as what is said. Cold, technical language may not be trusted, but when the message sounds caring and sincere, it feels more authentic. It humanises the content and shows that the person behind the message understands the emotional gravity of the situation. Tone creates emotional bridges that link the health system to the public. PROF5 captured this balance well: “Fear is effective but hope is more effective. The best posts balance both” (PROF5). This implies that emotional encouragement does not need to be overwhelming or manipulative. Fear can generate urgency, but hope fuels motivation. Posts that are too negative risk producing anxiety or resistance. However, when messages include positive emotions, such as stories of recovery, community strength, or resilience, they help people feel empowered and inspired. This dynamic between urgency and optimism is what makes emotional content not only effective, **but ethically grounded**.

The secret to effective health communication is not neutrality but **intentional emotional framing**. Whether it is presented as a tearjerking video, a powerful story, or a passionate message of support, emotional content resonates most deeply with people and prompts the strongest responses. It creates a shared experience and builds a connection between the public and the health system. Emotional messages are also more likely to be shared on social media, amplifying their impact. “Influence Valence” implies that the emotional quality of a message matters. Messages that trigger empathy, hope, or even despair are not only informative, **they mobilise and motivate**, shifting public attitudes. Ultimately, they turn passive readers into active participants. These are emotional stories that give meaning to statistics and substance to public health guidance.

By carefully calibrating tone, narrative, and visual content, public health campaigns can build trust, inspire action, and help people feel seen, heard, and supported. Emotion is therefore at the core of what makes messages meaningful and impactful. It transforms public health communication from simple information delivery into **emotionally resonant engagement**.

4.7.3 Code: Compliance Drift

“Compliance Drift” explains how people come to emulate health behaviours simply by seeing others do the same, especially in digital environments like social media. It is a subtle, often unconscious shift toward adherence, not prompted by direct pressure but because visible actions from peers and networks make the behaviour appear normal. People begin adopting health behaviours such as vaccination, mask-wearing or attending check-ups not only in response to official guidance but also because these actions are presented as standard practice within their social circles. PUBL02 **said**, “After everyone was posting shots of their vaccine, a selfie felt normal to post.” This sense of organic involvement is reinforced by the community dynamic of online compliance. As PROF3 **noted**, “When compliance becomes visible, it’s contagious,” illustrating how the visibility of prosocial behaviours encourages others to follow suit.

The central idea in Compliance Drift is that social cues from others act as emotional and behavioural anchors, nudging individuals to align with what appears typical or expected. In this study, Compliance Drift demonstrates how social media platforms, by making behaviour visible, can strongly influence what people perceive as normal or acceptable in the context of public health. People’s decisions are shaped not only by personal judgement or scientific evidence but also by what others around them are doing. PUBL18 **explained**, “You wouldn’t want to be the stick in the mud, if your clique behaves, you do too.” This insight highlights how the desire to belong can be a powerful motivator for health-related action. Rather than positioning themselves outside the group, people tend to follow the norm that emerges within it.

This alignment is not necessarily formal. It stems from observing and responding to social cues. PUBL03 **said**, “Health behaviour went viral, literally. It was all over my feed.” The term “viral” reflects the speed and scale with which these behaviours spread once they are replicated and shared online. Vaccine selfies, hashtags and stories appear repeatedly and this repetition transforms compliance into something that feels not only correct but expected. The behaviour takes on a life of its own, not through instruction but through example. This is a form of soft power that makes health decisions feel socially endorsed.

This influence can be powerful even without deep understanding. PUBL07 **added**, “I didn’t feel like looking into it, I trusted who others were going with.” This reflects how people often replace introspective analysis with social observation. When enough

people in one's immediate circle project confidence and positivity, it fosters a sense of security in imitating them. It is not blind following but a form of listening to the crowd, which serves as a shortcut to truth and safety during times of uncertainty.

The emotional safety tied to compliance is also central. PUBL20 **stated**, "It sure is safe to be following the herd during troubled times." This illustrates how following others is not just about agreement. It is about seeking security. When the surrounding world feels unstable and people are unsure of what to believe, watching others act provides direction and steadiness. It replaces fear with confidence, not through persuasion but through modelling. Observing others sends a silent message that a behaviour is safe and acceptable.

PROF2 **observed**, "It's not peer pressure exactly, it is more like social permission to join the group." This shows that Compliance Drift is not driven by guilt or force. It is about having a door opened. When people see others joining public health campaigns, they receive an emotional signal that it is acceptable to do the same. They feel invited into the behaviour rather than coerced. The action becomes socially approved and psychologically easy. This ease is exactly what makes Compliance Drift effective. The visibility of others practising health behaviours provides silent encouragement.

PUBL15 **explained**, "The group dynamic on Instagram made me feel so confident that it was the right thing to do." This captures how digital environments circulate not only information but also confidence. Every time someone shares a post about getting tested, wearing a mask or getting vaccinated, it contributes to the idea that responsible people take these actions. That emotional reassurance acts as a small nudge toward compliance. These voices form a strong picture of how social media does not merely reflect behaviour but shapes it. Once people see health behaviours being displayed and repeated, they are likely to adopt them too, often without conscious deliberation. This drift toward compliance is not driven by policy or fear but by the quiet logic of belonging. As more people take part, the momentum builds. What begins as individual choices gradually becomes group identity.

Compliance Drift shows how social pressure operates subtly but powerfully within digital spaces. It makes health behaviours visible, repeatable and emotionally safe. Instead of instructing people directly, it shows them what others are doing and creates an invitation to join in. This is a mechanism for achieving widespread participation

without conflict, which is vital for effective public health communication. When compliance feels not only like the right thing but also like the normal thing, people are far more inclined to participate and to motivate others to do the same.

4.7.4 Code: Self Efficacy Loop

“Self Efficacy Loop” describes the pathway through which people build confidence, or self efficacy, to carry out health behaviours once they receive clear, supportive and actionable information, particularly through digital platforms. It shows that empowerment is not just about motivation, but critically about **having enough clarity to know what to do**. When guidance is delivered through simple steps, relatable stories and straightforward instructions, people’s belief in their own ability increases.

As PUBL04 put it, “When the post tells me not just why, but how to do it, then I feel like I can” (PUBL04). This shows that explanation alone is not enough, **because practical support is essential for confidence**. PUBL05 echoed this point, recalling, “The visual step by step guides on social media gave me confidence to take action” (PUBL05). These examples demonstrate that belief is fostered when health systems move beyond simply telling people what is important, and instead show them how to act in response. PROF8 captured this well: “The clearer the instructions, the better, clear instructions help make a lower mental barrier” (PROF8). The theme is therefore no longer simply about confusion, but about clarity and inclusion. This reinforces the idea that anyone can take part in health decisions when they are given the right tools and messages.

The Self Efficacy Loop identified in this study illustrates how social media plays a central role in empowering individuals to take health related action. Participants consistently reported that clear, supportive digital content helped them to feel more confident, more prepared and more willing to practice health behaviours. PUBL04 expressed this plainly: “When the post shows me how to do, instead of just why, I feel like I can” (PUBL04). This highlights a key lesson, which is that motivation alone is not enough. People need direction, and when they receive it, they are empowered to act. **Motivation combined with clarity creates readiness.**

This emotional strength of guidance is reiterated by PUBL05, who explained, “Step by step visual guides by social media gave me the confidence to do something”

(PUBL05). Here, the video is just as important as the message itself. An image or infographic that walks someone through complex actions makes those actions feel manageable. This removes fear, reluctance and doubt, and instead of asking “What if I mess it up” people start thinking “I know what to do.” These small enabling nudges help people move from thinking to doing.

Professionals recognised the value of this too. Clear instructions make all the difference because they lower the mental barrier, as PROF8 pointed out (PROF8). That barrier may be invisible, but it is very real. It is the hesitation that comes from unclear steps. Even when people want to follow health recommendations, they may hold back if the steps feel too complex or unclear. But once uncertainty is removed through effective communication, action becomes easier and more natural.

This cycle of support building confidence is echoed by PUBL13, who stated, “I’m more likely to act when I feel I know what I’m doing, and social media helped with that” (PUBL13). This shows that effective communication does not only shift attitudes, it also builds capability. Social media networks that provide timely, clear and repeated instructions help people move from doubt to confidence. It is not just about telling people what matters, but about showing them how to act without fear of failure.

Real life stories also play a key role in this process. PUBL09 shared, “The pictures of people doing awesome health routines online inspired me to think that I could do so as well” (PUBL09). This illustrates how seeing others succeed can nurture belief in one’s own ability. It is a quiet form of encouragement, a form of social proof that fosters self belief. When someone sees an ordinary person, someone who looks like them, taking action, the behaviour becomes more real and more attainable. These stories transform abstract health messages into relatable and doable scenarios.

PUBL06 offered a strong example of this transition: “I saw a post explaining home testing, and that put me over the edge in terms of feeling empowered” (PUBL06). It is a statement with both practical and emotional clarity. One well crafted post, if clear and informative, can close the gap between intention and action. It offers what people need most, **reassuring direction that reduces doubt**. It shows how digital content can turn hesitation into readiness simply by clearly and warmly showing the way forward.

Another professional perspective, from PROF4, reinforces this idea: “When they post FAQs and visual explainers, it takes out the guesswork” (PROF4). Guesswork is often what stands between intention and action. People hesitate when they are uncertain. But when that uncertainty is replaced with clear answers, diagrams or checklists, confidence grows and action becomes more feasible. This process creates a reinforcing loop. Clarity builds confidence, confidence leads to action and action strengthens trust in the system. This reinforcing cycle was captured by PUBL20, who said, “You just grow more confident when the system stands by your side with clarity and consistency” (PUBL20). This sentiment pulls the point together. One off information is not enough. The system must offer guidance that is consistent, reliable and emotionally reassuring.

Taken together, these voices suggest that the Self Efficacy Loop is about emotional and psychological readiness. The more people feel supported, informed and prepared, the more likely they are to access health systems, follow guidance and support others in doing the same. When a social media post is designed to inform and teach, **it becomes a tool of empowerment rather than a simple announcement**. The outcome is not only increased engagement, but deeper community confidence. To be effective, health communication should be clear, relevant and consistent. The Self Efficacy Loop shows that people are more likely to act when they are given information that is clear and meaningful to them. Fundamentally, people act when they feel ready to act, and that preparedness develops step by step through visual guides, simple instructions, familiar stories and shared social cues. When media empowers people, they act with confidence, independence, and a sense of contribution to a healthier society.

4.8 THEME 5: DIGITAL PRESERVATION

Responses at both system and individual levels to themes and concerns on social media reveal important dynamics. This theme focuses on the reflexivity of public health agencies as they engage with social media dialogue. It contributes to Objectives 1 and 4 and supports Research Questions 1 and 4 by showing how institutions evolve by responding to community priorities.

Echo Governance illustrates that institutions are willing to listen and adapt. From introducing new languages to hosting live sessions, **participants observed that public feedback directly shaped system actions**, reflecting forms of dynamic governance informed by ongoing feedback loops, social proof, and iterative learning.

Sentiment Flux captures the fluctuations of public emotion. Institutions monitored citizen attitudes and **adjusted both tone and communication style** to maintain trust, aligning with the accumulation of normative feedback and emotional tuning as mechanisms for engagement and trust-building. The study shows that public narratives, rather than institutions alone, frequently drive conversations. Topics such as mental health and transparency advanced through public discussion, following the narrative flow of user-centred leadership outlined in Social Influence models. Spoken feedback is part of the co-creative nature of messaging. Participant comments, critiques, and suggestions were not overlooked but **actively incorporated into messaging strategies**, strengthening institutional communication. This **adaptive communication model** highlights the system's capacity to evolve in response to real-time public engagement.

4.8.1 Code: Echo Governance

Echo Governance refers to how community voices are heard and acted upon by public health institutions, especially through social media. It captures the shift from one way governance to a feedback loop where community input shapes institutional actions, language, tone and services. This code marks a transition in public health communication from static top down approaches to **adaptive and dialogic** ones. As PUBL14 noted, "Once we shared apprehensions about the vaccine slots, it took only 24 hours for them to update the app. That felt heard" (PUBL14). This highlights the value of community input leading to visible system level changes. PROF3 also observed, "I saw the Ministry change their messaging on a campaign following backlash. That was a pivot in response" (PROF3). These examples illustrate how Echo Governance involves institutions filtering and reflecting digital feedback to build a more human and responsive relationship between systems and people.

Research findings clearly show Echo Governance emerging as a dominant force in health communication. Abu Dhabi's public health authorities no longer make decisions

in isolation; they are increasingly influenced by the voice of the community, amplified by social media. PUBL14 explained this clearly: “The same day we called out the vaccine slots, the app was different. It was reassuring” (PUBL14). This responsiveness builds confidence. People feel validated when their opinions lead to real changes, such as vaccine slots opening following spontaneous demand. The government appears more accessible and willing to listen, embracing a more participatory form of governance. PROF3 further described, “I saw the Ministry change the campaign message after a strong response and apology backlash. That was a quick recovery” (PROF3). This rapid response shows strategic flexibility in policymaking, revealing institutions that are adaptable rather than rigid. **Such adaptability signals an evolving communication culture**, where health guidance increasingly involves the public in managing emotions and shaping future decisions.

Public impact is also evident in the tone of communication. PUBL07 wrote, “The tone in official sources tends to soften and become ‘less official, more human like’ in general as community input surges” (PUBL07). This marks a shift from distant, bureaucratic language to more personal and conversational styles. When officials adjust their tone, they recognise the public’s emotional state and try to meet people where they are. This creates trust and makes institutions appear more sympathetic. Content formats and communication channels are similarly shaped by public demand. PUBL13 noted, “The real time Q and A feature on Instagram became a thing due to popular demand” (PUBL13). This demonstrates that the communication process is increasingly co created with the audience. People do not only want information, they also want to participate, ask questions and receive answers in real time. Institutions that meet this demand show respect and transparency, reinforcing that governance is a dialogue rather than a one way broadcast.

This shift is recognised within institutions as well. PROF2 explained, “We repurpose our content calendars according to what gets emotional or informational traction” (PROF2). This shows content is no longer planned in isolation, but created based on audience preferences. Emotional traction refers to posts that provoke strong reactions such as joy, outrage or sadness, while informational traction refers to posts that are clear and useful. Both factors influence what content is disseminated. The evolving landscape of public health communication is captured by PROF6: “Public health isn’t

static anymore, it's constantly morphing to fit the social echo" (PROF6). The metaphor of a "social echo" highlights how public voices on platforms like Instagram or Twitter send ripples that institutions must respond to. Monitoring what content gains engagement and what messages evoke emotional responses guides the next round of communication. This ongoing recalibration is not optional; **it has become essential for maintaining trust and relevance.**

Critically, Echo Governance also involves emotional intelligence in responding to criticism. PUBL18 observed, "The government's response to negative comments was measured, they listened and did not become defensive" (PUBL18). This highlights a level of maturity and confidence in the system. Instead of suppressing criticism, the government welcomed it as an opportunity to improve, listening without defensiveness. This creates emotional safety and signals to the public that their input matters, even when critical. Inclusiveness is another vital aspect of responsive governance. PUBL09 noted, "They started including several languages in their releases after people complained about accessibility" (PUBL09). This reflects how institutions adapt to the diversity of society. **Language becomes not only a communication tool but a symbol of recognition.** Multilingual public health messaging makes information more accessible, respectful and effective across communities.

Together, these insights illustrate how institutions are becoming more agile in learning from and reflecting their audience in real time. Echo Governance transforms public health messaging into a living conversation, where every interaction, comment, repost and reaction feeds back into the system and shapes what comes next. This creates a health system that is not only reactive but **increasingly introspective**, responding to public needs as soon as they surface. Echo Governance therefore represents an emerging era of public health systems and communities working in tandem. By adopting multilingual communication, offering real time engagement and reshaping posts based on public sentiment, Abu Dhabi's institutions demonstrate that they are speaking as well as listening, hearing and acting. Social media becomes not simply a platform but a mirror, reflecting public needs and guiding institutional behaviour. Such an adaptive feedback loop strengthens relationships between people and systems,

deepens trust and helps ensure public health communication is timely, respectful and grounded in real experiences.

4.8.2 Code: Sentiment Flux

“Sentiment Flux” refers to the rapid and often unpredictable shifts in public emotion on social media during health crises or periods of heightened concern. The concept illustrates how public sentiment, which ranges from optimism and trust to fear, doubt or frustration, can change suddenly as a consequence of events, tone or even a single post. The digital community becomes emotionally reactive, with platforms such as Instagram, Twitter and WhatsApp acting as mirrors for collective feelings. PUBL04 noted, “One week it’s optimism, another it’s scepticism, it all depends on tone and timing” (PUBL04), highlighting the volatile nature of digital mood.

This emotional noise is not merely background; it directly influences how messages are received and what actions people take. PROF7 reinforced this by saying, “Public mood can flip overnight, especially immediately following a major event or announcement” (PROF7). Sentiment Flux therefore reflects the emotional climate in which public health communication occurs and reminds us that **timing, tone and contextual sensitivity** are as critical as the information itself.

In this study, Sentiment Flux demonstrates that emotional responses online are never neutral but constantly shifting, particularly during public health crises. People react strongly to announcements, tone, visuals and even institutional silence. These emotions spread digitally, generating echo chambers that ripple through social networks and escalate into waves of reaction that institutions must learn to monitor and navigate.

PUBL04 captured this well: “One week there’s optimism, the next, scepticism, it just depends on tone and timing” (PUBL04). This reinforces the idea that even well crafted messages can fail if the surrounding emotional environment is unreceptive. A message that offers hope one day may provoke doubt the next, depending on prevailing feelings and how events unfold. PROF7 added nuance to this by explaining, “Public mood can flip overnight, especially following a significant event or announcement” (PROF7). This indicates that external events such as shifts in

regulations, changes in case numbers or political announcements can rapidly reshape public emotions. Social media accelerates these shifts by providing real time platforms for expressing and amplifying feelings, allowing moods to escalate within hours.

In such a fluid landscape, health communication must be flexible and responsive rather than fixed. The full range of emotions, including fear, joy, frustration and relief, resonates throughout online spaces. PUBL20 observed, “Fear, relief and even joy, social media mirrors the emotional turbulence of a health crisis” (PUBL20). These emotions can be irrational yet remain genuine. A single post or story can spread hope, sadness or anxiety to thousands, highlighting the need for institutions to communicate **facts with emotional awareness**. People want to feel understood, not only informed.

During moments of intensity, this emotional tension becomes even more visible. PROF5 observed, “The collective anxiety of people during lockdowns was evident online, it informed our digital strategy” (PROF5). This demonstrates how institutions had to adjust their tone, shifting from rigid updates to calmer, clearer and more empathetic messages. If communication is too vague or too harsh, it risks amplifying fear rather than reducing it.

PUBL03 illustrated how fragile communication can be: “An off key campaign or hitting a wrong note, and the community responds with frustration lightning quick” (PUBL03). This shows that poorly phrased or ill timed messages can provoke strong backlash, undermining trust and damaging credibility. Public health messages must therefore be tested not only for accuracy but also for **emotional resonance**, since misjudged communication can spread faster than corrections. This emotional rhythm is visible even in routine social media exchanges. PUBL06 commented, “There’s a rhythm to the emotional waves, you can feel it in comment sections” (PUBL06). These comment sections provide real time emotional cues and immediate feedback. By observing these cues, institutions can gauge the community’s pulse and adapt their communication strategies. The speed and openness of these signals give health authorities insight into how messages are landing at any moment.

Acknowledging these emotions with empathy is essential. PUBL11 said, “I have more faith in the system when it shows some empathy, particularly when people are venting” (PUBL11). When people feel distressed or overwhelmed, they are not only seeking

information; they are seeking recognition. Institutions that respond with warmth rather than authority build trust and reduce emotional tension. Even individual voices can shape collective feeling. PROF8 explained, “Every once in a while, one influencer post is going to create a mood swing across an entire digital community” (PROF8). Key figures such as celebrities, affected individuals or respected professionals often hold emotional weight and can sway public sentiment quickly. Public health systems must therefore monitor these influential voices, both for collaboration and for early detection of mood shifts.

Sentiment Flux captures this rapid and potent emotional change transmitted through social media and reveals its long lasting impact on public reactions to health messaging. It also shows that communication must be **emotionally intelligent, timely and sensitive to shifting public moods**. Messages should be carefully phrased and timed to align with public feelings; otherwise they risk being misunderstood or dismissed. Institutions that track emotional highs and lows in real time, listen to feedback and respond with empathy are more likely to maintain connection and trust. By doing so, they can better manage public sentiment even in emotionally charged and uncertain times.

4.8.3 Code: Discourse Contours

“Discourse Contours” describes how themes, ideas and conversations evolve naturally over time in public digital spaces, especially within user generated content on social media. These contours show that the public is not merely a passive recipient of official messages but an active co creator of the public health narrative. Instead of institutions solely driving the agenda, people contribute through stories, hashtags, discussions and reactions, shaping new focal points for health conversations that institutions must recognise and adapt to.

As PUBL10 observed, “Mental health was hardly talked about until the public kept making it, and now it’s a focus” (PUBL10). This illustrates how certain issues gain importance only after the community decides they matter, and it links directly to the way digital spaces elevate repeated concerns. PROF4 echoed this, noting, “Nutrition, vaccine safety and misinformation naturally emerged as dominant themes” (PROF4), meaning these topics gained prominence because they resonated with people rather

than being pre determined by institutions. Together, these examples show that Discourse Contours are shaped by emotional, cultural and societal forces grounded in the public rather than the system.

In this study, Discourse Contours highlight the fluid and crowd driven nature of public health discussions in the digital age. Participants noted that “when people become aware of issues, that’s when change is possible, it all starts on social media” (participant). PUBL10 provided a clear illustration: “Mental health was hardly mentioned until the public raised it, now it’s a focus” (PUBL10). This demonstrates that ordinary users can reshape the health agenda through repetition and persistence. Once enough people cared and “shared their outrage on social media,” as one participant explained, scattered concerns began to converge and gain traction.

This natural emergence of key themes is reflected in PROF4’s observation that “nutrition, vaccine safety and misinformation became the prevalent themes organically” (PROF4). The word “organically” is therefore significant because it shows that the evolution of topics was not orchestrated from above but grew from lived experiences, everyday concerns and shared stories. These bottom up patterns push institutions to adjust their priorities and communications in response to the public mood.

Shifting public discourse also unfolds in stages. PUBL15 explained, “It’s moved on, from access, to transparency, to trust” (PUBL15). This shows how the nature of health conversations evolves with time, beginning with practical concerns, then transitioning to emotional needs such as honesty, reassurance and institutional accountability. These shifts reveal underlying changes in how the community interprets public health messages. The rise of storytelling is one key driver of these shifts. PUBL02 noted, “Stories, not stats, began to be shared. It was a complete narrative change” (PUBL02). This indicates that personal stories carry more persuasive force than graphs or data. When many individuals share similar narratives, they form a collective emotional chorus that pushes particular themes forward. The direction of discourse becomes people led rather than institution led.

Institutions are increasingly learning to monitor this evolution. PROF1 said that hashtags allow them to observe recurring concerns, adding, “It’s incredibly

enlightening” (PROF1). Hashtags act as digital markers of collective sentiment, helping institutions trace what issues are gaining attention and why. This transforms communication from a one way broadcast into an interactive environment where listening becomes just as important as speaking.

Listening also means accepting that the public can set the agenda. PUBL17 explained this clearly: “The public picked the topics, institutions simply rode the tide” (PUBL17). This reflects a reversal of traditional communication roles. Public health institutions are now required to follow the direction set by public discourse if they want to remain relevant and trusted. PUBL19 added, “The story arc is a constantly evolving one, user generated content is constantly influencing that direction” (PUBL19). This underscores the fluidity of digital communication. Themes move, shift and expand depending on what people feel, share or debate. Institutions may begin with one message, yet find it reshaped by the public as the conversation continues.

This dynamic is further illustrated by PROF9, who said, “It used to be a fear driven narrative, now it’s all about recovery and resilience” (PROF9). This tonal shift aligns with broader social changes. Fear may dominate the beginning of a crisis, but as people gain more understanding and support one another, the narrative gravitates toward hope and resilience. Institutions must adapt their communication or risk appearing disconnected.

Taken together, these reflections show that Discourse Contours are shaped by the ongoing negotiation between public voices and institutional responses. Social media empowers people to raise issues, share experiences and reshape public conversations, and institutions that track these patterns, recognise emerging themes and adjust their messages accordingly can build trust and strengthen community engagement. Discourse Contours demonstrate that today, health communication is shaped as much by public discourse as by official announcements. The community, through emotional storytelling, recurring themes and shared experiences, guides the tone and agenda. Institutions that understand and adapt to these shifting contours remain more relevant, more trusted and more effective in guiding public health behaviour. In a digital landscape, **the conversation becomes the compass**, and institutions that read its direction can respond with clarity, empathy and strategic awareness.

4.8.4 Code: Narrative Feedback

“Narrative Feedback” is the process by which public health messages are continuously refined and adjusted based on how the public actually responds to them online. This highlights the dynamic and iterative nature of digital communication, where comments, reactions and engagement levels are **treated as essential inputs** that shape the next round of messaging. This approach reveals a collaborative form of communication in which the audience actively helps shape and reshape the story.

PROF6 summed it up simply: “We use tiny online sites to test messages then scale depending on reaction” (PROF6), showing that institutions now view public platforms as living laboratories. PUBL12 reinforced this by stating, “Comments are like real time focus groups, we tweak messaging almost instantly” (PUBL12), illustrating the rapid integration of feedback to adjust tone, visuals and copy. Together, these descriptions define Narrative Feedback as an adaptive and audience driven rhythm that builds trust and effectiveness by making messages more empathetic, emotionally attuned and respectful.

In this article, Narrative Feedback is one of the clearest examples of how social media has transformed health communication from a one way broadcast into interactive dialogue. Health officials in Abu Dhabi do not merely observe public response, they refine their communication based on what they see in real time. This process values the audience’s voice and recognises communication as a two way street.

PROF6’s statement, “we test messages in small online spaces and then scale based on reactions” (PROF6), captures this well. Before launching major campaigns, organisations use digital platforms to gauge audience interest and then tweak messages for tone, clarity and resonance. PUBL12 compares public comments to “real time focus groups” and notes, “we tweak messaging almost instantly” (PUBL12). This shows how feedback has become central to communication strategy. Rather than following fixed plans, communicators now adapt in response to digital reactions such as likes, shares, silence or criticism. This approach **keeps health messaging aligned with how people actually think, feel and live.**

The shift in tone driven by this feedback loop is noted by PROF2, who explained, “At first no posts were too clinical. Now they are more empathetic because of feedback loops” (PROF2). This highlights a crucial point: factual accuracy alone does not ensure engagement. When people say messages feel cold or robotic, institutions adjust the tone as a sign of respect and responsiveness. PUBL08 described this emotional recalibration clearly: “We chased vaccine fatigue by seeing people disengage, and shifted to uplifting stories” (PUBL08). Here, the public’s declining engagement signalled the need for a more positive, hopeful narrative. The change was not guesswork, but grounded in reactions from the audience. These adjustments helped reconnect people who had emotionally withdrawn.

Over time, active listening and iteration produce more sophisticated communication strategies. PROF5 observed, “Our communications have matured as we have learned from our digital audience” (PROF5). The word “matured” emphasises that institutions have moved beyond basic information delivery to focus on relationship building. They no longer speak at people, they speak with them. Each round of engagement becomes an opportunity to refine approaches and build credibility.

One of the most insightful perspectives comes from PUBL16, who said, “The community essentially co authors the health narrative through reaction” (PUBL16). This reframes the communication dynamic entirely. Public discourse is no longer dominated by institutions, but co created by those who interact with it. Health messaging is therefore fluid and collaborative, reflecting the lived experience of the community.

PUBL21 added an important dimension: “Feedback is more than criticism, it is design input” (PUBL21). This shows a shift in how institutions interpret complaints and suggestions. Instead of seeing negative reactions as obstacles, they now view them as opportunities. Each comment helps refine language, imagery, structure and emotional impact. PROF1 summarised the importance of this cycle: “We learned it the hard way, without closing the feedback loop trust breaks down” (PROF1). This means that simply listening is not enough. Institutions must respond, adjust and demonstrate that public input leads to change. When people see their voices reflected in later messages, trust deepens. When they feel ignored, trust erodes.

Taken together, these voices demonstrate that Narrative Feedback is not a secondary task, but central to how digital public health systems operate today. The story is no longer top down. It is co created, continuously revised and shaped by emotion, engagement and interaction. This makes health organisations appear more flexible, more approachable and more trustworthy.

Narrative Feedback shows that public health communication has become a collaborative and cyclical process. Institutions must listen attentively, adapt quickly and speak with empathy. Each message becomes part of a conversation, and every response becomes an opportunity to learn. This two way engagement creates a synergy in which people and systems become more connected and communication remains aligned with public needs. When health institutions treat feedback not as data but as **co authorship**, they appear more human, more responsible and more effective at guiding public health behaviour.

4.9 DISCUSSION

The next section builds on theoretical foundations from the Health Belief Model and Social Influence Theory to explain the implications of the findings within frameworks of interactive, emotional, and co-constructed public health discourse. Focusing on the mechanics of interactive communication and participatory engagement, this section situates the findings within existing scholarship on behaviour change, trust dynamics, and institutional messaging practices. Drawing on extensive literature, it describes how digital platforms have transformed public health communication in Abu Dhabi from a one-way broadcast of information into a mutual, empowered engagement. The section conceptualises the interplay between individual agency, community norms, emotional resonance, and institutional legitimacy in shaping public health at a micro level through social media. In this sense, the analysis demonstrates how communication has shifted from a **top-down model to a reciprocally shaped digital dialogue**, where meaning is influenced jointly by institutions and the public.

4.9.1 Interactive Communication and Community Participation

The subtheme **Dialogic Resonance** highlights the interactive nature of social media and shows how two-way communication has shifted public health messaging in Abu Dhabi from **a traditional top-down transmission model to a more dialogic and participatory form of engagement**. Respondents consistently characterised this transition as a movement from broadcasting to conversation. As one public participant (PUBL13) stated, “I posted on waiting times and hours later the health department replied directly – you felt they were listening.” This interaction demonstrates the connection between institutional responsiveness, relational trust, and perceptions of inclusivity within governance.

This finding mirrors an international shift in digital healthcare communication. Burgess et al. (2021) emphasise the importance of participatory community engagement as a critical component of effective public health response. Azer and Alexander (2022) similarly note that **bidirectional communication grounded in empathy and timeliness elicits stronger behavioural responses** than one-way information delivery. These principles reflect the emerging Abu Dhabi approach, which appears increasingly human-centred.

Experienced professionals also recognised this transformation. As PROF4 observed, “We learn from the public just as much as we educate.” This demonstrates what Fedorowicz et al. (2022) describe as the integration of “citizen voice” into health decision-making through digital dialogue. The consultative feedback loop described here aligns with Campbell and Rudan’s (2020) argument that meaningful engagement turns public communication from persuasive into collaborative.

The theme **Discursive Pulse** also appeared frequently, referring to the tone and rhythm of communication. PROF5 noted, “If the message is robotic, we lose them; empathy keeps people reading.” PUBL05 similarly reflected that official posts now use “simpler, kinder language — less jargon, more clarity.” These statements reflect Chaudhri et al. (2021), who argue that reputational trust in healthcare communication is strengthened when messages are accessible and emotionally warm. Likewise,

Eghtesadi and Florea (2020) report that empathetic, accommodating online tone enhances compliance and trust during crises.

These narratives indicate that word choice and emotional tone are not stylistic additions but **critical indicators of authenticity and credibility**. As PUBL10 explained, “When you see them respond fast, what it does it builds up trust in them — this is a live thing happening; it’s not automated.” These findings affirm Li et al. (2022), whose work suggests that timeliness and information richness directly support public participation during health emergencies. In this way, Abu Dhabi’s communication approach can be seen as symbiotic, with institutions and publics co-generating communicative meaning.

The second subtheme, **Participatory Echo**, demonstrates how online participation influences institutional behaviour. PUBL03 stated, “We raised the issue of older people not being able to get to vaccine centres. And a week later... it was taken up in a livestream — that’s not much but it is proof our voice matters.” This reflects the democratising capacity identified by Duku et al. (2022), who found that structured community engagement enhances institutional responsiveness and policy legitimacy. PROF8 added that “a single post can incite collective change,” mirroring Ohmer et al. (2022) who observed that interactive digital platforms allow communities to co-produce solutions rather than merely consume them.

Participants described a new sense of conversational power. PUBL16 observed, “I feel like finally I can say something and be heard — agency we’ve never had before.” PROF3 echoed this sentiment: “Empowerment starts when a person feels they have been heard.” These experiences are consistent with Alicea-Alvarez et al. (2016), who found that participatory discussion fosters empowerment by helping individuals see themselves as contributors to social change. Naeem and Ozuem (2021) similarly argue that social media dissolves hierarchical boundaries, creating “horizontal engagement spaces.”

Perceived institutional responsiveness strengthened this sense of agency. PUBL11 said, “The ability to speak, be heard and see change — that’s agency in practice.” This resonates with Farley et al. (2021), who show that visible feedback mechanisms maintain confidence and build trust, particularly among marginalised groups. PROF6

added, “It’s not only one way, it’s being accessible to people, and letting people access us,” echoing Massey et al. (2021), who report that reciprocal communication enhances engagement and sustained compliance with public health guidance.

These digital exchanges also reflect components of the Health Belief Model (HBM). Participants’ experiences indicate strengthened perceived benefits and cues to action when messages were empathetic and interactive. As PUBL04 explained, “If they respond in my language or with empathy then I want to listen.” This aligns with Alhaimer (2022), who found that culturally framed, emotionally grounded messages increase public self-efficacy and comprehension. Alejandro et al. (2022) also show that HBM-based communication increases behavioural intention when delivered through relatable social media content.

Social Influence Theory (SIT) offers further insight. PROF2 reflected, “When people see that their post is shared by the health department, they feel as though they are contributing.” This interactional validation mirrors the internalisation mechanisms identified by Tian et al. (2023), where recognition of user participation strengthens trust and engagement. Thus, media culture shifts from broadcast to what can be understood as a **conversational commons**, shaped by trust, empathy, and co-creation.

References to rhythm and timing indicate that emotional cadence sustains engagement (“the right tone keeps engagement alive,” as PROF5 noted). Chaudhri et al. (2021) show that tone and accessibility build authenticity and relational capital. Jeyaraman et al. (2023) similarly argue that social-media storytelling allows professionals and communities to co-produce narratives of care that reinforce relationships. Respondents also observed a redistribution of communicative power. As PROF7 put it, “They have changed the winners of who can speak. It’s no longer top-down — it’s shared.” This aligns with Pangborn et al. (2023), who documented how social-media storytelling allowed frontline workers to reshape health narratives and foster emotional support. Collectively, these findings show a dual paradigm shift: from hierarchical circulation to dialogic co-creation, and from institution-dominated messaging to **mutually constructed communication shaped by real-time interaction**. This maturation aligns with global evidence for participatory digital health engagement (Mao et al., 2023; Vatcharavongvan et al., 2023).

4.9.2 The Effect of Social Media on Consumer Confidence and the Public Perception

The growing importance of social media as a communication channel in the public health arena provides an active space to build trust, influence perception, and engage the public in informed, participatory discourse. This section examines how people construe digital health messages and how these interpretations shape, and are shaped by, perceptions of institutional credibility and public trust. Drawing on qualitative interview data and thematic constructs, including perceptual cartography, dialogic resonance, and communal ethos on one hand, and mobilised cognition and digital reverberation on the other, the data reveal the **cognitive and emotional landscape** through which health narratives circulate across digital communication spaces.

Respondents emphasised that health communication via social media is particularly effective when it follows a dialogic approach, and where meaning is co-constructed through mutual exchange between institutions and the public. The concept of mutual respect was clearly illustrated when participants described how direct responses to their queries made them feel **both empowered and acknowledged**. This resonates with earlier work by Alamri et al. (2019), who contend that social media enables more direct interaction between the public and health authorities, fostering immediacy, trust, and transparency. Similarly, Heldman et al. (2013) highlight the shift from unidirectional messaging to dialogic communication, wherein the public becomes an active co-producer of discourse rather than a passive recipient of information. These discursive rhythms inform the tone, energy, and compassion of communication. When communication becomes human, timely, and relevant, **rather than distant or overly technical**, it ceases to function as generic messaging and becomes a connection that resonates with people.

The idea of a participatory echo, where public discourse refracts back through institutional channels, further complicates the conventional divide between citizen and sovereign. Multiple interviewees provided examples where social media activity such as comments, stories, or hashtags precipitated policy responses or institutional recognition. This finding aligns with work by Belt et al. (2013), who found that patients are more likely to trust health systems when their feedback is publicly acknowledged.

The reciprocity of such interaction fosters what can be described as conversational agency, enabling individuals to experience agency through the expression of concerns and the visible impact of their contributions. This aligns with Graffigna et al. (2021), who identify empowerment as a key mediator between digital engagement and adherence to health behaviours.

The emotional and cognitive rendering of messages, categorised here under the umbrella term perceptual cartography, reveals that people navigate public health communication using intuitive heuristics rather than statistical logic. Codes such as risk intuition show that emotional proximity to health events, for example hearing about peers who are unwell, often triggers stronger feelings of vulnerability than impersonal demographic data. Similar findings are evident in Health Belief Model studies by Al-Metwali et al. (2021), which show that perceived susceptibility is heightened by anecdotal evidence rather than formal recommendations.

The semiotics of trust, another key code, demonstrates how visual cues such as status markers, language, uniforms, official logos, or even typographic choices contribute to perceptions of legitimacy. These symbolic elements act as heuristics for distinguishing credible from non-credible information, a phenomenon also reported by Hita et al. (2023) in their study of trust mechanisms during pandemic communication. Anticipated benefits, or what participants referred to as the horizon of benefits, emerged as key motivators for action. Members of the public engaged in order to access information, seek reassurance, and identify clear routes to health-promoting behaviours. This finding corresponds with Davies and Mann (2023), who observed that social media campaigns were more effective when they conveyed a clear vision of the health gains associated with preventative action.

However, cognitive dissonance was a recurring theme, and participants frequently expressed confusion and frustration when credible sources delivered inconsistent information. Such inconsistencies undermined trust and led to disengagement. This was also observed by Gesser-Edelsburg (2021), who found that when messaging across platforms lacked coherence, public confidence was eroded. The collective belief in society theme demonstrated the impact of shared norms and social identity on community engagement. Normative adherence occurred when users adopted

health behaviours because such behaviours were visible and part of their network's norms.

Beyond behaviour, collective identity was constructed through symbolic interaction with health organisations. Individuals shared public health content not merely to inform, but to signal group membership — to say, “I’m here, I support this, these values speak to me.” This transliteration of public health branding into civic identity is explored by Sharp (2011), who argues that when public health branding reflects communal ethos and culture, it becomes tied to broader identity structures. One sub-theme of trust, relational trust, concerned the presence of relatable messengers. When viewers saw individuals who resembled them in language, ethnicity, or social context, they reported higher levels of trust in health content. This supports findings by Alhaimer (2022) and Vatcharavongvan et al. (2023), who emphasise that culturally grounded and demographically specific communication strategies foster greater receptivity and trust. Additionally, when the tone of messages was culturally meaningful, users perceived that their identity was respected, leading to symbolic alignment between institutional narratives and user worldviews.

Social media also generated a variety of cognitive and behavioural effects — collectively mobilized cognition. Action-staged reminders, stories, and visual cues had immediate impact on prompting actions such as booking appointments or attending check-ups. This is consistent with Yoo, Kim, and Lee (2018), who demonstrated that campaigns with strong cues to action significantly increase behavioural intention. It was also a matter of affect, meaning the emotional charge of the message. Emotional stories, especially about recovery or loss, inspired action more than statistics. This aligns with Alejandro et al. (2022), who contend that emotional appeals improve memory and have persuasive effects in health communication.

There was also evidence of behavioural contagion, with some participants becoming more motivated simply by observing others. Their responses showed that sharing was intentional due to common practice and peer behaviour, aligning with social learning theory and studies such as Azer and Alexander (2022). Self-efficacy acted as the bridge connecting intention with action. Visual graphics, FAQs, and peer testimony helped individuals feel capable of acting by reducing cognitive burden and increasing confidence. The role of instructional clarity in enhancing self-efficacy is highlighted by

Ayaz-Alkaya et al. (2020), who show that structured message content supports action in health settings.

At a macro level, this analysis highlights how institutions process and respond to public sentiment online. A crucial enabler of trust was echo governance, where feedback loops informed adjustments to policy or communication. When individuals saw that the system responded to their voices, for example by changing apps or refining messages, they perceived it as responsive and human. This form of adaptive governing is supported by Campbell and Rudan (2020), who argue that flexibility is vital for credibility during health crises. Sentiment flux also illustrated how emotionally volatile digital spaces can be, with moods shifting in response to message tone, timing, and context. This requires **communication plans that are adaptable**, as recommended by Landi et al. (2021).

Discourse contours illuminated the organic evolution of health themes on social media, as issues such as mental health or vaccine safety gained traction through persistent public discussion. This bottom-up thematic shaping aligns with Ni et al. (2021), who highlight the importance of grassroots discursive influence. Narrative feedback loops then enable messages to be refined in real time based on audience reaction. Institutions that succeeded were those that listened, adjusted tone, and co-authored public health narratives with their audiences. This iterative co-creation reflects the participatory models advanced by contemporary scholarship.

Taken together, this thematic synthesis demonstrates that social media is not merely a transmission platform but a site of participatory work where meaning, trust, and action are constantly co-produced. Institutional legitimacy is not imposed from above, but earned through **dialogic atonement**, emotional intelligence, and adaptive responsiveness. Public trust in health systems is built not just through information, but through shared identity, responsiveness, and affective engagement, a concept well anchored in contemporary theories of digital health communication.

4.9.3 Social Media Use and Public Health Results

The public health implications of social media engagement in Abu Dhabi are evident in the cognitive, emotional, peer, and design dimensions of digital messaging. The

findings show that when platforms stimulate both reasoning and feeling, they do not simply raise awareness but drive concrete health action. Mobilized Cognition captures this movement from passive reception to active participation through Action Catalysts, Influence Valence, Compliance Drift, and the Self-Efficacy Loop.

Action Catalysts were reflected in participants' accounts of timely reminders and visual cues that acted as behavioural prompts. PUBL08 described booking a check-up after a single tweet, and PROF6 noted that urgency in digital design produces a "drive to decide now." These findings align with Mao et al. (2023) and Li et al. (2022), who demonstrated that timely crisis communication increases public responsiveness. In Abu Dhabi, daily digital engagement provided fertile ground for countdowns, reminders, and interactive notices that translated directly into preventive behaviour.

Affective content, captured through Influence Valence, strongly predicted engagement and behavioural intention. Participants such as PUBL01 and PUBL10 highlighted how personal stories of illness or recovery inspired action, and PROF9 emphasised that messages balanced "urgency and empathy." These insights correspond with Pangborn et al. (2023) and Hita et al. (2023), who found that affectively encoded narratives are more likely to be internalised and shared, aligning with the Health Belief Model (HBM), which posits that emotional cues heighten perceived susceptibility and motivate action (Alejandro et al., 2022).

Social influence processes were equally important. Compliance Drift demonstrated that visible norms normalise compliance. Seeing vaccine selfies, check-ins, or shared routines influenced behaviour. PUBL02 noted that "it's a nature that's easy to follow," and PROF2 described this not as pressure but "social permission." This mirrors reference group behaviour (Kelley, 1955) and the collectivist, digitally literate social environment of Abu Dhabi. Studies such as Graffigna et al. (2021) similarly argue that visible community endorsement enhances behavioural uptake.

Yet motivation must be paired with capability. The Self-Efficacy Loop emerged when participants described how step-by-step guides, live demonstrations, and infographics reduced cognitive barriers. PUBL13 said, "If I feel like I know it, I'm more likely to do it," and PROF8 stressed that clear instructions "lower the mental barrier." This aligns with HBM constructs regarding the centrality of self-efficacy (Ayaz-Alkaya et al., 2020).

For a diverse population, simplified visuals and accessible formats ensured that people not only understood what to do but felt able to do it.

Public absorption of health messages is also participatory. Respondents framed their digital habits as socially grounded, shaped by observing others and recognising the cost of inaction. This reflects Dunn et al. (2018), who showed that youth engagement with prevention campaigns was driven by emotional resonance and peer endorsement, and Azer and Alexander (2022), who identified similar dynamics during COVID-19. The Abu Dhabi context stands out because institutions repeatedly adapted tone and content in line with digital feedback, generating a sense of co-authorship that strengthened trust. Cultural responsiveness further amplified trust. Participants noted higher confidence when messages included multilingual content, culturally familiar imagery, and national symbols. This supports Burgess et al. (2021), who emphasise that culturally attuned engagement enhances intervention success. Abu Dhabi's demographic diversity required a broad communicative repertoire capable of speaking to Emiratis and expatriates alike.

Sustained engagement was also important. Feedback loops allowed institutions to remain attuned to public sentiment, with repeated exposure to reminders, stories, norms, and guidance transforming momentary engagement into ongoing behavioural reinforcement. Duku et al. (2022) similarly argue that continuous community engagement is essential for influencing behaviour. HBM constructs such as perceived threat, benefits, and efficacy were shown to be dynamic, continually recalibrated through digital interaction. Mao et al. (2023) found that behavioural cues delivered through relatable narratives reinforce perceptions of risk and benefit—patterns strongly reflected in Abu Dhabi.

The convergence of immediacy, emotion, peer influence, and empowerment forms an ecosystem where digital engagement becomes behavioural architecture. Action Catalysts trigger behaviour; Influence Valence deepens emotional meaning; Compliance Drift normalises behaviour through visibility; and the Self-Efficacy Loop builds capability. Together, these mechanisms explain how digital communication shaped real-world action. Health behaviour emerged from an interdependent system of sense-making and value alignment. Normative adhesion was visible as individuals adopted health behaviours to integrate within their digital community, echoing

institutional isomorphism (DiMaggio and Powell, 1983). Social media made norms visible and transformed preventive practices into forms of social currency, consistent with Davies and Mann (2023).

Trust functioned as a critical compliance moderator. PUBL05 felt reassured when Emiratis featured in campaigns, reflecting findings by Freire et al. (2023), who argue that messages resonate more strongly when delivered by culturally proximate messengers. In a multicultural environment, authenticity and representational closeness were essential. Eghtesadi and Florea (2020) highlight the importance of culturally embedded crisis communication, a dynamic clearly present in Abu Dhabi. Echo Governance was central to building trust. Participants described visible policy changes following online feedback, an example of institutions demonstrating respect and inclusion. This is consistent with Franceschini and Marletto (2019), who argue that iterative institutional responsiveness strengthens trust. In Abu Dhabi, adaptations in language, app design, and campaign formats reinforced the perception of a listening system.

Sentiment shifts further influenced institutional decision-making. Rapid emotional fluctuations in comment sections, hashtags, and influencer posts shaped communication strategy. Pangborn et al. (2023) describe such narratives as barometers of collective affect, requiring institutions to recalibrate tone accordingly. Participants described how issues raised online were later addressed in livestreams or policy updates, reinforcing civic ownership (Duku et al., 2022). Cognitive dissonance emerged when conflicting messages circulated. Participants like PUBL13 described confusion when guidelines contradicted each other, echoing Hita et al. (2023), who linked inconsistent information to anxiety and erosion of trust. Institutions that maintained clear, consistent, emotionally intelligent messaging—such as those in Park et al. (2016)—secured higher engagement.

Symbolic alignment and Benefit Horizon showed that messaging must be aspirational as well as informational. Campaigns invoking themes of unity and resilience resonated deeply, positioning compliance within a shared cultural narrative. As Bornkessel et al. (2014) and Navarro et al. (2020) suggest, storytelling transforms public health messages into shared human truths. Ultimately, social media is a dynamic space where norms, emotions, and identities are continuously negotiated. In Abu Dhabi,

digital engagement contributed directly to higher adherence to vaccination, screening, and preventive behaviour. Public trust emerged through responsiveness, emotional resonance, and cultural attunement. These findings show that when used authentically, social media becomes not only a communication tool but a mechanism for behavioural change and health citizenship.

4.10 SUMMARY

Chapter 5 also elaborates on how public health communication in Abu Dhabi has been reframed through its convergence with social media, transitioning from a one-way apparatus of information distribution to a participatory model that is enacted collaboratively by institutions and the public. The chapter synthesizes five major themes in order to clarify how digital environments generate the emotional, cognitive, and relational conditions necessary for eudaimonic public health behavior. The chapter shows that on the basis of these findings public health communication has emerged as a dialogical communicative event resonating emotionally and culturally, reflecting shared values and beliefs in local communities.

The first important observation is that digital communication has redefined how people live and understand public health messages. Hearing participants consistently reported how responsiveness, tone and accessibility made institutional communication feel more human-like and trusting. Messages were no longer viewed as distant or administrative, but as conversations that took into consideration people and their emotions and cultural contexts. Trust was established, the message understood better and a sense of joint ownership developed through this relational process. The public's responses were observed to impact greatly on the way that institutional posts are drafted, phrased and delivered thus pointing towards a co-constructed communication model in which social voices genuinely influence how discussions lead.

A second key result pertains to the emotional infrastructure of digital public health communication. Content that had high emotional valence was consistently cited as being more memorable and impactful than numerical data. Recovery, loss or community solidarity stories increased perceived susceptibility to and inspired empathy for disease consequences, suggesting an urgency to act. Participants

explained how emotions of messages were either more or less congruent with the emotions of the public mood and that this strengthened or weakened trust as a result. This indicates that emotion attunement is not a matter of aesthetic preference but essentially a strategic necessity in how it shapes behavior and institution regulation.

Simultaneously, social signals and peer effects were observed to be strong drivers of behaviour. Observability of others behaviours – for example vaccine selfies, check-ins or shared testimonies – normalised adherence and generated a perception of social permission. Health behaviours were frequently discussed by participants as being undertaken because they are what other people do or socially practised within their network of society. This process of normative modelling was reinforced by cultural affinity and identification, particularly when communications used linguistically or culturally coherent communicators.

The cognitive aspect of communication also proved to be important. When messages provided clear instructions, step-by-step guides and practical reassurance people were more likely to act. Self efficacy was raised when uncertainty decreased and an act became doable by visual demonstrations, FAQs, or simple language. Participants viewed these types of direction as instrumental in translating intention to action and emphasized the necessity for clarity, smart design, and ongoing encouragement.

The chapter demonstrates that public health communication is not static but subject to feedback loops, emotional sw troughs and discursive fashion. Public opinion shaped campaign shifts, as organizations harnessed online reaction to tweak their approaches in real time. This reflective system generated an experience of civic engagement and reinforced the relational tie between the government and community, so that health was accepted as having more common ground; health for all became ideally.

CHAPTER 5: CONCEPTUAL FRAMEWORK

5.1 INTRODUCTION

The rise of social media as a disruptive influence on healthcare communication has reframed the way public health systems interact with communities, **nudge** behaviour change, and build trust. In Chapter 5, the “Digital Reverberations in Health Communication” framework is introduced and explained as an analytical tool, nested on multiple levels and conceived precisely to analyse the interconnectedness of health communication, community engagement, and public health branding from **ADTDAD’s** point of view. This framework is based on the constructs of the Health Belief Model (HBM) and Social Influence Theory (SIT), but provides a synthesised view that includes the psychological, social, and technological aspects of digital engagement. The model represents communication not as a linear process, but as a feedback loop — a reverberating system in which digital interactions perpetually mould and are moulded by public opinion, institutional narratives, and behavioural reactions. In doing so, it lays a solid groundwork for examining the ways in which social media functions as a trigger and facilitator of participatory public health discourse. This chapter describes the theoretical foundation, profile of constructs, and structure of the framework, and locates its application within the context of a multicultural, digitised public health system in Abu Dhabi. Through a combination of empirical research and theory-building, the chapter situates this framework as a pragmatic tool for policymakers and health communicators seeking to optimise the role that digital platforms can play in promoting public trust, increasing engagement, and improving health outcomes.

5.1 THEORETICAL FRAMEWORK INTERACTION WITH THE CONCEPTUAL FRAMEWORK

Figure 5.1, **Digital Reverberations in Health Communication**, is the product of, and a direct response to, the themes identified — a distillation of this study’s **processual** chapter in visual form. **Chapter Five 5.1 Thematic Analysis** set forth in Chapter 4 marks **that it** is not a conceptual model, but an empirically based representation in which every element, link, and direction of effect stems from codes/themes identified during systematic thematic analysis. The model **illustrates** the circular and multifaceted **forms through which** communication takes place among Abu Dhabi

public health agencies and communities through social media. It models a conversational environment that develops meaning, trust, and interaction through dynamic dialogic exchange, emotional resonance, and behavioural feedback. This figure is developed in the interpretivist logic described by the methodological framework, within which theory arises inductively from data rather than being imposed a priori (Naeem et al., 2023).

In the middle of the figure, Dialogic Resonance, which encapsulates the interpretive centre of the data, typifies dynamic reciprocity between health authorities and the community, traversing back and forth prominently in participant narratives. The dialogic core is encompassed by the concentric territories of Symbolic Exchange, Discursive Pulse, Participatory Echo, and Conversational Agency. These layers directly parallel the primary and sub-themes of Chapter 4. For example, Symbolic Exchange accounts for participants' descriptions of mediated meaning-making and emotional attunement to institutional health discourse, and Discursive Pulse measures the pulse or flow of online conversation as detected through sub-themed sentiment and tone. So too do Participatory Echo and Conversational Agency spring from participant accounts of the **rejection of viewing communication as passive reception**, in which participants **actively engage in shaping and redistributing digital material**. Together, these elements constitute the communicative life cycle of public health messages on social media — from institutional dissemination to public response, interpretation, and behaviour change.

The top part of the framework, Digital Reverberations, is the source of discourse and its propagation through social media. Such behaviour is linked directly to the code Narrative Feedback from Chapter 4, in which participants reflected on how posts, replies, and shares shaped the reach and meaning of a message. The paired outflows of Narrative Feedback and Discourse Contours give a visual sense of the feedback loops shaping broadcast into conversation. This top-down and bottom-up dynamic represents the dialogic structure of digital communication implicated in the data, whereby social media functions both as a transmitter of information (distribution channel) and a locus of participative reflection.

The left of the figure is Communal Ethos, **the** community sense of social norms, moral codes, and shared values among Abu Dhabi's multicultural public. It is based on the

conceptual evidence that public trust, cultural match, and emotional tone underpin engagement. **When** we respond to the cultural and moral dimensions of people's lives, then codes like Relational Trust, Symbolic Alignment, and Normative Adhesion are found here. It is in line with **findings** from Alhaimer (2022) and Burgess et al. (2021), suggesting that public health communication is effective only when it becomes culturally credible and empathetic. Conversely, the right-hand panel displays Perceptual Cartography, and it illustrates how people navigate and interpret whatever health cues they receive. It encompasses such elements as Benefit Horizon, Cognitive Dissonance, Risk Intuition, and Trust Semiotics, all of which were observable in participants' stories, where they articulated how their beliefs interacted with issues around credibility and risk when consuming online health information. The reciprocity of Communal Ethos and Perceptual Cartography, on one hand, and the balance between collective identity and cognitive self on the other, constitute, in turn, a horizontal axis of this model.

Mobilised Cognition is the lowest-level component of the framework, which reflects the conversion of digital context to real-world behaviour. It is also related to other sub-themes such as Influence Valence, Action Catalysts, Compliance Drift, and the Self-Efficacy Loop, which altogether provide a structure describing how taking part, trusting, and emotionally resonating translate into intention or action. This aspect of the framework captures the end point of the social media-based communication process, in which perception is translated into performance and understanding translates into health practice.

The form-and-content unity is reflected in the theoretical combination of HBM-SIT as a basis for interpreting the findings. The HBM provides the cognitive and motivational element of health behaviour, while SIT adds the social and normative processes that show how collective influence affects individual choices. The concepts of perceived susceptibility, **perceived** severity, benefits, and barriers are positioned most clearly in the P1 (Perceptual Cartography) space. Statements made by participants suggested that online forums, infographics, and shared posts served as cognitive maps through which they evaluated their susceptibility, the severity of disease, and the effectiveness of protective measures. This is in accordance with the study of Vatcharavongvan et al. (2023) and Youssef et al. (2022), in which online engagement increases risk

perceptions and leads to protective health behaviours. Risk Intuition and Cognitive Dissonance in the model therefore map directly to the relationship between perceived susceptibility and barriers that is, a psychological conflict between awareness and resistance.

Benefit Horizon also records the perceived benefits and cues to action, given that messages on social media such as stories with positive emotional content, visual information, and testimonial evidence — encourage health behaviours. This is similar to the results of Alhaimer (2022) and Mao et al. (2023), who showed the effectiveness of affectively laden health messages across social media, particularly ones couched in culturally appropriate and outcome-focused language as digital cues to action for boosting efficacy beliefs and normalising target behaviours. HBM's emphasis on confidence and agency individuals' belief in personal capabilities — is illustrated through the Self-Efficacy Loop (lower panel of the model), which is in line with HBM's focus on "confidence" and shows how successful exposure to others' beliefs and perceived responsiveness of institutions **can** strengthen or weaken these beliefs in their own personal capability. In doing so, the framework illustrates how social media **conceptualises** each HBM component, transforming intangible cognitive constructs into tangible communicative exchanges.

Social Influence Theory is the thread that runs through the model's left-hand-side constructs: namely, Communal Ethos, Symbolic Exchange, and Participatory Echo. The phenomena of compliance, identification, and internalisation, as characterised in SIT, are again echoed in the present focus group participants' discussions of social norms, peer acceptance, and image credibility. Finally, Normative Adhesion and Collective Ideals describe the social underpinning of behaviour, where individuals perform in congruence with the perceived norms of their group. This is supported by Naeem and Ozuem (2021), who examined how professionals digitally **negotiate** their identities in public health. Analogously, Symbolic Alignment relates to the SIT concept of identification people taking on the attitudes or behaviours of others whom they admire or consider high-status. Participants commonly recounted replying to messages shared by their revered doctors, religious authorities, or Emirati influencers, highlighting how messengers' attributed credibility facilitated adherence to recommended behaviours. This is consistent with Odunsi and Farris (2023), who

recently discovered that message source credibility significantly enhances behavioural intention when used in pandemic-related health campaigns.

The idea of Participatory Echo illustrates how social inspiration and community dialogue are interdependent. When participants like, comment on, or re-create content, they are affirming collectively and reinforcing socially both normative expectations and individual stake/commitment. This interactional feedback is the implemented version of SIT's internalisation: repeated practice turns external persuasion into inner conviction. In this sense, the arrows connecting Participatory Echo with Conversational Agency represent the transition from social reinforcement to independent action. Trust, situated at the interface of these two domains, becomes a mediating mechanism that links HBM and SIT. Studies such as Tetteh et al. (2022) and **Alhaimer** (2022) verify that trust functions as a simultaneous antecedent and consequence of health communication: it improves message receptivity while being enforced by regular, transparent engagement. The model captures this by showing that Relational Trust feeds into both Dialogic Resonance and Mobilised Cognition; trust accounts for the fact that dialogue is sustained (rather than switching off when a conversation is over) and also that someone might so wholeheartedly believe in what they have been convinced of through discourse that it affects their behaviour.

The diagram in Figure 5.1 is also a conscious interpretation of the original model introduced in Figure 2.1. While the original design of health communication in Figure 2.1 presents this process in a linear fashion linking government institutional messaging to public response, our revised model now encompasses circularity, reflexivity, and emotional resonance. The first developed model built on existing literature to posit a relationship between social media use and community engagement in public health. But empirical evidence revealed that the interactions are not unidirectional; they constitute a dialogic spectrum in which institutions and members of the public co-construct meaning through ongoing digital feedback. Figure 5.1 thus reframes communication away from transmission and towards a dynamic ecology of resonance, participation, and co-production. As opposed to the static relationship between variables presented in Figure 2.1, this model is dynamic, showing how messages are constantly enacted within affective and social forces in Abu Dhabi's digital assemblage.

Figure 5.1 is also different from that in **FPSC**, as it illustrates the complete cycle of communication (generation, dissemination, interpretation, and behavioural production), with ERIC at the top and Mobilised Cognition at the bottom of the matrix. This extension was based on two findings: firstly, respondents conceptualised social media as both a feedback mechanism and a trigger of behaviour. It directly answers the findings of Davies and Mann (2023) and Tian et al. (2023), who emphasise that digital interactivity serves as both an evaluative and motivational framework. The thematic evidence of ongoing message circulation, variation, and emotion at work underpins the need for a departure from the linear concepts in earlier models in this thesis to one more cyclical or recursive. In that sense, Figure 5.1 describes a theoretically motivated development based on empirical findings of social-media-facilitated health communication in the culturally diverse context of Abu Dhabi.

The arrows and relations between the elements in the framework show a two-directional flow of influence: horizontal, from community values to individual cognition; and vertical, from discourse to action. The cross-links A through D from Communal Ethos to Perceptual Cartography represent the balancing act between cultural norms and personal interpretation that has bearing on issues of trust, credibility, and meaning-making. **Digital Reverberations** — the vertical series from (top) Digital Reverberations to (bottom) Mobilised Cognition — indicates how messages transform in symbolic form into bodily action. Each intersection (e.g., between Normative Adhesion and Compliance Drift) is a place where social influence intersects with health belief — flipping determination to behaviour. This recursive trajectory corresponds to the interpretive notion in which meaning and behaviour are co-constructed by interaction, congruent with the study's epistemological position and method.

They also provide insight into how the impact of the message is moderated by emotional tone and cultural proximity. The alternation between Benefit Horizon and Cognitive Dissonance in Discursive Pulse parallels the psychological tug-of-war people are engaged in when attempting to judge whether a given health communication is genuine. Sensory empathy and sense-making factors were reported as key factors in whether people adopted or discarded messages. These results support the work of Chaudhri et al. (2021) and Egtesadi and Florea (2020), **who found** that the emotional resonance of messages and linguistic accessibility have a

significant impact on message credibility and memorability. Discursive Pulse, then, becomes the emotional heart-rate of communication — the way we can gauge the vitality of ideas. When trust, empathy, and cultural congruence are in tune, their heartbeat strengthens, drawing out behavioural activation; when they are out of sync, engagement diminishes.

By incorporating these theoretical and empirically derived components, the model captures a dynamic communication ecology that encompasses more than the structural dimensions often found in health belief or social influence models. It locates the two theories in the mediated urban landscape of Abu Dhabi, negotiated by multiculturalism, technological entanglement, and institutional re-creation. Although the HBM provides understanding of individual perceptions, and SIT helps explain group-level influence, these concepts independently fail to reflect recursive feedback loops and co-creative processes that accompany interaction on social media. By incorporating these approaches within a dialogic and constructionist system, Figure 5.1 overcomes this limitation and provides a more comprehensive paradigm in which beliefs, emotions, trust, and community identity are treated as interrelated variables that jointly contribute to communication outcomes.

In conclusion, this model has been developed to explain the progression of Abu Dhabi's public health communication environment — from knowledge delivery down a channel to the use of digital platforms for engagement. It illustrates the creation of a Third Space through social media, where sender and receiver are no longer separate entities; they share the communication process — a mutual adaptation process involving reciprocal learning, relationship, and trust within this meta-concept. The figure represents both a synthesis of theory and evidence that cognitive, social, and emotional factors in public health communication are intertwined elements of the same recursive system. It builds on the existing framework to translate abstract theoretical suppositions into a systematically grounded model that is both connected to participant experience and generative of new hypotheses, showing how, in an era of digitised life, engaging effectively with public health entails more than simply disseminating information; it requires resonance, reciprocity, and relational trust. In its overlapping, cyclical structure, Figure 5.1 is illustrative of the combination of beliefs, influence, and

discourse in Abu Dhabi's social media environment that coalesce to create a living, breathing form of health communication.

Based on the research objective — namely, to examine the potential and effectiveness of social media in enhancing communication and engagement — this approach offers an all-encompassing perspective to consider, appraise, and improve digitally mediated interactions in public health. At the heart of the framework is the arena of Dialogic Resonance, representing the dynamic dialogue between public health institutions and the community. This core is enveloped by three further layers — Symbolic Exchange, Discursive Pulse, and Participatory Echo — culminating in Conversational Agency, which describes how public discourse is shaped, circulated, refracted, and ultimately embodied by culture. These phases also illustrate how the passive receipt of information can be converted into action and behavioural modification, aligning with the goal of this study to explore whether community concerns and health outcomes are moulded by online engagement.

Situated above this dialogic nucleus, Digital Reverberations represent the genesis and iterative amplification of public health messaging on social media. It encompasses two primary directional outputs: Narrative Feedback and Discourse Contours, which describe how public reaction and content sharing contribute to feedback loops that shape future communications. This highlights the network effect not only as a means of distribution but also as a mechanism for refining messages at scale in response to user engagement — demonstrating the effectiveness of social media as a communication vehicle.

On the left side, Communal Ethos reflects the shared values and cultural narratives of Abu Dhabi's diverse population. It links to Symbolic Alignment, Relational Trust, and Normative Adhesion, underscoring that community identity and trust are foundational for engagement. These dimensions illustrate the need for public health communication to intersect with local cultural beliefs in order to promote compliance and collective behaviour, consistent with the research objective of investigating community dynamics.

On the right side, Perceptual Cartography represents the individual's internal compass for interpreting health information. This includes Benefit Horizon, Cognitive

Dissonance, Risk Intuition, and Trust Semiotics — cognitive tools individuals use to assess the credibility and personal relevance of health messages. These elements are key to understanding the effects of social media on both individual and community-level health behaviours.

At the bottom of the model, Mobilised Cognition is revealed as the end result — the shift from digital exposure to actual public health behaviours such as vaccination uptake, screening, or health activism. This includes forces such as Influence Valence, Action Catalysts, Compliance Drift, and the Self-Efficacy Loop, which bridge the gap between behavioural psychology and social influence theories. This figure therefore offers an inclusive and cyclical illustration of how digital interactions, community norms, social identity, and institutional communication feed into health behaviour and engagement. This finding aligns with the study's objective: to demonstrate how social media functions as a communicative infrastructure and participatory arena through which Abu Dhabi's health system is able to address public health issues, build trust, and enhance outcomes through effective digital participation.

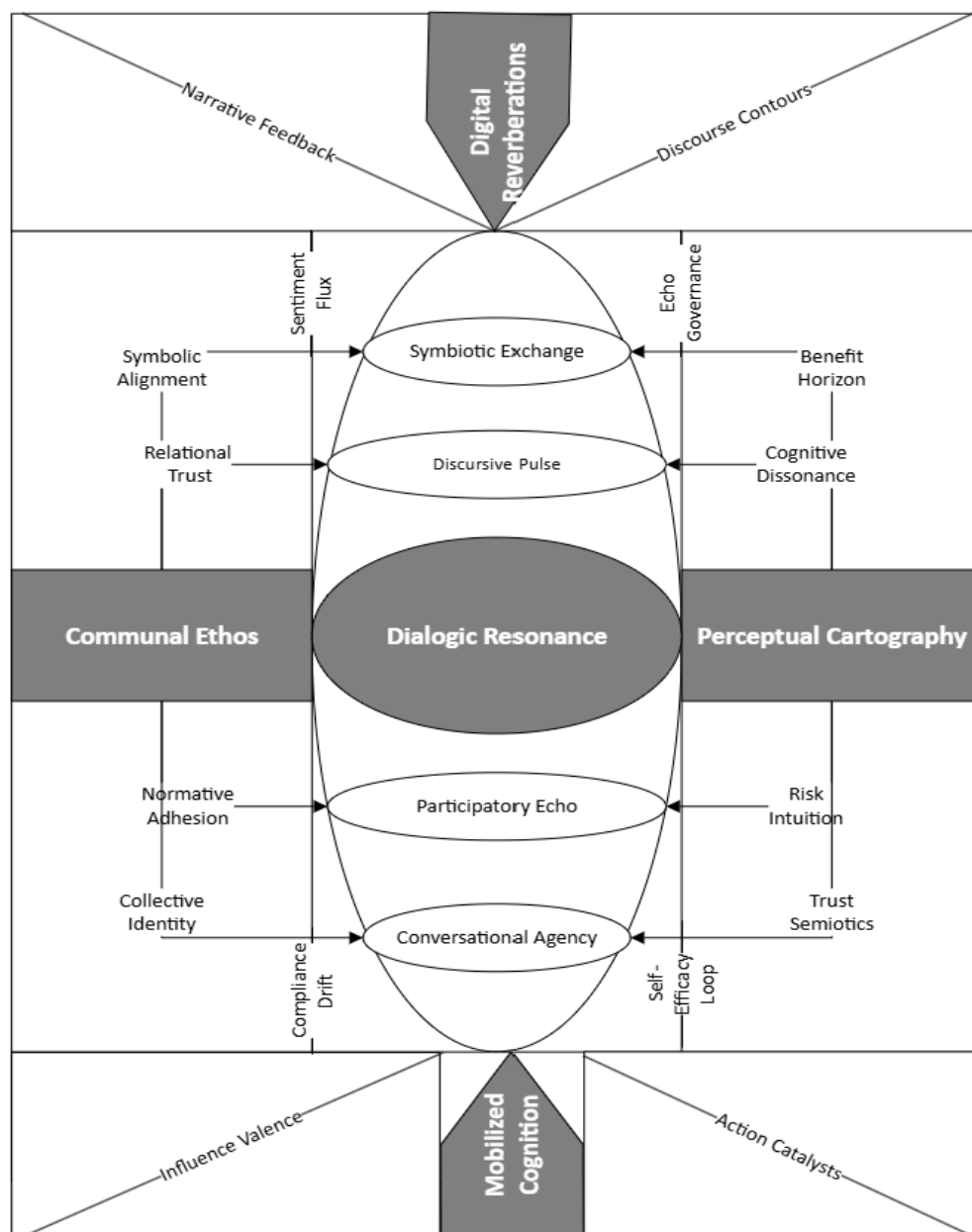


Figure 5-1 Digital Reverberations in Health Communication

As Digital Reverberations in Health Communication illustrates, the framework provides several *lenses* through which to evaluate social media as a communication and engagement mechanism in Abu Dhabi's public health system. It weaves together a multitude of issues and theoretical considerations to analyse the fluid dynamics and dialogic exchanges of feeling, belief, trust, and collective action on social media. Grounded in emerging findings from our primary data and connected with established health theories such as the Health Belief Model and Social Influence Theory, this

model helps explain how digital communication spawns a participatory spread of health awareness, public opinion, and behaviour change.

The concept of Digital Reverberations sits at the top of the framework, illustrating the feedback loop and cyclical processes of social media discourse. It is both the source and megaphone of public health stories. The streams of Narrative Response and Discourse Flow course outward, implying that once something is posted on social media, it enters public space and reacts to — and is reacted upon by — the public. This aligns with the cues to action in HBM, which refer to external stimuli that prompt health behaviours. Social media, in this context, becomes a perpetual conduit for **cued** information and affective messaging.

The vertical central spine of the framework consists of three encased domains — Symbolic Exchange, Discursive Pulse, and Participatory Echo — which lead toward and envelop Conversational Agency, while the overarching notion of Dialogic Resonance provides the wider context. This defines the essential real-time, dual-exchange loop of communication between society and public health, mirroring the construction of both mainstream and alternative narratives. It closely reflects perceived benefits and self-efficacy in HBM, as people interact with and use information they deem both beneficial and actionable.

Symbolic Exchange, located at the apex of the dialogical funnel, refers to the mediated exchange of meaning and values. This domain highlights Sentiment Flux and Echo Governance — the ways in which emotional tones evolve and messages persist across networks. These concepts mirror internalisation and identification in Social Influence Theory, which suggest that people assimilate messages when they align with personal values or group norms. From this emblematic convergence emerges the concept of Relational Trust — an index at the junction of Communal Ethos and Dialogic Resonance.

5.1.1 Discursive Pulse displays the rhythm and life of public discourse

The layer below this, Discursive Pulse, displays the rhythm and life of public discourse. It is the *beat* of the community's interpretive interaction with health stories. Benefit Horizon and Cognitive Dissonance impact the pulse, reflecting the struggle between

the prospect of benefits and the internal conflict that can result from conflicting beliefs. This dimension closely mirrors perceived severity and perceived barriers in HBM, in that inconsistent information — or content that does not align with one's beliefs — can deter behaviour change.

Further themes such as Participatory Echo also surface here, referring to messages of possibility and potential that are not simply received but **re-articulated** and spread through community action. Normative Adhesion links to compliance in Social Influence Theory. This suggests that, in many cases, participation in the *festivity* is motivated either by a desire for social aggregation or due to community expectation. Similarly, Community Identity, grounded in **Communal** Ethos, relates to identification, where users model and mimic the behaviours of the perceived in-group.

At the bottom of the funnel is Conversational Agency — a personification of empowerment through conversation. This reflects the capacity for people to direct the conversation, not just absorb it. This aligns with HBM as a form of self-efficacy — the belief in one's ability to effect change — and with the final stage of internalisation in Social Influence Theory, where behaviour is fully integrated into the self and aligned with a shared norm. At a more precise level, Mobilized Cognition represents the outcome phase where thought becomes action. It is facilitated by the convergence of Influence Valence and Action Catalysts, which emerge through the combination of personal perceptions (HBM) and social demands (Social Influence Theory). Mobilized Cognition represents the dawning of action, as trust, belief, identity, and engagement combine to generate public health action — such as vaccine uptake, adoption of preventive behaviours, or advocacy for healthcare policy.

5.1.2 Communal Ethos and Perceptual Cartography

Communal Ethos and Perceptual Cartography are the theoretical “edges” against which the side panels provide resistance. The former refers to the values, beliefs, and moral anchors of the community, while the latter reflects the cognitive maps that individuals use to navigate risk, trust, and belief in the credibility of information. These two expansive domains serve as bookends for the entire enterprise, situating online interaction within both a collective process of meaning-making (ethos) and an individual process of sense-making (perceptual cartography).

Risk Intuition and Trust Semiotics, located within Perceptual Cartography, are especially relevant to HBM's perceived susceptibility and the role of source credibility, respectively, as they mediate individuals' interpretations of messages concerning their personal health risks and institutional trust.

This complex interplay is further emphasised through the horizontal axes of the framework. On the left, the relationship between Symbolic Alignment, Normative Adhesion, and Compliance Drift illustrates a trajectory from passive value agreement to behavioural enactment. On the right, the path from Benefit Horizon through Risk Intuition to Self-Efficacy Loop indicates that individual health action is influenced by awareness of benefit, interpretation of risk, and the perceived ability to act.

The cyclical design of this model — fuelled by digital echoes — suggests that communication end products (e.g., health behaviour change or public engagement) are not final outcomes but rather re-enter the ecosystem as fresh storytelling inputs. This recursive interplay underscores the dynamism of the social media health communication space, where every public response informs future strategies and continually reshapes the contours of discourse.

The framework positions social media not as a mere broadcast medium but as a dialogic space — a site of symbolic exchange and cognitive mobilisation. It provides an example of how Abu Dhabi's public health system can harness digital engagement dynamics to build trust, promote behavioural change, and improve health outcomes. By combining the Health Belief Model (HBM) with Social Influence Theory (SIT), it links individual-level psychological determinants with broader sociocultural and communicative processes, offering a holistic framework for assessing and guiding social media's role in public health engagement.

Underlying digital-era health communication is the interplay between individual cognition, social environment, and technological intervention. Abu Dhabi's public health system, marked by high levels of digital penetration and demographic diversity, presents a unique context in which to understand how individual and collective attitudes intersect through social media to shape engagement, trust, and health outcomes. This framework — evolved through the integration of HBM and SIT — offers

a comprehensive, multilayered model for analysing the dynamic interplay of health communication, public participation, and behaviour change in the emirate.

According to the Health Belief Model (HBM), individual health behaviour is typically determined by a combination of perceived susceptibility, severity, benefits, barriers, cues to action, and self-efficacy (Ellithorpe et al., 2022; Al-Metwali et al., 2021). These factors can determine the extent to which individuals undertake health-related behaviours. In the context of Abu Dhabi, the circulation of health messaging via platforms such as Twitter, Instagram, and government-endorsed WhatsApp alerts engages these perceptual dimensions in quite distinct ways. For example, during the COVID-19 pandemic, the public's willingness to be vaccinated or comply with quarantine measures was largely shaped by perceived vulnerability (susceptibility), perceived severity of illness, and belief in the effectiveness of interventions (vaccines) (Vatcharavongvan et al., 2023; Youssef et al., 2022).

As such, social media can be said to function as a trigger for behaviour and a tool to reinforce self-efficacy using visual stories, testimonials, infographics, and influencer messaging (Mao et al., 2023). Aspects such as the credibility of information sources, the clarity of content, as well as the alignment of social media with personal beliefs and cultural norms, are key factors in public receptivity (Tian et al., 2023; Odunsi & Farris, 2023). For Abu Dhabi's diverse population, this points to the need for culturally tailored messages to promote behavioural change. This also addresses common criticisms of the HBM, particularly its limited cultural sensitivity and emotional range (Alejandro et al., 2022; Nooriani et al., 2019).

On one hand, perceptions of health threats and benefits (HBM) lead to perceptions of social norms and influence (SIT), forming the circular structure proposed in this study. Interactive communication flows between these two domains, with social media acting as both modulator and amplifier. It provides both a content context and a social context, where ongoing acts of acknowledgement — likes, shares, comments, endorsements — routinely reinforce and reshape perceived realities. During times of uncertainty, individuals often turn to online groups for validation and guidance as a form of informational social influence. Such behaviours impact how messages from institutions such as Abu Dhabi's Ministry of Health are heard, interpreted, and acted

upon. These messages are filtered **through** both belief systems and social influence processes.

5.1.3 Underpinning of Social Influence Theory

Social Influence Theory, first established by Kelman (1958), extends this model by focusing on compliance, identification, and internalization as key behavioural mechanisms. These mechanisms can be strategically utilised by health authorities in the emirate of Abu Dhabi. For instance, health promotion campaigns that employ respected local figures or community members as spokespersons foster identification and compliance among target audiences. When value-based messages are delivered consistently, people may come to see themselves as the kind of person for whom taking preventive health actions, such as vaccination or regular screening, feels more authentic and meaningful. This perspective becomes even more relevant when considering the various types of social influence: normative influence, driven by the need for social acceptance, and informational influence, driven by the desire to make correct decisions (Liu et al., 2020). In Abu Dhabi, normative influence is reflected in collective behaviours such as participation in community vaccination campaigns, while informational influence is evident in expert-led live question and answer sessions and the circulation of fact-checked infographics about symptoms and prevention (Naeem and Ozuem, 2021). These forms of influence reinforce each other. Interaction with trusted content in the public sphere promotes greater uptake of health-promotive actions, contributing to improved health outcomes at the population level.

Community engagement is also a key feature of this iterative framework for comprehensive CPG development. Social media shares news and allows for feedback, expression of concern, and peer-to-peer discussion. Hita et al. (2023) examined media cues to action during the COVID-19 pandemic and demonstrated how content shared on social media platforms can encourage behaviours such as panic purchasing or social distancing. In Abu Dhabi, tracking these trends could enable public health officials to identify misinformation early in the process before addressing community concerns and altering communication strategies. This is where the circular nature of the framework is crucial. Engagement through social media contributes to a more nuanced construction of health perceptions and social influence

dynamics, which in turn shape individual behaviours and open up further communication needs. This feedback loop means that effective public health communication is a continuous and adaptive process. For instance, if a social media campaign reveals high levels of perceived barriers, such as fear of side effects, health officials might respond by sharing the stories of individuals who chose to be vaccinated by debunking rumours or myths (Odunsi and Farris, 2023; Davies and Mann, 2023), or by partnering with medical influencers who can increase self-efficacy.

Role of trust as a mediating variable between HBM and SIT
The model acknowledges the role of trust as a mediating variable between the Health Belief Model and Social Influence Theory in Abu Dhabi. Confidence in health institutions, bolstered by regular and culturally relevant communication, enhances the perception of benefits and strengthens social norms (Tetteh et al., 2022). People who have confidence in their health authorities are more likely to follow guidelines, share accurate information, and encourage others to do the same. Studies have shown this consistently (Alhaimer, 2022). In this way, trust is not only an outcome but also a condition, situated at the midpoint of the conceptual circle.

The model also considers the emotional and cultural dimensions of behaviour, components frequently criticised as lacking in the HBM. In rapidly developing Abu Dhabi, where the population is diverse, interpretations of health and illness are refracted through various cultural lenses. Social media provides a valuable platform for customising information to incentivise behaviour change across languages, cultural values, and demographic segments. Campaigns that are mindful of local values, include familiar figures, and use locally relevant metaphors have the potential to change not only attitudes but also behaviours (Nooriani et al., 2019; Youssef et al., 2022).

Crucially, the model suggests that the impact of social media on health communication relies on a level of harmony between personal health beliefs and social influence. Communication strategies should target both, appealing to cognitions, such as perceptions of health risks and benefits, as described by the Health Belief Model, as well as social norms, emotional resonance, and community identity, as described by Social Influence Theory.

The framework presents several considerations for the public health system in Abu Dhabi. First, content must specifically address relevant Health Belief Model constructs, such as decreasing perceived barriers or increasing cues to action. Social campaigns can also leverage influencers and local figures to reinforce compliance, identification, and internalisation. Message content should incorporate social proof and normative references, framing protective behaviour as beneficial, socially expected, and endorsed. In addition, platforms should facilitate feedback loops to help health authorities capture emerging sentiment, correct misinformation, and adapt strategies in real time.

The circular confluence of the Health Belief Model and Social Influence Theory in this conceptual framework provides a strong and dynamic vantage point from which to study and improve social media's role in Abu Dhabi's health communication and engagement. The model is not linear but recursive, recognising that perceptions of health, social factors, and communication behaviours continuously evolve in relation to one another. This comprehensive model offers policymakers, public health professionals, and communicators a theory-driven framework for creating, testing, and refining health promotion strategies that are effective, inclusive, and culturally relevant in an era of pervasive digital media use.

5.1.4 Integration of Social Media Communication, Corporate Social Responsibility (CSR)

This thesis has implications for understanding how social media communication, community engagement, and public health branding are intertwined within Abu Dhabi's healthcare sector. For ease of analysis, a multidirectional model is proposed that includes social media as an endorser, social media as a facilitator, and social media as a disseminator. This model is cyclical and integrated, marking a significant departure from more traditional top-down public health communication models by giving greater voice and agency to the patient.

Interactive internet communication via social media is the central feature of the model and acts as the main driver for community involvement and public health branding. Community engagement is framed as a dynamic and interactive process through

which patients and the public communicate and express ideas and insights via social media while also responding to suggestions and comments with tangible implications for practice.

In contrast, public health branding emerges through enduring and meaningful digital exchanges, helping to build reputation and trust in the health services offered in Abu Dhabi. This interrelated view is also supported by the literature, which documents the historical evolution of social media in health from a source of information in the early two thousands to a platform for engagement and branding today (Belt et al., 2013). This evolution is essential to understanding the significance of the conceptual framework in the present day. As Chaudhri et al. (2021) and Gesser-Edelsburg (2021) suggest, the branding of health services through digital engagement strategies is critical to creating a robust and trusted public health system, especially in multiethnic urban locales such as Abu Dhabi.

The addition of the concept of CSR into a discussion of social media and public health communication must be explicitly justified in order for it to align with the theoretical underpinnings of the Health Belief Model and Social Influence Theory that drive this study. Within this context, CSR is seen not simply as a corporate or marketing construction but as a communicative strategy that serves to uphold institutional legitimacy, social trust, and collective identity. These concerns are central to public health engagement in Abu Dhabi. The allusion to CSR is therefore conceptually warranted provided it is conceived of as a tool of social influence and relational trust among public institutions and citizens.

Within Abu Dhabi's health ecosystem, CSR-based communication includes government-driven programs, healthcare collaborations, and corporate sanctioned awareness efforts that work toward improving community health. Such initiatives can also help provide health information, but more broadly they serve as a way to promote institutional social responsibility through the use of social media. This institutional image in turn feeds public trust in the efficiency, good intentions, and ethical orientation of the organisations concerned. Trust, as integrated in this work, represents the mediating construct between individual beliefs described by the Health Belief Model and collective behaviour described by Social Influence Theory. CSR that communicates health messages contributes to this trust because such information

signals responsibility, empathy, and public commitment from organisations. These qualities facilitate credibility, message acceptance, identification, and conformity within the community (Injodey et al., 2019).

Theoretically, CSR fits with Social Influence Theory in the mechanisms of compliance, identification, and internalisation. When a health organisation, whether public or private, undertakes CSR activities such as funding vaccination drives, supporting wellness campaigns, or backing health education initiatives, it does more than display moral responsibility or community integration. These efforts are interpreted by audiences as genuine acts of concern and public solidarity. This perception drives normative influence, causing people to adopt behaviours that align with health actions endorsed by society. It also fosters identification, as community members see themselves reflected in shared moral values. Repeated exposure to socially responsible digital communication can lead to internalisation over time, through which health promoting behaviours become part of an individual's personal or cultural identity. CSR therefore constitutes a permissive environment in which trust is both created and sustained, bolstering the upward spiral between Communal Ethos and Perceptual Cartography that is represented in Figure 5.1.

Theories of planned behaviour highlight that CSR also has a cognitive component that resembles factors found in constructs from the Health Belief Model. Humanising institutions and increasing perceived benefits of participation reduce perceived barriers. Participants are more likely to perceive messages from health workers as prosocial and trustworthy when health organisations demonstrate social responsibility, resulting in greater willingness to act on the advice. This is consistent with Ellithorpe et al. (2022) and Youssef et al. (2022), who found that credibility and trust were critical in mediating the pathway from awareness to behaviour change. Accordingly, CSR-based communication operates as a cue to action, a signal that activates both belief and behaviour through affective trust and moral reciprocity.

Given these theoretical overlaps, CSR is not an additional factor but a latent aspect within the construct of social influence and public trust in the model. Its legitimacy is evidenced by its broad socioethical focus, which accompanies health communication in Abu Dhabi and considers institutional reputations and civic responsibilities as interconnected components of public engagement. Rather than being removed, a

reference to CSR should be preserved but can be reformulated as a trust-building mechanism linking organisational ethics with influence over behaviour. In this distilled sense, CSR represents the ethical reach of social power, describing how institutional benevolence reconfigures into social legitimacy and lived health citizenship.

The “Digital Reverberations in Health Communication” model offers a theoretical reconceptualisation of public health institutions and their relationship to the communities they aim to serve through digital media. The integrated model, developed to combine the core constructs of the Health Belief Model and Social Influence Theory, provides a comprehensive theoretical account of the psychological and social drivers of public health behaviours in the digital context. It therefore positions social media as both a communication tool and a transformative ecosystem in which information, identity, and influence converge to shape behaviour and perception.

Applied to a specific population such as the digitally connected and multicultural population of Abu Dhabi, the customisable nature of the framework offers both an effective model and a map for strategic health communications in a specific time and context. It highlights the necessity for dialogic communication, community involvement, and culturally relevant messaging to drive successful health promotion and institutional trust. With its focus on evidence-based interventions and real-time responsiveness, the model underscores the importance of feedback loops that adapt messaging based on public sentiment and behavioural outcomes.

In light of challenges ranging from misinformation to the erosion of public trust in health systems globally, the application of this framework in Abu Dhabi serves as a valuable example and major research finding. This chapter presents the foundation both as a set of guiding principles and as an operational blueprint for delivering inclusive, trust-based, and impact-oriented health communication in the age of digital disruption.

5.2 SUMMARY

Chapter 5 offers a first-of-its-kind theoretical and empirical overview submission on how social media interaction constructs, reforms, and sustains public health communication in Abu Dhabi. We present the Digital Reverberations in Health Communication framework, which reimagines communication as a perpetual and

iterative process of co-creation of meaning between public institutions and communities through sustained digital interaction. The chapter frames social media not as an adjunctive artifact, but part of a communicative ecology where trust, identity, affect and behaviour meet. Through the lens of health belief model and social influence theory, this chapter demonstrates how health communication is a two-way street- both psychological and sociocultural—where perception-individual difference as well as norms-collective identity serve to determine public health in real time.

The model is based in the constructed thematic analysis found in Chapter 4. It is the interpretive nexus between Dialogic Resonance, that signifies lived in exchanges where public voices and institutional responses anthropomorphically move from platform to platform. Around this core, the model traces how meaning is generated through Symbolic Exchange, Discursive Pulse and Participatory Echo — the emotional, linguistic and behavioural flows that typify Abu Dhabi's digital health presence. These layers demonstrate how this communication travels from when it gets shared, to its interpretation, during resharing, through emotional processing and finally into behaviour. The cyclic nature of the model is in stark contrast to older linear communication theory, which suggests that public reaction is an end point – when it's actually a new beginning that will inform future messaging.

The chapter contributes to the broader understanding of how people make sense of health messages by Perceptual Cartography -the way in which individuals map informational cues, such as: risk perception; trust markers; cultural resonance; and personal relevance. At the same time, the concept of Communal Ethos indicates that cultural identity, collective beliefs and moral expectations imply a normative condition which in turn directs communication. The horizontal and vertical dimensions of the model express the ongoing process of negotiation between personal beliefs about health, on one side, and influences from others surrounding one, on the other; it illustrates that individual cognitive (mental) capacities and social forms of organization are inextricable in digital engagement.

Mobilised Cognition at the bottom of model represents the behavioural output where how people engage with digital media ends up as real-world behaviour. These factors include the impact of emotional narratives, the motivational value of prompts, the role of perceptible norms and visible targets and the potency of self-efficacy when people

become more confident in their ability to accept health behaviours. The chapter also demonstrates that trust is serving as a mediating factor connecting cognitive appraisal and social influence. Trust is also enhanced when the institutions shows cultural understanding and emotional warmth, which directly increases perceived benefits and social conformity.

The chapter finishes by showing how social media communication and CSR for trust building of CSG for forms participatory engagement to make up a relational culturally embedded and adaptive public health communication system. The model provides for policy makers and health communicators an evidence-based and contextually relevant framework to inform strategic approaches that are inclusive, dialogic, and culturally anchored. In this respect, Chapter 5 provides a theoretical base from which to begin to consider the role of digital ecosystems in strengthening public trust, enabling reciprocal social and behaviour change & lending greater effectiveness to present day public health activities.

CHAPTER 6: RECOMMENDATIONS AND CONCLUSION

6.1 INTRODUCTION

This study aimed to examine the function and impact of social media as a platform for communication and community engagement in the public health system of Abu Dhabi. The results suggest a varied but hopeful environment in which digital interaction intertwines with public health outcomes. The analytical lens of “Digital Reverberations in Health Communication,” generated in this study, explains how digital communication operates not as a unidirectional transmission but as a reciprocal process, both relying on and continually shaping societal understandings, practices, and organisational dynamics. In Abu Dhabi, which is a digitally sophisticated and demographically heterogeneous environment, social media has emerged as an active forum for both dialogue and participation. This setting allows health institutions to engage with the public, build trust, and motivate health-related behavioural change in ways that are immediate, inclusive, and more dynamic than those available through more traditional modalities.

The framework is built around the notion of dialogic resonance between health organisations and the public, linking communicative actions in both directions. Thematically, it is organised around a surrounding ring of constructs, such as symbolic exchange, discursive pulse, and participatory echo, which account for how meaning, affect, and engagement circulate and transform within digital ecosystems. These constructs, although theoretical, are rooted in the specific features of Abu Dhabi’s health communication context. For example, during public health emergencies like the COVID-19 pandemic, platforms such as Twitter, Instagram, and WhatsApp played a key role in spreading cues to action. They shaped risk perceptions and established social norms, and they found resonance through storytelling, testimonials, and culturally appropriate content. This created momentum towards behavioural intention and societal mobilisation.

This study aimed to examine the use of social media as an instrument of interactivity, community involvement, and trust-building in the Abu Dhabi public health system. The study was conducted qualitatively and interpretatively, using not only interviews with professional and community actors but also focusing on how dialogic communication

affects public understanding, engagement, and health-related behaviour. The results highlight the extent to which communication between public health institutions and citizens has been revolutionised by digital platforms, especially Twitter, Instagram, and WhatsApp. Across these platforms, not only one-way transmissions but reciprocal, participatory, and affective communications are forged that reframe the organisation-community relationship.

The study found that conducive health communication in Abu Dhabi rests on three intersecting factors: cultural saliency, institutional legitimacy, and dialogical adaptability. Culturally relevant messages promote inclusivity and relevance, credible communication builds trust, and a dialogic approach to interaction through feedback loops fosters responsiveness to public opinion. Combined, these factors turn virtual engagement into a power that changes behaviour and delivers tangible results such as awareness-raising, health guidance compliance, or institutional reputation enhancement.

The constructed theoretical mechanism of this process, Digital Reverberations in Health Communication, depicts these phenomena as a mutually serving and circular model. It is evidence that social media operates as a communication structure, a trust-creating device, and a space for participation. This finding challenges conventional linear models of public health messaging and situates Abu Dhabi as a flagship for digitally enabled governance aimed at promoting citizen participation and community welfare.

6.2 CONTRIBUTION

6.2.1 Theoretical contribution

Perhaps the essential theoretical contribution of this project is the development of a model, Digital Reverberations in Health Communication (see Figure 5.1). This is a model that integrates the Health Belief Model and Social Influence Theory, through an empirically based synthesis built upon themes from actual experiences. It envisions digital health communication as a recursive ecology rather than a linear information cascade. By integrating the Health Belief Model's focus on individual cognition and perceived benefits with the Social Influence Theory's emphasis on social identification

and normative pressure, the model offers a hybrid explanatory platform that can explain both psychological and social determinants of digital engagement.

This integration is important because, until the current study, the Health Belief Model and Social Influence Theory had been infrequently integrated in a digital communication environment, especially within a multicultural Middle Eastern public health setting. What makes this contribution unique is that it shows that when these theories are combined, they provide an explanation of how people are motivated to act upon health messages and how digital environments support action through trust, emotion, and social validation. Figure 5.1 presents an image of this process in the form of a living, dialogic cycle in four domains: Dialogic Resonance, Communal Ethos, Perceptual Cartography, and Mobilised Cognition.

Each domain is a layer of meaning that was found in the empirical data. Dialogic Resonance refers to the essence of mutual exchange in which institutions and citizens co-author stories through dialectic interaction. Communal Ethos offers a moral and cultural account of such interactions by explaining how trust and identity are intensified through common values and local forms of symbolism in Abu Dhabi's society. Perceptual Cartography theorises the interpretive practices by which people assess risk, normative legitimacy, and benefit, mediated through both rational cognitive calculation and emotional heuristics. Mobilised Cognition brings these interpretive actions to the level of behaviour, showing how digital engagement moves awareness into actionable health practices.

The originality of the framework also lies in its structural invention. By shifting from a linear depiction to a circular, self-referential loop in Figure 5.1, the constant feedback effects of communication in digital spaces are represented. Messages do not have an end but become the beginning of other messages; in this perspective, audience feedback becomes part of message production. This recursive motion is a more accurate representation of the nature of social media communication, in which institutional and public discourse co-evolve. In doing so, the model provides a theoretical lexicon to articulate feedback-based, emotion-modulated communication in contemporary networked societies.

The theoretical development of Dialogic Resonance also advances the Health Belief Model and Social Influence Theory. Traditional health models tend to conceptualise communication as persuasion, while Dialogic Resonance reconceives it as a collaborative act based on emotional attunement and symbolic exchange. It highlights that resonance, rather than reach or frequency, is the key to understanding how health communication works in the digital era. This theoretical step addresses a gap in the literature that has long failed to consider affective and participatory communication within behavioural theories.

Perceptual Cartography also introduces a new idea by mapping how people navigate the digital health space. It transcends cognitive rationality by integrating trust, emotions, and cultural interpretation as coordinates that influence decisions. The framework contributes to health communication theory by noting that people do not receive messages in the same way; they build meaning through social validation and personal salience. For a city like Abu Dhabi, where language, faith, and identity intersect, this insight has significant explanatory power.

The third theoretical novelty, Mobilised Cognition, explains how behaviour does not arise from single exposures but from repeated, emotionally charged interactions. It integrates the self-efficacy concept from the Health Belief Model with the internalisation mechanism from Social Influence Theory, suggesting that behavioural adoption is driven less by rational deliberation than by sustained dialogic reinforcement. The model extends the conventional notion of behaviour change by reframing it as a process shaped by culturally embedded communicative momentum.

In short, Figure 5.1 is the most comprehensive conceptual framework of the thesis, integrating cognitive, social, and emotional processes into one dynamic system. It is novel because it transforms static behavioural constructs into a responsive, interactive framework for health communication in a digitally networked and culturally plural society.

6.3 CONTRIBUTION TO PRACTICE

This study offers public health managers a responsive communication model that prioritises two-way, real-time interaction with the wider community. By applying the *Digital Reverberations* framework, health communication in Abu Dhabi can move beyond a traditional broadcast approach to an updated approach that listens, reacts, and adapts to public sentiment. Such a move enables a more agile response to misinformation, clearer policy messaging, and the cultivation of improved public trust. Managers can apply the model to design communication strategies that are responsive to feedback and which promote a more transparent, trust-based dialogue aligned with both institutional aims and community expectations.

Second, the study offers practical insights into how institutional trust might be better enhanced through socially engaged media. It demonstrates how public health institutions can use culturally resonant storytelling, influencer collaboration, and peer-to-peer conversation to create stronger *relational trust*. To achieve this, communication teams should be trained in sentiment analysis and trust-building techniques across diverse demographics. By fostering relationship-based communication that reflects community values, public health managers can help make institutions appear more trustworthy. In turn, they can promote behaviours that are more aligned with health guidance.

Third, the research highlights the potential of digital engagement to co-create health systems. Through concepts such as *Participatory Echo* and *Conversational Agency*, the study outlines how digital platforms can support citizen involvement in policy and service design. Managers might go on to use tools like online consultations, polls, and open feedback channels to gather public perspectives and incorporate these into decision-making. This participatory process supports patient-centred care and fosters shared ownership in terms of tackling public health issues.

Fourth, the framework points to a need for targeted interventions supported by data-driven strategies. Insights from *Perceptual Cartography* can enable managers to interpret behavioural patterns, emerging community concerns, and perceptual shifts. In addition, tools such as social listening and analytics dashboards can help tailor content to specific groups based on age, ethnicity, or risk perception, thereby rendering communication more relevant and effective. In diverse contexts like Abu Dhabi, such precision is key to avoiding the pitfalls of one-size-fits-all messaging.

The model stresses that long-term behavioural change depends on *trust-building* and *value congruence*. The concept of *Mobilised Cognition* implies that campaigns should both encourage immediate uptake and also embed lasting public health habits. This requires consistent messaging, visible positive outcomes, and the reinforcement of collective norms. Public health managers should incorporate behavioural insights into communication planning. They should set objectives that go beyond clicks or impressions toward sustained behavioural impact to demonstrate multi-generational engagement that supports long-term public health outcomes.

6.4 LIMITATIONS AND FUTURE DIRECTIONS

6.4.1 Limitations

The most obvious limitation of this study is the fact that it has taken place in a specific context. Theoretical and empirical links are made with the sociocultural and institutional context of Abu Dhabi, which is a linguistically diverse, high digital literacy ecosystem with a centralised governance structure. These particular characteristics are informative but **do** confine the transferability of the model to other political frameworks where communication infrastructure, cultural norms, and public healthcare systems are significantly divergent. For example, the assumptions of this research, **such as** that a state has strong capacity and well-developed digital infrastructure or a multilingual population, may not be applicable to less centralised or relatively underdeveloped technology settings. Accordingly, the Digital Reverberations in Health Communication model should be considered as contextually situated rather than universally generalisable, and its applicability more generally must be empirically evaluated.

Another limitation concerns the **time-related** design of the study. This is a cross-sectional qualitative study; it provides a “snapshot” of the communication dynamics and not a “moving picture” over time. Public engagement, trust building, and digital participation are dynamic activities **that take place** in response to changing policies, crises, and cultural accommodation. The lack of longitudinal data limits the extent to which we can comment on how attitudes and behaviours change over time or in response to repeated institutional communication. Although the theory implies cycles, the empirical specification does not take cyclicity over time into account. Such a temporal constraint implies that while the model offers interpretive depth, it cannot

claim predictive validity with respect to how trust will evolve in the long term, **nor** how behavioural adaptation will unfold.

Moreover, the research is limited by the nature of its qualitative data size and scope. Although sufficient for interpretive depth, the number of interview participants is not representative of the complete demographic heterogeneity of the population in Abu Dhabi. Some groups might be under-represented, **such as** transient workers, elderly residents, or those with low digital literacy. The use of purposive sampling, while logical in a qualitative framework, diminishes the statistical generalisability of this investigation. This methodological design valued meaning and interpretation over coverage and measurement, but **it** may bring limitations in terms of demographic representation and generalisability.

Another limitation is that the proposed model has been conceptually, not empirically, validated. The model combines elements of the Health Belief Model and Social Influence Theory as thematic constructs, yet no hypotheses were tested through quantitative analysis. Although such a synthesis provides strong theoretical coherence, it does not provide data to show that these constructs mediate the processes of trust building, engagement, and behavioural mobilisation. Concepts such as Dialogic Resonance, Perceptual Cartography, and Mobilised Cognition are theoretically strong but are interpretative categories rather than statistically verified variables. The absence of numerical validation leaves the nature and direction of these associations, as well as their underlying pathways, open to additional empirical scrutiny.

While the interpretivist nature of the study enables deep contextual understanding, it does compromise objectivity through researcher interpretation. Reflexivity was established at each stage of the analysis; however, qualitative thematic analysis requires interpretive judgment. Therefore, these findings are credible and theoretically grounded, but **they** might be interpretatively situated due to the researcher's positioning, cultural frame of reference, or methodological orientation. It is not the intention that such limitations be interpreted as failings, but rather as inherent aspects of a qualitative exploration of meaning and interactions within a particular social networked environment.

6.4.2 Future research direction

Future research needs to further develop, test, and elaborate the Digital Reverberations in Health Communication model beyond the qualitative nature of its current construction. A promising direction consists of testing the framework's main constructs with mixed-method or quantitative designs. In order to measure trust formation, **structural** equation modelling (SEM), accompanied by social media analytics and surveys, could be employed for the quantification of variables such as interactivity, emotional response, and behavioural intention. This would enable researchers to measure the degree and direction of relationships between Symbolic Exchange, Discursive Pulse, Relational Trust, and Mobilised Cognition. This would empirically underpin the causal logic of the model and improve its predictive utility in varied public health contexts.

Another critical direction is the study of longitudinal research. While communication and digital trust are dynamic phenomena whose value may change over time, longitudinal or cohort studies could explore the long-term effects of interactive communication during extended health campaigns or policy changes. Monitoring shifts in sentiment, engagement, and compliance at different public health intervention stages would illuminate how continued dialogic communication fosters enduring behavioural and attitudinal change. Longitudinal data would also allow testing of the cyclic nature of the model, **for example**, whether repeated exchanges build trust and engagement over time or reach a plateau.

Comparative work across cultural and institutional contexts is also important. It would be valuable to apply the framework to other areas, particularly those with less digital capability and more decentralised governance, and test its transferability as well as understand how context specific it is. Studies in low-income settings, federated health systems, or culturally different societies might consider testing whether constructs such as Communal Ethos and Dialogic Resonance operate comparably across different sociopolitical contexts. Such comparisons would sharpen the framework for transnational applicability and protect its sensitivity to the local.

Another promising avenue is the integration of automated and qualitative analysis. The framework's domains may be explored using digital trace data, such as social media feeds, sentiment mapping, and discourse analysis, in order to illuminate how specific engagement patterns and emotional expressions unfold over time. Combining big data

with qualitative interpretation in mixed-method research would not only validate the constructs but also provide useful action-guiding information to policy-makers and communication experts interested in optimising public engagement.

Future work might also consider how machinic mediation and practices of algorithmic accountability shape dialogic resonance and trust building. As AI increasingly curates and moderates digital content, this shapes how citizens see and interpret public health information. Exploring the interrelationship between human agency and algorithmic systems in public health communication may help to further the theoretical scope of the model, as well as connect it with recent trends in digital sociology and media ethics.

The model could also be applied to health education and risk communication contexts beyond Abu Dhabi to improve its disciplinary applicability. Demonstrating that the Digital Reverberations in Health Communication framework functions in a mental health promotion context, and also for immunisation campaigns or chronic disease management, would illustrate its relevance across different public health areas. Trial investigators might explore whether dialogic and culturally situated models of communication lead to engagement outcomes as well as **more easily** quantifiable health-related effects, such as adherence to preventive behaviours or uptake of screening services.

The cyclical nature of the model lends itself to further research on feedback within adaptive governance. Further examination of how organisations learn from and respond to public discourse could help to elucidate the mechanisms through which communication influences policy change. Combining insights from governance studies, behavioural science, and digital anthropology might help develop a complementary view of how participatory communication helps to turn health systems into reflexive learning organisations.

Across these lines of enquiry, including quantitative evaluation, longitudinal examination, international application, computational evidence, and interdisciplinary expansion, the Digital Reverberations model can develop from a context-sensitive heuristic into an internationally generalisable theory of digital public health communication. In this process, it would not only deepen academic knowledge of trust, engagement, and participation but also generate practical tools for building adaptive

communication systems that are responsive to culture and able to address the complexities of public health in a digital era.

REFERENCES LIST

- Alamri, A., Rogers, P., Kearns, C., Doke, T., Al-Habib, A., Servadei, F., Hutchinson, P. J., Kolias, A. G., & Uff, C. (2019). Social media for dissemination and public engagement in neurosurgery—the example of Brainbook. *Acta Neurochirurgica*, 161(1), 5–9.
- Alamri, W., 2019. Effectiveness of Qualitative Research Methods: Interviews and Diaries. *International Journal of English and Cultural Studies*, 2(1), pp. 65-70.
- Alejandro, A. L., Bruce, M., & Leo, C. (2022). Parents' awareness of antimicrobial resistance: a qualitative study utilising the Health Belief Model in Perth, Western Australia. *Australian and New Zealand Journal of Public Health*, 46(6), 764–770.
- Alhaimer, R. (2022). The health belief model: Evaluating governmental public health messages on social media aimed at preventing a COVID-19 epidemic in Kuwait. *Cogent Business & Management*, 9(1).
- Alicea-Alvarez, N., Reeves, K., Rabelais, E., Huang, D., Ortiz, M., Burroughs, T., & Jones, N. (2016). Impacting Health Disparities in Urban Communities: Preparing Future Healthcare Providers for “Neighborhood-Engaged Care” Through a Community Engagement Course Intervention. *Journal of Urban Health*, 93(4), 732–743.
- Al-Metwali, B. Z., Al-Jumaili, A. A., Al-Alag, Z. A., & Sorofman, B. (2021). Exploring the acceptance of COVID-19 vaccine among healthcare workers and general population using health belief model. *Journal of Evaluation in Clinical Practice*, 27(5), 1112–1122.
- Ayaz-Alkaya, S., Yaman-Sözbir, Ş., & Terzi, H. (2020). The effect of Health Belief Model-based health education programme on coping with premenstrual syndrome: a randomised controlled trial. *International Journal of Nursing Practice*, 26(2), e12816-n/a.
- Azer, J., & Alexander, M. (2022). COVID-19 vaccination: engagement behaviour patterns and implications for public health service communication. *Journal of Service Theory and Practice*, 32(2), 323–351.
- Bejtkovský, J. (2021). edIn, a vocational social network, as a tool for promotion in selected healthcare service providers. *Management & Marketing (Bucharest, Romania)*, 16(3), 286–299
- Bell, E., Bryman, A., & Harley, B. (2022). *Business research methods* (Sixth). Oxford University Press.
- Belt, T. H. v. d., Engelen, L. J., Berben, S. A. A., Teerenstra, S., Samsom, M., & Schoonhoven, L. (2013). Internet and Social Media For Health-Related Information and Communication in Health Care: Preferences of the Dutch General Population. *Journal of Medical Internet Research*, 15(10), e220–e220.
- Berger, Z. (2019). Metrics of Patient, Public, Consumer, and Community Engagement in Healthcare Systems: How Should We Define Engagement, What Are We Measuring, and Does It Matter for Patient Care? Comment on "Metrics and Evaluation Tools for Patient Engagement in Healthcare Organization- and System-Level Decision-Making: A Systematic Review" *International Journal of Health Policy and Management*, 8(1), 49–50.
- Bernard, H. R. (2017). *Research methods in anthropology: qualitative and quantitative approaches* (Sixth). Rowman & Littlefield Publishers.
- Bertrand, I., & Hughes, P. (2018). *Media research methods: audiences, institutions, texts* (Second). Palgrave.
- Bhattacharya, S., Srinivasan, P., & Polgreen, P. (2017). Social media engagement analysis of U.S. Federal health agencies on Facebook. *BMC Medical Informatics and Decision Making*, 17(1), 49–49.

Biggs, R., Preiser, R., Clements, H., Vos, A. d., Maciejewski, K., & Schlüter, M. (2022;2021;). *The Routledge Handbook of Research Methods for Social-Ecological Systems* (R. Biggs, R. Biggs, A. d. Vos, A. de Vos, R. Preiser, R. Preiser, H. Clements, H. Clements, K. Maciejewski, K. Maciejewski, M. Schlüter, & M. Schlüter, Eds.; 1st ed.). Routledge.

Bornkessel, A., Furberg, R., & Lefebvre, R. C. (2014). Social Media: Opportunities for Quality Improvement and Lessons for Providers—A Networked Model for Patient-Centered Care Through Digital Engagement. *Current Cardiology Reports*, 16(7), 504–504.

Braun, V. & Clarke, V., 2012. Thematic analysis. In: H. Cooper, et al. eds. *APA handbook of research methods in psychology*, Vol. 2: Research designs: Quantitative, qualitative, neuropsychological, and biological. Washington, DC: American Psychological Association, pp. 57-71.

Breakwell, G. M., Wright, D. B., & Barnett, J. (. p. (2020). *Research methods in psychology* (Fifth). SAGE.

Brennen, B. (2021). *Qualitative research methods for media studies* (Third). Routledge.

Buckler, S., & Moore, H. (2023). *Essentials of research methods in education*. SAGE.

Burgess, R. A., Osborne, R. H., Yongabi, K. A., Greenhalgh, T., Gurdasani, D., Kang, G., Falade, A. G., Odone, A., Busse, R., Martin-Moreno, J. M., Reicher, S., & McKee, M. (2021). The COVID-19 vaccines rush: participatory community engagement matters more than ever. *The Lancet (British Edition)*, 397(10268), 8–10.

Cairns, P., & Cox, A. L. (2008). *Research methods for human-computer interaction*. Cambridge University Press.

Callaghan, K. A., & Horne, R. (2023). The Dynamics of Local Participation. *The American Behavioral Scientist (Beverly Hills)*, 67(4), 476–491.

Campbell, I. H., & Rudan, I. (2020). Effective approaches to public engagement with global health topics. *Journal of Global Health*, 10(1), 01040901–01040901.

Cassell, C., Cunliffe, A. L., & Grandy, G. (2017). *The SAGE handbook of qualitative business and management research methods*. SAGE.

Chaudhri, V., Oomen, T., Pridmore, J., & Joon, A. (2021). “CARE” in social media: perceptions of reputation in the healthcare sector. *Journal of Communication Management (London, England)*, 25(2), 125–141.

Chen, L., Sivaparthipan, C. B., & Rajendiran, S. (2021). Unprofessional problems and potential healthcare risks in individuals' social media use. *Work (Reading, Mass.)*, 68(3), 945–953.

Chen, X., Lun, Y., Yan, J., Hao, T., & Weng, H. (2019). Discovering thematic change and evolution of utilizing social media for healthcare research. *BMC Medical Informatics and Decision Making*, 19(Suppl 2), 50–50.

Choo, E. K., Ranney, M. L., Chan, T. M., Trueger, N. S., Walsh, A. E., Tegtmeyer, K., McNamara, S. O., Choi, R. Y., & Carroll, C. L. (2015). Twitter as a tool for communication and knowledge exchange in academic medicine: A guide for skeptics and novices. *Medical Teacher*, 37(5), 411–416.

Clark, T. (L. in research methods), Foster, L., Sloan, L., Bryman, A., & Bryman, A. (2021). *Bryman's social research methods* (Sixth). Oxford University Press.

Coe, R., Waring, M., Hedges, L. V., & Arthur, J. (2017). *Research methods & methodologies in education* (2nd ed.). SAGE.

Coe, R., Waring, M., Hedges, L. V., & Ashley, L. D. (2021). *Research methods and methodologies in education* (3rd ed.). SAGE.

- Coolican, H. (2024). *Research methods and statistics in psychology* (Eighth). Routledge.
- Cui, X., Huo, B., Lei, Y. & Zhou, Q., 2019. The influence of team social media usage on individual knowledge sharing and job performance from a cross-level perspective. *International Journal of Operations & Production Management*.
- Davies, C., & Mann, A. (2023). Factors influencing women to accept diet and exercise messages on social media during COVID-19 lockdowns: A qualitative application of the health belief model. *Health Marketing Quarterly, ahead-of-print*(ahead-of-print), 1–19.
- Denscombe, M. (2021). *The good research guide: research methods for small-scale social research projects* (Seventh). Open University Press.
- DiMaggio, P. J., & Powell, W. W. (1983). The iron cage revisited: Institutional isomorphism and collective rationality in organizational fields. *American sociological review*, 147–160.
- Doherty, C. (2023). Using web log analysis to evaluate healthcare students' engagement behaviours with multimedia lectures on YouTube. *PloS One*, 18(4), e0284133–e0284133.
- Duku, S. K. O., Nketiah-Amponsah, E., Fenenga, C. J., Janssens, W., & Pradhan, M. (2022). The effect of community engagement on healthcare utilization and health insurance enrollment in Ghana: Results from a randomized experiment. *Health Economics*, 31(10), 2120–2141.
- Dunn, H. K., Pearlman, D. N., Beatty, A., & Florin, P. (2018). Psychosocial Determinants of Teens' Online Engagement in Drug Prevention Social Media Campaigns: Implications for Public Health Organizations. *The Journal of Primary Prevention*, 39(5), 469–481.
- Durey, A., McEvoy, S., Swift-Otero, V., Taylor, K., Katzenellenbogen, J., & Bessarab, D. (2016). Improving healthcare for Aboriginal Australians through effective engagement between community and health services. *BMC Health Services Research*, 16(1), 224–224.
- Dutta, T., Agle, J., Meyerson, B. E., Barnes, P. A., Sherwood-Laughlin, C., & Nicholson-Crotty, J. (2021). Perceived enablers and barriers of community engagement for vaccination in India: Using socioecological analysis. *PloS One*, 16(6), e0253318–e0253318.
- Eghtesadi, M., & Florea, A. (2020). Facebook, Instagram, Reddit and TikTok: a proposal for health authorities to integrate popular social media platforms in contingency planning amid a global pandemic outbreak. *Canadian Journal of Public Health*, 111(3), 389–391.
- Ellithorpe, M. E., Aladé, F., Adams, R. B., & Nowak, G. J. (2022). Looking ahead: Caregivers' COVID-19 vaccination intention for children 5 years old and younger using the health belief model. *Vaccine*, 40(10), 1404–1412.
- Eugenio-Vela, J. d. S., Ginesta, X., & Kavaratzis, M. (2020). The critical role of stakeholder engagement in a place branding strategy: a case study of the Empordà brand. *European Planning Studies*, 28(7), 1393–1412.
- Fagerström, C., Wickström, H., & Tuveusson, H. (2022). Still engaged - healthcare staff's engagement when introducing a new eHealth solution for wound management: a qualitative study. *BMC Health Services Research*, 22(1), 103–103.
- Fapohunda, T., 2013. Towards Effective Team Building in the Workplace. *International Journal of Education and Research*, 1(4), pp. 1-12.
- Farley, J. E., Dangerfield, D. T., LaRicci, J., Sacamano, P., Heidari, O., Lowensen, K., Jennings, J. M., & Tobin, K. E. (2021). Community engagement and age to care efforts by peer community-health workers to increase PrEP uptake among sexual minority men. *Public Health Nursing* (Boston, Mass.), 38(5), 818–824.
- Fedorowicz, S., Riley, V., Cowap, L., Ellis, N. J., Chambers, R., Grogan, S., Crone, D., Cottrell, E., Clark-Carter, D., Roberts, L., & Gidlow, C. J. (2022). Using social media for patient and public involvement and engagement in health research: The process and impact of a closed Facebook

group. *Health Expectations : an International Journal of Public Participation in Health Care and Health Policy*, 25(6), 2786–2795.

Franceschini, S., & Marletto, G. E. (2019). The dynamics of trust during public participation. *Papers in Regional Science*, 98(4), 1713–1729.

Freire, Y., Sánchez, M. G., Suárez, A., Joves, G., Nowak, M., & Díaz-Flores García, V. (2023). Influence of the use of social media on patients changing dental practice: a web-based questionnaire study. *BMC Oral Health*, 23(1), 365–365.

Gannon, R., & Fauchon, M. (2021). *Illustration research methods*. Bloomsbury Visual Arts.

Gayle, V., & Lambert, P. (. (2021). *Quantitative longitudinal data analysis: research methods*. Bloomsbury Academic.

Gesser-Edelsburg, A. (2021). How to Make Health and Risk Communication on Social Media More “Social” During COVID-19. *Risk Management and Healthcare Policy*, 14, 3523–3540.

Graffigna, G., Palamenghi, L., Barelo, S., Savarese, M., Castellini, G., Lozza, E., & Bonanomi, A. (2021). Measuring citizens' engagement during emergencies: Psychometric validation of the Public Health Engagement Scale for Emergency Settings (PHEs-E). *PloS One*, 16(12), e0261733–e0261733.

Grubert, T., Steuber, J. & Meynhardt, T., 2022. Engagement at a higher level: The effects of public value on employee engagement, the organization, and society. *Current Psychology*.

Gruss, R., Abrahams, A., Song, Y., Berry, D., & Al-Daihani, S. M. (2020). Community building as an effective user engagement strategy: A case study in academic libraries. *Journal of the Association for Information Science and Technology*, 71(2), 208–220.

Guillaume, M., & Loufrani-Fedida, S. (2023). Stakeholder engagement in inclusive employability management for employees whose health at work is impaired: empirical evidence from a French public organisation. *Personnel Review*, 52(1), 121–144.

Hackett, P. (2019). *Quantitative research methods in consumer psychology: contemporary and data driven approaches* (First). Routledge, Taylor and Francis Group.

Hair, J., Joseph F., Page, M. (P. of management), & Brunsveld, N. (2019). *The essentials of business research methods* (Fourth). Routledge.

Hammond, M., & Wellington, J. J. (2020;2021;). *Research methods: the key concepts* (2nd;2; ed., Vol. 1). Routledge.

Hansen, A., & Machin, D. (2019). *Media and communication research methods* (2nd ed.). Red Globe Press.

Hansen, A., & Machin, D. (2019). *Media and communication research methods* (2nd ed.). Red Globe Press.

Heldman, A. B., Schindelar, J., & Weaver, J. B. (2013). Social Media Engagement and Public Health Communication: Implications for Public Health Organizations Being Truly “Social”. *Public Health Reviews*, 35(1), 1–18.

Heldman, A. B., Schindelar, J., & Weaver, J. B. (2013). Social Media Engagement and Public Health Communication: Implications for Public Health Organizations Being Truly “Social”. *Public Health Reviews*, 35(1), 1–18.

Hennink, M. M., Hutter, I., & Bailey, A. (2020). *Qualitative research methods* (Second). SAGE.

Hita, M. L. R., Grégoire, Y., Lussier, B., Boissonneault, S., Vandenberghe, C., & Sénécal, S. (2023). An extended health belief model for COVID-19: understanding the media-based processes leading to social distancing and panic buying. *Journal of the Academy of Marketing Science*, 51(1), 132–152.

Howitt, D. (2019). *Introduction to qualitative research methods in psychology: putting theory into practice* (Fourth). Pearson.

Howitt, D., & Cramer, D. (2020). *Research methods in psychology* (Sixth). Pearson.

Hu, G., Han, X., Zhou, H., & Liu, Y. (2019). Public Perception on Healthcare Services: Evidence from Social Media Platforms in China. *International Journal of Environmental Research and Public Health*, 16(7), 1273.

Huo, J., Desai, R., Hong, Y.-R., Turner, K., Mainous, A. G., & Bian, J. (2019). Use of Social Media in Health Communication: Findings From the Health Information National Trends Survey 2013, 2014, and 2017. *Cancer Control*, 26(1), 1073274819841442–1073274819841442.

ijsen, V., Maddux, M., Lavertu, A., Gonzalez-Hernandez, G., Ram, N., Reeves, B., Robinson, T., Ziesenitz, V., Shakhnovich, V., & Altman, R. (2020). Science: The Potential and the Challenges of Utilizing Social Media and Other Electronic Communication Platforms in Health Care. *Clinical and Translational Science*, 13(1), 26–30.

Jeyaraman, M., Ramasubramanian, S., Kumar, S., Jeyaraman, N., Selvaraj, P., Nallakumarasamy, A., Bondili, S. K., & Yadav, S. (2023). Multifaceted Role of Social Media in Healthcare: Opportunities, Challenges, and the Need for Quality Control. *Curēus* (Palo Alto, CA), 15(5), e39111–e39111.

Johnson, E., DeJonckheere, M., Oliverio, A. L., Brown, K. S., Van Sparrentak, M., & Wu, J. P. (2021). A theoretical model of contraceptive decision-making and behaviour in diabetes: A qualitative application of the Health Belief Model. *Diabetic Medicine*, 38(6), e14434-n/a.

Jonsdottir, I. J., & Kristinsson, K. (2020). Supervisors' Active-Empathetic Listening as an Important Antecedent of Work Engagement. *International Journal of Environmental Research and Public Health*, 17(21), 7976.

Kapur, R., 2018. Corporate Community Engagement and Social Responsibility. [Online] Available at: https://www.researchgate.net/publication/323695242_Corporate_Community_Engagement_and_Social_Responsibility [Accessed 26 July 2018].

Kara, H. (2020). *Creative research methods: a practical guide* (Second). Policy Press.

Kara, H. (2020). *Creative research methods: a practical guide* (Second). Policy Press.

Kara, H., Lemon, N., Mannay, D., & McPherson, M. (2021). *Creative research methods in education: principles and practices*. Policy Press.

Keller, B., Labrique, A., Jain, K. M., Pekosz, A., & Levine, O. (2014). Mind the gap: social media engagement by public health researchers. *Journal of Medical Internet Research*, 16(1), e8–e8.

Kelley, H. H. (1955). The two functions of reference groups. In *Readings in social psychology* (pp. 410–414). Holt, Rinehart, & Winston.

Khani Jeihooni, A., Mohammadkhah, F., Razmjouie, F., Harsini, P. A., & Sedghi Jahromi, F. (2022). Effect of educational intervention based on health belief model on mothers monitoring growth of 6–12 months child with growth disorders. *BMC Pediatrics*, 22(1), 1–561.

Khokhar, S., Pathan, H., Raheem, A. & Abbasi, A., 2020. Theory Development in Thematic Analysis: Procedure and Practice. *International Journal of Management and Social Sciences*, 3(3), pp. 423–433.

Kiesler, S., Collins, B. E., & Miller, N. (1969). Attitude change: A critical analysis of theoretical approaches.

Kendon, S. L., Pain, R., & Kesby, M. (2010). *Participatory action research approaches and methods: connecting people, participation and place* (Vol. 22;22:;). Routledge.

- Király, G., & Miskolczi, P. (2019). Dynamics of participation: System dynamics and participation—An empirical review. *Systems Research and Behavioral Science*, 36(2), 199–210.
- Klinga, C., Hasson, H., Andreen Sachs, M., & Hansson, J. (2018). Understanding the dynamics of sustainable change: A 20-year case study of integrated health and social care. *BMC Health Services Research*, 18(1), 400–400.
- Koumpouros, Y., Toulas, T. L., & Koumpouros, N. (2015). The importance of patient engagement and the use of Social Media marketing in healthcare. *Technology and Health Care*, 23(4), 495–507.
- Lai, K. P., & Chong, S. C. (2023). Influences of time perspectives and perceived values on continuance intention to engage in social media amongst older adults for healthcare-related purposes. *Universal Access in the Information Society*, 22(2), 315–329.
- Landi, S., Costantini, A., Fasan, M., & Bonazzi, M. (2022). Public engagement and dialogic accounting through social media during COVID-19 crisis: a missed opportunity? *Accounting, Auditing & Accountability Journal*, 35(1), 35–47.
- Li, K., Zhou, C., Luo, X. (., Benitez, J., & Liao, Q. (2022). Impact of information timeliness and richness on public engagement on social media during COVID-19 pandemic: An empirical investigation based on NLP and machine learning. *Decision Support Systems*, 162, 113752–113752.
- Ljajic, S. & Pirs, D., 2021. The Role Of Internal Communication And Workplace Language In Positioning Of Organizations. *DHS*, 1(14), pp. 441-452.
- Madhumathi, J., Sinha, R., Veeraraghavan, B., & Walia, K. (2021). Use of “Social Media”—an Option for Spreading Awareness in Infection Prevention. *Current Treatment Options in Infectious Disease*, 13(1), 14–31.
- Malhotra, G., 2017. Strategies in Research. *International Journal of Advance Research and Development*, 2(5), pp. 172-180.
- Mannay, D. (2015). *Visual, narrative and creative research methods: application, reflection and ethics*. Routledge.
- Mao, Z., Wang, D., & Zheng, S. (2023). Health belief model and social media engagement: A cross-national study of health promotion strategies against COVID-19 in 2020. *Frontiers in Public Health*, 11, 1093648–1093648.
- Marginson, S. (2016). The worldwide trend to high participation higher education: Dynamics of social stratification in inclusive systems. *Higher Education*, 72(4), 413–434.
- Martínez-Parra, A. G., Pinilla-Alfonso, M. Y., & Abadía-Barrero, C. E. (2018). Sociocultural dynamics that influence Chagas disease health care in Colombia. *Social Science & Medicine* (1982), 215, 142–150.
- Massey, D., Huang, C., Lu, Y., Cohen, A., Oren, Y., Moed, T., Matzner, P., Mahajan, S., Caraballo, C., Kumar, N., Xue, Y., Ding, Q., Dreyer, R., Roy, B., & Krumholz, H. (2021). Engagement With COVID-19 Public Health Measures in the United States: A Cross-sectional Social Media Analysis from June to November 2020. *Journal of Medical Internet Research*, 23(6), e26655–e26655.
- Matias, C. E. (2021). *The handbook of critical theoretical research methods in education*. Routledge.
- May, T., & Perry, B. (2022). *Social research: issues, methods and process* (Fifth). Open University Press.
- Men, L., O'Neil, J. & Ewing, M., 2020. Examining the effects of internal social media usage on employee engagement. *Public Relations Review*, 46(2), pp. 1-20.
- Mertens, D. M. (2023). *Mixed methods research: research methods*. Bloomsbury Academic.

Miah, S. J., Hasan, N., Hasan, R., & Gammack, J. (2017). Healthcare support for underserved communities using a mobile social media platform. *Information Systems (Oxford)*, 66, 1–12.

Mishra, K., Boynton, L. & Mishra, A., 2014. Driving Employee Engagement: The Expanded Role of Internal Communications. *International Journal of Business Communication*, 51(2), pp. 183-202.

Nabi, M. N. U., Zohora, F. T., & Misbaudhin, S. M. (2023). Social media s with social capital to trust in healthcare facilities: empirical evidence from Bangladesh. *Library Hi Tech*, 41(1), 210–228.

Naeem, M., & Ozuem, W. (2021). Exploring the use of social media sites for health professionals' engagement and productivity in public sector hospitals. *Employee Relations*, 43(5), 1029–1051.

Naeem, M., Ozuem, W., Howell, K., & Ranfagni, S. (2023). A Step-by-Step Process of Thematic Analysis to Develop a Conceptual Model in Qualitative Research. *International Journal of Qualitative Methods*, 22.

Naeem, M., Smith, T., & Thomas, L. (2025). Thematic Analysis and Artificial Intelligence: A Step-by-Step Process for Using ChatGPT in Thematic Analysis. *International Journal of Qualitative Methods*, 24.

Naeem, Muhammad (2021) *Do social media platforms develop consumer panic buying during the fear of Covid-19 pandemic*. *Journal of Retailing and Consumer Services*, 58. Article no. 102226. ISSN 0969-6989

Nassaji, H., 2015. Qualitative and descriptive research: Data type versus data analysis. *Language Teaching Research*, 19(2), pp. 129-132.

Navarro, S. M., Mazingi, D., Keil, E., Dube, A., Dedeker, C., Stewart, K. A., Ncube, T., Rickard, J. L., Lavy, C., & Tuttle, T. M. (2020). Identifying New Frontiers for Social Media Engagement in Global Surgery: An Observational Study. *World Journal of Surgery*, 44(9), 2881–2891.

Neuman, W. L. (2014). *Social research methods: qualitative and quantitative approaches* (Pearson new international;Seventh;). Pearson.

Ngai, C. S. B., Singh, R. G., & Lu, W. (2020). Exploring drivers for public engagement in social media communication with medical social influencers in China. *PloS One*, 15(10), e0240303–e0240303.

Nguyen, H. T., & Connelly, L. B. (2014). The effect of unpaid caregiving intensity on labour force participation: Results from a multinomial endogenous treatment model. *Social Science & Medicine* (1982), 100(Jan 2014), 115–122.

Ni, L., De la Flor, M., Wang, Q., & Romero, V. (2021). Engagement in context: Making meaning of the Latino community health engagement process. *Public Relations Review*, 47(2), 102036.

Nirenberg, J., 2007. From team building to community building. *National Productivity Review*, 14(1), pp. 51-62.

Nooriani, N., Mohammadi, V., Feizi, A., Shahnazi, H., Askari, G., & Ramezanzade, E. (2019). The effect of nutritional education based on health belief model on nutritional knowledge, Health Belief Model constructs, and dietary intake in hemodialysis patients. *Iranian Journal of Nursing and Midwifery Research*, 24(5), 372–378.

Nwosu, C., & Ward, K. D. (2022). Comment on Alharbi et al. Adoption of Health Mobile Apps during the COVID-19 Lockdown: A Health Belief Model Approach. *Int. J. Environ. Res. Public Health* 2022, 19 , 4179. *International Journal of Environmental Research and Public Health*, 19(24), 16846.

Odunsi, I. A., & Farris, K. L. (2023). Predicting College Students' Preventative Behaviour During a Pandemic: The Role of the Health Belief Model, Source Credibility, and Health Literacy. *The American Behavioral Scientist (Beverly Hills)*, 276422311640.

- Ohmer, M., Mendenhall, A., Carney, M. & Adams, D., 2022. Community engagement: evolution, challenges and opportunities for change. *Journal of Community Practice*, 30(4), pp. 351-358.
- Omona, J., 2013. Sampling in Qualitative Research: Improving the Quality of Research Outcomes in Higher Education. *Makerere Journal of Higher Education*, 4(2), pp. 169-185.
- O'Neill, M., & Roberts, B. (2020). *Walking methods: research on the move*. Routledge.
- Opoku Duku, S. K., Nketiah-Amponsah, E., Fenenga, C. J., Janssens, W., & Pradhan, M. (2022). The effect of community engagement on healthcare utilization and health insurance enrollment in Ghana: Results from a randomized experiment. *Health Economics*, 31(10), 2120–2141.
- Owusu-Addo, E., Renzaho, A. M. N., & Smith, B. J. (2018). The impact of cash transfers on social determinants of health and health inequalities in sub-Saharan Africa: a systematic review. *Health Policy and Planning*, 33(5), 675–696.
- Ozuem, W and Azemi, Y (2018) *Handbook of Digital Marketing Strategies for Fashion and Luxury brands*, Hershey: IGI Global
- Ozuem, W and Rafangni, S (2021) *The Art of Digital Marketing for Luxury and Fashion Brands: marketplaces and marketplaces*, London: Palgrave Macmillan
- Ozuem, W and Willis, M (2022) *Digital Marketing Strategies for value co-creation: Online Brand Communities*, London: Palgrave Macmillan
- Ozuem, W, Datson, P and Howell, K and Lancaster, G (2021b) Beyond the game: perceptions and practices of Sports Sponsorship in German SMEs , *Qualitative Market Research Vol. 24 (5)*
- Ozuem, W, Patten, E and Azemi, Y (2020) *Harnessing Omnichannel retailing strategies for fashion and luxury brands*, Florida: BrownWalker Press
- Ozuem, W, Ranfagni, S and Willis, S (2023) *Digital Transformation for Luxury and Fashion Brands: Theory and Practice*, London: Palgrave Macmillan
- Ozuem, W, Willis, M, Howell, K, Helal, G, and Lancaster, G (2021a) Differential effects of Millennial online brand community and customer loyalty in the fashion industry, *Psychology and Marketing*, Vol 38 (5) pp. 774- 793 (CABS 3*)
- Ozuem, W, Willis, M, Howell, K, Lancaster, G and Ng, R (2021c) Determinants of online brand communities' and millennials' characteristics: A social influence perspective, *Psychology and Marketing*, Vol. 38 (5) pp. 794 -818
- Ozuem, W., and Bowen, G (2019) *Leveraging Computer-mediated marketing environments*, Hershey: IGI Global
- Pangborn, S. M., Boatwright, B. C., Miller, C. L., & Velting, M. N. (2023). "I Don't Feel Like a Hero": Frontline Healthcare Providers' Social Media Storytelling during COVID-19. *Health Communication*, 38(8), 1508–1518.
- Park, H., Reber, B. H., & Chon, M.-G. (2016). Tweeting as Health Communication: Health Organizations' Use of Twitter for Health Promotion and Public Engagement. *Journal of Health Communication*, 21(2), 188–198.
- Pizzuti, A. G., Patel, K. H., McCreary, E. K., Heil, E., Bland, C. M., Chinaeke, E., Love, B. L., & Bookstaver, P. B. (2020). Healthcare practitioners' views of social media as an educational resource. *PloS One*, 15(2), e0228372–e0228372.
- Poth, C. N. (2023). *The SAGE handbook of mixed methods research design*. SAGE.
- Powell, R. E., Doty, A., Casten, R. J., Rovner, B. W., & Rising, K. L. (2016). A qualitative analysis of interprofessional healthcare team members' perceptions of patient barriers to healthcare engagement. *BMC Health Services Research*, 16(1), 493–493.

- Qasim, A., Kharbat, F., Razmak, J. & Alshawabkeh, A., 2018. Enhancing internal communication in organisations using enterprise social networking. *International Journal of Economics and Business Research*, 15(1), pp. 72-86.
- Qu, S. & Dumay, J., 2011. The qualitative research interview. *Qualitative Research in Accounting & Management*, Volume 8, pp. 238-264.
- Quinlan, C., Babin, B. J., Carr, J. C., Griffin, M., & Zikmund, W. G. (2019). *Business research methods* (Second). Cengage Learning.
- Rask, M., Mačiukaitė-Žvinienė, S., Tauginienė, L., Dikčius, V., Matschoss, K., Aarrevaara, T., & d'Andrea, L. (2018). *Public Participation, Science and Society: Tools for Dynamic and Responsible Governance of Research and Innovation*. Taylor & Francis.
- Reboe-Benjamin, M., Brindamour, M., Leis, K., Hanson, J., Verity-Anderson, L., Gomez, M., Baerg, M., & Leis, A. (2023). Refugees' Care Experiences, Self-Reported Health Outcomes and Transition to Mainstream Health Care After One Year at the Refugee Engagement and Community Health (REACH) Clinic. *Journal of Immigrant and Minority Health*.
- Rivera, Y. M., Moran, M. B., Thrul, J., Joshu, C., & Smith, K. C. (2022). Contextualizing Engagement With Health Information on Facebook: Using the Social Media Content and Context Elicitation Method. *Journal of Medical Internet Research*, 24(3), e25243–e25243.
- Robson, C., & McCartan, K. (2016). *Real world research: a resource for users of social research methods in applied settings* (Fourth). John Wiley & Sons, Incorporated.
- Saldana, J. (2021). *The coding manual for qualitative researchers* (Fourth). SAGE.
- Salim, A., 2022. Internal Communications and Organization Performance in Zanzibar Public Institutions. *Asian Journal of Economics Business and Accounting*, 22(20), pp. 1-15.
- Saunders, M. N. K., Lewis, P., & Thornhill, A. (2023). *Research methods for business students* (Ninth). Pearson.
- Saunders, M., Lewis, P., & Thornhill, A. (2016). *Research methods for business students* (Seventh). Pearson Education.
- Scott, W. R. (2013). *Institutions and organizations: Ideas, interests, and identities*. Sage Publications.
- Shah, R., 2019. Effective social constructivist approach to learning for social studies classroom. *Journal of Pedagogical Research*, 3(2), pp. 38-51.
- Sharp, J. (2011). Brand awareness and engagement: a case study in healthcare social media. *Frontiers of Health Services Management*, 28(2), 29–33.
- Shawky, S., Kubacki, K., Dietrich, T., & Weaven, S. (2019). Using social media to create engagement: a social marketing review. *Journal of Social Marketing*, 9(2), 204–224.
- Sloan, L., & Quan-Haase, A. (2017). *The Sage handbook of social media research methods*. Sage.
- Smith, J. A. (2015). *Qualitative psychology: a practical guide to research methods* (3rd ed.). SAGE.
- Starman, A., 2013. The case study as a type of qualitative research. *Journal of Contemporary Educational Studies*, Volume 1, pp. 28-43.
- Subramanian, K., 2017. Influence of social media in Interpersonal Communication. *International Journal of Scientific Progress and Research*, 38(109), pp. 70-75.
- Suomi, K., Saraniemi, S., Vähätalo, M., Kallio, T. J., & Tevameri, T. (2021). Employee Engagement and Internal Branding: Two Sides of the Same Coin? *Corporate Reputation Review*, 24(1), 48–63.
- Tajiki, I., Vizeshfir, F., & Keshtkaran, Z. (2022). The effect of training program based on health belief model on burn prevention knowledge in mothers of children aged to 1–3 years: A randomized controlled. *Burns*, 48(4), 808–815.

Takacs, J., Ciotti, M., Tsoleva, S., Wiltshire, E., Baka, A., Kinsman, J., de Vries, D., Cremers, L., Rios, M., & Angrén, J. (2019). Community engagement in public health emergency preparedness. *European journal of public health*, 29(Supplement_4), 574.

Taylor, S. J., Bogdan, R., & DeVault, M. L. (2016;2015;). *Introduction to qualitative research methods: a guidebook and resource* (Fourth;4th;4;1; ed.). John Wiley & Sons.

Tetteh, E. K., Combs, T., Geng, E. H., & McKay, V. R. (2022). Public Health Information Seeking, Trust, and COVID-19 Prevention Behaviors: Cross-sectional Study. *Journal of Medical Internet Research*, 24(9), e37846–e37846.

Tian, X., He, W., He, Y., Albert, S., & Howard, M. (2023). Using health belief model and social media analytics to develop insights from hospital-generated twitter messaging and community responses on the COVID-19 pandemic. *Journal of Enterprise Information Management*.[Link](#)

Torous, J., Ungar, L., & Barnett, I. (2019). Expanding, Augmenting, and Operationalizing Ethical and Regulatory Considerations for Using Social Media Platforms in Research and Health Care. *American Journal of Bioethics*, 19(6), 4–6

Vatcharavongvan, P., Boonyanitchayakul, N., Khampachuea, P., Sinturong, I., & Prasert, V. (2023). Health Belief Model and parents' acceptance of the Pfizer-BioNTech and Sinopharm COVID-19 vaccine for children aged 5–18 years Old: A national survey. *Vaccine*, 41(8), 1480–1489.

Vaux, D. E., & Wang, D. (2021;2020;). *Research Methods for Interior Design: Applying Interiority* (D. E. Vaux & D. Wang, Eds.; 1;1st;1st; ed.). Routledge.

Vaux, D. E., & Wang, D. (2021;2020;). *Research Methods for Interior Design: Applying Interiority* (D. E. Vaux & D. Wang, Eds.; 1;1st;1st; ed.). Routledge.

Walliman, N. (2016). *Social research methods: the essentials* (2nd ed.). SAGE.

Walliman, N. (2018). *Research methods: the basics*. Routledge.

Walliman, N. (2021). *Research methods* (Third). Routledge.

Walliman, N. (2022). *Research methods: the basics* (Third). Routledge, Taylor & Francis Group.

Xu, J., Xie, B., & Chung, B. (2019). Bridging the Gap between Affective Well-Being and Organizational Citizenship Behaviour: The Role of Work Engagement and Collectivist Orientation. *International Journal of Environmental Research and Public Health*, 16(22), 4503.

Yoo, S.-W., Kim, J., & Lee, Y. (2018). The Effect of Health Beliefs, Media Perceptions, and Communicative Behaviors on Health Behavioral Intention: An Integrated Health Campaign Model on Social Media. *Health Communication*, 33(1), 32–40.

Young, A. & Hinesly, M., 2014. Social Media Use to Enhance Internal Communication: Course Design for Business Students. *Business and Professional Communication Quarterly*, 77(4).

Youssef, D., Abou-Abbas, L., Berry, A., Youssef, J., & Hassan, H. (2022). Determinants of acceptance of Coronavirus disease-2019 (COVID-19) vaccine among Lebanese health care workers using health belief model. *PloS One*, 17(2), e0264128–e0264128.

Yuan, M. et al., 2021. Community engagement in public health: a bibliometric mapping of global research. *Archives of Public Health*, 79(6).

APPENDICES

APPENDIX 1: INTERVIEW QUESTIONS

7.1.1 Professional

Please revise the following demographics, because we cannot ask what is your age or gender so give some options to the participants, Note: keep in mind the these interviews will be done in UAE

Demographic information

Age Group:

- Which of the following age groups do you belong to?
 - 18-24
 - 25-34
 - 35-44
 - 45-54
 - 55-64
 - 65 and above

Gender:

- Please indicate your gender:
 - Male
 - Female
 - Prefer not to say
 - Other (please specify)

Educational Level:

- What is the highest level of education you have completed?
 - Primary education
 - Secondary education
 - Diploma/Technical qualification
 - Bachelor's degree
 - Master's degree
 - Doctorate/Ph.D.
 - Other (please specify)

Employment Status:

- What best describes your current employment status?
 - Employed full-time
 - Employed part-time
 - Self-employed
 - Student
 - Unemployed
 - Retired
 - **Other (please specify)**

Nationality:

- Please select your nationality:
 - UAE National
 - Expatriate (please specify country)
 - Other (please specify)

Length of Residence in Abu Dhabi:

- How long have you lived in Abu Dhabi?
 - Less than 1 year
 - 1-2 years
 - 3-5 years
 - 6-10 years
 - More than 10 years
 - I don't live in Abu Dhabi

Language Preference:

- What language(s) do you prefer to use in daily communication?
 - Arabic
 - English
 - Other (please specify)

7.1.2 Interview questions -public

- Can you describe how you usually use social media in your daily life?
- Have you ever learned something about health from social media? Can you share an example?

- How do you interact with health information on social media? Do you read, like, share, or comment on posts?
- When you see health information on social media, how do you decide if it is trustworthy?
- Has any health information you've seen on social media influenced your health behaviours? For example, did you change your diet, exercise, or go to the doctor because of something you saw online?
- Do you follow any health-related authorities or organizations on social media? What kind of information do they share?
- Do you think the way health information is shared on social media is suitable for everyone in Abu Dhabi, considering our diverse cultures?
- During health emergencies, like the COVID-19 pandemic, how helpful was the health information you found on social media?
- What would you suggest to improve the way health information is shared on social media in Abu Dhabi?
- What kind of health topics would you like to see more of on social media?

Appendix 2 : Ethic certification completion



Certificate of Completion

This certifies that

Khulood Almarzooqi

has successfully completed the CPD Course:

Introduction to Research and Professional Ethics

Delivered by Anglia Ruskin University

Date:

07/12/2024

6754909c5369f405290227

Appendix 3: Organizational permission

Dear Khulood,

Greetings from DOH,

As per our meeting , please note that you may proceed with conducting the survey. We have no objections, but ensure that the survey includes consent from the participants.

For any clarification please don't hesitate to contact us.

Best regards,

7.2 APPENDIX 4: INTERVIEW TRANSCRIPT

PROF1

Q1: Can you describe how you usually use social media in your daily life?

Certainly. I also happen to be a healthcare administrator and social media is my life. I live on it — not only passively but actively, following developing conversations, observing and reading community sentiment and even shaping public relations strategy. Platforms like Twitter and Instagram are especially useful for spotting early signals from the community about current public health issues. I also follow LinkedIn and Facebook to see how the stories split between platform as well as demonstrative demographic.

Over the years, I've gotten to see that public feedback is a valuable input. And to cite an example, "public feedback on Twitter has affected our pharmacy queuing system"—and I regard that change as "invaluable." It wasn't a response to an official report, but rather a series of repeated posts from users that illustrated how frustrating the wait time was for some. Those insights, if gleaned through regular surveys alone, may have taken months to compile.

Q2: Have you ever learned something about health from social media? Can you share an example?

Absolutely. I remember when COVID-19 first hit, how multiple doctors started anonymously at first, sharing their bedside experiences each day. These weren't formal studies, but their warnings and pictorial posts about complications informed us of some problems long before official briefings. And, I hate to add, "it's not stats that sell us; it is scary stories that make the threat real. We've seen that firsthand in the case of young victims who were sent out by community pages — they did more to drive urgency than any chart or briefing.

Q3: How do you interact with health information on social media? Do you read, like, share, or comment on posts?

All of the above. But, more importantly, I see the shape of engagement. I don't just see what's being shared — I see who is sharing it, how it's framed and the language that is being used. For my own use, I share posts that are verified — and particularly if it is issued by the official health department or from someone who is known to be an expert.

But I've since discovered something else: ones with human faces, stories and flesh-and-blood emotion behind the scenes tend to go farther. Data visuals and infographics may have their place, but personal stories are where engagement flows. That's part of what we're working to change internally — better aligning formal messaging and real human connection.

Q4: When you see health information on social media, how do you decide if it is trustworthy?

Trust is multi-layered. First, it's about the source. Verified accounts, government pages and posts with official logos naturally gain further credibility. But even so, tone and consistency are important.

Unfortunately, “conflicting sources of information corrode our intuition about health. “So even when it’s with the best of intentions, it blows trust.” That sucked in the early days of the pandemic — you’d have one department saying one thing and a local clinic sharing something else. For professionals it was even confusing. For the public, it was chaos. That’s why after mid-2020 we moved to standardize more aggressively response formats and verification practices.

Q5: Has any health information you've seen on social media influenced your health behaviours?

Although my own actions are mostly based on medical guidelines and policy, frankly I would be lying to say that social media hasn’t had an influence. For example, a quick post on a possible outbreak or public concern is often followed closely by internal audits and policy reviews. Moreover, it drives personal reflection. One instance: a story that went viral of a frontline worker having lost a loved one to testing delays led to my forcing out an internal memo around prioritization of vulnerable community members.

Once more, “It’s not numbers that convince us—it’s stories that bring the risk home.”

Q6: Do you follow any health-related authorities or organizations on social media? What kind of information do they share?

Yes, multiple. I track the Department of Health—Abu Dhabi, SEHA, WHO and several other regional hospitals’ accounts. They share advisories, event updates, vaccination schedules and more, these days often in videos with people who look like real patients or health care workers or community members — not just English speakers.

We pushed for this change. Consistency and authenticity were the guts of the thing early on. There was a time when content just smacked of bureaucracy. But we have drifted toward something more fitting. We have the campaign we have because that’s the tone we now share as a country.

Q7: Do you think the way health information is shared on social media is suitable for everyone in Abu Dhabi, considering our diverse cultures?

That’s an ongoing challenge. Messaging needs to be diverse without watering down clarity. Certain groups do better with visual layouts and others need to hear story telling. But for all of them, emotional relevance and linguistic familiarity count.

This is why we now do cultural adaptation checks on all campaigns before they’re launched. We have learned that people want to feel spoken to — not just spoken at. And the data bears it out: Authentic posts get more engagement and, surprisingly, also a tone of respect. “There’s a sort of unifying in terms of the framing: It wraps people,” he said. That’s been a goal and a learning point.

Q8: During health emergencies, like the COVID-19 pandemic, how helpful was the health information you found on social media?

By then, social media had transformed into a real-time crisis communication machine. Our response depended on it — for messaging and for feedback. That two-way stream was

invaluable. And I would say it helped contain panic and misinformation better than some classic press strategies. public feedback on Twitter has influenced our pharmacy queuing system,” calling that kind of interaction “invaluable. It’s not the stats that sell us; it’s the scary stories that make the threat feel real.

But again, quality varied. In some cases, emotional story telling succeeded over detailed policy explainers. We found that when they saw someone who looks like them, they engage quicker. “I didn’t need a chart to feel the risk — those social posts with young victims made it real.” I think that kind of guttural communication saved lives, I really believe that.

Q9: What would you suggest to improve the way health information is shared on social media in Abu Dhabi?

First, clarity over quantity. Second, humanize the messenger. Third, avoid contradiction and Consistency is the stone trust is built on; inconsistency shatters it. We are also working on incorporating feedback channels for the community — comment prompts, direct messaging response guidelines and sentiment dashboards.

But perhaps trickiest of all, we have to relinquish control. Social media is not just a broadcast medium anymore — it’s a conversational space. And we have to treat it like that, not sending information out but taking it in just as earnestly. It is not statistics that persuade us — it is stories that make the threat personal. It’s incredibly enlightening.

Q10: What kind of health topics would you like to see more of on social media?

Preventative care, mental health and elderly care. These continue to be underrepresented, despite being demands from society. When a post is emotional, it is spreading faster, and it is sticking deeper. Also, behind-the-scenes tales from the health care work force. What people see they believe — if we show the humanity of our system, we help build stronger bridges. Clashing sources undermine our intuition related to health. Even when people have the best intentions, it blows trust. We learned it the hard way, without closing the feedback loop trust breaks down.

We have learned that communication is not just about informing — it’s about building belonging. In this arena, “it’s not the statistics that sell us; it’s the scary stories that make the threat feel real.” And if we want to encourage the kind of public engagement that gets results, we need to keep infusing the human voice into our messages.

